

COURT FILE NO.: 02-CV-20559

DATE: 2005/06/15

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

SUZANNE MONKS

Plaintiff

- and -

ING INSURANCE COMPANY OF
CANADA

Defendant

Peter Cronyn, for the Plaintiff

Debbie Orth, for the Defendant

HEARD: January 10, 11, 12, 13, 14, 17,
18, 19, 20, 21, 24, 25, 26, 27, 28 and 31,
and February 1, 3, 4, 7, 8, 9, 10, 11, 16 and
17, 2005

REASONS FOR JUDGMENT

LALONDE J.

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The Relevant Dates

First accident – February 16, 1993
Second accident – July 2, 1995
Third accident – December 23, 1998

First decompression surgery – September 27, 1999
Second decompression surgery – January 2001

Settlement of Zurich action for 1995 motor vehicle accident – April 29, 2002
Designated assessment centre's findings – July 20, 2001

This claim is brought for the following relief:

- a) declaration that Suzanne Monks sustained a “catastrophic impairment” as defined in the *Statutory Accident Benefits Schedule – Accidents on or after November 1, 1996*, O.Reg. 403/96 as amended (hereafter “SABS-96”) as a result of the motor vehicle accident of December 23, 1998;
- b) declaration that Suzanne Monks is entitled to receive from ING Insurance the statutory benefits which a catastrophically impaired person is entitled to receive pursuant to the SABS-96, including but not limited to:
 - 1. Income Replacement Benefits;
 - 2. Medical and rehabilitation expenses;
 - 3. Attendant care benefits;
 - 4. Housekeeping and home maintenance expenses;
 - 5. Expenses of visitors;
 - 6. Caregiver benefits;
 - 7. Costs of examinations; and
 - 8. Case manager services.
- c) an order that ING Insurance pay to Suzanne Monks all amounts for any such enumerated statutory benefits owing to her since May 31, 2002 to date;
- d) an order that ING Insurance pay interest on all such amounts which have been owing since May 31, 2002 pursuant to s. 46 of SABS-96;
- e) an order that ING Insurance pay to Suzanne Monks on an on-going basis all amounts for any such statutory accident benefits as have been proven at this trial;
- f) an order that ING Insurance pay aggravated damages to Suzanne Monks.

Dysesthesia – A condition in which a disagreeable sensation is produced by ordinary touch, temperature or movement. *It usually refers to altered feelings such as burning, wetness, electric shock, pins and needles, itching, creepy-crawly sensation caused by neurological malfunction.*

Edema – An excessive accumulation of fluid generally in *the* extracellular space.

Foramen – A natural opening or passage in bone. In the case of the spine an *opening, (intervertebral foramen)* allowing for the egress of spinal nerve roots from between two vertebrae.

Foramina (plural of foramen)

Functional overlay – signs and symptoms that are not explained by objective findings.

Intractable pain: pain not able to be controlled by medication.

Myelomalacia – softening of the spinal cord.

Myelopathy – any functional or pathologic disturbance in the spinal cord.

Neurogenic bladder – defective functioning of the bladder due to impaired innervations.

Neurogenic pain – originating or caused by the nervous system or nervous impulses.

Parasthesia – tingling or burning sensation in the extremities.

Prophylactic Surgery: performing surgery to prevent and avoid some further injury because the spinal canal is narrowed even though the patient is asymptomatic.

Quadriplegia – paralysis of all four limbs.

Quadriparesis – weakness of all four limbs.

Radiculopathy – disease of the nerve roots in or near the spinal canal as a result of direct pressure from a disc, or inflammation of the nerve roots due to disc or spinal joint disease.

Signal change – paramount change to spinal cord.

Stenosis – reduction in the diameter *of any hollow structure in the body including* the spinal canal, and in the case of the spinal canal *congenital or acquired* due to new bone formation which may result in pressure on the spinal cord or nerve roots.

Overview

[1] Suzanne Monks (“Ms. Monks”), a 48-year-old Registered nurse/insurance adjuster/rehabilitation caseworker, sustained injuries in three different motor vehicle accidents.

[2] The first accident occurred on February 16, 1993, when her vehicle was involved in a relatively minor accident. She developed a whiplash type of injury that resolved itself within two or three months of massage therapy and physiotherapy treatments. Plain x-rays showed that Ms. Monks suffered from an onset of degenerative disc disease, but not of a significant nature.

[3] Ms. Monks was involved in a second motor vehicle accident on Montreal’s south shore, on July 2, 1995, when her vehicle, stopped at a red light, was hit from behind. She immediately became aware of numbness and paraesthesia (tingling) in both upper extremities, lasting from 15 to 20 minutes. Until that time, Ms. Monks had been working 70 hours per week at Voc-Care as a rehabilitation consultant. During the next two years, Ms. Monks was tested for an increased risk of spinal cord injury with minor trauma, as her doctors discovered, through Magnetic Resonance Imaging (“MRI”), that she was suffering from a narrowing of her spinal canal to, at one level, about 6.0 mm. This meant that there was no reserve room in Ms. Monks’ spinal cord and sudden extensions of her neck could further compromise the canal diameter, due to inward buckling of posterior ligaments. This could lead to irreversible spinal cord injury and Ms. Monks was told to avoid activities that would place her at risk for falls and related neck injuries.

[4] Unfortunately, Ms. Monks was involved in a third motor vehicle accident when, on December 23, 1998, the sport utility vehicle she was driving hit a slippery patch during a

snowstorm, in Pennsylvania, U.S.A. and struck a bridge abutment. She suffered an exacerbation of her symptoms. She suffered pain in her neck and discomfort in her arms.

[5] Because of increasing severity of her symptoms, Ms. Monks underwent a surgical intervention of the spine on September 27, 1999, (from the anterior) at two levels in her cervical spine, in order to prevent further deterioration. Although there were no immediate complications from that surgery, Ms. Monks suffered eventually symptomatology and required further neurological surgery, in January 2001, (from the posterior) this time to forestall further decline. As a result of this operation, she has developed spastic quadriparesis (weakness in all four limbs) particularly in the lower extremities, chronic pain in her neck and shoulders, extreme fatigue, gait ataxia (decreased balance, trunk strength and stability), gastrointestinal dysfunction (persistent nausea and vomiting), neurogenic bladder (requires catheter management), neurogenic bowel (requires digital evacuation and enemas), decreased concentration and memory, sleep apnea/narcolepsy symptoms and depression.

[6] For the 1995 and 1998 accidents, only statutory accident benefits are available to Ms. Monks. The 1995 accident occurred in the Province of Quebec and even though the liability for the accident rested entirely with the young man who rear-ended Ms. Monks' vehicle, no tort claim could be advanced, because the law of the jurisdiction where the accident occurred applied, and the law of Quebec prohibits bringing lawsuits for damages arising out of motor vehicle accidents. The 1998 accident was a single-vehicle accident and Ms. Monks was the driver.

[7] Ms. Monks applied for benefits from Zurich Insurance, her insurer at the time of the 1995 accident. The insurance regime in effect at the time of that accident was Bill 164. Zurich denied that it had any responsibility to pay any benefits to Ms. Monks arising out of the 1995 accident and litigation ensued. Ultimately, Zurich settled its claim with Ms. Monks on the eve of trial on April 29, 2002. The money received by Ms. Monks at that time was the first money she received from Zurich at any time following the 1995 accident. The parties agreed to the following breakdown:

- a) Loss of Income Benefits:
 - i) for past benefits (“IRB”) - \$100,000.00
 - ii) for future loss of earning capacity (“LEC”) - \$500,000.00
- b) Supplementary Medical Benefits:
 - i) for past benefits - \$40,000.00
 - ii) for future medical expenses - \$150,000.00
- c) Future Rehabilitation Benefits:
 - i) for home renovations - \$75,000.00
 - ii) for housekeeping - \$110,000.00
- d) For future attendant care - \$300,000.00

[8] Ms. Monks who also had an insurance policy with ING Insurance applied to ING Insurance for benefits, following the 1998 accident. That accident was governed by a different insurance regime – Bill 59. ING did provide benefits to Ms. Monks on an on-going basis leading up to the settlement of April 29, 2002. However, ING always took the position that it was doing so on a without prejudice basis. It was ING’s intention to seek indemnity from Zurich for whatever benefits it had paid to Ms. Monks.

[9] To that end, ING had itself added as an intervenor to the litigation between Ms. Monks and Zurich Insurance on July 27, 2001. ING participated in that litigation right up to the settlement between Ms. Monks and Zurich.

[10] In late 2001, Zurich Insurance and ING Insurance entered into an agreement whereby, amongst other things, ING would acquire all the personal lines of insurance of Zurich Insurance, effective April 1, 2002. Ms. Monks’ 1995 policy was one such policy. The settlement funds received by Ms. Monks, in April 2002, were from a cheque from ING not Zurich.

[11] ING made a 'strategic decision' not to pursue Zurich for any benefits it had paid to Ms. Monks. It was clear that by April 2002, they would simply have been taking money from one pocket and putting it in another.

[12] Instead, ING chose to terminate all benefits payable to Ms. Monks, as a result of the April 2002 settlement, and to claim back from her what it alleged had been overpayments on their part. This decision was conveyed to her in a letter, dated May 31, 2002 (Exhibit 15).

[13] ING took the position at that time that the CAT DAC, which was delivered 10 months earlier, on July 20, 2001, concluded that: "you are not catastrophically impaired on the basis of the December 23, 1998 accident". (More will be said of the CAT DAC conclusion later in these submissions, but it must be pointed out that the wording in the conclusion of the CAT DAC was in fact: "we cannot reliably conclude that this patient is catastrophically impaired on the basis of the December 23, 1998 accident").

[14] In regard to medical/rehabilitation benefits, ING noted that on the basis of the report of Scott Puddicombe, dated May 29, 2001, home modifications would cost \$146,911. It was noted that Ms. Monks had received \$75,000 toward home modifications in the settlement with Zurich. ING chose to pay nothing toward home modification and paid instead the amount of \$50,786.88 to Ms. Monks, which represented the balance owing for medical and rehabilitation benefits to the limit of \$100,000 for a non-catastrophic impairment.

[15] Concerning attendant care, ING took the position that it only had the obligation to provide attendant care benefits for 104 weeks, arguing Ms. Monks was suffering a non-catastrophic impairment. It also argued that any amounts, which Ms. Monks had received in the Zurich settlement, were deductible from what ING would be obliged to pay. As a result, ING informed Ms. Monks that she was required to repay the amount of \$33,064.37 for what ING considered to be an overpayment of attendant care benefits.

[16] ING took the same position when it calculated Ms. Monks' entitlement to housekeeping expenses, as no benefits were payable after 104 weeks and, since the amounts received in the

Zurich settlement were deductible, it claimed a further repayment from Ms. Monks, in the amount of \$1,811.

[17] At the beginning of trial, counsel for ING stated that it would not be pursuing these two claims. However, between May 31, 2002 and the commencement of trial, no letter had been sent to Ms. Monks to this effect and no one from ING had advised her, or her counsel, that ING would not be continuing to seek repayment of these amounts until then. This course of action will be revisited in the submissions regarding aggravated damages, submitted by both counsel.

[18] When considering income replacement benefits, ING noted that Ms. Monks had received \$600,000 for income benefits in the Zurich settlement. ING took the position that these benefits were deductible from the income replacement benefits she had received from ING and, accordingly, sought repayment of \$17,730.44. ING immediately began to deduct 20% from the amount it was currently paying, for income replacement benefits, in order to recover this amount.

[19] Prior to this, Ms. Monks and ING had reached an agreement that the weekly income replacement, which was attributable to the loss of income sustained, following the 1998 accident, was \$340.96. Following the May 31, 2002 letter sent by ING and right up to the first day of trial, ING continued to deduct 20% from this weekly amount on the basis of the argument presented in the May 31, 2002 letter. On the first day of trial, ING agreed that the weekly income replacement benefit should have been \$340.96 all along and, repaid all of the amounts held back since May 2002, with interest.

[20] As a result, ING has now agreed that Ms. Monks is entitled to continue to receive an IRB in the amount of the \$340.96 per week and that such an agreement should be part of the judgment in this case.

ISSUES

1. **Who or what caused Ms. Monks' injuries, which are the subject of the present claim for benefits?**

- a. What role should the 1993 and 1995 accidents and Ms. Monks' alleged pre-existing medical condition play in the causation of injuries?
 - b. Did the 1998 accident directly cause Ms. Monks' injuries?
2. How should the funds received by Ms. Monks in 2002 (following her settlement with Zurich Insurance concerning the 1995 accident) be taken into account?

Should Ms. Monks be required to spend the entire amount of funds received in 2002 before being entitled to receive any additional amount?
3. What benefits should be provided by ING to Ms. Monks?
 - a. What amounts should be received for past benefits dating from May 2002 to the present?
 - b. What amounts should be received for current on-going benefits?
 - c. How should Ms. Monks' claims for "accommodation" benefits (such as vehicular and home modifications) be resolved?
4. Is Suzanne Monks entitled to aggravated damages because ING delayed the payment of various accident benefits to her?

The Expert Evidence at Trial

On Behalf of the Plaintiff

Dr. Howard John Lesiuk

[21] Dr. Howard John Lesiuk ("Dr. Lesiuk") became a medical doctor in 1982, graduating from the University of Manitoba. He did his neurosurgery residency at the same university between 1982 and 1990, and became a fellow of the Royal College of Surgeons, in 1989. He also completed a two-year fellowship post-residency training in the subspecialty of cranial vascular neurosurgery. In 1992, he moved to Ottawa, where he became an associate professor for the Department of Surgery at the Neurosurgery division of the University of Ottawa. He is also a neurosurgeon on staff at the Ottawa Hospital. He stated that 60% of his surgical procedures involve the spine and 40% of his surgical procedures involve the brain. He performs between 250 to 300 surgeries each year. He estimated that he has completed 2,300 surgeries over the past 14 years. Dr. Lesiuk was Ms. Monks' treating neurosurgeon and he has filed

medical reports with the Court. I qualified Dr. Lesiuk as an expert in neurosurgery with experience in spinal cord injuries.

Dr. Anik Vandewaetere

[22] Dr. Anik Vandewaetere (“Dr. Vandewaetere”) graduated in medicine from Ottawa University, in 1989. She completed her residency in internal medicine at Queen’s University, as well as a two-year specialist course in family medicine. She also obtained a certification in family medicine with the Canadian Council for family practice. From 1991 to the present, she has maintained an office in a group practice with other doctors, in Ottawa, Ontario. Her patients range from children to the elderly, and the services rendered to her 4,000 patients are comprised of physical examinations, referrals to specialists, triaging the seriousness of medical complaints, and making diagnoses and findings on a patient’s medical condition. She deals with problems touching paediatrics, obstetrics, neurology, orthopaedics, psychiatry and general medicine. She has studied spinal cord functions and degenerative disc disease. She stated that 10% to 15% of her patients consult her concerning back problems that involve degenerative disc disease.

[23] Dr. Vandewaetere was qualified to give evidence as a general medical practitioner, specialized in family medicine. Dr. Vandewaetere testified that eight years ago, the Ontario government encouraged the creation of her specialty and gave family medicine specialists the role of gatekeepers of the medical system. While she treated Ms. Monks, Dr. Vandewaetere received information from neurologists and neurology surgeons. Her opinions in neurology are well informed, but cannot be given the same weight as the opinions of the neurologists and neurology surgeons who treated Ms. Monks.

[24] Dr. Vandewaetere gave a demonstration of how a person’s spinal cord functions. The spinal cord carries nerves that branch out at different levels of the body, giving off information, sensations and motor commands, all carried down from the base of the skull. As Ms. Monks suffered from degenerative disc disease, she summarized the disease as follows:

- Degenerative means the shrinkage in height of a disc found between two vertebrae.

- As a disc shrinks, it squeezes out and creates a bulge that can go backwards and enter the spinal canal; the disc oozing out can go to the right, as well as backwards.
- Osteophytes (a bony outgrowth or protuberance) are the nature's way to heal the wear and tear of discs, by laying down calcium in the affected area.
- The disc degenerative disease is ubiquitous and universal in all persons and it consists of 80% of water at age 20 years and, as a person grows older, the disc has less moisture and hydration with time such that 70-year-old persons have discs with a 60% water component only.
- Degenerative disc disease can be a component of trauma. The spinal column has foramen (a natural opening or passage in bone, an opening allowing for the egress of spinal nerve roots from between two vertebrae). If the aperture is reduced and the nerve exit is impinged upon, then pain is felt with sensory and motor involvement.

[25] Dr. Vandewaetere explained that radiculopathy could occur (a disease of the nerve roots in or near the spinal canal, as a result of direct pressure from a disc, or inflammation of the nerve roots due to disc or spinal joint disease). Myelopathy is also another condition that can develop from trauma (any functional or pathologic disturbance in the spinal cord). Dr. Vandewaetere reminded the Court that the spinal cord carries much information through the exiting nerves from the spinal cord and, as a result of trauma many parts of the body are at risk. By way of examples, she cited disturbance of leg movement and the functioning of the bowel and the bladder.

[26] Ms. Monks had consulted Dr. Vandewaetere in 1993, when Dr. Vandewaetere worked in a walk-in clinic, in Orleans, an eastern suburb of the City of Ottawa. No notes are available from those visits and Dr. Vandewaetere stated that she remembered Ms. Monks as the lady who had thirteen miscarriages. In January 1997, Ms. Monks attended her walk-in clinic, now moved to Hunt Club, a suburb south of Ottawa.

[27] As the determination of the issues rests on Ms. Monks' medical condition before, and after, the 1993, 1995 and 1998 motor vehicle accidents, I find it is appropriate to follow the development of Ms. Monks' medical condition by following Dr. Vandewaetere's chronological chart entries. I will also refer to what other experts have said concerning Ms. Monks' symptoms,

before and after each accident, to arrive at a determination as to which of the three accidents caused Ms. Monks' incomplete paraplegia.

For the Defence

Dr. Bruce Stewart

[28] Dr. Bruce Stewart graduated from the Toronto Medical School in 1958. He began neurology at St. Michael's Hospital, in Toronto, and then completed his training at the Neurological Institute at Columbia University, in New York. He returned to Canada to join the teaching staff at the University of Toronto in neurology. After practising in neurology in Scandinavia, for one and a half years, he returned to teach at St. Michael's Hospital, in Toronto.

[29] In 1966, he obtained his fellowship in neurology from the Royal College of Physicians. He then became an assistant professor in neurology at the faculty of medicine at the University of Toronto. He was chief of medicine at Etobicoke General Hospital and he is currently a consultant neurologist at Westpark Hospital, a rehabilitation hospital. I qualified Dr. Stewart as an expert in neurology. Dr. Stewart also gave this Court a written report on Ms. Monks' injuries, having reviewed what other experts had said on the progression of her symptoms in relation to all three accidents.

Dr. Robert Hansebout

[30] Dr. Hansebout graduated as a doctor at the University of Western Ontario, in London. He went on to study at McGill University and obtained a Masters of Science degree in neuropathy and he obtained his diploma in neurosurgery, in 1966.

[31] He trained in neurosurgery at the Montreal Neurological Institute and as Dr. Lesiuk did, he spent one and a half years doing research on the spine at the National Research Council, in Ottawa. He then moved to Hamilton, Ontario, where he did neurosurgery at Hamilton's four hospitals. He was chairman of the Department of Surgery at McMaster University for 10 years, commencing in 1988. He is the author of 64 publications and he testified that 75% of his publications concerned problems with the spine. He claimed that he had performed 4,000

surgeries and 3,000 of the surgeries were done on the spine or peripheral nerves. I qualified Dr. Hansebout as an expert in neurosurgery.

Dr. Markus Besemann

[32] Dr. Besemann stated that he is a specialist in physical medicine and rehabilitation. He works at the Hull Regional Hospital, which is the main trauma referral centre for the whole Outaouais region, a catchment area of 350,000 people. He received his medical degree at the University of Ottawa, in 1988, did his internship at the Ottawa General Hospital, in 1989, and then did a four year stint as a medical officer with the Canadian Armed Forces.

[33] Dr. Besemann did his residency in physical and rehabilitation medicine when he returned to Ottawa, at the University of Ottawa. He completed three years of residency dealing with spinal cord injury, amongst other type of injuries. He also had three months training in electromyography with Dr. Pierre Bourque. Electromyography is the measurement of nerve conduction velocities and insertion of needles into muscles to diagnose nerve system lesions.

[34] Currently Dr. Besemann is Chief of Physical Medicine and Rehabilitation at the Hull Hospital and at the Rehabilitation Centre, next to the hospital. He is also a consultant physiatrist at the Work Wise Designated Assessment Centre (“DAC”), in Ottawa. He performs independent medical examinations, Catastrophic Designated Assessment Centre (“CAT DAC”) evaluations, among other duties. He essentially fulfills the role of an arbitrator when there are disputes between insurers and plaintiffs, regarding treatment. To do a medical rehabilitation assessment at the government Designated Centre is specifically to answer questions as to the reasonableness and necessity of specific treatments relating to accident-caused injuries. I qualified Dr. Besemann as an expert in the field of physiatry and as a doctor that does many medical rehabilitation and disability assessments each year. He testified that he had seen only one or two patients besides Ms. Monks, who have suffered whiplash injuries that resulted in neurologic compromise.

Names of witnesses testifying or giving a medical opinion

For Plaintiff

Suzanne Monks (Perrozzino)
Dr. Pierre Bourque, neurologist
Dr. Howard Lesiuk, neurosurgeon
Dr. Walter Walsh, gastroenterologist
Dr. Anik Vandewaetere, family medicine
Laurie Warren, friend
Lynn Parker, friend
Dr. Maxine Morrison, friend
Dr. B. Guyot, neurosurgeon
Dr. Harold Becker, family medicine
Dr. W. Snook, family physician
Dr. L. Scully, gastroenterologist
Dr. Leahy, urologist
Phyllis Bergmans, consultant health care
Sandra Guest-Poulin, occupational therapist
Dr. Donald Johnson, orthopaedic surgeon

Dr. Donald Chow, orthopaedic surgeon
Dr. Jack Klein, psychiatrist
Dr. D. McKim, respirologist
Fiona Smith-Bradley, occupational therapist
Scott Puddicombe, master carpenter – design consultant
Dr. Markus Besemann, physiatrist

For Defendant

Dr. Lynne MacGregor, physiatrist
Dr. David Attack, neurologist
Joanne Beehler, case manager ING
Dr. Robert Hansebout, neurosurgeon
Dr. Bruce Stewart, neurologist
Dr. Ross Roussev, neurologist
Dr. Anthony Weinberg, general medicine
Dr. Mark Besemann, physiatrist
David Borthwick, design consultant – D.A.C. assessor
Hamid Riahi, real estate agent
Kompass Claims Management Group – Phyllis Bergmans – consultant for insurance companies

ISSUE NO. 1 – What caused Suzanne’s injuries and what role did the 1993, the 1995 accidents and Suzanne’s pre-existing condition play on her present condition?

Ms. Monks’ Evidence

Education and Employment History

[35] Born in Quebec City, on September 29, 1955, Ms. Monks married John Perrozzino, in 1977, and moved to the City of Ottawa. She is the mother of two children, Jonathan, now 23 years of age, and Shaugn, now 15 years of age. Only Shaugn currently resides with Ms. Monks and her husband. She obtained a Bachelor of Science degree in nursing, at McGill University, in Montreal, in 1977.

[36] Ms. Monks testified that she worked as a nurse at the Ottawa General Hospital for six months and as assistant head nurse at the Children’s Hospital of Eastern Ontario until 1981,

when her son, Jonathan, was born. She held a number of positions in the health field until her second son, Shaugn, was born in 1989. From 1990 to 1995, she worked as an occupational Health and Safety Officer for the Canadian Brotherhood of Railway Transportation and General Workers' Union. Her work involved developing appropriate vocational strategies, and implementing a rehabilitation plan, with the cooperation of the employer and the employee, to help disabled employees to reintegrate the work place.

[37] Ms. Monks also testified that, during the years 1992 to November 1994, she served as a vocational rehabilitation consultant with Voc-Care Rehabilitation Management. She helped individuals receive disability benefits from automobile insurers and/or from the Canada Pension Plan. From November 1994 to September 1997, she worked as an independent adjuster at Renaud and Senecal adjusters, in Ottawa, Ontario, specializing in disability and rehabilitation problems for accident victims. She defined her work as being an advisor to clients, health practitioners, employers and other rehabilitation professionals, in order to develop and implement a rehabilitation plan to assist clients achieve an optimal level of functioning.

[38] As Renaud and Senecal adjusters was sold in 1997, Ms. Monks stated that she incorporated Polaris Rehabilitation and Advisement Inc. ("Polaris") in May 1997, and that she kept all her old files that she had handled at Renaud and Senecal. From 1997 to the time she stopped work, because of her own disabilities, in March 2001, Ms. Monks said she was hired by Dynamic Rehabilitation of Ottawa ("Dynamic"). She closed down Polaris, as she became too busy developing rehabilitation services in Eastern Ontario and parts of Western Quebec for Dynamic. She was then providing vocational rehabilitation and case management to individuals who were receiving disability benefits, due to illness or accident injuries.

[39] Ms. Monks testified that she followed a number of courses and was highly successful in achieving accreditations to do her work, either as an adjuster or as a vocational and rehabilitation specialist. She became an associate of the Insurance Institute of Canada when she joined the firm of Renaud and Senecal. In addition, she obtained the following accreditations:

1995 and 1996 – Associate Insurance Institute of Canada Award for highest level of achievement in first year program.

1996 – Courses for Life Skills Certificate taken at Ottawa for several weekends during a six-month period to help clients relearn basic skills.

1996 – A one-week long course in Halifax, Nova Scotia, on Brain Injury.

1996 – Accreditation with Canadian Association of Rehabilitation Professionals.

1998 – Certified Rehabilitation Registered Nurse – an American certification required by the Ottawa Rehabilitation Centre that required a lot of studying and the writing of an exam in December 1998.

1998 – Corporate Crisis Intervention Certification following a four-day course, taken in Las Vegas, Nevada.

1998 – The certification on Threat of Violence also involved a four-day course, in Las Vegas, given by the American Medical Association.

She testified that all her accreditations were obtained between her 1995 accident and her 1998 accident.

The 1993 Accident

Evidence of Suzanne Monks

[40] Ms. Monks stated that this accident occurred on Highway 401, on a trip to Toronto, in February 1993. She was driving a chaser van for Laidlaw and a snowstorm developed. Her vehicle was broadsided when a woman, who had been passing vehicles ahead, lost control of her vehicle, ended up on the median, and had increased the speed of her vehicle to return to the road when she entered into a collision with the van driven by Ms. Monks. As the van could still be driven, she stated that she proceeded to Toronto to collect the company's bus drivers, who had preceded her on the road. As a result of the accident, she did not go to a hospital. While in Toronto overnight, she had applied ice bags to her neck that had become stiff. On her return to

Ottawa, she consulted her family physician, Dr. Wayne Snook. Dr. Snook diagnosed a cervical strain that affected the rotation of her head. She obtained neck massages at Voc-Care Rehabilitation Management where she worked, and stayed off work for March and April of 1993.

[41] Ms. Monks maintained that the impact of the 1993 motor vehicle accident had been negligible, as it had not interfered with her work at Voc-Care Rehabilitation Management, where she had been “busier than ever”. At home, she claimed she did everything that concerned the family, as, unfortunately, her Italian husband was only good, at the time, to fold the laundry.

[42] She stated that her recreational activities included biking with the children, two or three times per week, playing slow pitch ball in a league during the summer months. She recalled that she was downhill skiing, four to five times per season, golfing almost every weekend in the spring and summer months, and swimming, either at the Royal Canadian Mounted Police pool or at her home pool in the summer, as well as at her cottage in East Aldfield, Quebec.

[43] Under cross-examination, Ms. Monks admitted that her x-ray, taken following the 1993 accident, showed she had degenerative disc disease. She stated that, as most persons’ discs are continually shrinking between the ages of 20 years and 70 years, this fact did not bother her. She agreed that in 1993, she had consulted Dr. Jean-Pierre Laflèche for a sore right knee, diagnosed as a collateral ligament strain.

[44] She also admitted that in 1992, she had a slip and fall accident and that the Workmen’s Compensation Board file contained an employer’s report that she had a wrenched neck.

[45] She also agreed that Dr. Snook, her attending physician at the time, had noted that on June 16, 1993, she had reported mild low back pain, stretching to her buttocks. During the month of August 1993, Ms. Monks had a fall and had fractured her right tibia. Dr. Vandewaetere was asked whether it was possible that Ms. Monks, who weighed 204 lbs. on October 8, 1993 and who had increased blood pressure, could do sports activities such as slow

pitch ball, swimming, biking and golf. Dr. Vandewaetere replied that Ms. Monks' weight was not an impediment in her sporting activities.

[46] Ms. Monks was also cross-examined on her injuries that pre-dated her 1993 motor vehicle accident. She was asked why she had not told her treating physician, Dr. Vandewaetere, about past injuries, since Dr. Vandewaetere's role is to collect information on patients and relay such information to specialist doctors. She answered that she had not recalled the 1980 incident when she had put her car in a ditch. The record of her rheumatoid arthritis need not have been mentioned, as it had been a false diagnosis. Ms. Monks had also suffered a 1992 slip and fall incident that she had not mentioned because, as she stated, it did not necessitate a medical follow-up. She also stated that her mononucleosis pneumonia that made her miss her grade 10 promotion in school was not relevant to her condition in 1993, and that she did not remember what had caused her low back pain in June 1993.

Evidence of Laurie Warren

[47] Laurie Warren is an occupational therapist who graduated from Queen's University, in Kingston, with a Bachelor of Science degree. She has been practising as an occupational therapist for the past 12 years, in Ottawa, following her graduation in May 1990. Her *curriculum vitae* is filed as Exhibit 21. She often met Ms. Monks at Ottawa Valley Adjusters Association meetings ("OVAA") and social functions such as golf tournaments. She testified that in 1993, Ms. Monks was energetic, sturdy, tomboyish and upon meeting her, one would not think that she had any serious medical problems.

Evidence of Lynn Parker

[48] Lynn Parker graduated with a B.A. (Psych) from Queen's University and an MBA from the University of Ottawa. Her professional profile (Exhibit 22) lists her occupation as a Certified Life Planner as she is involved in the preparation of life care plans for catastrophic and complex injuries for Kompass Life Care Group Inc. From 1996 to 2001, she was also involved as manager, supervising all Statutory Accident Benefit Schedule ("SABS") claims in Eastern Ontario for AXA Insurance. She had also worked with Renaud and Senecal adjusters, in 1995.

She worked with Travellers/Zurich Canada from 1987 to 1995 and during that time met Ms. Monks. Her meetings with her took place at lunchtime seminars given by Voc-Care Rehabilitation Management (“Voc-Care”), and those meetings were attended by case managers. She described Ms. Monks as an articulate, perceptive person, having exceptional knowledge in the vocational and rehabilitation field.

[49] From 1992 to 1994, Ms. Parker stated that she was entrusting Ms. Monks with her most complex files and she stated that she had handled the files quickly and with efficiency. She described Ms. Monks’ work ethic as above average, yielding astonishing results, because of her high productivity level. She stated that she could not see any physical impairment in Ms. Monks when she worked at Voc-Care, from September 1992 to November 1994. When Ms. Monks went to work for Renaud and Senecal adjusters, from November 1994 to September 1997, Ms. Parker stated that she kept on sending motor vehicle claims’ files to her. Ms. Parker stated that she also gave Ms. Monks case management files involving very complex injuries and that she could not be compared to other adjusters. Ms. Monks was working night and day at that time.

The Medical Evidence for the 1993 Motor Vehicle Accident

[50] On May 13, 1997, Dr. Lesiuk reported to Dr. Vandewaetere on Ms. Monks’ condition and he commented on her 1993 motor vehicle accident:

Thank you for asking me to evaluate this very pleasant 40-year-old, right-handed insurance adjuster. As you know, she was well until she was involved in a motor vehicle accident in 1993 sustaining a whiplash type of injury. She subsequently developed localized cervical pain, which resolved over the next 3-4 months with conservative therapy. (Exhibit 1, Tab A-2)

[51] Dr. Pierre Bourque, a neurologist, saw Ms. Monks on July 16, 1997. He had been asked by Dr. Lesiuk to provide a second opinion on her condition. Dr. Lesiuk testified that “Dr. Bourque had not attached a high significance to Ms. Monks’ 1993 motor vehicle accident and neither do I”. Dr. Lesiuk also commented on Ms. Monks’ other pre 1993 ailments. He said that the episode about her having driven her car in a ditch and the episode of her missing her grade 10 on account of misdiagnosed rheumatoid arthritis; such incidents were not important things for him to have known, as they had not led to active problems by the time he examined her.

[52] Dr. Hansebout, a neurosurgeon, testified and he did not have much to say about the 1993 motor vehicle accident. He commented on an x-ray report (Exhibit 6, Tab 1) done on March 15, 1993, of Ms. Monks' cervical spine:

It shows a straightening of the spine. Usually that's because the muscles in the neck are tight and they straighten the spine, it usually indicates that there is neck pain. In this case, it's obviously what she had, neck pain, that's what straightened it. There is also degenerative narrowing of the C6-7 disc, which means that the disc is starting to get thinner due to a reduction in the water content of the disc, which usually occurs with aging. So it just means that her spine already is showing a degeneration. I think she was about 38 then."

[53] Dr. Lesiuk stated that he disagreed with Dr. Hansebout in that Ms. Monks' degenerative disc condition was normal for a 38-year-old woman.

[54] John Perrozzino ("Mr. Perrozzino") testified that, following the 1993 motor vehicle accident, his wife Suzanne Monks continued to do all the housework and that "she was okay three to four months later". He stated also that she continued her work at Voc-Care, but did not return to drive buses. He described his wife's sporting activities. They included tobogganing with the children, cross-country and downhill skiing in the winter and camping, swimming, hiking and golfing in the summer. He stated: "There is not much she could not do."

[55] He stated that Suzanne ceased driving buses for Laidlaw, as she could not turn her neck sufficiently nor could she lift the hood of the bus. He claimed that he did not hear his wife complain about any physical ailments, following the 1993 motor vehicle accident.

[56] Dr. Maxine Morrison testified. She holds a B.A., M.A. and Ph.D. in psychology. She has developed a practice in clinical psychology over the past 20 years. She has known Ms. Monks since 1994 and she described her as a friend and as an active and energetic person who worked phenomenal hours. She stated: "I do not know how she did it. I could not keep up with her." She had not noticed any physical problems with Ms. Monks, following the 1993 motor vehicle accident.

The 1995 Accident

[57] Ms. Monks testified that she was involved in a motor vehicle accident on July 21, 1995, when she had stopped her car at a red light on the highway, on Montreal's south shore. Her vehicle was rear-ended by a vehicle driven by a young man, who was installing a CD in his car's playing machine, and failed to see the light was red and that her vehicle was stopped. She was wearing her seatbelt and the force of the collision had pushed her back in her seat and then forward on the steering wheel. She claimed that her arms had gone numb right away for a 15 to 20 minute period, followed by a period of time when she felt pins and needles in her arms. She recalled having said to the young man, who had caused the collision, "young man, you injured my neck". She recalled also that the pain in her neck and shoulder was in the trapezius area.

[58] Ms. Monks stated that, back in Ottawa, the same day of the accident, her neck got stiffer and she had muscle spasms. She recalled that she was off work and treated herself with ice, heat and took Tylenols, over a two-week period, following the accident. She also stated that her upper back was sore and that she had some difficulties with her right arm. Her right hand felt "clumsy". Dr. Snook had diagnosed a cervical strain and she stated she went the massage therapy route. She stated that she used over the counter medication to deal with the pain, such as Robaxacet.

[59] Ms. Monks also recalled that she had returned to work two weeks after the accident. However, she felt worried, as she had trouble writing with her right hand.

[60] Ms. Monks described her symptoms, leading to a 1996 MRI, as muscle spasms in the neck, the back and the arm. She stated that the pain that caused her headaches, in September 1995, was located in her neck, in the trapezius area. At work, during the same period of time, she had to slow down, as she could not maintain the same pace at work that she had maintained in 1994, following her first accident. Ms. Monks maintained that, during 1994, she had worked insane hours and by that she meant 18 to 20-hour days, at Renaud and Senecal adjusters.

[61] She remembered that she reduced her hours of work a year later by focusing on old files and refusing new files. During the fall months of 1995, the hours spent at work were in the 50 to

60 hours per week range and she also worked on weekends. She recalled working that many hours per week in 1996, although many hours were not billable hours. She also recalled taking courses and being involved on the board of directors of her son's daycare facility.

[62] At home, Ms. Monks stated that things had changed. Her husband, John, was no longer only involved in folding clothes out of the dryer. Ms. Monks recalled that she needed more help and her husband became involved in picking up her son from the daycare and in preparing meals, as her level of activity dropped by 50%.

[63] In 1996, she was worried about the lack of strength in her right hand, and she also stated that she was dropping things. Ms. Monks confirmed that Dr. Snook had stated, in his 21st of September 1995 x-ray report that she had "a mild narrowing of the 5-6 and 6-7 disc spaces with minimal marginal osteophytes in keeping with mild degenerative disc disease". According to Ms. Monks, Dr. Snook had told her that the x-ray was normal and in keeping with a mild degenerative disc disease (Exhibit 6, Tab 2).

[64] Ms. Monks recalled that, on January 18, 1996, she suffered from neck discomfort on and off, and it meant that the driving she had to do, as a claims adjuster, to meet with clients, was difficult.

[65] Ms. Monks further recalled that, on July 27, 1996, she underwent an MRI investigation at the Ottawa General Hospital. She also recalled that, in August 1996, she had discussed paraesthesia in her right arm with Dr. Snook (abnormal sensation, ex. burning). She claimed that the 1996 MRI showed her discs herniating at the centre and not at the root of her spinal cord, which resulted in undesirable compression of the spinal cord on the bone.

[66] Dr. Lynne McGregor was consulted by Ms. Monks, on November 14, 1996. She stated that Dr. McGregor's diagnosis was that her pain was more muscular than neurological. Ms. Monks testified she understood that her pain was not coming from the spine and that she had to follow Dr. McGregor's advice and not worry so much about her MRI. She recalled that she was told to reorganize her workstation to meet better posture requirements, and that she was referred

to a facility that conducted nerve studies, to eliminate the possibility of a nerve trapped in the wrist or the spine.

[67] As the year 1996 closed, Ms. Monks recalled that she had not made progress, nor lost ground, with her symptoms. She stated that her writing problems, due to poor strength in her right hand, were worse, although it was hard to judge that. She was working at Renaud and Senecal adjusters and she was falling behind in her work. She was becoming discouraged and depressed, because her work and other responsibilities, such as the budget for her son's daycare corporation, were falling behind. She stated that it was at that time that she was not able to write her final exam for her ACCI accreditation. The exam was in December 1996, and she recalled that the organization did not allow her to use a computer to answer the exam questions.

[68] Ms. Monks visited Dr. Vandewaetere during the month of January 1997. She testified that she had problems with her right hand, problems with sleeping and with fatigue. She stated that she was not meeting her responsibilities at home and at work, and that she had felt depressed. Ms. Monks was referred to Dr. Lesuik, a neurosurgeon, by Dr. Vandewaetere, to have her hand problem evaluated. Dr. Lesuik, according to Ms. Monks, found weakness at two fingers on her right hand and he recommended another MRI. She had understood his diagnosis as being a moderate degenerative spondylosis change, affecting the lower cervical segments C 4/5, C 5/6 and C 6/7. Dr. Lesuik also found a mild canal stenosis at C 4/5 and to a lesser extent C 6/7 and a more substantive canal stenosis at the C 5/6 level.

[69] Ms. Monks' new MRI was reviewed in Dr. Lesuik's report to Dr. Vandewaetere of August 12, 1997. She had also seen Dr. Bourque, a neurologist, on July 16, 1997.

[70] Ms. Monks recalled that she had complained to Dr. Bourque about her right hand, left upper arm fatigue and urinary frequency. She stated that Dr. Bourque planned to treat her symptoms conservatively and that she was to report any symptom change.

[71] Ms. Monks testified that Dr. Lesuik stated there were no changes on the new MRI, when it was compared with the previous one. This, she understood, meant that her condition was

stable. Surgery was discussed with Dr. Lesuik, who felt that surgery would only be necessary if Ms. Monks' neurological symptoms progressed, but Dr. Lesuik felt that, as there was no signal change on the new MRI, no surgery was mandated.

[72] Next, Ms. Monks described her work habits between the 1995 and the 1998 motor vehicle accidents. During the summer of 1997, she had cleaned-up old adjustment files, because she had left the firm of Renaud and Senecal. She worked for Polaris, a company she incorporated, and she also did work for Dynamic. She was working from her home. She decreased her hours of work in early 1998 and tried voice dictation. She further reduced her hours of work to 35 to 40 hours per week, leading up to the month of April 1998.

1995 to 1998 Employment Description

[73] Ms. Monks worked for Renaud and Senecal, covering Eastern Ontario and West Quebec, as an adjuster, but she dropped her dual role of adjuster and rehabilitation consultant. She kept rehabilitation files for clients coming in to Ottawa from outlying areas served and was assisted by other workers and, in addition, did some marketing.

[74] Ms. Monks stated that there was an overlap between her Renaud and Senecal job and her Polaris job. She taught computer skills in the morning, and did rehabilitation work in the afternoon, with Dynamic Rehabilitation. She took clients to hockey games, splitting 44 games with another worker. She also played some golf with clients in a women's league that allowed her to play with seniors, as her golf had deteriorated. She played one evening per week plus a tournament in September, riding in a cart. If her arms became fatigued, she then limited her golf to putting for the rest of the game.

Education 1995 – 1998

[75] Ms. Monks maintained that she had no problems with her intellectual capacities between 1995 and 1998, as she completed many courses. During the month of December 1998, she passed her Certified Rehabilitation Registered Nurse ("CRRN") certification with honours. In

1996, she had received her accredited Rehabilitation Professional Certificate. She had also completed her Association Insurance Institute of Canada studies, in 1995 and 1996, and received the Institute's award for highest level of achievement in the first year program. She completed several other courses enumerated in the summary of her qualifications filed as Exhibit 7.

Suzanne Monks' Housekeeping Abilities in 1995-1998

[76] Ms. Monks testified that she did almost 100% of her housekeeping chores before 1995. Things changed, as she could not do the heavier housekeeping chores after 1995, even though she liked a clean and organized home. Her home, after 1995, was not the same anymore. She stated that doing the vacuuming was not a good idea and that she had only managed light chores and Mr. Perrozzino had replaced her in transporting Shaugn to his daycare. A cleaning lady had to be hired. She testified she could do the laundry and cooking, but that while, pre 1995, she did most of the housework, after 1998 her husband, Mr. Perrozzino, did most of the chores. She did the shopping at Price Club, she said, and Mr. Perrozzino did the shopping at Loeb's.

[77] During the years 1995 to 1998, Ms. Monks stated that, following the death of her father, she had taken the responsibility of looking after her mother. She took her mother to medical appointments, to do groceries, went over to her house to bathe her, took her to a hairdresser and, at times, to enjoy a meal at a restaurant. She stated that her mother suffered from arthritis, had depended on her dad for outings, and now did not want to go out alone. Her mother did not like strangers looking after her, and homecare was not an option. Her mother died before Christmas 2002.

Suzanne Monks' Recreational Activities 1995-1998

[78] Ms. Monks confirmed that, as she had no spinal canal reserve, Dr. Lesiuk had warned her that she could not do physical activities, such as biking. She also stated that, during the month of July 1997, Dr. Vandewaetere had been worried about her diving in the lake at the cottage as "any further injury could cause catastrophic and permanent injury".

[79] She testified that Dr. Lesiuk also wanted her to refrain from swimming and biking, as he feared that she could damage her spine in a fall. She recalled Dr. Lesiuk had said: “No sports, no fall, no accidents”.

[80] Ms. Monks stated that she had stopped all sporting activities the first time Dr. Lesiuk had mentioned the restrictions to her during the summer of 1997. She understood that because of the narrowness of her spinal canal (it was down to 6.0 mm.), no neck extension was possible, as she understood that it would lead to an irreversible spinal injury. She testified that after 1995, she had done some skiing with her sister from Kentucky and swimming in the lake, but those activities were not as good as before and her right arm did not pull as well while swimming. She did walk for exercise, although it was reduced to $\frac{3}{4}$ of an hour to one hour. In November 1998, in Barsto, California, U.S.A., Ms. Monks testified that, while shopping with friends, she used a wheelchair, as she was holding them up. The shopping was an annual trip and it took place when she attended a conference, as both she and her friends would arrange free time to do their Christmas shopping the day before, or a day after, the conference. The previous year, everything had been fine, but she felt that in 1998, she had suffered from leg fatigue. A wheelchair was also used in transferring from one plane to another on her return trip to Ottawa, as sitting for the five-hour flight proved to be too long for her.

[81] She was fine with everyday living activities and she did not have to use a cane. She could climb up and down stairs, bathe herself and dress herself without problems. She also stated that there were no problems in getting in and out of her Ford Expedition car.

[82] During cross-examination, Ms. Monks was asked if she recalled being taken to hospital by ambulance on November 13, 1995, suffering from back pain in the lumbar region and having a numb feeling at the back of her leg. She replied that it had happened and that she had returned for a Doppler and bone scan the next day. However, she stated that she was healthy enough to write an exam the next day and pass it. She was also confronted with Exhibit 12, Tabs 5, 6 and 7, which contain reports from the Orleans urgent care facility, detailing numbness in the fingers and pain in her shoulders, as a result of a slip and fall injury while at work. Lori A. Lee, a

registered massage therapist, had written, on her behalf to Zurich Canada, describing her injuries, on July 7, 1996 and had stated:

Sue has a large workload at the office, not to mention responsibilities at home and she seems to be gradually running herself down, with discomfort and headaches presenting as the final result.

[83] Ms. Monks replied, when shown Ms. Lee's report, that she was attempting to have her insurers respond to a request for the help of a physiatrist to determine if there were any neurologic components to her difficulties, and to request an ergonomic assessment of her work station. She said that Zurich never responded to Ms. Lee's letter.

[84] Ms. Monks stated that, between July 1995 and December 1995, she controlled her pain with ice, massages and Robaxacet. When it was suggested to her that she had more difficulty in controlling her pain between January 1 and July 1, 1996, Ms. Monks stated that she had managed to control her pain with over the counter medication. She was asked if on May 13, 1997 she had told Dr. Lesiuk that she had experienced severe neck pain and headaches, immediately following her 1995 motor vehicle accident, and that those symptoms had been fluctuating since the accident. She acknowledged having said that to Dr. Lesiuk and that the conversation had taken place during her first meeting with Dr. Lesiuk, after the results of the 1995 MRI had become known. She reiterated that there was no myelopathy diagnosis and that no surgery had been discussed at that meeting. She agreed with defence counsel that her symptoms were more prominent in May 1997 than they had been in 1996.

The Discussions about Surgery

[85] Defence counsel directed Ms. Monks to a letter she wrote, on October 10, 1995, to Zurich. The letter is reproduced in part and stated:

At this time, I have incurred expenses pertaining to this loss as a result of the injury that I sustained. I am currently having symptomatology that is affecting my ability to perform my occupation effectively.

I am attempting to maintain my caseload, however, at this time I am having considerable difficulty performing tasks requiring fine motor control.

At this time, my physician is conducting tests to determine the cause of the abnormality that has developed at the base of my skull. On first examination, the physician indicated that it was his impression that I had sheared tendons from the base of my skull at impact and these were reconnecting with accompanying bony overgrowth. Unfortunately, the skull x-rays did not confirm this diagnosis and as such, further testing is required. The physician has recommended that I take time off work, however, at this time this option is one that I would prefer to avoid, however, with increasing headaches and neurological symptoms, I may have no other choice.

[86] Ms. Monks replied that, as the two-year limitation period to make a claim against Zurich was approaching and, as she understood her physical problem of spinal canal narrowing better, following visits with Dr. Lesiuk, she had hired a lawyer. Zurich had not responded to her concerns in the one and a half years that had elapsed since the accident. When asked if she had wanted Zurich to cover the cost of surgery, she stated that, as surgery was a possibility in the future, she needed to know if Zurich was “going to be there for her”. When asked if the only reason she had not undergone surgery in 1997 was because Zurich would not pay, she answered “absolutely not”.

[87] The examinations for discoveries were then referred to (page 45 of Ms. Monks’ examination, question 2634 et al). She had said that, because she had been scared, following her meetings with Dr. Lesiuk, and that her fears were increased, due to the fact that Dr. Lesiuk had intended to refer her to Dr. Fehlings for a second neurologist’s opinion, she had given surgery some thought. She then stated that her friend, Lynn Parker, an insurance adjuster, was urging her, at the time, to protect herself and that explained her attempts to have Zurich commit to cover whatever expenses she might have to incur. Ms. Monks also replied that the next MRI investigation showed “no signal change” and that Dr. Lesiuk would not have operated on her back, even if she had asked him for an operation. She indicated to defence counsel that Dr. Lesiuk had only forbidden her to do certain types of activities at that point. She pointed to Dr. Lesiuk’s report (Exhibit 1, Tab A-1) and Dr. Bourque’s report (Exhibit 1, Tab G-2), to establish that both doctors had not recommended surgery. She stated that Dr. Lesiuk had said three things had to happen before he would do surgery:

1. An MRI would have to show “a signal change” on her spine;
2. The presence of intractable pain; and
3. Hard clinical findings of myelopathy or radiculopathy.

[88] Ms. Monks agreed, during cross-examination, that during her discoveries (question 2703, page 463), she had said that Dr. Lesiuk had brought up the question of surgery at their first meeting. She had also answered during discoveries that, if Zurich had offered to pay for surgery, she might have had surgery.

[89] Next, she was referred to Exhibit 17, a letter she wrote on August 12, 1999 to Halifax Insurance. She had written:

As I indicated to you, prior to the December motor vehicle accident, I was aware that surgery was going to be needed, however, no surgical date had been set pending resolution of my dispute with Zurich.

To this, she replied during trial:

With hindsight, I would have had surgery in 1997, because I knew the difficulty I was in.

[90] Defence counsel asked her about her increased frequency of headaches between 1995 and 1998. She replied that the headaches were related to the eight hours per day she spent at the computer in sustained arm and shoulder movements. She stated that her headaches were not caused by her spinal cord compression, but were myofacial in nature, thus muscular in nature. In response to her suggestion of going off work, as stated in her letter to Zurich (Exhibit 16, Tab 8), Ms. Monks testified that she was still able to write at that time. She recalled that she had brought her workload down to a reasonable level, to better manage her life. She felt that, prior to her 1995 accident, she had worked 100 hours in some weeks. She agreed that this was reflected in Dr. Vandewaetere’s notes of June 10, 1997 (Exhibit 3, Tab 1, page 12).

[91] Ms. Monks agreed, in cross-examination, that she had hired a housekeeper, between 1995 and 1998, because of the 1995 motor vehicle accident. She had removed some of the frustration resulting from the fact that her husband was not able to pick-up more than 50% of her housekeeping role. Later, the housekeeper was dropped, partly because Zurich, the insurer

responsible to pay benefits, did not want to pay the housekeeper's cost and partly because her husband became better at housekeeping. She stated that her husband did most of the meal preparations between 1995 and 1998, "as I had trouble cutting through veggies".

[92] Mr. Perrozzino in his cross-examination stated that he had not been doing all of the housework, prior to the 1998 motor vehicle accident.

Evidence of John Perrozzino

[93] He confirmed Ms. Monks' work habits and her involvement in sporting activities. He stated that he got involved in housekeeping between 1995 and 1998. He remembered that his wife was "dropping things on the floor", and she had asked him to take over the washing of floors. He stated that he had concluded that his wife was undergoing pain at that time, although she had not complained. She had maintained long hours after the 1995 motor vehicle accident at Renaud and Senecal, but when she worked at Dynamic, she took twice as long to do the same work. He testified that he did the floors, the laundry and the vacuuming in the house. He did not remember discussing the first MRI investigation with his wife, as he did not think it had much significance. During cross-examination, Mr. Perrozzino remained resolute in stating that, between May 1997 and December 1998, nothing had changed in their home once he had agreed to help more. He maintained that, following Dr. Lesiuk's report, after his wife's second MRI investigation, he had not discussed surgery with her. He stated that both he and his wife concentrated on a reduction of activities to avoid accidents and confirmed that her October 25, 1995 letter to Zurich had been written, not for an immediate surgery, but to protect against the high cost of possible surgery in the future.

The Experts

Dr. Anik Vandewaetere – 1995 motor vehicle accident

[94] For Dr. Vandewaetere's examination-in-chief at trial, I have reproduced her summary taken from her report to plaintiff's counsel (Exhibit 1, Tab E-1), as the evidence I received at trial was similar to what she had previously reported in her notes.

[95] Dr. Vandewaetere testified that Ms. Monks visited at her Care Medics Hunt Club Clinic for a consultation, on June 9, 1997. She said that she was working as a rehab worker for the insurance companies. She had injured her coccyx, hitting the corner of the door while getting into her Explorer, on December 31, 1996. Dr. Vandewaetere explained that Ms. Monks had been her patient in 1992 while she was working in an Orleans clinic before setting up her practice in Hunt Club. In June 1997, Ms. Monks had talked briefly of her involvement in a motor vehicle accident, first in 1993 and, subsequently, in July 1995. She had undergone an MRI, which according to Ms. Monks had showed herniated discs at C5-6 and C6-7 with some compression of the cord. She had also seen Dr. Lynne MacGregor with complaints of repeat hand weakness and numbness and had been referred for EMG studies. Ms. Monks had found it difficult to write more than half a page. She had been on claim for a short while in 1993 and then the symptoms resolved themselves.

[96] Dr. Vandewaetere stated that Ms. Monks had complained, during her January 24, 1997 visit, of fatigue, decreased energy and motivation, which were frustrating to her because she was working a reduced amount of time of 55 hours per week. She had regained 40 of the 50 lbs. she had lost with Ensure supplement and fluids. She had had poor sleep because she was ruminating about work. Despite these complaints, Ms. Monks continued to work. She had been sent for x-rays of her Lumbar spine and coccyx, and blood work was ordered. Some Desyrel had been prescribed on a “prn” “as needed” basis to help with sleep and Anaprox for inflammation, a wrist splint for her right wrist and Senecot for constipation. She had been seen again on January 24, 1997, to review her symptoms of depression. She had repeated her symptoms of poor energy, poor motivation, poor sleep and rumination. She had also discussed a decrease in her mood and having the occasional anxiety attacks. In addition, Ms. Monks had mentioned a decreased ability to concentrate and that her friends had spoken of this to her. Ms. Monks had agreed to a trial of anti-depressants. Dr. Vandewaetere said she had started Ms. Monks on a trial of Effexor, 37.5 mg, and Restoril at night, as needed.

[97] Dr. Vandewaetere testified that Ms. Monks had seen Dr. MacGregor on November 17, 1996, for EMG studies, which showed some carpal tunnel syndrome. As she had also wanted to

see Dr. Lesiuk, Dr. Vandewaetere had referred her to him. On her follow-up visit February 17, 1997, she had complained of visual symptoms that were felt to possibly be secondary to the Effexor medication and so it had been discontinued. Her blood pressure had been assessed and treated with Renedil. She had had difficulty writing more than a half page and it was suggested that she use a computer. A note had been given to her for computer use during exams.

[98] On March 27, 1997, Ms. Monks had again complained of poor sleep and lack of energy and motivation. She had felt irritable. (All these things were unusual and out of character for Ms. Monks and she found it both frustrating and disconcerting.) She felt that the Prozac was helping with her anxiety and she had felt more calm. Her Prozac was increased to 40 mg. per day.

[99] On May 27, 1997, Ms. Monks had discussed her visit with Dr. Lesiuk. He had stated that he was worrying about some cervical myelopathy and that he had noted some decreased motor strength of her right finger extensors.

[100] She had been quite stressed because her “company” had been sold and because she had been having legal difficulties with increased costs, working on a “New Deal” with the previous company. She had needed to decide in the next few days “what to do with her life”. The possibility of surgery had been raised by Dr. Lesiuk, but Ms. Monks thought this would be difficult, because it could interfere with her work and negotiations. Her blood pressure was up again and Altace was changed to Coversyl.

[101] Dr. Vandewaetere recalled that she had reviewed Dr. Lesiuk’s letter of May 13, 1997. Dr. Lesiuk had raised the question of early cervical myelopathy, however, he wished to get a more up to date MRI and to have the case reviewed by a neurologist, Dr. Bourque, and to report further “as events unfold”.

[102] Dr. Vandewaetere stated that, on June 10, 1997, she saw Ms. Monks again at her office. Ms. Monks had continued to have much situational stress and some depressive symptoms. Her Prozac was changed for a trial of Serzone.

[103] On July 24, 1997, Dr. Vandewaetere reviewed with Ms. Monks, Dr. Bourque's note to Dr. Lesiuk in which he stated that the symptoms described by the patient (arm and leg fatigue and urinary urgency) might be in keeping with the MRI findings, but that, at that time, there was little significant, objective, evidence of neurologic deficit, other than minor reflex asymmetry. He favoured a conservative approach and stated that the patient was well aware of the usual indications for surgical interventions. These are intractable pain or progressive neurologic deficit, either a cervical radiculopathy, or progressive myelopathy. Dr. Lesiuk's letter of August 12, 1997, in which he had stated that the repeat MRI had not shown a significant interval change, was also reviewed with Ms. Monks.

[104] According to Dr. Vandewaetere, Dr. Lesiuk had noted that the combination of disc protrusion and osteophyte formation had resulted essentially in a spondylitic bar formation at C4/5, C5/6 and C6/7. This had resulted in mild canal stenosis at C4/5 and to a lesser degree at C6/7 and moderate canal stenosis with foraminal stenosis at C5/6. Very importantly, he had also stated that the AP diameter (of the spinal canal) was reduced to 6 mm. There had not been signal change within the cord to suggest parenchymal (cord) injury. He had stated that Ms. Monks had been indeed correct in her surmise that central cord injuries were associated with relatively minor extension injuries in the setting of canal stenosis. Dr. Vandewaetere stated that what wasn't mentioned in her July-September 1997 notes was that she had been concerned about the progression of Ms. Monks' symptoms, that her level of concern increased after reading Dr. Lesiuk's note and that she had suggested to Ms. Monks "off the record" that she should advocate for herself with the neurosurgeons (Dr. Lesiuk and Dr. Fehlings). Dr. Vandewaetere remarked that no neurosurgeon is ever keen to do these surgeries because of the level of risk involved with the surgery. However, she stated that she was very worried for Ms. Monks' safety. She had been told that she shouldn't even dive into the water at her cottage and Dr. Vandewaetere feared that any further injury could cause catastrophic and permanent injury.

[105] Dr. Vandewaetere noted that, while Ms. Monks fractured her right foot in October 1997, this had not affected her neck.

[106] Dr. Vandewaetere said she had received a further letter of explanation from Dr. Lesiuk, dated April 6, 1998. She had specifically urged Ms. Monks to ask more questions. That report is very important in that it explained clearly Dr. Lesiuk's thinking, regarding Ms. Monks' condition at that time, and that he still hesitated to perform surgery without 'classic indications' for decompressive surgery. He had again mentioned in answer to question 12 that she should not partake in certain sports because the cervical spinal canal "was significantly narrowed down to about 6 mm. at one level. This distorted the shape of the spinal cord somewhat and left absolutely no reserve room. Sudden extensions of the neck could have further compromised the canal diameter due to inward buckling of the posterior ligaments. This could have lead to sudden and potentially irreversible spinal cord injury."

[107] Dr. Vandewaetere had seen Ms. Monks again on June 19, 1998, concerning her neck pain and canal stenosis. She said that her life was quite restricted, in that she could not type, and had difficulty with the computer mouse. She could not do sports, and could not even "cake decorate". She was still having some stress and urge incontinence. Her nerve conduction studies had shown only mild right carpal tunnel syndrome. She was still having problems with the insurance company and Dr. Fehlings' office had not made an appointment for her. She also said that, in May 1998, she had had one week of vertigo symptoms. Dr. Vandewaetere testified that she had called Dr. Lesiuk's office and had been able to speak to him on July 22, 1998. She had explained that Ms. Monks was having some vertigo symptoms with neck extension, some urinary urge incontinence and sometime urinary retention. Dr. Lesiuk stated that the vertigo symptoms were rarely seen in spinal stenosis and that he would look into having a second opinion from Dr. Fehlings.

[108] Dr. Vandewaetere explained that she had not seen Ms. Monks between July 27, 1997 and June 19, 1998. She stated that this meant that she was feeling well and had no need to see her. She confirmed that, during that time, Dr. Lesiuk's letter had indicated that Ms. Monks had not presented with a clinical change and that Dr. Bourque had found no objective sensory motor deficits. She stated that she usually deferred, as she did in this case, to the opinions of neurosurgeons and neurologists. She stressed that in 40% of the population, symptoms

progressed and it explained why Ms. Monks was forbidden to play sports. In June of 1998, because her spinal canal was restricted to 6.0 mm., it raised red flags in her mind and she had to pay special attention to what Ms. Monks was telling her.

[109] During cross-examination, Dr. Vandewaetere was asked why Ms. Monks had wanted to see Dr. Lesiuk between her two visits to her, in 1997. She replied that Ms. Monks had nothing specific to go on, but had felt there were differences in her symptoms. Dr. Vandewaetere also agreed with the suggestion that, since Ms. Monks had not been able to complete her exam without a computer in 1997, and since she had had to change her lifestyle, that meant her symptoms had progressed. Dr. Vandewaetere also replied that, on May 13, 1997, Ms. Monks had not advised her that her hand clumsiness had become more noticeable.

[110] Next, Dr. Vandewaetere was referred to Dr. Bourque's report of July 16, 1997 (Exhibit 1 G-2) in connection with Ms. Monks' lack of reports made to her and to Dr. Lesiuk, concerning problems with her lower extremities. Dr. Vandewaetere replied that the report referred to "legs relatively intact, less endurance, both legs tightened up" and it meant, by implication, fatigue and motility.

[111] Responding to questions about Ms. Monks' description of her symptoms, in a letter she wrote to Zurich, Dr. Vandewaetere stated that the term 'neurological symptom' is a vague term, and even though she was not Ms. Monks' family doctor then (October 10, 1995), it is not a choice term and she would not have used it.

[112] Dr. Vandewaetere was not impressed with suggestions that she had not been appraised of Ms. Monks' past medical history in 1997. She stated that she did not spend time on complaints that she knew well and that she concentrated on new complaints. Addressing the situations of Ms. Monks putting a car in the ditch, injuring her knee, tearing ligaments while doing slow pitch, past slip and falls, misdiagnosed rheumatoid arthritis, she stated that it only concerned her if the patient continued to suffer *sequelae*. She reiterated that degenerative disc disease is universal in every person and every person would have a certain amount of the disease showing in his or her x-rays, as a person's discs are always shrinking.

[113] Dr. Vandewaetere was asked if obese persons could golf, slow pitch, swim and ski, and she replied that weighty people are athletic. She also stated in cross-examination that the fact that Ms. Monks had not completed an exam meant that her symptoms of hand clumsiness had progressed. Dr. Vandewaetere confirmed that, from May 13, 1997 to June 10, 1997, she had received no complaints from her concerning her physical problems in the lower extremities. Dr. Vandewaetere also stated that she would have made a note of such complaints, had she received them from Ms. Monks. She also agreed that Dr. Bourque had not mentioned in his report that Ms. Monks' spinal cord problems had caused her urinary problems.

Dr. Lesiuk – 1995 motor vehicle accident

[114] Dr. Lesiuk described Ms. Monks' symptoms, following the 1995 motor vehicle accident, in his report of May 2001 (Exhibit 1, Tab A-10).

She experienced immediate numbness in both arms, which resolved over the next 15 minutes. She also experienced immediate severe neck pain and headache. Since that time, she has had fluctuating neck pain and headaches. More concerning, since the fall of 1995, she has had increased fatigability of the upper right extremity and to a lesser extent, the left upper extremity.

[115] Dr. Lesiuk explained that, since both arms were involved producing diffuse numbness, the symptoms did not identify with a particular nerve root. The reference to transient disturbance in the nerve roots meant that you could have permanent spinal cord injury, as a consequence of ongoing symptoms. The symptoms are falling away from the neck to the right and left limbs, and this, together with the fatigability, points to some nerve dysfunction, as opposed to pain due to a mechanical dysfunction.

[116] Dr. Lesiuk felt there was a problem with nerve root or with the spinal cord, but could not tell which one of them was involved. He then explained his reference in the first report, dated May 13, 1997 (Exhibit 1, Tab A-2), to "weakness of the right finger extensors" and to "there were equivocally positive Hoffman reflexes, more prominent on the right than left, raising the possibility of an early myelopathy". As these findings are central in the debate concerning Ms. Monks' medical condition, on May 13, 1997, I reproduce Dr. Lesiuk's evidence at trial:

Q. And when you look at the symptoms that she was talking about with respect to her right arm and left arm, what sort of concerns - - and when I'm asking this question, what particular parts of the nervous system would be concerned about by virtue of what she was reporting?

A. Either a problem with the nerve roots arising from the spinal cord and exiting through the spine, to go down the arms and subserve those functions, or of the spinal cord itself.

Q. Is that, in terms that are used in your profession, either radiculopathy or myelopathy?

A. Yes.

Q. radiculopathy would be - - -?

A. Refers to the nerve root problem, and myelopathy refers to a spinal cord problem.

Q. At this stage are you able to say which would be involved?

A. No.

Q. All right then. Carry on with what you observed and spoke about with her?

A. Well, we discussed what had been done to that date, and I mentioned another, that she had a variety conservative measures, those will generally include rest, analgesics, painkillers, anti-inflammatory agents and often physiotherapy-type interventions. And unfortunately such interventions as she had had, had not solved the problem.

Q. Okay. So at the bottom of the page you did an examination. And what did you do in the way of an examination?

A. Well, I assessed the range of motion of her cervical spine. I inspected it and palpated it for areas of tenderness or sensitivity or deformity, and found that her range of motion was mildly reduced. That's a non-specific finding that can occur in any situation that is painful in the neck. On examination, I was concerned about some weakness of the finger extensors of the right hand, those are the muscles that allow you to straighten your fingers out and maintain them in a straightened out against resistance if the examiner is pushing on them.

Q. What's the significance of that?

A. The significance of that is that that implies some neurologic involvement, not simply mechanical pain arising from structures in the neck. To effect the muscles in the hand and cause some weakness, there would have to be nerve or nerve root involvement, or spinal cord involvement.

She had, I note from the report, she had had electrical studies of nerve function and muscle function, which had shown a very mild carpal tunnel syndrome, which can cause numbness and tingling in the hands independently of any other condition, and can cause some weakness in the hands, but not this.

Q. Why is that?

A. Because the finger extensors - - a carpal tunnel syndrome involves pressure on the media nerve that goes to some muscles, particularly in the region of the thumb at the wrist. The nerves that goes to the muscles that extend the fingers, those muscles are in fact in the forearm, and the nerves are in the forearm, and they are going to be affected by pressure on the media nerve in the carpal tunnel.

So, as I noted in my report, those EMG findings did not account for the symptoms; in fact, did not disclose signs of a more proximal nerve problem.

Q. When you use the term "proximal", what do you mean?

A. Closer to the neck.

Q. All right?

A. But it must also be recognized, but the ...

Q. When you're talking about that test, is that the Hoffman reflex test that you're describing?

A. No. The EMG I was referring to was a test carried out by another physician at the rehabilitation centre, and the results of which I think had been reported to me in the referral note by Dr. Vandewaetere.

Q. I see. What I mean though, you said you did a test of her pushing on her fingers?

A. No, that's a test of motor strength. The Hoffman reflex test is a different test. That involves observing the response of the fingers to sudden forced extension of the tip of the finger; basically involves having the patient relax

their hand and flicking one of their fingers upwards so that it stretches the muscles and tendons in the finger. That can initiate a reflex response which causes the fingers to flex. In normal circumstances there are suppressive pathways that suppress that particular reflects, so a normal person doesn't have a Hoffman reflex.

The presence of a Hoffman reflex suggests what is termed medically an upper motor neuron lesion. What that means, simply, is something going wrong either in the spinal cord or in the brain, not in the nerve itself.

Q. That would mean more myelopathy as opposed to radiculopathy?

A. That's correct. So when you see a Hoffman reflex, you become concerned that there may be spinal cord involvement. I would note at this point that the response was equivocal, that's because it was rather weak, and it is well described as some people who are normal have weakly positive responses in the absence of a clear pathologic process.

Q. When you use the term "equivocal" what did you mean?

A. What I mean by that is that her response was weakly positive. So that I was at that point uncertain of its significant, I was uncertain whether it simply represented a variant of normal, or whether it actually represented an early sign of spinal cord dysfunction.

Q. And you indicated as well in your report that the reflexes were 2 plus bilaterally and symmetrically; is there any significance to that?

A. That's normal.

Q. In terms of doing that test, why would you be doing that?

A. In terms of doing those reflex tests, again it's part of an evaluation of the nervous system, of her neurologic function. A usual sort of neurologic exam involves examination of the sensory system, looking for areas of numbness and altered sensation, an examination of the motor system looking for altered strength or tone in the muscles or muscle bulk. And also, an examination of the reflexes. A nerve root damage for example can result in reduction of the reflexes. In contrast, a spinal cord injury can result in an abnormal increase in the reflexes.

[117] Dr. Lesiuk was asked to comment on an x-ray taken in September 1995 (Exhibit 6, Tab 2), and on an MRI taken on July 27, 1996 (Exhibit 6, Tab 3). He explained that on the x-ray, the

report notes a cervical lordosis or “a normal gentle C-shape curve of the spine that we all ought to have normally”. He testified that the straightening of the spine is a non-specific finding. It is typically found in patient having cervical spine pain of any cause. It often results in narrowing and he noted that the radiologist had described, “a little degenerative narrowing of the C-6 C-7 disc. He commented that this was fairly common for Ms. Monks’ age group”.

[118] He then explained a second x-ray taken in 1995 (Exhibit 6, Tab 2). He stated that two years after the 1993 x-ray, there was a little bit of narrowing between C5-6 disc space, as well as C6-7 disc space. He told the Court the difference between an x-ray and an MRI.

A plain x-ray is a two dimensional rendering of basically the bony structures in the neck; you cannot see the nerves; you cannot actually see the discs; you can make some very crude conclusions about the state of the discs, by evaluating the gaps between the bones where the discs lie. You can’t see the ligaments, and you can’t really see any of the other non-bony tissues. So, in comparison with an MRI, an x-ray examination is a very limited one.

The MRI will show you all of the things I’ve mentioned above; it will basically show all of the anatomic structures of the neck; it will show the spinal cord; it will show the cushioning fluid around the spinal cord; it will show the leathering covering, called the dura matter that encases the spinal cord and the nerve roots; it will show all of the bony structure; it will show the ligaments; it will show the joints; it will show the surrounding muscles; it will show the nerves as they arise from the spinal cords and exit through the channels in the bone.

[119] Throughout their reports, the experts referred to disc herniation and protrusion and how that condition can affect Ms. Monks’ spinal cord. The definitions given by Dr. Lesiuk are necessary to understand the findings of the radiologists after reading the x-rays and the MRIs. The elaborate explanations that follow in the next ten pages are needed to understand the written reports filed with the Court and to understand the oral evidence of the doctors who testified.

[120] Dr. Lesiuk explained the findings recorded on the radiologist report (Exhibit 6, Tab 2):

And then they described the changes. The most significant changes alluded to in the bottom paragraph, at C5-6, a moderate left paracentral disk herniation is present, which produces moderate central and left-sided cord compression. They note that the neural foramina - - those are the side channels in the bone

that the nerves pass out after they've arisen from the spinal cord, they pass out through side channels in the bone called the neural foramina - - they've noted that those are widely open, but they're patent, they're fine.

Q. Now tell us the significance of what you just read at C5-6?

A. That there is a significant structural abnormality now that's been documented.

Q. What is a paracentral disk herniation?

A. Well, disk herniations, as you remember when we were having our general discussion of the degenerative disk disease, I said that a disk has an outer fibrous retaining ring and an inner gelatinous core; and that if there was enough wear or damage to the outer fibrous retaining ring, the weight of the head pressing down on the inner gelatinous core, and that force being transmitted to the weak and the retraining ring, could cause the weak and retaining ring to bulge out.

Typically the area where the retaining ring becomes damaged is at the back and slightly to one side or the other, that is where structurally the normal retraining ring is most susceptible to injury.

So a disk protrusion usually takes place slightly to one side of the middle or to the other. It is less common, although certainly not rare, for a disk to occur completely centrally, for the bulge to occur right in the middle, not favouring - - favouring one side or the other.

Q. Is that what "paracentral" means?

A. Yes.

Q. When you characterize it as "moderate", typically what are the ranges that you look at when you look at MRIs?

A. Well, obviously, this is only semi-quantitative, but it is also typical of a radiologist's or a clinician's interpretations, that they will divide the disks herniations into mild, moderate or severe. There are clinical implications for that, in that a mild disk protrusion, one that is classified as "mild" is often not obviously compromising the neural elements, in other words the spinal cord or the nerve roots.

So when a radiologist reports on an MRI "mild disk protrusion", he's often implying that the probability that this is causing significant problems is low.

When he reports “moderate”, he is now saying that the probability that this could cause problems is higher, although he’s still implying, I think, that there is an element of uncertainty in that.

When he gets to the point that it’s “severe”, there is also the implication with that clinical problems are likely to be associated with that. That’s a rough guide and I think something of an oversimplification, but I think it does have the gist of it.

Q. When it says “a moderate left paracentral”, what does that mean?

A. Well, if it’s to the left side of centre you would expect that for the most part it would affect things on the left, in other words, it’s more likely to affect a nerve root on the left hand side than one on the right hand side; it is more likely to affect the spinal cord on its left hand side than the right hand side, so it’s likely to cause weakness more on the left than the right.

Unfortunately the situation gets more complicated because some of the sensory pathways cross over at that level, so it’s also possible for it to produce sensory findings, particularly in the area of pain and temperature on the right hand side of the body.

Q. And when it means left paracentral, does that mean it’s central but with a bit more to the left?

A. Yes. It generally means that it is more or mostly to the left, may reach the middle, but doesn’t particularly go over to the right side.

Q. All right. And so then going on and saying “produces moderate central and left-side cord compression”, what does that mean?

A. First, that it contacts the cord and, in the moderate range, has had some effect on the spinal cord in that the spinal cord has changed its shape in some way, to adapt to this narrow canal or rotated in some way, which is another way the spinal cord can adapt to this. It implies the potential for causing spinal cord injury is there. But again the implication of the term “moderate” is that the degree of compromise of the bony channel, the degree of changes taking place there are not such that they would certainly be associated with clinical problems.

Q. I’m just going to ask you to stop for a second, because you’re saying at that point it implies contact between the disk protrusion and the cord itself.

A. Yes.

Q. Typically, what will you find inside the canal where the spinal cord actually resides?

A. Okay. Well, typically, what you would find, of course, is centrally within the canal is the spinal cord itself. It is normally surrounded by a protective cushion of watery fluid called the cerebral spinal fluid. Typically there will be a couple millimetres, at least, in front of and behind the spinal cord before you get to the leathery protective covering that encases and retains that fluid, which is called the dura matter, which is like a layer of thin leather surrounding this spinal cord and this fluid.

Then surrounding the dura matter, typically there will be small amounts of fatty tissue there, which also provides a bit of a cushion and some additional spare room. We are built with some redundancy and some spare room in this area normally. Then surrounding that would be the bones and the ligaments that form the boundaries of the spinal canal itself.

Q. So when you say there could be contact between the disks protrusion and the cord, what portions, or what portion of what you just described would actually be in contact?

A. Well, the disk protrusion would have to be sufficiently large to displace the small amount of fat lying between the disk and the leathery covering, to indent the leathery covering, to push it up against the spinal cord, to displace the cushioning fluid that surrounds the spinal cord and actually cause - - the disk doesn't actually itself come in contact with the spinal cord, it pushes the leathery covering, which remains between it and the spinal cord into contact with the spinal cord.

Q. And then if we can just go back up and look at what happens at the C4-C5, and you can tell us what the significance of what that discloses?

A. Well, again, it now shows that there has been some significant anatomic developments here that are not normal, they're not as severe as the ones at C5-C6 and are less likely to be clinically significant because the terms used are generally mild. So that there is a broad disk bulge. The fact that the radiologist has used the term "bulge" rather than herniation, there is also some implication that it is more mild; people often use the term "bulge" rather than herniation when they're referring to a more modest amount of displacement of the disk from its normal position.

Q. When it says a "broad disk bulge", what does that tell us?

A. It means that it's rather more diffuse. The implication is it's on the right side, in the middle, on the left side somewhat as well. It's its broadness means it encompasses the entire spinal canal from side to side, okay. And the fact is also mentioned that it's slightly worse on the right than the left.

Q. Okay. And that's where it says this is somewhat more prominent to the right of the mid-line?

A. Yes.

Q. And producing mild right-sided cord compression?

A. Yes.

Q. Given what you told us about moderate cord compression at C5-C6 level, what does the radiologist mean when he or she says that it's a mild right-sided cord compression?

A. It often means it's still displaced those other structures as I described for the moderate one, but the effect on the spinal cord in terms of causing it to rotate or change its shape or that is minimal, it is barely in contact with it.

Q. It also makes mention of an osteophytic bar?

A. Yes.

Q. You have told us what osteophytes are. What is meant by an osteophytic bar?

A. Okay. This reflects part of the broader nature of degenerative disease, okay. It implies a bony change, when we were discussing a general description of a degenerative spondylosis, I said degenerative changes in the disks were one aspect of it, and that this repair process that causes the body to attempt to lay down extra bony material was also part of it.

The disk herniations can come and go quite quickly because you're dealing with a soft disk material that can shrivel up, lose water content and shrink back in, or, conversely, in the setting of stress on the disk or some trauma that damages the disk, the disk can bulge out quite suddenly, it can be quite an acute event.

An osteophytic bar implies a protrusion of bony-type tissue. That doesn't occur suddenly, okay, that gets built up over some period of time, generally an extended period of time, months to years. And the other aspect of it is that an

osteophytic bar or spondylotic bar is, to the extent that it is bony, because often the term “spondylotic bar” encompasses both an element of soft disk protrusion as well as this laying down of bone. The soft component of it can fluctuate quite relatively quickly. But the bony component is not going to go away. Once it’s there, it’s generally there for good.

Q. Where would this osteophytic bar - - or where was it present when he says it’s present, where is it actually physically - - where was it located?

A. In the same place as the disk. The bar is generally at the upper end or the lower edge of the disk.

Q. Okay. And again, in looking at this, what is the significance, if any, of the presence of this bulge from a clinical perspective?

A. Well, as I say, it’s less likely to be causing significant clinical symptoms than the more substantial protrusion at C5-6. It is an indicator of more degenerative change, and it does raise the issue in comparison with the old plain radiographs that things have progressed and the possibility that they have progressed rather more quick quickly than one would typically anticipate, although I would emphasize that those comparisons between an MRI and plain radiographs, in evaluating the degree of change, must be quite limited and must be interpreted with caution.

Q. All right. So then it goes on to say that the neural foramina at the C4-C5 level show mild left and mild to moderate right foraminal stenosis. Can you explain what that means?

A. Yes. In addition to the encroachment on the main spinal canal where the spinal cord lies, we’re now talking about encroachment on the side channels, where the nerve roots that are coming off the spinal cord exit through the bony canal. So when they’re talking about stenosis, or narrowing - - for stenosis is simply a medical term for narrowing - - of those channels, they’re implying the potential to put pressure on the nerve roots. And again, the qualifying nature of the terms “mild”, “moderate” imply that a mild narrowing has some possibility of putting pressure on the nerve route but is unlikely.

“Moderate” carries a greater probability of putting pressure on the nerve root but is still quite uncertain. The typical nerve root channel in its normal state is several times larger than the nerve root itself. So you can sustain some narrowing, and there is still enough room for the nerve root. It generally takes fairly substantial narrowing in the moderate to severe range before you actually start to get ongoing pressure on the nerve root.

Q. When the radiologist uses the terms “mild” and “moderate” and “severe” in the context of stenosis of the foramina, is there a way to sort of interpret what they’re saying?

A. Well, as I say, again, it tends to be that this is something of an oversimplification because they are after all referring to the structural issues. But there is something of an implication there that if it’s mild it is unlikely to be causing symptoms. If it’s moderate, it may be causing symptoms but it’s uncertain. If it’s severe, it is quite likely to be causing symptoms.

Q. What about mild to moderate, we’ve got a half-way measure there?

A. Well, then, I think that refers to the structural abnormality that the radiologist is saying it’s on the edge of what he would normally describe as mild and what he would normally describe as moderate in terms of the structural change. He’s waffling a bit in that area.

Q. And then it says from uncontrovertebral [sic] osteophytes. And what does that mean?

A. Uncovertebral osteophytes.

Q. Excuse me.

A. What they’re referring to, in simple terms, is the location of the osteophytes in the area of the disk edge, not in the midline at the back but off to one side where they typically occur, the osteophytes forming in this so-called uncovertebral region. The usual area that these osteophytes do form and they’re at the disk edge off to one side or the other.

Q. Now you indicated that you had actually seen these films?

A. Yes.

Q. In terms of the report that we’ve just gone through, from your own analysis of the films, does that report accurately reflect what you saw?

A. Yes, I think it does. In my own notes there, I’ve noted that over all there is moderate degenerative change affecting the lower cervical segments with disk space narrowing, disk bulging at C4-5, C5-6 and C6-7 associated with osteophyte formation or osteophytosis and spondylotic bar formulation, which are synonyms for the terms the radiologist has used. And I have also noted that there is some enlargement of the joints and ligaments in that area. As a result of this, there is mild narrowing, in my view, of the spinal canal at C4-5, and

mild narrowing in fact at C6-7 due to these changes. But more substantive narrowing at C5-6.

Q. In looking at that paragraph, what significance does all of that tell you at that point?

A. Well, it leads to my concluding paragraph that, based on that combination of seeing those changes and seeing Ms. Monks, hearing her complaints, and having examined her and having had this equivocal Hoffman reflex and this weakness of the finger extensors, raised my concern that she possibly has some problem going on with her nerve root or her spinal cord, that again I'm concerned about those possibilities. And that my feeling is that we should repeat the MR images to see whether there has been any progression and whether or not we can be anymore definitive about these findings, which, because they're in the moderate range, could be causing the problem. It's not for certain that there is a problem. And my clinical findings are somewhat equivocal in that there could be a problem with the nerve roots or spinal cord here, but I'm not convinced that these findings are solid and incontrovertible.

So as a result of that, I asked for another MRI and I asked for a second opinion from a neurologist.

Q. I'm going to stop you there for a second. Because in your report you said the MR images were taken last July and her symptoms have become more prominent since then. What does that tell you, or what were you thinking about in terms of - - -

A. If the symptoms are becoming more prominent, it means that the structural problem, of course, may be coming more prominent. In fact the MRI done, I guess almost a year ago by this point, may not reflect the situation as it exists now.

[121] Having viewed the first MRI, Dr. Lesiuk stated that he was concerned because of the possibility of a nerve root problem or a spinal cord problem. He had decided that a second MRI was required to determine if there was progression in Ms. Monks' symptoms and he also had wanted her to get a second opinion. He could not recall having recommended surgery to her at that point; otherwise, it would have been indicated in his notes.

[122] The next MRI investigation was done on May 25, 1997, and it showed "no internal change". He stated that, reviewing the films for the 1995 and 1997 MRIs, the uncertainty of

recommending surgery, remained. As no structural changes were seen, he felt that the changes, if any, were not severe enough for him to tell Ms. Monks that she needed an operation.

[123] Dr. Lesiuk then interpreted Dr. Bourque's report for the Court (Exhibit 1, Tab G2). Dr. Bourque, in examining Ms. Monks on July 16, 1997, had not found anything definite that would have led him to be certain about a spinal cord or nerve root dysfunction. He had not found significant objective neurologic deficit other than a minor reflex asymmetry. Dr. Lesiuk felt that Dr. Bourque had not found compelling evidence that decompression surgery was warranted and had recommended a conservative approach. Dr. Lesiuk stated: "'conservative' is a nice way of saying not involving surgery".

[124] Dr. Lesiuk described Dr. Bourque as having practised as a neurologist for 15 to 20 years and explained that he is one of the most well respected neurologists in the Ottawa-Neuroscience Community. He is known for his thoroughness and his diagnostic acumen with a particular interest in nerve disorders.

[125] Dr. Lesiuk noted that Dr. Bourque had stated in the same report, that Ms. Monks was well informed and aware that the indications for surgical intervention would be: intractable pain or progressive neurologic deficit, either radiculopathy or myelopathy. Dr. Lesiuk then explained what Ms. Monks was supposed to look for. He stated that intractable pain was "basically severe, persistent, uncontrolled pain, that cannot be managed by activity modifications or medications, and is rendering her life an ongoing misery". The pain would have to be located in a region other than the neck. Dr. Lesiuk was referring to a pain extending in the arms or legs, especially in the arms, and if it started to follow a nerve distribution, that would suggest a compromise of a nerve root.

[126] Regarding what Ms. Monks would be looking for for progressive neurologic deficit, either at the cervical radiculopathy, or progressive myelopathy, Dr. Lesiuk said it would be in the nature of "numbness or tingling in the arms or legs". If it was in the arms, it could arise from a nerve root problem; if it was in the legs that would certainly imply a spinal cord problem and if it was related to the neck, Ms. Monks would experience weakness, or clumsiness, increasing

fatigability, less balance, or difficulty in walking. Dr. Lesiuk also confirmed that the symptoms do not necessarily tell you if it's myelopathy or radiculopathy, as they overlap. Involvement of the arms can occur either through the nerve roots or through the spinal cord. Involvement of the legs would imply either involvement of the spinal cord or involvement of other nerve roots coming out from other levels of the spine, in the lower back.

[127] As there are many references in several medical reports, filed in this trial, referring to urinary urgency and the spinal cord structure, I have quoted Dr. Lesiuk's explanations on these matters.

Q. One thing he mentions (referring to Dr. Bourque's report), and he did mention it in his report, is this issue of the urinary urgency?

A. Uh'hmm.

Q. What's the significance of that?

A. Well, the potential significance is that, of course, the spinal cord not only carries messages to the skin and muscles providing for control of the muscles and sensation in the skin, but also carries messages that go to internal organs, including the bowel and bladder and the muscular sphincters [sphincters] that control them, so that allow us to have normal bowel movements and allow us to empty our bladders normally.

When you get systems of urinary urgency, it raises the possibility that there is some neurologic involvement, particularly in the setting of a cervical spine problem of the spinal cord giving rise to that. However, in this context, that urinary urgency is not a specific symptom; is not diagnostic. There are many other things that can give rise to urinary urgency, ranging from urinary tract infections to bladder mechanical problems that are of the sort that can be associated with difficult or repeated pregnancies or simply the aging process.

Q. Okay. And the nerves that you just described that deal with, for example the bowel and bladder and many of the organs, what are those called?

A. That aspect of the nervous system, the nerves and their associated ganglia and that that result in those internal controls are referred to as the autonomic nervous system.

Q. That's A-U-T-O-N-O-M-I-C?

A. Yes.

THE COURT: Autonomic what?

MR. CRONYN: Autonomic.

THE COURT: What's the other word? Autonomic what?

A. Autonomic nervous system.

MR. CRONYN:

Q. In terms of how - - just to help out a little bit on that as well. Can you just give us a brief description of the structure of the spinal cord itself as it goes down through the canal in terms of the various nerve bundles and how they're packed?

A. Yes. The spinal cord structure consists of, crudely speaking, a central core of so-called grey matter. Grey matter is where nerve cell bodies lie. White matter is where the actually nerve fibres that connect the cell bodies with each other and with the rest of the body lie. So the spinal cord structure is, although complex, a rather simpler one than the brain; it has a central core of grey matter, which rather has an 8 shape to the central core, and an outer surround of white matter.

And as the spinal cord is in essence, crudely speaking, a communication cable. The dominant feature of the spinal cord is the white matter, the communication cables itself. Within the white matter, like most things in the nervous system, everything is right for left, back for front and upside down. So that all the sensory inputs to the spinal cord come into the back, and all the motor inputs - - again, this is a bit of an oversimplification - - go out the front.

The fibres that have to do with fine touch and that are located in the white matter at the back. The motor fibres and the fibres that go to the autonomic nervous system, to the internal fibres, are located at the side. And fibres that have to do with signalling for pain and temperature are at the front.

And the fibres are layered according to what part of the body they go to. As you're travelling down in the spinal cord, the fibres that are destined to go to nerves that come out the cervical region tend to be more towards the centre. And then the ones that go to the thoracic region are layered on top of that, outside of that. Then the ones that go to the lumbar region are outside of that; and the ones that go to the sacral region are outside of that.

Q. Is that a fairly good; I won't take you any further than that. But just to give a sense of how the bundles move?

THE COURT: It helps, yes.

[128] Dr. Lesiuk measured Ms. Monks' spinal canal diameter reduction at 6 mm. He explained that it was a significant reduction, as a canal diameter is typically 12 to 13 mm. At 10 mm., it is an abnormally narrow canal, a stenotic canal. Dr. Lesiuk characterized Ms. Monks' spinal cord canal and advice he gave her in 1997:

Q. Okay. How do you characterize that level of compression?

A. I would characterize it as moderate as well; it's getting to the upper end of the moderate range, but it still falls within that range in my view.

Q. So that when you say that there's some abnormal compression, what is the implication of that for the patient?

A. Well, two things: Firstly, ongoing substantive pressure on the spinal cord can lead over the medium to long term to progressive spinal cord injury. When I use the term "injury", I'm using it in the broad sense of the word - - of damage.

Q. Okay.

A. So that ongoing pressure on the spinal cord has the potential to lead to damage over the medium to long term.

And secondly, it places her at increased risk should she sustain any significant trauma to the spine. Because when we're involved in some sort of trauma that involves the spine, there is dynamic motion, in other words the bones can potentially shift on each other. That is one of the reasons that we have reserve room, I think, to allow for that, so that people can have these shifts that are going to occur with varying degrees of trauma, and not end up inevitably with a spinal cord injury. That's why we're not all quadriplegic after every whiplash injury is because we have reserve room.

If that reserve room disappears progressively, then a person is rendered at increased susceptibility to a spinal cord injury if they're involved in a trauma.

Q. And I know that later in a report you address this issue. But did you talk about that issue with Suzanne Monks in August of 1997?

A. Yes. By this point I would have discussed with her.

Q. What did you tell her in terms of your recollections at that time?

A. Well, - - -

Q. Just about that particular aspect?

A. Yes. My advice - - I would have advised her of the increased risks she's at if she's involved in that sort of trauma. I would have said that the sort of injuries that are worst for this sort of thing are those in which there is forcible extension of the neck, in other words your neck is snapped back for some reason. And the reason for that is the ligaments that line the back part of the spinal canal will buckle inwards when your neck is forced back. So that if you already have no spare room, when those ligaments buckle inwards, they are literally going to punch the spinal cord.

So I would have advised her that she should try to avoid putting herself in a situation where that sort of thing happens. That in some respects is difficult advice to be comprehensive about. So I would typically recommend, you know, 'You shouldn't be going downhill skiing, horseback riding or involved in contact sports where you're likely to end up taking a tumble'.

Having said that, unfortunately, the most likely way that a person is going to sustain this sort of injury in modern society is in some sort of motor vehicle accident where you get rear-ended, and there is not much you can do about that.

Q. Now you gone on and say appropriate management decisions are not entirely forward. What do you mean by that?

A. I think that's bit of a typo. I think it's "straight forward".

Q. So what do [you] mean by it?

A. Well there is this concern that she has this reduced canal diameter, and she's at increased risk. And the risk of that, that hypothetical risk if she's involved in some sort of trauma, has to be taken into consideration, but also has to be balanced against the risk of undertaking surgery. We're now talking about a spinal decompression, which in and of itself has some significant

risks associated with it, including the risk of exacerbating it or producing a spinal cord injury.

Q. Let me stop there. You're talking about the prospect of spinal cord decompressive surgery?

A. Yes. Well, that would be the treatment.

[129] At this point in his evidence, Dr. Lesiuk explained the different types of surgeries that are possible to decompress the canal diameter. He concluded that the surgery is very risky and although the pressure is relieved, the canal diameter may not be restored to its normal state. Depending on the surgical approach taken, the patient's spine resistance to trauma may not be restored or maintained at a normal state. It becomes dangerous for a patient to be involved in some sort of trauma, following surgery.

[130] Dr. Lesiuk was asked about the third paragraph of his letter to Dr. Vandewaetere, dated April 6, 1998 (Exhibit 1, Tab A-3). The Court heard from Dr. Vandewaetere that she had encouraged Ms. Monks to ask Dr. Lesiuk all the questions that bothered her concerning her future health. Dr. Lesiuk had answered 13 questions for Ms. Monks and he was reporting to her family doctor. On her condition, Dr. Lesiuk had written:

There has not been a great deal of clinical change but she does report a number of features which raise the possibility of subtle deterioration in her overall status. She reports increased fatigability of the right upper extremity, as well as increased muscular cramps/spasms in the proximal left upper extremity particularly with activity. She finds that her legs fatigue more easily on her with long periods of walking, but she is still able to walk for over an hour without being compelled to stop. She has noted some subtle urinary urgency. Certainly, her most prominent complaint is that the right upper extremity fatigues more easily with repetitive tasks.

[131] Dr. Lesiuk explained what he meant by a number of Ms. Monks' features that raised the possibility of a subtle deterioration. The report of increased fatigability in the upper right extremity did not enable him to determine if that complaint meant Ms. Monks was in the category of hard radiculopathic or myelopathic symptom. Ms. Monks had asked Dr. Lesiuk if surgery was recommended and he testified that, in April 1998, despite the many months that had

gone by since his August 1997 report, he felt that no change had come about in Ms. Monks' condition that would have led him to recommend surgery.

[132] Then, Dr. Lesiuk had been asked (Question no. 7) to ascribe the role of the 1995 motor vehicle accident in the changes seen on the MRIs and in her symptoms. He had written:

The cervical spine films taken in 1993 did indeed demonstrate minimal degenerative changes. The present difficulties are the result of a degenerative condition and there is no one feature of the present MRI that can be identified as being the obvious result of trauma. However, in view of the onset of symptoms, in relation to the second MVA and the rather brisk pace of accumulation of degenerative changes which has been documented since (a phenomenon which has been described in association with trauma), on balance, I think it is reasonable to ascribe some significant role to the trauma in the development of the present situation.

[133] He testified that there was nothing to this point in the MRIs that could be identified as the result of a trauma. All the changes to this point had been degenerative in nature and could have occurred in the absence of trauma. Then, he stated that Ms. Monks had another accident in 1995, and developed immediate significant symptoms, including signs of transient spinal cord dysfunction and the rather brisk pace of degenerative changes, following that accident. At trial, Dr. Lesiuk remained resolute in his conclusion that it was reasonable to ascribe some significant role to the trauma in the development of Ms. Monks' present condition.

[134] Dr. Lesiuk went on to say that there was no emergency for Ms. Monks, in April 1998, and had she undergone surgery at that time, it would have been elective surgery.

[135] Then came Ms. Monks' question 11:

Question 11: Without surgery, what disease progression would you anticipate and what additional symptoms would you anticipate would affect my level of functioning?

Answer: Disease progression in the setting of spinal stenosis is difficult to predict with certainty. In the absence of clear myelopathic features, clinical symptom progression may occur in approximately 40% of individuals. Once myelopathic features appear the tendency towards to symptom progression is

substantially higher. Symptom progression may take the form of pain and/or sensory motor loss.

[136] Dr. Lesiuk clarified that he was referring to patients who do not have myelopathic features and proceed to surgery on the basis of spinal stenosis. 60% of them will stabilize without surgery and 40% will go on to have surgery. Those patients who develop myelopathic features go on to have surgery. At that point in time (April 6, 1998), Dr. Lesiuk felt that Ms. Monks' symptoms would stabilize at some level and that she would not need surgery; she would be in the 60% category.

[137] I have reproduced Ms. Monks' question 12:

Question 12: You have recommended that I stop participating in certain sports activities such as horseback riding, downhill skiing, skating and competitive sports such as softball. Could you comment on your reasons for the request?

Answer: I have indicated that the size of the cervical spinal canal is significantly narrowed, down to about 6.0 mm. at one level. This distorts the shape of the spinal cord somewhat and leaves absolutely no reserve room. Sudden extensions of the neck can further compromise the canal diameter due to inward buckling of posterior ligaments. This can lead to sudden and potentially irreversible spinal cord injury. For this reason, I have recommended the avoidance of activities that place one at risk for falls and related neck extension injuries.

[138] As this matter was addressed earlier in this judgment by Dr. Lesiuk, no further explanation is required. Dr. Lesiuk went on to testify that the following ailments had likely nothing to do at that point with Ms. Monks' spinal condition:

- Ms. Monks' report of July 22, 1998, to Dr. Vandewaetere concerning her vertigo ailment;
- Ms. Monks' urine incontinence and sometimes retention (Ms. Monks was referred to an urologist, Dr. Walsh).

[139] During cross-examination, Dr. Lesiuk was referred to the New Patient Registration form that Ms. Monks had completed on her first visit to his office, in 1997. (Exhibit 4, Tab 5, page 95). She had listed the medication that she was taking in 1997 and Dr. Lesiuk told the Court that Anaprox is a non-steroidal anti-inflammatory agent. He also stated that Advil is another popular

trade name for that. Haprosyn was another trade name for that drug and Dr. Lesiuk testified he believed that at the time it could only be obtained by prescription. Altace was listed also and it is an anti-hypertensive blood pressure medication to treat high blood pressure. Fional completed Ms. Monks' medication list and Dr. Lesiuk stated it is a drug prescribed for headache conditions and migraines. He could not recall if she had stated, during her visit, whether or not she was still taking that medication.

[140] Ms. Monks' past history was read to Dr. Lesiuk, who stated that for the most part he was unaware of her slip and falls, her fracture of the tibia, fibula and left ankle, and the tearing of her ligaments in both her right and left knee. He stated that he remembered she had had a fall and landed on her coccyx. He also recalled that she had suffered with problems in her lumbar spine. He answered that it would have been interesting to know all of Ms. Monks' past medical history and he added: "I don't know that any critical point turns on it".

[141] Dr. Lesiuk disagreed with Dr. Hansebout that the mild narrowing of disc spaces shown on Ms. Monks' 1993 and 1995 x-rays depicted the back condition of a much older person. Dr. Lesiuk stated that minor degenerative changes are quite common in middle aged persons.

[142] As Dr. Lesiuk had last seen Ms. Monks between her April 1998 visit to his office and her post 1998 motor vehicle accident visit, in January 1999, he was asked if a fall by Ms. Monks, while suffering from vertigo, could cause "a signal cord change". Dr. Lesiuk read from his report of January 28, 1999 that "she remained relatively clinically stable until she was involved in a motor vehicle accident that took place December 23, 1998.

[143] Defence counsel pointed out that Dr. Vandewaetere's notes of June 19, 1998 stated:

"Her life is restricted. Can't type. Difficulty with mouse. Can't do sports. Can't even cake decorate".

[144] She asked Dr. Lesiuk why he would attribute these functional changes to the 1998 motor vehicle accident when they existed prior to Ms. Monks' accident in 1998. Dr. Lesiuk replied that all Dr. Vandewaetere had done was to indicate the clinical impression Ms. Monks gave at the time. Nothing could have been gained by receiving that information at the time. He

stated that what Ms. Monks told him in January of 1999 on her level of symptoms was that the level of symptoms had become significantly and substantively worse after the 1998 motor vehicle accident. He also stated that, after the 1998 accident, Ms. Monks' symptoms were not different in distribution or character, but that they were more severe. Dr. Lesiuk concluded that there had been a substantive change attendant to that accident.

[145] Finally, Dr. Lesiuk would not agree that urinary retention could only be caused by degenerative disc disease, as it can also be a sign of other things.

Dr. Robert Hansebout – 1995 Motor Vehicle Accident

[146] Dr. Hansebout filed a 49-page report (Exhibit 1, Tab G-9) and gave evidence for the defendant. He summarized at page 47 of his written report what he felt happened to Ms. Monks, following her 1995 motor vehicle accident.

On July 21st, 1995, her car was rear ended by another car, following which she complained of numbness of both arms, followed by neck pain and again a reduced range of neck motion. Subsequently she complained of low back pain and sciatica. X-rays revealed degenerative disc disease in the lumbar region and osteophytes in the lower thoracic region. Her legs gradually became numb. She continued to complain of muscle pain in the neck. She also began to complain of tingling in both arms. Headaches were a problem. She had a skull X-ray which was normal. She also has X-rays of the neck which showed degenerative disc disease at the C 5-6 and C 6-7 level. A year later, she was still complaining of neck pain, limited neck motion and tingling in her arms in addition to left foot and ankle pain. An MRI of the cervical spine was done in July, 1996, which showed degenerative disc disease at the C 5-6 and C 6-7 levels with osteophytes and foraminal stenosis. Subsequently, her right hand weakened and she had difficulty completing an examination for an insurance adjuster's license. As a consequence of her problem, she became depressed and tired. Nevertheless, according to 1 source, she was able to bill \$280,000. in salary and commissions in 1996 and did not appear to others to have problems at work.

On January 9th, 1997, she slipped, and fell against the door of her as SUV and thereafter complained of a sore tailbone. A lumbar spine X-ray series showed scoliosis, osteophytes at various levels in the lumbar spine and also in the lower thoracic spine. An X-ray of the coccyx was normal.

In view of her many problems, she was sent by her family physician to see Dr. Lesiuk, neurosurgeon on March 13, 1997. He diagnosed low back pain, a reduced range of neck motion and also weakness and clumsiness of the right hand. By that time she complained of tired arms and depression. Another cervical spine X-ray was done in view of the fact she had continuing neck pain, which suggested narrowing of the spinal canal in the neck. The company for which she worked sold out and therefore in 1997, she opened her own business. This was called Polaris Consultants. She subsequently made an arrangement with Dynamic Rehabilitation to handle cases in Ottawa.

Dr. Lesiuk felt that she had a narrow spinal canal and the beginnings of cervical spondylosis with myelopathy. He sent her for a second opinion to Dr. Bourque who indicated she complained of tired arms and tired legs. However, he did not find enough neurological abnormalities to recommend surgery. Accordingly, for a third opinion Dr. Lesiuk referred the patient to Dr. Fehlings, neurosurgeon [neurosurgeon] in Toronto. Dr. Fehlings' secretary gave an appointment date and passed this on by letter to Dr. Lesiuk whose staff apparently did not advise the patient. Meanwhile, the patient continued to deteriorate from the standpoint of tired extremities and also she developed urinary urgency.

[147] Dr. Hansebout testified that the 1995 accident caused Ms. Monks' neck to go backwards and extend. As she already had degenerative disease of the spine and her neck extended backwards, the nerve roots can stretch very easily. He stated that he was referring to the nerves that go through the intervertebral foramina to the muscles. He surmised that the result the accident produced immediately came out in Ms. Monks' complaint at the time.

And the stretching, and possibly the stretching of the spinal cord over an osteophyte in the back, caused damage, a little bit of damage, possibly in the spinal cord, but probably in the nerve roots going out from that level, and that caused tingling to occur in her arms and numbness. And that lasted for about 15 minutes. And it just means that that accident did probably cause some damage to nervous structures, notably the nerves coming off the spinal cord.

Q. Now does that tell you anything with respect to the narrowness of the spinal cord at all?

A. Well, yes. Because if the spinal canal is not narrow, and the neck extends, there is nothing for the spinal cord to stretch up against, so that you wouldn't get this situation normally, in a normal person. Usually it means that there is degenerative change already in the spine and some narrowing.

[148] Reviewing the September 19, 1995 x-ray (Exhibit G, Tab 2), Dr. Hansebout referred to a narrowing of C5-6 and C6-7 disc spaces, with minimal marginal osteophytes, in keeping with mild degenerative disc disease. He found that this result showed progression if compared to the 1993 x-ray results. Dr. Hansebout then commented on the July 27, 1996 MRI investigation, as follows:

A. July 27, 1996; it was done at the Ottawa General Hospital, and it's a magnetic resonance imaging scan of the neck. This shows, at the C4-5 level, between the 4th and 5th bones in the neck, the disk is bulging out broadly, that means it's a wide bulging, with an osteophytic bar present. That means there is a bar of bone sticking near the disk protruding backward. And it's more prominent to the right of the mid-line. And, not only that, but it's already starting to press on the patient's spinal cord.

Now, what it also shows is that the foramina, that is the little holes through which the nerves go out to the muscles and skin, that they're already getting smaller due to the bone growth degenerating and making them smaller, which means that the nerves - - it may be that they're going to get squeezed soon, or are getting squeezed already.

And then at the level below, there's also a disk protruding and it's sticking out towards the left side, and that's compressing the other side, the left side of the spinal cord. But the little holes through which the nerve passed out or okay on that level.

At the lowest level in the neck, the C6-7 level, there is also a third disk bulge, also with osteophytes degenerated boney [bony] spicules, and this is compressing on the cord broadly. That's quite a lot of problems for a woman I think who's, what, 40, 41, that's a lot of problems. That's pretty significant stenosis and spinal degeneration already.

Q. Now does that reflect a change from the previous x-ray that we looked at, the September 95?

A. Yes, because now three levels are involved. The first x-ray in 93 was C6-7. The second x-ray, in 95 I think, was C5-6 and C6-7. Now we've got C4-5 involved, C5-6 and C6-7. That's three levels involved. And there are osteophytes now at two levels. That represents progression, at least in the bone.

Q. You described the osteophyte at C4-5, it has an osteophytic bar. Is that indication of increased growth in the osteophytes, the fact that it's a bar?

A. What it means is, it's not just a little spicule sticking out, it's maybe a couple of centimetres wide. It's a big thing. It's not just a little point sticking out of bone, it's a broad thing.

Q. And that's a change from what we saw in September of 1995?

A. Yes.

Q. If I could ask you, please, to provide the witness with Exhibit 12. And if I could ask you to turn to tab 9 of Exhibit 12. (January 10, 1997 MRI)

A. I have it.

Q. Could you please advise the court what this particular x-ray is telling us and how it is relevant?

A. Well, it says there is as mild scoliosis, in other words, the spine, if you look from the front at the patient, it's bending, it's bending slightly towards the left side. And the spine is straightening, which may mean some muscle spasm in the back. And you also - - it looks like there is boney [bony] spurring, and that's another name for osteophytes at the L1-2, that's between the first and second lumbar bones, between the second and third lumbar bones, and between the third and fourth disks. But the disk spaces aren't narrow.

What's also very unusual, extremely unusual, is that she's got an osteophyte at the D11-12 level, that's thoracic spine. That's very, very unusual. Maybe you see one of those in a neurosurgical practice every two or three years. It's very unusual. She's got one of those, too. What that means, since there is still spinal cord up there, that could also be pressing on the spinal cord, that is at the D11-12 level.

But in the lumbar levels there is no spinal cord, it's just a bunch of nerves coming from the bottom of the spinal cord. So it's called the cauda equina, which is Latin for horse's tail. It means the nerves coming from the bottom of the spinal cord.

[149] Dr. Hansebout was then asked to explain the term "facet and ligamentous hypertrophy" referred to in Dr. Lesiuk's letter of May 13, 1997 (Exhibit 1, Tab A-2). He answered as follows:

A. Well it's facet, F-A-C-E-T. And where the bones join together, that's facet it's an upper and a lower facet forming a joint. So that's a facet.

As I mentioned before, those facets, where the were the vertebrae slide, one on top of the other, are getting arthritic and they're getting larger. So hypertrophy means enlargement. So those joints are showing increasing arthritis.

But at the back of the spinal cord there is a ligament called the ligamentum flavum, F-L-A-V-U-M, the yellow ligament, which defines the back of the spinal canal. And this one also has gotten a lot bigger, which is a change that we see in aging. So that it's very significant. What this means is that it makes the spinal canal even smaller. So the spinal stenosis is increased as a consequence of those changes.

[150] Dr. Hansebout was also asked to explain Dr. Lesiuk's last paragraph of the same letter, raising the possibility of an early cervical myelopathy. Dr. Hansebout indicated further deterioration progression in Ms. Monks' spine:

Q. Now Dr. Lesiuk, in the last paragraph, indicates:

"Certainly on the basis of these images and the clinical findings, the possibility of an early cervical myelopathy is raised."

What is your opinion with respect to - - -

A. This is what I have just explained, that the Hoffman responses, plus the x-ray findings of very narrow spinal canal, to me - - and in addition to her complaints of clumsiness in the right hand, to a lesser extent in the left hand, means that her spinal cord is getting early compression already, it's getting squeezed, which means that the blood supply to the spinal cord is reduced, and it's not working as well as it should be.

In the early stages that's all you get, that things aren't working quite right: maybe your legs are a little weak, maybe your hands are a little weak. Later on you get changes within the spinal cord called myelomalacia, which means that the inner aspect of the spinal cord actually dies, like a stroke. And on the MRI this shows - - it's called increased signal intensity. But already this woman is progressing, both from the standpoint of changes in the spine and clinically.

[151] Dr. Hansebout had this to say concerning Dr. Lesiuk's comment of August 12, 1997 (Exhibit 1, Tab A-1) that the diameter of the canal is reduced at this point to approximately 6 mm.:

A. This is, to a neurosurgeon, I would describe it as shocking. It is shocking, because when the spinal cord is compressed to an anteroposterior, from front to back, diameter of 11 millimetres, we consider it already compressed sufficiently to consider surgery.

This woman has a compression of - - she's got a narrowing of the spinal canal to only 6 millimetres, which means her spinal cord is getting flattened, it's really suffering. This is - - with her complaints of clumsiness in her right hand and beginning problems with the left, this to me is a very good indication that surgery needs to be considered.

[152] Dr. Hansebout criticized Dr. Lesiuk for not recommending immediate surgery. He explains Dr. Lesiuk's findings in this way:

Q. There is reference in the next line under the one that I just read to you, "Nonetheless, there are not hard myelopathic or radiculopathic findings." What does it mean by "hard myelopathic"?

A. It means something that you can check and you can find a significant reproducible weakness or neurological sign, that's what that means. And it's a sign that you could say this is a true finding.

So what he's saying is, that so far he's only suspicious of these problems. To him, it's not an absolute indication, that her spinal cord is being compressed he says, you know, that's what I gather that he's saying.

He's saying that the neurological findings are not significant enough for him to really get worried about it, I guess; I guess you would have to ask him. I don't know what he exactly what he means. But he's saying that he's not 100 percent sure about his clinical findings.

Q. Based on what you've reviewed, are you in agreement with that?

A. Well, she's got clumsiness of her right hand, she can't write anymore; her left hand is getting affected. She's got pain. She's got a Hoffman sign. She's got a 6 millimetre -- her spinal cord is being compressed from 11 or 12 millimetres down to 6. And in order to judge whether you need to do something about a

patient, from a surgical standpoint, you need clinical findings, which she's got, radiologic findings, which she's got, and it all has to correlate: radiological, clinical, and the history, what they're telling you. So all these things are quite worrisome to me.

So I think that surgery needs to be considered, and he does too, because I think he's sent her to Dr. Bourque, and he wanted to send her to Dr. Fehlings, F-E-H-L-I-N-G-S in Toronto, who's a specialist in spinal problems, he's a spinal neurosurgeon.

Q. Now if I could ask you please to turn to tab 3, A3, that's Dr. Lesiuk's report of April 6, 1998?

A. I have it.

Q. And if we turn to the last page of that report, Question 11 is asking:

"Without surgery, what disease progression would you anticipate and what additional symptoms would you anticipate would affect my level of functioning"?

And he answers:

"Disease progression in the setting of spinal stenosis is difficult to predict with certainty in the absence of clear myelopathic features clinical symptom progression may occur in approximately 40 percent of individuals. Once myelopathic features appear, the tendency towards symptom progression is substantially higher. Symptom progression may take the form of pain and/or sensory motor loss."

Do you agree with that?

A. Yes I do.

Q. If you were to put Ms. Monks in a category, would she fall in the 40 percent of individuals or 60 percent of individuals?

A. Forty

[153] Dr. Hansebout was asked if he thought that, had Ms. Monks taken an MRI in the fall of 1998, just prior to her third accident, the MRI would have shown further deterioration of her spinal canal. I asked Dr. Hansebout if in giving his answer he would be guessing, and I

disallowed the question when Dr. Hansebout stated that indeed he would be guessing if he answered the question.

[154] Dr. Hansebout's evidence was clear on the need for surgery prior to Ms. Monks' 1998 motor vehicle accident.

Q. Doctor, I will again ask you then: In your opinion, did Ms. Monks require spinal surgery before the 1998 accident occurred?

A. In reading that sentence that you have alluded to, what I'm saying is that I didn't think that the December 1998 accident had much bearing on her disability. And she would have required surgery at some point, either before or after in any event, that's what I meant.

Q. Thank you. What is the basis of that opinion?

A. The basis of that opinion is that she was complaining of problems with her right hand, she had an x-ray -- she had an x-ray evidence of marked compression of the spinal cord, which was affecting her ability to work and enjoy life; in fact she couldn't write and she was I think an insurance adjuster at that time.

I think she needed -- you know, I would have put to her that 'You've got problems. We should consider surgery.' As a matter of fact Dr. Lesiuk had considered it, he wanted to send her to Dr. Michael Fehlings in Toronto because it was a tough case, he knew it.

[155] I allowed Dr. Hansebout to testify outside his report, as nowhere in his 49-page report had he stated that Ms. Monks required surgery prior to the 1998 motor vehicle accident. This worked some unfairness to counsel for the plaintiff, but as the order of witnesses had been reorganized, Dr. Lesiuk was to testify later and could address the problem.

[156] During cross-examination, Dr. Hansebout agreed that two years after the 1995 motor vehicle accident, Ms. Monks had reported to Dr. Lesiuk increasing fatigability in the right upper extremity and to a lesser extent the left upper extremity, and that was an indicator of radiculopathy (the nerve roots being affected as they leave the foramen). In myelopathy, it is the entire spinal cord that is affected. He also agreed with defence counsel that what Ms. Monks was reporting to Dr. Lesiuk, following the 1995 motor vehicle accident, could have been

radiculopathic or myelopathic as stating “yes, that’s fair”. Dr. Hansebout also agreed that what Dr. Lesiuk had on May 13, 1997 (Exhibit 1, Tab A-2), was a single MRI showing a potential nerve involvement at that point. Dr. Hansebout reluctantly agreed, during cross-examination, that absent clear myelopathic features in Ms. Monks, she had to be classified in the 60% group that stabilizes and not in the 40% group where Dr. Hansebout had classified her during his examination-in-chief.

[157] Looking at Exhibit 6, Tab 3, the first diagnostic report that Dr. Lesiuk and Dr. Bourque had to make in their reports, Dr. Hansebout agrees that there is evidence that at C4-C5 and C5-C6 and C6-C7, the spinal cord is being compressed at all three levels by disc bulges. Looking at Exhibit 6, Tab 4, the next MRI investigation, Dr. Hansebout had to agree that the radiologist had reported no internal change (1 year later, 1996 to 1997). Dr. Hansebout also conceded that the MRI findings largely explain Ms. Monks’ symptoms.

[158] Dr. Hansebout questioned whether Ms. Monks, even with her nursing background, could have monitored intractable pain and the progression of neurologic deficits. He admitted that he had never met her and it was not fair for him to doubt Dr. Lesiuk’s statement.

[159] The surgery involved for Ms. Monks’ condition is risky surgery. Dr. Hansebout agreed that a patient could die on the operating table, could receive further damage to the spinal cord and become a quadriplegic. For the interior surgery, going through the mouth and tossing the vocal cords aside, could, among other things, cause damage to the throat and neck area. He also agreed that both Dr. Lesiuk and Dr. Bourque were not completely wrong in concluding that: “Suzanne was not a classic candidate for surgical intervention at that point”.

[160] Dr. Hansebout did not want to admit that Ms. Monks had not suffered intractable pain prior to the 1998 motor vehicle accident. It was one of three things she had been told was a signal as further progression of her spinal canal problem. He was reminded that Ms. Monks stated that she did not take over the counter medication prior to her 1998 motor vehicle accident.

[161] In addition, Dr. Bourque, in his report, had informed Ms. Monks to watch for any increased symptoms of radiculopathy, meaning further symptoms of numbness or weakness in the upper extremities. Dr. Hansebout agreed that Ms. Monks had the training to enable her to watch for such increased symptoms.

[162] As regards myelopathy, Ms. Monks would be watching the upper extremities, but also anything lower than that, like lower extremities or even autonomic functions. Dr. Hansebout reluctantly agreed that both Dr. Lesiuk and Dr. Bourque had discussed the risks of surgery and, as Dr. Lesiuk mentioned in his report, “her concerns are not fanciful or ill-informed or irrational.

[163] Dr. Hansebout had to agree that between neurosurgeons, there is room for a difference of opinion. He questioned whether Ms. Monks’ decision would have been a decision for elective surgery at that point. He conceded that, if she had a choice of going or not going for surgery, it meant surgery was elective.

[164] Between April 6, 1998 and August 11, 1999, Dr. Hansebout agreed that something happened to Ms. Monks that made what was once an elective surgery decision, a mandatory surgery decision. He also agreed that, aside from looking at x-rays, a neurosurgeon would be talking to the patient and watching the patient’s symptoms. He admitted that the treating physician is in the best position to evaluate if the symptoms are progressing, stabilizing or regressing. He had not met with Ms. Monks and he had to conclude that in terms of being able to ascertain whether or not the recorded statements about her symptoms before or after the accident were correct or accurately reported, he had had no opportunity to confirm this information with Ms. Monks.

[165] Dr. Hansebout further conceded that the information contained in his report, that Ms. Monks missed one month of work after the 1993 accident, was wrong. Dr. Hansebout also had to concede that, in attributing 5% of her condition to the 1993 motor vehicle accident, he had surmised incorrectly that Ms. Monks was off work for a month.

[166] Then Dr. Hansebout agreed that, in his report, he wrongly associated the fact that Ms. Monks had urinary stress incontinence or urinary retention with a spinal cord stenosis problem, which pre-dated her operation in 1999. Dr. Vandewaetere (Exhibits 3, Tab 1, page 14) spoke to Dr. Lesiuk in July 1998, and received the opinion that the urinary problem was not connected to spinal stenosis. Dr. Vandewaetere referred Ms. Monks to an urologist, Dr. Walsh, who reported that her incontinence and retention problems were due to an irritable bladder.

[167] Next, Dr. Hansebout admitted he was wrong on how many hours Ms. Monks worked after the 1995 motor vehicle accident. Ms. Monks had reduced her working hours from 75 to 80 hours per week, to 35 to 40 hours per week, as she told Dr. Lesiuk (Exhibit 1, Tab 3, page 2). Dr. Hansebout had based his statement on Dr. Atack's report that Ms. Monks' working hours had been reduced from 80 to 90 hours per week before the 1995 motor vehicle accident, to about 20 to 25 hours per week after the accident. Dr. Hansebout agreed it changed his conclusion, if Ms. Monks kept working the hours she had told Dr. Lesiuk and, in addition, had attended a number of educational seminars. Dr. Hansebout was informed that, between 1995 and 1998, Ms. Monks had taken AIIC Certification courses, and in 1998, two 5-day courses out-of-town. In 1996, she had also taken the Ontario Brain Injury Association Certification and had become an accredited rehabilitation professional.

[168] Dr. Hansebout had to correct his statement that Ms. Monks did not finish an exam because she could not write, as the evidence was that she had not been allowed to use a computer to write the exam. Dr. Hansebout had concluded in his report that she had deteriorated significantly because she had had to use a wheelchair on occasion. He agreed that his statement was not correct when told that Ms. Monks used a wheelchair once and that her friends had testified it had happened one month prior to her September 1999 surgery.

[169] Inaccuracies were noted in Dr. Hansebout's report, where Dr. Hansebout claimed that Ms. Monks had a history of transient numbness in the arms after the 1995 accident. Faced with Dr. Lesiuk's report (Exhibit 1, Tab A-3), Dr. Hansebout stated that all Ms. Monks had, at the time, were cramps and spasms, and no fatigability in the left arm. Dr. Hansebout also corrected a statement he made at page 30 of his report, where he had stated that Ms. Monks had reported

numbness in her legs. It was brought to his attention that Dr. Lesiuk, following Ms. Monks' April 6, 1998 and May 13, 1997 visits, had not mentioned numbness in the legs.

Evidence of Suzanne Monks' husband and friends – 1995 Motor Vehicle Accident

Laurie Warren

[170] Laurie Warren testified that during the month of June 1988, she had been without work as an occupational therapist for three months. On July 6, 1998, she commenced work at Dynamic and remained at Dynamic until December 2002. For the first three months, she worked in Ms. Monks' home. She stated that, although Ms. Monks had intermittent pain in her arm (every hour to 45 minutes), she continued to work. Ms. Monks also stated that the pain she had was a result of the 1995 motor vehicle accident. She described Ms. Monks' work as meeting with clients and doing extensive research for future care costs. Ms. Monks' work at rehabilitating clients, took her out of her home to client locations. Ms. Warren described a trip made to a golf tournament and B-B-Q, in Toronto, during the month of August 1998. Ms. Monks drove to Toronto and back. She also confirmed that Ms. Monks was attempting to have Zurich Canada commit to Income Replacement Benefits so that if surgery became necessary, she would be covered. Ms. Warren testified that there was only a possibility of surgery and that it was not imminent.

Dr. Maxine Morrison

[171] She had met Ms. Monks in 1994. She stated that Ms. Monks had mentioned suffering from chronic pain, following her 1995 motor vehicle accident, but that it had not interfered with her work. Ms. Morrison also said that Ms. Monks had not complained about her physical problems. Between 1995 and 1998, Dr. Morrison knew that Ms. Monks was taking medication and therapeutic massages. She described Ms. Monks' social life as very active, noting that she had taken trips to Las Vegas, New York, and to Reno, Nevada. She stated that she went out for dinner at restaurants and to movies with Ms. Monks. Dr. Morrison stated that, in November 1998, she took a course and workshop in Las Vegas with Ms. Monks. The course involved assessing dangerousness in the work place and crisis intervention. She told the Court that Ms.

Monks had been a power shopper and while she got tired, Ms. Monks kept on going. She remained resolute that Ms. Monks did not use a wheelchair.

Lynn Parker

[172] She testified that Ms. Monks had discussed her 1995 motor vehicle accident with her. She stated that Ms. Monks thought she was suffering from a whiplash type of injury. She knew that Ms. Monks had complained of numbness in her fingers and, as the condition had persisted, she had sought medical help. She recalled that Ms. Monks had reorganized the ergonomics of her workstation so that she did not exacerbate her physical problems. She had discovered that Ms. Monks had back spasms, but she added that it did not affect Ms. Monks' work. She felt that Ms. Monks had had no problems with walking prior to her 1998 motor vehicle accident and had not resorted to mobility aids. Ms. Parker stated that, at the time, Bill 164 was the insurance law in Ontario and she had urged Ms. Monks to obtain a legal opinion to make sure any expenses would be covered in the future.

[173] Ms. Parker mentioned the shopping trips she shared with Ms. Monks in Las Vegas, Los Angeles and the like. Those trips involved Maxine Morrison, Suzanne Monks, Laurie Warren and herself. Ms. Monks had no difficulties in keeping up to her friends during the shopping sprees. She confirmed that Ms. Monks had no assistive device, like a wheelchair, during the November 1998 trip. Ms. Monks had done the driving on the open highway for the three or four hours it took, with a van, to get to factory outlets, to do some shopping.

[174] John Perrozzino during his cross-examination confirmed that Ms. Monks had continued to work at Dynamic between May 1997 and December 1998, and had worked twice as long to get the same amount of work done. He stated that, following his wife's 1995 motor vehicle accident, he had been called upon by his employer to work up north, in the Arctic. When he returned home, three weeks later, he recalled that her worst pain was over and that she was back at work.

[175] He noticed that between 1995 and 1998, Ms. Monks was dropping things. He confirmed that he was taking over heavy housework at home to give his wife a break. He also

confirmed that she had been doing some sporting activities until Dr. Lesiuk recommended that she abandon her sporting endeavours. Nothing affected her concentration, as most of her courses were achieved during those years. He felt that, once he and his son decided to help in the house, not much had changed prior to the 1998 motor vehicle accident. He was resolute in saying that he was not doing all the housework by a “long shot” before the 1998 motor vehicle accident.

Dr. Bruce Stewart – the 1995 Motor Vehicle Accident

[176] Dr. Stewart met with Ms. Monks, in Toronto, on May 29, 2002. He stated that Ms. Monks had told him that her head was thrown back at the time of the accident. There was an immediate tingling in her arms, which she felt had subsided over a 15 to 20 minutes period. It was followed by headaches, a stiff neck and some discomfort under the bra line. Dr. Stewart testified that, during his 2002 meeting with Ms. Monks, she said to him that “in general, I saw progression,” referring to a progression of symptoms she felt prior to her 1998 motor vehicle accident.

[177] Then Dr. Stewart was asked about drugs Ms. Monks was taking.

Q. Did she tell you anything about the medications that she was taking prior to the 1998 accident?

A. Yes. She told me that at that time she was using the drugs Naproxen, Advil and Robaxacet.

Q. Can you advise whether any of those medications are prescriptive; and could you advise what those medications would be for?

A. Yes. Naproxen is an anti-inflammatory oral non-steroidal anti-inflammatory. It's only purchasable on prescription, in many sizes of course.

Advil can be purchased in low strength, 200 milligrams over the counter, and is frequently purchased by individuals, but doctors prescribe it always as 400 milligram. I have no idea which one she was using. It is also an anti-inflammatory. These are non-narcotic, non-steroidal - - without steroids, anti-inflammatories - - so they help pain. Robaxacet is a combination of a muscle relaxant, Robaxin, or I believe it's cyclos - - I've forgotten - - leave that out. Robaxin is a muscle relaxant, and acetazolamide [sic], which is Tylenol, and it is a

muscle relaxant and mild pain reliever. And Robaxin and Robaxacet I believe can be purchased over the counter without prescription.

[178] Dr. Stewart expressed the opinion that the x-rays taken March 12, 1993 of Ms. Monks' spine (Exhibit 6, Tabs 1 and 2) and the x-ray taken on September 21, 1995, show degenerative changes beyond those he would expect in a woman at ages 38, 40 and 42 (if the x-ray of 1997, Exhibit 12, Tab 9, is taken into account).

[179] Dr. Stewart expressed the opinion that a treating physician always tries to give his patient the benefit of the doubt, but that an examiner such as he is, is always objective and entitled to be believed.

[180] During cross-examination, Dr. Stewart was reminded that he was retained by the defendant, who saw issues differently from the plaintiff. Dr. Stewart had to admit that he was paid \$3,600 for his initial evaluation, \$2,500 for his Court appearance, and \$2,800 for his preparation for trial.

[181] Dr. Stewart engaged in a long argument with the plaintiff's counsel. He had stated in his report that it was reasonable for Ms. Monks to have surgery when Dr. Lesiuk saw the 1999 MRI. (It took half an hour of the Court's time to have Dr. Stewart admit he did in fact say that in his report. Dr. Stewart had corrected his report in his examination-in-chief, to say surgery should have been undertaken prior to the 1998 motor vehicle accident.)

[182] Dr. Stewart mentioned in his report that he had removed Ms. Monks' collar and testified he had not seen a large scar on her neck. When it was suggested to Dr. Stewart that the reason he had not seen Ms. Monks' scar was because the scar was beneath the collar, Dr. Stewart refused to back down and insisted he had removed her collar. Dr. Stewart had also stated in his report that Ms. Monks was using a wheelchair when she saw him at his office. It was pointed out to him that Ms. Monks was getting around, at that time, with a wheeled walker.

[183] Ms. Monks had written back to Dr. Stewart, following her visit to him. He had suggested that she get medical attention for edema – water in her legs. He reluctantly agreed that

Ms. Monks had mentioned that she had water in her lungs, as well as in her legs. The fact that Ms. Monks had water in her legs would have affected his observations of her walking. Contrary to what Dr. Lesiuk had stated, he also maintained that Ms. Monks' x-rays showed her spine in a condition normally found in older female patients.

[184] A long debate took place during cross-examination concerning Ms. Monks' need for surgery prior to the 1998 motor vehicle accident. Dr. Stewart could not find a way to challenge Dr. Lesiuk's in his comparisons of the radiologist's findings in the MRI's and could not counter the statement "there is no significant objective deficit". Dr. Stewart finally accused Dr. Lesiuk of having made an error. (Page 68, transcript of evidence)

[185] As both Dr. Lesiuk, a neurosurgeon, and Dr. Bourque, a neurologist, were in agreement that a conservative approach to surgery was indicated, Dr. Stewart had to agree that there is room for a difference of opinion on the matter. Dr. Stewart disagreed with the suggestion that, given the high-risk situation Ms. Monks was in, she could have been in the 60% range of persons who could go on and stabilize their condition without surgery.

The December 23, 1998 Accident

The evidence of Suzanne Monks

[186] Ms. Monks recalled that a single car accident occurred when she was driving the family car in Erie, Pennsylvania. She was en route to Lexington, Kentucky, to visit her sister. Her husband John and their two children were passengers, as was her mother sitting in the front passenger seat. Ms. Monks recalled that they had left Ottawa at 10:15 p.m., after purchasing a Reiss Trailer that they hauled behind their Ford Expedition van. All passengers were wearing seat belts. She had taken the wheel of the car from John and had driven through Rochester, Buffalo and Erie. A snowstorm developed that caused the Reiss transporter to swing into the side of the car. It caused the car to swerve and Ms. Monks stated that she over adjusted the wheels of the van and that the van had struck a bridge abutment.

[187] She stated that even though the speed limit was posted at 55 mph, she was travelling very slowly at 55 km per hour. Ms. Monks stated that she had hit the steering wheel at impact and that her body had snapped back. The police had been called and after three hours of waiting, the police took her and her family to a room at a nearby motel.

[188] As her mother was not well, the police suggested calling an ambulance for her mother to go to the hospital. Her mother had trouble breathing and had neck pain, and a second ambulance was then called to take Ms. Monks to the same hospital from the motel.

[189] Ms. Monks stated that she stayed an extra week to rest in Lexington and then took two days to return to Ottawa. She did some driving on the return trip to Ottawa from Erie, Pennsylvania, to Buffalo (2 to 3 hours duration).

[190] Ms. Monks testified that immediately at the motel, following the accident, her neck gave her pain, as did the bruising on her chest from the seat belt. She stated that she had a tingling sensation in her right arm and it felt sore, more than usual.

[191] She reported that immediately, following the accident, she felt more pain and had more sensations from her left arm; she described it as a numbness, not a normal feeling. She stated that she could not tell the doctor if the feeling was worse than before. She recalled that the medical staff at St-Vincent's Hospital, in Pennsylvania, gave her a soft collar.

[192] Ms. Monks stated that, in the days that followed the accident, the bruising across her chest and right breast was sore and purple in colour, and she stated she developed a bad headache. The heaviness in her arms had disappeared.

[193] After she arrived in Lexington, Kentucky, on December 24, 1998, she said that her symptoms got worse. The range of rotation of her head was restricted, she had a burning sensation in her neck, and her right arm bothered her more than usual. The pain was ten times worse than before. The fact that her left arm bothered her more than previously was something new. She also began dropping things from her right hand. She described a neurologic pain that felt like an electric shock type of pain, like being bitten by bugs, and sometimes the pain gave

her a burning sensation. She described a pain that travelled from her upper back and down her arm that kept her awake at night both while in Lexington and upon her return to Ottawa.

[194] Back in Ottawa, she stated, the bruising on her chest took three months to go away. The headaches she suffered were worse than the type she had endured after the 1995 accident and some tasks she was used to doing, were aggravating her symptoms. Her symptoms were the same as after the 1995 accident, but more intense ... a burning and tingling sensation in her right arm down to her fingers. She described her strength as weaker than prior to the accident. She was dropping things, and had difficulty with lifting. She also described her left arm as not very symptomatic prior to the 1998 accident, but after the accident the left arm felt like the right arm prior to the 1998 accident, with a tingling sensation coming down to her fingers.

[195] She stated that her neck remained stiff after the accident and that during the month of April 1999, she underwent physiotherapy to help with the range of motion in her neck.

[196] Ms. Monks also stated that the physio at the Canadian Back Institute facility ("CBI") helped the range of her neck motion, but did nothing for her arms or for her headaches. She claimed that she had pain in her neck and shoulders. She recalled that the pain after the 1998 motor vehicle accident became disabling ... a type of pain she did not have after the 1995 accident. The massage therapy she had undertaken after the 1995 accident had reduced the pain and the pain had eventually disappeared. Although she could not remember at what point in time the pain had left her.

[197] In 1999, the headaches were constant and not improving, and she stated that she was taking over the counter medication.

[198] In January 1999, she stated that she consulted Dr. Vanderwaetere, who expressed concern as she had been warned against getting involved in another accident. In 1995, no prescription medication had been required to control pain, following the accident, but in 1999, painkillers and muscle relaxants were prescribed for her.

[199] Dr. Donald Chow is an orthopaedic surgeon working full time at the Canadian Back Institute (“CBIO”). In April 1999, Dr. Chow suggested physiotherapy to improve her neck motion. Ms. Monks stated that, as the CBI facilities were located in the same building where she worked, she did physio until the end of that summer.

[200] In 1999, ms. Monks testified that the medication prescribed for her headaches consisted of over the counter Tylenol 3 during the day, as she was driving a car. At night, as Tylenol 3’s were not very effective, she took Flexeril, and Flexeril lost its effectiveness due to frequent use. As Flexeril made her sleepy, she could not use the product during the day. The drug Percocet was also used. Percocet relieved her pain, but did not eradicate it; her pain was down to a 5 from a scale of 10. Ms. Monks also stated she took Oxicontin that allowed her to rest at work, at lunchtime.

[201] Ms. Monks testified that she saw Dr. Lesiuk during the month of August 1999 and that he had determined that there was swelling on her spinal cord. Dr. Lesiuk recommended surgery and Ms. Monks also stated that it was not a matter of choice. The disc herniation had increased and it caused more pressure on her spinal cord ... that was a signal change and Dr. Lesiuk had previously discussed that eventuality with her. Dr. Lesiuk, according to Ms. Monks, had discussed her file with a colleague, Dr. Guyot, who had confirmed that surgery was mandatory. Ms. Monks was told that the operation would be through the neck, down her throat and that her voice box would be tossed aside. She stated that she was left with a gravelly voice, following the operation. The list of potential complication was long. She could be left as:

- Quadriplegic
- If the spinal cord continued to swell, she could suffer paralysis.
- The dead cells could cause surrounding good cells to die.
- And she could suffer effects from the anaesthetic.

[202] Ms. Monks stated that Dr. Lesiuk planned to remove a damaged disc, patch her up with bone taken from her hip or from cadavers, and fuse the spine together with bone plugs at the C5, C6 and C6/C7.

[203] The procedure used would be the Cloward procedure. Ms. Monks stated that Dr. Lesiuk was concerned that the operation would leave her worse off. Dr. Vanderwaetere had also seen her and Ms. Monks stated she received the message that, without an operation, she would be paralysed, had she not been operated upon prior to Christmas. The paralysis would affect the upper part of her body and she would become paraplegic (Paraplegic – is the paralysis of both lower extremities and, generally, the lower trunk).

[204] On September 27, 1999, Ms. Monks' first surgery took place. Dr. Lesiuk had considerable difficulty at the C6 and C7 level and Ms. Monks was told by Dr. Lesiuk that, as he had not been able to do the Cloward procedure, he did the Smith-Robinson procedure instead. As a result, Ms. Monks testified that the operation had extended over a longer period of time. (An explanation of both procedures used in surgery is not required to understand this case.)

[205] Ms. Monks stated that she ended up with a fused area in her back and a different approach to life. Her legs no longer functioned, and she could not stand up on her own, as she discovered the day after the operation. Her legs felt heavy and they did not feel quite right. She was vomiting more than after any other surgery she had had in the past. She stayed in the hospital for two weeks and during that time the vomiting never stopped. At the hospital, she did physio and she graduated to walking with the assistance of a walker, but she did not have the balance and strength to walk.

[206] Ms. Monks recalled that Dr. Lesiuk had not anticipated that her spinal cord would go into shock, and he had told her the spinal cord should go back to normal within three months.

The Sequelae of the September 1999 Operation

[207] After the September 1999 operation, Ms. Monks testified, she had to catheterize every two to three days, as she had no sensation to void and she had abdominal pain when her bladder

was too full. She had accumulated 1,000 ccs of urine in her bladder when the nurse in the hospital had catheterized her. Ms. Monks also stated that she had developed a neurogenic bladder. This meant that her bladder could not relax and would empty at unexpected times. This had not been expected by the doctors, after the operation.

[208] Ms. Monks also stated that the condition in her right arm had not changed, though she had been told that, while the operation would not repair her arm, it would stop the damage progression. The left arm was now in the same condition as the right arm, although the removal of the compression at the core was supposed to have relieved the pain.

[209] The right arm was clumsier, but she could not recall if the left arm was also clumsier. She had an increase of pain in the neck and shoulder, post operation. Her headaches increased and a medical team attempted to find medication that would help. She was placed originally on morphine to stop the vomiting but once the morphine was stopped, the vomiting continued.

[210] She testified that the bladder dysfunction brought about a bowel dysfunction. Prior to 1998, she had a fussy bladder – it was a simple bladder problem, not the neurogenic bladder she suffered post surgery.

[211] Once she had healed a bit, Ms. Monks testified, she had lost sexual satisfaction. Her whole body was different after the operation and she had difficulty getting around the house.

[212] The vomiting prevented the neck from healing, so Dr. Lesiuk re-admitted her three weeks after she had left the hospital after surgery. She had a fever and a urinary tract infection. On leaving the hospital again, she was told she would have to catheterize herself. At the hospital, she was vomiting 8 to 12 times daily and her electrolytes were so low that she had to drink potassium. Ms. Monks estimated that drinking potassium to that extent could have led her to suffer heart failure.

[213] Ms. Monks recalled that her right arm gained strength, as did her left arm. However, the pain in her shoulders was disturbing. She suffered from migraine headaches and took an oral form of morphine that reduced pain to a 5 from a scale of 10 (10 being the intolerable).

Concerning her continuous vomiting, she stated, as above, that she was re-admitted to the hospital for two to two and a half weeks to meet with a gastroenterologist to test for “a vegis nerve damage”. As solid foods did not move from her stomach to her intestine, the result was that the food had no alternative but to move upwards, causing vomiting. She added that some physicians, at the hospital, claim that spinal cord problems affect nerves that send signals to the stomach.

[214] She had to continue to catheterize to void urine, as her bladder emptied spontaneously, and she testified that she believes that this is a problem related to her spinal cord injury. As a result, she has undergone a CT Scan with an urologist, every six months, to ensure that her kidneys keep to a normal size.

[215] Fiona Smith Bradley is a registered member of the Canadian Association of Occupational Therapists. She graduated from Queen’s University with a Bachelor of Science in occupational therapy. She specialized in assessing individuals with disabilities in order to facilitate their functioning within their normal lives. She has assessed over 600 clients in order to make recommendations on increasing their function in their homes and/or work places. She has experience with individuals suffering from spinal cord injuries. She filed a report to Halifax Insurance, found in Exhibit 1, Tab B-1, dated December 28, 1999, that summarized Ms. Monks’ situation at that time. Her report reads as follows:

Purpose of Referral:

1. To assess Ms. Monks’ ability to perform her pre-accident normal life activities.
2. To assess Ms. Monks’ work stations at both her home office and her external office.
3. To complete an analysis of Ms. Monks’ pre-accident employment position.
4. To make recommendations regarding assistive devices and/or services that would assist Ms. Monks with the performance of her normal life activities.

Date of Contact: December 16, 1999

Details: This therapist met with Ms. Monks in her home in order to complete the initial assessment.

PRESENT PHYSICAL STATUS

Accident-Related Injuries: Cervical multi level stenosis with cord compression.

Intervention: On September 27, 1999 Ms. Monks underwent a fusion surgery of the C3-C6-7 vertebrae, and was required to wear a hard collar at all times post-accident. Ms. Monks has 3 different hard collars which she alternates wearing due to the development of pressure sores on her neck if she wears only one brand of collar. Ms. Monks wears a Philadelphia collar in the bathtub only, and alternates between a Miami J collar and an Aspen collar during the day.

Ms. Monks had a follow-up appointment with Dr. Lesiuk on December 21, 1999. Dr. Lesiuk reportedly stated that a MRI test performed earlier that month revealed damage to the spinal cord, and that Ms. Monks will have to wait to see the implications of the damage as they arise over the next year. Dr. Lesiuk has also recommended that Ms. Monks begin to gradually decrease the wearing of the hard collar and intermittently wear a soft collar. Gentle progressive physiotherapy was also reportedly recommended, which Ms. Monks will begin in the New Year.

Symptomatology: Ms. Monks reports that she is presently experiencing the following symptoms:

- Neuro-cognitive fatigue during activity performance, affecting concentration, alertness.
- Marked right-sided stiffness of the upper and lower extremity, as well as fatigue on the right side on a continual basis. Decreased endurance with right-sided activity.
- Left-sided weakness compared to pre-accident ability; weakness is prevalent during repetitive exercises.
- Poor balance.
- Sporadic nausea with frequent vomiting.

[216] Ms. Monks stated that she continued to vomit and continued to be incontinent. There was no improvement three months post surgery. The hospital recommended she take the food supplement "Ensure". She did not work prior to January 2000, as she was following physiotherapy to learn how to walk independently. Ms. Monks testified that she suffered the Proprio Reception after her first surgery; it gave her a sensation that her body was in space and that she did not know where her legs were. This, in turn, had caused her to lose her balance, obliging her to relearn how to walk.

[217] Ms. Monks added that, as her office was located in the same building as the Canadian Back Institute, she used the equipment in the spinal cord injury unit, under supervision, three times per week. She was attempting to work full days in January 2000, but she said she was not accomplishing what she wanted. Some days the vomiting was so bad that she could not drive herself to work and stayed home. She testified that she stopped working at the end of February 2000.

[218] Dr. Vanderwaetere's January 20, 2000 note reads as follows:

Doesn't feel capable of continuing to work; too much pain; exhausted; works – then goes to bed; bladder – catheterizes 3 to 4x/day; nausea continues – vomits up to 3x per day; still can't eat; has been very upset with the realization that she can't work; physio for neck and legs; still use walker to mobilize; legs get too tired, cramps and give out, like muscular fatigue – they just won't go, then has to switch to wheelchair.

Suzanne Monks' Work after the 1998 Motor Vehicle Accident

[219] After the 1998 accident, Ms. Monks stated that she reintegrated her work on a gradual basis. She asked Laurie Warren to take over some hardening work program files for her.

[220] She said that she was slowed down at work. She applied hot packs to her neck and lay down on the floor for one to two hours before going out to clients or writing a report.

[221] In 1995, the symptoms in the hands, the arms and shoulders were present, but now, in 1999, they were not only present when using the mouse, but they were present when she used the keyboard on her computer, as well.

[222] In April 1999, Ms. Monks was advised that she had to change her method of working. She had to abandon marketing and she also abandoned going to hockey games with clients. She delegated her work to an associate. Her out-of-town meetings for example, to Toronto, were done by teleconference. Her work hardening programs were passed on to an associate. Her billable hours decreased ... she was at the office longer, but billed less, as she took longer to accomplish her tasks. In 1995, her work interruptions were infrequent, but after the 1998

accident, she claimed she was constantly fatigued and she had to rest at work. She stated she had no choice in the matter. She worked until her surgery of September 27, 1999, and she was told that she would be off work for 12 to 16 weeks after surgery.

[223] Ms. Monks testified that, on November 18, 1999, she returned to work with a walker. The insurance company provided transportation for her to work. By 11:00 a.m. at work, she said she could barely keep her eyes open. Her right arm gave her considerable difficulty, as it was painful and inflexible at the computer keys. It was more a pain issue than a functional issue, and it was a neurogenic pain. Shoulder and back pain had started to improve, meaning that they were not as acute.

[224] Fiona Smith Bradley reported on December 29, 1999: “Suzanne’s trying to put out reports and is falling asleep. She did not have problems like that before the operation. She suffers from marked right-sided stiffness of upper and lower extremities, plus fatigue on right side on a continuous basis. Her left side weakness, following her surgery, is worse compared to her pre-accident condition.”

[225] Ms. Monks recalled that she had been working full days in December 1998 and, in 1999, because she was not able to work full days, she realized that she was on a descent and not accomplishing what she wanted.

[226] She stopped working at the end of February 2000. Her work was hampering her recovery. She had estimated that three or four months would have brought her strenght back with physio, but that didn’t happen, and she stated that she never returned to work.

[227] Ms. Monks stated that, later, after her 2001 discharge from the Rehabilitation Centre, she still wanted to return to work. Dynamic was still around and it employed an occupational therapist. Now in 2005, a return to work is not realistic, due to her poor concentration, difficulty getting anywhere in a reasonable amount of time, and her inability to give clients 100% of her capabilities. She added that she couldn’t think of anything she could do with a reasonable amount of success.

At Home after the 1998 Motor Vehicle Accident

[228] Ms. Monks stated that she had to cut out more tasks in housework than in 1995. She could not pick-up her mother to do groceries, or take her mother to her medical appointments. She also stated that her husband John was obliged to replace her in doing the housework and meals. If someone brought down the laundry, she could sort the loads, but could not take the clothes out of the washer or dryer. She still drove her car, but could not shop alone at Price Club, as lifting parcels in and out of the car, was out of the question. She had difficulty cutting up vegetables.

[229] After the September 1999 surgery, she required assistance to walk in her home and obtained a walker from home care. At times, she testified that she stayed in the upstairs part of her home until her next doctor's appointment, three weeks away.

[230] During the month of January 2000, she experienced difficulty getting in and out of her car, so she purchased a van to help with that problem, and she was getting in and out of bed with the help of her husband.

[231] Ms. Monks testified that Dr. Donald Chow, orthopaedic surgeon at the Canadian Back Institute, summarized her condition in his written report to Dr. Vanderwaetere of November 15, 2000, found in Exhibit 1, Tab D-1:

At this time I was shocked to hear that she has had considerable deterioration since I last saw her. She has recently had several falls, and her walking has deteriorated considerably. She has been using a walker since the surgery. She also has a neurogenic bladder, and required catheterization. She says she catheterizes herself four times a day. She also has quite severe bowel motility problems. I understand that she is seeing Dr. Scully for the bowel problem, and may be referred to a specialist in Hamilton. She had also seen Dr. Leahy with regards to the neurogenic bladder.

With regards to her pain, she has seen Dr. Wynne in the Pain Clinic and is now on MS-Contin 120 mgs. t.i.d. as well as other antiemetics. I see that she is taking intravenous antiemetics at this time. Other medications that she has also been taking include Zoloft as an anti-depressant and Bromocriptine with regards to a prolactinoma (a pituitary tumour).

[232] Dr. Chow also noted that Ms. Monks weighed 230 lbs. at that time. She stated that at the time of Dr. Chow's report, she had fallen going down the stairs in her two-storey home, had somersaulted and landed on "her butt". As she could not use a walker in the house, she remained in the upstairs part of her home. She left her home only if she had appointments with doctors. She was taking MS-Contin, a derivative of morphine, to control the pain. To combat nausea, she was prescribed Zofran and she was also taking Zoloft to fight depression.

[233] Ms. Monks stated that "her muscles had gone on her hips" and that in June 2000, she was still going to the Canadian Back Institute three times per week, for gait training with the goal of becoming independent of assisting devices for walking. She felt she had made no improvement on her accident symptoms and that some symptoms had increased. She was taking medication to fight nausea, Oxicontin for pain, and Neurontin to combat headaches. As most medications had side effects, Ms. Monks testified the medications combined to play havoc with her system.

[234] Zofran was administered intravenously for better absorption, but Ms. Monks stated it never got rid of her nausea. Zofran intake was installed with a small catheter in one vein and gave rise to infections, even though nurses came every day to change the bag. As a result, a PICC line was installed (Peripheral Intravenous Central Catheter). This, she recalled, had occurred in the fall of 2000. The installation was made on her subclavian vein, the main one leading to her aorta, and was installed at the Ottawa Hospital, Civic Campus. Now the dosage could cover a whole day. It stayed 14 months on her right arm and four and a half months on her left arm. The PICC line caused inflammation of the vein and blood clots.

January 11, 2001 Surgery

[235] The second surgery took place on January 11, 2001, and Dr. David Attack summarized the procedure in his report of June 6, 2001, as follows:

Dr. Lesiuk had examined Suzanne and his assessment on November 28, 2000 indicated right sided weakness in both the upper and lower extremities with a sudden-give component undoubtedly induced by pain. There was also subjective

alteration to pin prick and light touch below the elbows and knees bilaterally but worse on the right. He also reported diffusely brisk deep tendon reflexes with bilateral positive Hoffmann reflexes but with plantar reflexes still flexor (normal). Gait was said to be somewhat wide based. He reports the MRI scan as showing degenerative changes with stenosis causing flattening of the spinal cord and moderate stenosis of the neural foramina at C4/C5. At C5/C6 a flattened cord without evidence for extrinsic compression was present (compressions previously seen seemingly relieved by previous surgery in September 1999). Some foraminal stenosis was also seen. At C6/C7 degenerative changes with flattening of the spinal cord and moderate stenosis of the neural foramina was again noted. There was also a continuing signal change in the spinal cord tissue possibly more diffuse than previously. It should be noted that the Neuroradiologist reviewing the MRI scan at that point did not appreciate any significant worsening.

On January 11, 2001 Dr. Lesiuk performed a posterior decompression of the cervical spine involving a partial laminectomy at C3 and full laminectomies from C4 through C7 with bilateral foraminotomies at C4/C5, C5/C6 and C6/C7. Postoperatively the patient recalls (documented by Dr. Lesiuk) a significant exacerbation of neck pain again spreading into the interscapular area. She was also aware of low back pain. She was hospitalized at the Civic Campus for two months and then transferred to the Ottawa Rehabilitation Centre for further inpatient rehabilitation. She was discharged from the Rehab Centre on April 13, 2001.

[236] Dr. Attack, in his report of June 6, 2001, reported the list of Ms. Monks' problems, as follows:

The patient summarized her residual impairments and limitations as follows:

1. She reported symptoms of chronic ongoing neck pain.
2. She reported extreme fatigability requiring extended rest periods during the course of the day.
3. She reported upper extremity numbness and weakness with activity as well as poor dexterity of her hand and finger movements.
4. Gait ataxia (imbalance) and lack of stability requiring a walker or cane for mobility for short distances and a scooter for long distances.
5. Decreased endurance and walking tolerance even with a walker. She notes that she will walk five minutes and then require a rest period before continuing.
6. Decreased cervical range of motion.
7. Decreased strength in trunk making it difficult to maintain balance.
8. Difficulty rising from lower surfaces such as a low chair or toilet due to hip extensor and flexor weakness.
9. Inability to raise arms past 90 degrees with side lift.

10. Right hip pain.
11. Plantar fasciitis.
12. Persistent gastrointestinal problems resulting in frequent bouts of nausea and vomiting requiring intravenous medication to manage.
13. PICC line in place to manage nausea.
14. Neurogenic bladder requiring catheter management.
15. Bowel dysfunction requiring enemas or digital removal.
16. Decreased concentration and poor short term memory secondary to fatigue and medication.
17. Stress and anxiety regarding uncertainty about the future.

[237] Ms. Monks explained that a porth-a-cath system was then installed on her person. It consists of a receptacle the size of a quarter, a one-inch long cylinder, surgically inserted under the muscles in her chest with a catheter. The skin is then allowed to grow over it. It was installed at the local hospital during the month of March 2002. A pump extracts from an IV bag, the medication fed through a gropper. With the PICC line, she received medication three times per day and with the porth-a-cath system, she can disconnect and re-prime the pump three times per day. It stays on for one to two hours every eight hours. She also stated that she consulted Dr. Jack Klein, a psychiatrist, who helped her to surmount difficulties encountered after the 2001 surgery.

[238] Ms. Monks suffered the following problems after April 2002. Some conditions had been present since the earlier of the two surgeries and some following the January 2001 surgery: she was hospitalized for septicemia caused by the changes of the clipper on her porth-a-cath, she suffered from pneumonia, nausea (she ran out of Zofran for three days and her nausea was unbearable), vomiting several times each day, having to avoid solid foods, and colitis (blood in stools). Colitis started in September 2004 and never stopped. She was hospitalized for the removal of her gall bladder and what should have taken one week for recovery, in her case, took over one month.

[239] The medication taken by Ms. Monks, following the 1998 motor vehicle accident, was listed by Sandra Guest-Poulin, Life Care Planner, hired by the defendant and found in Exhibit 8, Tab 1, page 13 (stamped 16):

• Medication and Dosage	• Purpose	Intermittent Need	Continuous Need
Statex 10 mg. 1 to 2 q4h - prn	Pain Management		√
Zoloft 200 mg. at bed	Anti-depressant		√
Pantaloc 40 mg. 1 tab at a.m.	Inhibits stomach acid		√
Motilium/ 20 mg. 2 q.i.d.	Gastric Utility		√
MicroLax – one a night	Spastic Bowel		√
Robaxin 750 mg, q.i.d. - prn	Muscle Relaxant	√	
Lorazepam – prn	Helps with sleep	√	
Neurontin, 1500 mg. 5x/day	Neuropathic pain		√
Zofran, 4 ml. qh8 – IV Administration	Nausea		√
PMS Colace, 2 caps t.i.d.	Stool Softener		√
Apo Ferrous Glucomate – t.i.d.	Anaemia	√	
Uripass 100 mg. 2 t.i.d. – 10 days a month	Spastic Bladder		√
Duragesic 250 mg. 1 patch every 72 hrs.	Pain control		√
Xenical 120 mg. 1 cap t.i.d.	Lipid Inhibitor	√	
Proctosedyl – 2x/day prn	Bowel Suppository		√
Conversyl 4 mg. 1 a day	High blood pressure		√
Wellbutrin 450 mg. 1 b.i.d.	Anti-depressant	√	
Stemetil 10mg. 1 q.i.d.	Anti Nausea		√
Cabergoline .5 mg – 2 tab/week	Pituitary Tumour		√
Furosemide 20 mg. 3 tabs a.m.	Controls oedema		√
Potassium Chloride 8 mg. 1 bid	Furosemide depletes Potassium		√
Amoxil	3 to 4 bladder infections/yr.	√	
Westcort & Hydrocortisone Cream	Patch adhesive allergy	√	
Ensure – 4 to 6 cans a day	Food supplement	√	

[240] Ms. Monks described her phlebitis in the legs in 2001 and 2002, as a loss of sensation in the legs that changed the blood circulation. As messages from her nerve system were not carried properly, her legs did not work as well. She had surgery on her legs in 2004, as she had constant swelling (edema). She also had pulmonary edema in February 2004. She claimed that her edema had set in, following the 2001 surgery, and she took Laysic, a medication that gets rid of extra fluid. She listed those problems as problems resulting from a spinal cord malfunction.

[241] Following her 2001 surgery, Ms. Monks described a breaking pain that could come and go ... like an electric shock, lasting 10 to 15 minutes. She had the pain in her shoulder muscles, upper back trapezius, arms down to her fingers and in her legs. She had developed a high

tolerance for morphine that handled the incision pain caused by the surgery incision down her back, but morphine did nothing for her neuropathic pain.

[242] She was given a Fentanyl patch at the Civic Hospital and the medication is absorbed through the skin. Following her January 2001 operation, she described her pain on a scale of 0 to 10, as being a 20 without medication.

[243] During the year 2001, Ms. Monks explained, she developed respiratory problems. She was tested and treated for sleep apnea, as her oxygen level dropped (not taking enough breaths) at night to low levels. Following that discovery, Ms. Monks stated, she developed Narcolepsy (a lack of oxygen to the brain). She would fall asleep anywhere: while undergoing a CAT Scan, in the bathroom, etc. Her husband, John Perrozzino, testified that he left for work one morning while she was in the bathroom and found her still asleep in the bathroom when he returned home from work at night.

Description of Health Problems from 2001 to 2005

[244] Ms. Monks explained that she has lost the ability to concentrate while reading and has to re-read paragraphs over and over again. She also has trouble finding words. She complained of a myofacial pain that can last one to one and a half hours, two to three times per week, that goes to her head and gives her migraines. She controls the pain at a manageable level with Advil and Fentanyl patches. For breakthrough pain, Ms. Monks uses morphine, listed as Statex on the medication list.

[245] Ms. Monks further identified the following ongoing residual problems to Sandra Guest-Poulin, a certified disability consultant, on December 13, 2004 (found in Exhibit 8, Tab 4, pages 229 to 233). I have reviewed Ms. Monks' evidence at trial and it confirmed this report. (This report was not tendered by the plaintiff, at trial, to show her entitlement to future benefits, but on the issue of aggravated damages.)

Lower Extremity Problems – Ms. Perrozzino reports chronic musculoskeletal low back, and bilateral knee, leg, and feet numbness, and pain. Her gait is ataxic due to muscular weakness, poor trunk strength, and stability. She reports a

diagnosis of right hip bursitis with hip extensor and flexor weakness. She has experienced numerous falls in the home and the community.

Ms. Perrozzino reports that her leg muscles are weaker. She describes a neurological pain that radiates down her legs, described as “one thousand fire ants biting her legs. She describes painful feet, with the sensation of “walking on glass” when weight bearing, and the feeling of “thick rubber bands wrapped all around her feet” even when lying in bed. At its’ worse, she feels an “electric cattle prod” sharp and constant pain. She no longer uses a walker to walk in the community due to the pain she experiences when weight bearing, amplified by problems she has poor balance and the risk of falls, and having reduced physical tolerances for standing and walking.

She experiences frequent episodes of her knees “buckling”. Dr. Johnson, Orthopaedic Surgeon, diagnosed progressive degeneration of her knees comprising of bilateral patellofemoral degeneration, medial compartment collapse, and osteophytes (more left than right). He has indicated that she may require joint replacements in the future, although her history of septicaemia could preclude joint replacement in the near future. She reported that the edema in her legs, knees, and feet prevents the fitting of an appropriate brace to support her knees. While her knee problems are not directly related to the injuries and aftermath complications of the motor vehicle accident, her inability to obtain appropriate treatment for her knees is directly due to the physical and medical complications that followed her accidents.

Ms. Perrozzino’s personal safety is a significant concern when she is in her home and in the community. According to her husband, she has frequent falls whether standing or sitting because she will fall over while sitting. He reports an episode where she fell out of her walker seat, landing on the floor and injuring her coccyx. She reported falling off the toilet and not being able to rise from the floor one night. She stated that she did not want to call her husband for help because he had to work the next day. He found her asleep on the bathroom floor the next morning. Her husband reports that she frequently loses her balance while ascending and descending the stairs.

Upper Extremity Problems – She reports decreased cervical and shoulder range that limits reaching activities. She reports upper extremity numbness, loss of strength, and poor dexterity that requires a variety of assistive devices to accomplish simple activities of daily living. All activities take her longer to complete. She requires assistance to enter and exit her bathtub for bathing, and reports poor dental hygiene because she is unable to floss her teeth. At the time of our meeting, she had four dental appointments to crown a fractured tooth. She requires increased dental visits for dental cleaning.

Neurogenic gastro paresis – Ms. Perrozzino requires intravenous Zofran to control nausea and reflux because of gastro paresis. Her gastrointestinal symptom has resulted in a 40-pound weight loss. She had increased nausea and stomach pains on February 15, 2003, in which she vomited for over 10-hours and taken to hospital. Examination revealed an infected gall bladder that required removal through laparoscopy surgery the next day. The anaesthesiologist intubated her while she was awake because he wanted her [to] let him know if he was overextending the neck and compressing her spinal cord further. She remained on a respiratory machine for two days in ICU. She reports having significant pain, before, during, and after this episode. She reports that the incision ruptured three times before healing, and that she now has a small intestinal hernia.

Porth-a-Cath Management – As reported last year, thrombosis of her PICC Line resulted in the insertion of a porth-a-cath, which she continues to require. The life expectancy of porth-a-cath use could extend from nine to 68 months; however, studies indicate that the average time for the presentation of complications (infection, tachycardia, and thrombosis) was 10.7 to 13.07 months, requiring the removal of the catheter. There will be a need to return to PICC line administration if Ms. Perrozzino develops complications in the future. As seen in the prior Life Care Plans, the cost for Picc Line nursing, supplies, and medication administration is a larger expense than that of porth-a-cath management.

Neurogenic Spastic Bladder – Ms. Perrozzino self-catheterizes because of urine retention and bladder spasms. At the time we met in December 2004, Ms. Perrozzino was concluding Septra antibiotic therapy for another urinary tract infection. Her medical and prescription records indicate that she experiences frequent urinary tract infections that require careful attention to hygiene practises for bladder care equipment. She reported frequent nighttime incontinence and uses adult diapers to prevent soiling her bed.

Neurogenic Spastic Bowel – Ms. Perrozzino reports increased bowel irregularities, including rectal bleeding of which she is awaiting an assessment by Dr. Champion, GI Specialist. She reports having haemorrhoids due to bowel straining and constipation, and an excoriated anal area due to constipation caused by medication and irritation caused by her practice of digital disimpaction.

Apnea, Narcolepsy – Ms. Perrozzino has Sleep Apnea, in which she uses a Bi-Pap machine due [to] prevent oxygen desaturation. She experiences disturbed sleep [disturbance] due to pain, nausea, and episodes of narcolepsy. They described incidents where she would fall to sleep while in the bathroom attending to her personal and medical care, wherein she would fall forward and hit her head on the desk that is set across her. In one incident, while sitting on her walker, she fell to sleep [asleep] and fell to the floor. Mr. Perrozzino reports a number of narcoleptic episodes in 2004, including her falling to sleep [asleep] in a cafeteria

after purchasing a sandwich, while in an elevator, and while sitting in a snack shop with her husband and son. In another instance, she fell asleep in the car and could not be awoken enough to walk into the house. Reportedly, she stayed sleeping in the car from 2:00 p.m. to 11:00 a.m. (nine hours) before the family were able to get her to manage going into the home. I observed a narcoleptic incident when Ms. Perrozzino excused herself from our meeting to use the washroom and fell asleep on the toilet. After a half-hour wait, I asked a home support worker to check on her. These events support the importance of Ms. Perrozzino having 24-hour attendant care to ensure her safety from risk of falls because of her narcolepsy. She ingests Alertec to combat narcoleptic episodes.

Dermatological Problems – Ms. Perrozzino continues to have skin ulcers and contact irritant dermatitis that initially began on her hands as an allergic reaction to medication and medical care supplies (adhesives). She has scarring on her face, nose, and breast. She uses numerous medicated creams to manage open sores.

Respiratory Problems – Ms. Perrozzino's immune system is poor, leaving her susceptible to colds and pneumonia that has required hospital attention and increased attendant care.

Cardiovascular Health – Dr. Chandler diagnosed an enlarged left ventricle. She reports variable blood pressure due to the medications she requires for her pain, edema, nausea, and neurological difficulties. Her problems with labile blood pressure, reported to Dr. Skinner in October 1993, indicate varying from a low 60/30 to highs 230/120. According to Ms. Perrozzino, Dr. Chandler "is somewhat convinced that her edema in her legs and arms is of a neurogenic origin".

Carpal Tunnel Syndrome – As previously reported, nerve conduction testing taken in April of 2003 confirmed bilateral ulna nerve damage and she underwent carpal tunnel surgery to her right wrist on October 7, 2003. She will require further surgery to her left wrist and will not be able to manage her own personal care following surgery. This is another instance where 24-hour attendant care support would be essential to ensure that Ms. Perrozzino receive the appropriate level of care.

Cognitive Functioning – Previous neuropsychological testing confirmed problems with immediate and visual memory and mental multi-tasking (multi-factorial – pain, medication, sleep disorder), which will affect Ms. Perrozzino's capacity to self-medicate and care for her physical self in the future. When interviewed in January 2004, Ms. Perrozzino would often go off on a tangent out of context of the conversational topic. She appeared to have difficulty staying on topic and the interview process became more difficult as she fatigued. Her husband reports that she will make errors in purchases (gift shopping for a baby

girl when the gift should have been for a baby boy). She insisted on having her husband attend our meeting this month because she is forgetful about details.

Attendant Care Needs – The January 2004 Life Care Plan indicated an increased need for attendant care support due [to] Ms. Perrozzino susceptibility to falls and reduced capacity to be independently mobile. Between 2001, when first assessed for a Life Care Plan, and January 2004, when last assessed, she advanced to requiring (100%) total assistance for stair climbing, (100 to 75%) maximal assistance with ambulation, (50%) moderate assistance with wheelchair locomotion and (25%) assistance with sit and stand transfers and balance. Her Attendant Care needs increased from 60 hours per week in April 2001 to 81 hours a week in early 2003.

Despite an increased need for attendant care, Ms. Perrozzino reports that she reduced private attendant care in the fall of 2003, and stopped hiring attendant care support by the spring of 2004 because of limited financial resources to afford attendant care. The family has instituted a Life Line system, in which she can call for help in emergencies. While this program is helpful for calling for help, the immediacy of receiving help is dependent the monitoring station reaching someone to attend her at her home.

Presently, CCAC provides a home support worker for 7-hours a week, and a nurse visit, twice weekly for about an hour each visit. Ms. Perrozzino pays for housekeeping at 5 hours a week. Therefore, there are only 14 hours of attendant care provided by outside providers a week, augmenting her husband's evening, night, and weekend attendant support (110 hours). She is alone for over 30 hours a week, where she is bedridden while she awaits her husband to return from work to assist her.

Analysis of Records – Ms. Perrozzino attended 34 medical and diagnostic appointments between March and December 2003. Ms. Perrozzino requires attendant care support to get to community appointments and is not always able to rely on her husband as he copes with full time employment. She requires both an attendant care provider and accessible vehicle to get to appointment or the use of private ambulance services to provide transportation given that the likelihood of her ever being an independent driver in the future is unlikely.

As previously mentioned, I observed extensive edema in both of Ms. Perrozzino's legs from knees to feet where the lower part of her legs were purple in colour and the skin was cracking in the spring of 2003. At that time, Ms. Perrozzino also described that the bottom of her feet are hyper sensitive whenever she is weight-bearing. Additionally, she underwent surgery for ingrown toenails on December 9, 2003 due to the edema in her feet. I was advised that Dr. Greenberg recommended nursing support to change medication creams and dressings, twice

a week; however CCAC was unable to meet that request for additional nursing services.

[246] During cross-examination and re-examination, Ms. Monks explained that she took stronger medication after her 1998 motor vehicle accident than before. Her reference to having taken strong medication prior to the 1998 motor vehicle accident was referring to the fact that she was mixing over the counter Robaxacet with Tylenol 3. She had not taken narcotic medication.

[247] On the matter of the emergency room doctor's notes at St-Vincent's Hospital in Erie, U.S.A., Ms. Monks pointed out the errors in the report, concerning a neurological exam. She stated no neurological exam had been made by the attending physician, and the examination report, which noted "neuro – no focal deficits", listed also "follow-up with physician consider further evaluation of paresthesias or pain" (Exhibit 5). Ms. Monks also indicated to defence counsel that the emergency department note, on the page that followed, contained another error. It had been impossible to do as described the morning of the 1998 motor vehicle accident, namely: "go down an embankment – walked and went to a hotel". Ms. Monks stated she and her family got off the highway by the nearest exit in a police vehicle and they were taken to the town, to a motel.

The Evidence of Dr. Vandewaetere – 1998 Motor Vehicle Accident

[248] Dr. Vandewaetere saw Ms. Monks on January 15, 1999. Ms. Monks' last visit had taken place on July 22, 1998. Dr. Vandewaetere testified that she had assumed Ms. Monks had no need to see her during that six months period. (I have already reproduced Dr. Vandewaetere's note for that visit, in reviewing Ms. Monks' evidence for January 1999.)

[249] Dr. Vandewaetere stated that the impact of the 1998 motor vehicle accident had to have a significant force to cause the injuries that she saw on Ms. Monks' breasts. She also stated that she had to worry about what the accident did to Ms. Monks by way of neck extension. Ms. Monks' complaints about pain in her left arm were new, as she had not received such a complaint prior to the 1998 motor vehicle accident. Dr. Vandewaetere's report to Ms. Monks'

counsel, dated September 5, 2002 (Exhibit 1, Tab E-1) covering Ms. Monks' visits to her office from May 11, 1999, is as follows:

Suzanne was next seen at my office on May 11, 1999. She was complaining of some low back pain with radiation to her right posterior hamstring thigh and I wondered if she had a right L5 nerve root irritation. She was having a lot of migraines. She had seen Dr. Chow who ordered another MRI of her neck and was scheduled to see Dr. Lesiuk that week on June 21, 1999.

She was seen for a physical exam. She had undergone her MRI and Dr. Lesiuk wanted her to consider surgery for her canal stenosis. She was finding it nearly impossible to keep up with her work load. She had a lot of neck pain and headaches and required Tylenol # 3 and flexeril to sleep. She was not tolerating physiotherapy well. She had some reduced range of motion of her cervical spine.

Suzanne's condition quickly deteriorated. She was no longer coping at work. She was in terrible pain. She had weakness in her arms and much reduced stamina. She was anxious about her condition.

At the July 8, 1999 visit we reviewed the MRI of her c-spine done on June 10, 1999. It showed a "C4-5 broad based posterior disc protrusion that was mildly compressing the anterior aspect of the spinal cord, slightly more prominent on the right of midline similar to that of May 25, 1997.

At C5-C6 a large left paracentral disc protrusion retrodisplaced and moderately compressed the spinal cord more to the left of midline. There was an increase in this discal protrusion, as well as, an increase in the degree of cord compression since the 1997 examination. There was a suggestion of a small zone of increased T2 and decreased T1 signal in the spinal cord at the C5-C6 level suggesting a small area of cord edema reflecting the increasing degree of cord compression". This was particularly alarming as it signalled spinal cord injury for the first time.

At C6-C7 there was a central disc protrusion extended more to the right compressing the anterior aspect of the spinal cord thereby producing a milder degree of cord compression and central spinal stenosis than demonstrated at the C5-C6 level.

I was alarmed at these findings and worried that the damage to her spinal cord could be permanent. Her pain was not controlled with Tylenol # 3 and so percocet was added.

Dr. Lesiuk wrote to me a letter on August 11, 1999. He too felt that the MRI changes were highly significant as these suggests that "neural tissue damage was

beginning to manifest itself". He felt there was a clear indication for intervention and he recommended a 2 level decompression surgery at C5-6 and C6-7.

Mrs. Perrozzino-Monks also felt that her pain and symptoms had increased so significantly, since the automobile accident of December 1998, that she felt she could no longer avoid surgical intervention.

She was seen again on July 22, 1999 when she informed me that she was being scheduled for surgical decompression on August 16, 1999. She stated that Dr. Lesiuk had said that "with the amount of spinal cord compression she had, that she must get surgery or paralysis would be a certainty".

She requested a wheel chair, walker, hospital bed and bath seat and chair post-operatively.

She was seen one more time on September 20, 1999. Apparently, the surgery had been rescheduled and was to take place in a week's time. She was quite anxious about the whole thing and requested some ativan, as well as needing renewals of her other medications.

Suzanne was seen post-operatively on November 30, 1999. Her post-operative course was complicated. She suffered from intractable vomiting, urinary retention and inability to void without catheterization, ongoing right sided weakness both upper and lower extremities and ongoing severe pain. She was seen by a variety of specialists in hospital including the "pain team" and was given multiple medications to help with her nausea and pain. She was in physiotherapy to try and help with her right sided weakness. I was worried that the surgery had not been able to alleviate the damage caused by the December 1998 motor vehicle accident. Since the surgery had not alleviated her symptoms, I feared that the spinal cord had been irreparably damaged and that Suzanne would have permanent catastrophic and irreversible spinal cord injury.

On her next visit December 23, 1999, we had a discussion about her pain medication. Suzanne was finding it difficult to work but she was trying nonetheless. She was falling asleep at her computer. She was only able to write 5 mins. at a time and 30 mins. at the computer.

She was still self-catheterizing and the urologist had told her to be patient and continue doing this for 6 months more. Dr. Lesiuk had done an MRI one week prior to our visit and said that the surgical area was healing and had suggested she begin some "gentle physio" in January/2000.

On her next visit January 20, 2000, Suzanne was completely dejected because in spite of her efforts, she did not feel she could continue working. She simply

had too much pain. She was completely exhausted. (The result of weakness post surgically, pain and medication). She was self-catheterizing 3-4x per day. Her nausea was continuous and she vomited 0-3x/day. She was unable to eat solids and had lost 9-10 more pounds. She was very upset with the realization that she could no longer work. "I was in tears. I can't believe that I couldn't keep going."

She was using both a wheel chair and walker to mobilize. She could mobilize for 10 mins. with the walker and if she took a rest she could continue for 30 mins. Her legs would cramp and fatigue and "just won't go". Then she would switch to a wheel chair. She was having great difficulty getting up stairs.

Her right arm was fatiguing quickly and could not even hold a book more than 15 mins. and computer use was limited to 15 mins. She also had a sensation of numbness both in her right hand and forearm and right leg below the knee.

Her husband assumed all of the responsibilities for house work and child care. He had to help Suzanne with most of her self-care that required above the shoulder hygiene and getting dressed. She stated that her MRI had in fact shown permanent damage.

On the visit of February 14, 2000 Suzanne was very upset because she had had to leave her work. It had been very difficult for her to come to the realization that she actually could not work. She found herself crying easily which is unusual for her. Her bowels were not moving without enemas, nor her bladder without catheterization. She was in constant pain and had constant nausea. Her balance was poor. She was taking a multitude of medication and physio to try and alleviate her symptoms and her prognosis was a "guess" according to Dr. Lesiuk. They would have to wait 6 months to allow for her healing to be complete but he expected she would likely not return to normal function. Lastly she felt trully "depressed" as anyone would under these circumstances and she was placed on disability for a minimum of 6 months to allow her the most rest and treatment while awaiting the final outcome of her post-operative healing.

In summary, this previously vibrant, active, strong, energetic hard working and intelligent woman not only is incapable of working she is no longer able to care for her home, her husband, her children or herself. She cannot walk properly, use her arms, or go to the bathroom (bowels and bladder). She cannot eat normally because of nausea. She can't read because the books are too heavy to hold for longer than 15 minutes and she fatigues. She cannot use the computer more than 30 minutes at a time. She is in chronic severe pain and is exhausted and dejected. I believe this woman's life was catastrophically altered.

I also believe that the motor vehicle accident she was in on December 23, 1998 was the final blow to her spinal cord and was the proverbial “straw that broke the camel’s back”.

Her condition continued “as is” and she received further cervical laminectomies in January/2001 in hopes of bringing about a better outcome. Suzanne spent from March 1 to April 12, 2001 in the Rehabilitation Unit but in the end her condition did not improve. She is on a host of medications to control her symptoms including zofran, neurontin, fentanyl, zoloft, wellbutryn pantoloc, urispas, dulcolax, microenema, lasix and morphine.

The need for these medications required a more permanent IV access. Suzanne therefore received a PICC line. Unfortunately this PICC line became thrombosed and infected and so she took coumadin to prevent further clotting of her PICC line. Since the PICC line was so much trouble, it was decided that she really would be better off with a porta-cath.

Unfortunately she suffered a pneumonia and septicemia while waiting her porta-cath surgery likely due to PICC line contamination, infection and required months of IV medication.

[250] Dr. Vandewaetere continued to see Ms. Monks at her office throughout March, April and June 2000. She concentrated on getting the correct medication to control Ms. Monks’ pain, nausea and vomiting, and she referred Ms. Monks to Dr. Scully, a gastroenterologist, concerning Ms. Monks’ problems with gastric emptying. On August 9, 2000, Ms. Monks had complained to her that she fell backwards in her house and that her neck was stiff. She noted that Ms. Monks’ hip was weak, she has positional imbalance, and was tender in the cervical spine area. Ms. Monks had only 50% in range of head motion to the left. Ms. Monks had been on a diet of milkshakes, according to Dr. Vandewaetere’s notes of September 15, 2000, as “it is the only food Ms. Monks can keep down”.

[251] Following Ms. Monks’ surgery in January 2001, Dr. Vandewaetere has noted “incomplete quadriplegia”, which she stated meant that all symptoms of a quadriplegia are present, but not with the same severity. Ms. Monks had maintained some functions, she could move her arms, she can void, and she can walk.

[252] The notes Dr. Vandewaetere made of Ms. Monks' May 7, 2001 visit are elaborate. Ms. Monks had two swollen feet (Bi-Pedal Edema), had a lot of pain and was allergic to the Fentanyl patches.

[253] She had prescribed more medication to Ms. Monks to control the water in Ms. Monks' body. She noted that Ms. Monks had reported falling down the stairs in her own home. She referred Ms. Monks to Dr. McKimm concerning her snoring. She explained that Ms. Monks was suffering from congestive heart failure. The heart is a pump and the inflow has to equal the outflow of fluids. If the inflow is stronger, fluids accumulate in the lungs and the rest of the body and typically in the lower limbs. The accumulation of fluid in the lung gives a person congestive heart failure and it affects a person's respiration, causing shortness of breath and oxygen desaturation.

[254] Ms. Monks' visits to Dr. Vandewaetere continued almost monthly, throughout 2001 and 2002, and Dr. Vandewaetere's notes reveal the problems Ms. Monks enumerated in her evidence and Dr. Vandewaetere's efforts to control them.

[255] During cross-examination, Dr. Vandewaetere admitted that Ms. Monks had blood pressure problems prior to the 1998 motor vehicle accident. She also stated that she was unaware that Ms. Monks had requested Zurich Canada to pay for surgery prior to her last accident and that the insurance company had refused. She agreed that she did not ask Ms. Monks to go through her symptoms between Ms. Monks' visit on June 19, 1998 and Ms. Monks' accident in December 1998 and, as a result, would not know what had transpired during that period of time. Dr. Vandewaetere explained that her reference to Ms. Monks' significant accident in December 1998, came from the fact that Ms. Monks' breasts were bruised, not only from the seat belt, but by hitting the steering wheel during the accident. Dr. Vandewaetere agreed that the reference to Ms. Monks' suffering of neurogenic pain did not appear in her notes between Ms. Monks' January 15, 1999 visit and Ms. Monks' first surgery in September 1999. The first time neurogenic pain is mentioned is on June 5, 2000, following the first surgery.

[256] Defence counsel asked Dr. Vandewaetere if the reference in Exhibit 1, Tab A-5 (Dr. Lesiuk's August 11, 1999 report) about "cramping pain in the upper extremity and weakness and clumsiness in the right arm" identified symptoms that existed before the 1998 motor vehicle accident. Dr. Vandewaetere replied that Ms. Monks had not presented to her cervical pain nor cramping before the 1998 motor vehicle accident. She refused to attribute Ms. Monks' gastric problems to her obesity.

[257] On mobility, Dr. Vandewaetere pointed out that she visited Ms. Monks at her home, as sometimes a home visit provides the physician with a wealth of information, as it did in this case. Dr. Vandewaetere stated that the medication taken by Ms. Monks might have caused Ms. Monks' sleep apnea, according to Dr. McKimm.

[258] During re-examination, Dr. Vandewaetere stated that the 1998 accident likely caused the changes seen in Ms. Monks' 1999 MRI investigation, because she had the symptoms and after the 1998 accident, those symptoms increased in character and distribution. Ms. Monks had developed headaches and required medications. She did not have the ability to cope with work at a time when she was keen on her work and had just started a new company. Dr. Vandewaetere pointed out that even after prescribing Ms. Monks the percocet medication, a medication 10 times stronger than Tylenol 3s, she had not managed to get Ms. Monks' pain under control prior to her September 1999 surgery.

Evidence of Dr. Lesiuk and 1998 Motor Vehicle Accident

[259] Dr. Lesiuk summarized his involvement with Ms. Monks following her last motor vehicle accident in a report to Ms. Monks' counsel, dated May 29, 2001, and found in Exhibit 1, Tab A-10.

I reviewed the situation again on January 28, 1999. She had not seen Dr. Fehlings (although I had previously received confirmation from Dr. Fehlings' office that an appointment was forthcoming). Mrs. Perrozzino-Monks had been involved in a second motor vehicle accident on December 22, 1998. She was the driver of a sport utility vehicle that struck a bridge abutment on a slippery road. She experienced an immediate exacerbation of her symptoms. The character and distribution of symptoms was unchanged but the severity was significantly worse, particularly in regard to the weakness and clumsiness in the right upper extremity. Her exam findings were unchanged except for a questionable increase in weakness of the right finger extensors. She found that she was having great difficulty coping with her work duties, which involved considerable keyboarding. This represented deterioration in functional status. Because of these changes I recommended repeat MRI (carried out on June 10, 1999).

I reviewed the situation again on August 11, 1999. Mrs. Perrozzino-Monks felt her symptoms were continuing to insidiously worsen. The new MRI showed that the previously documented disc protrusion and C5/6 had increased significantly in size, causing correspondingly worse spinal cord compression. There was slight signal change within the spinal cord tissue suggesting edema and perhaps demyelination. I felt this to be highly significant as it indicated neural tissue damage was occurring and represented a clear indication for surgical intervention. In addition, at the C6/7 level the right-sided component of the previously documented disc protrusion was worse increasing the degree of compromise of the right C7 root. I had reviewed the clinical situation and MRI with my partner, Dr. Ben Guiot who concurred with my assessment. We both agreed that surgical decompression from an anterior approach addressing both the C5/6 and C6/7 levels was indicated. I recommended this to Mrs. Perrozzino-Monks and she agreed to proceed.

The decompression was carried out on September 27, 1999. At the C5/6 level the decompression was accompanied by anterior interbody fusion with a Cloward style cylindrical dowel graft. At C6/7, which was more difficult to access because of the patient's body habitus, a Smith-Robinson style rectangular graft was used. Following the surgery the patient manifested a number of significant problems:

- Ongoing cervical pain, which required an extended course of parenteral narcotic analgesics prior to discharge.
- Severe nausea, recurrent vomiting, and intermittent abdominal pain. A gastroenterologist (Dr. Scully) was consulted and undertook investigation and management of this. A clear cause for this has not been elicited. Autonomic dysfunction associated with her neurologic injury may play a role. It has remained a substantial problem ever since and she is compelled to markedly limit her normal food intake

and to maintain adequate nutritional balance with concentrated liquid dietary supplements.

- Urinary retention, seemingly neurogenic in origin, which has been followed by a urologist and has required Mrs. Perrozzino-Monks to perform intermittent catheterizations of her urinary bladder several times a day ever since.
- Unsteadiness of gait.
- Easy fatigueability.

Post-operative radiographs and CT images of the cervical spine demonstrated good decompression at C5/6 and C6/7. There was good bony alignment and progressing graft incorporation.

When I reviewed the situation on December 19, 1999 there had been no improvement in any of her symptoms. She felt the pain and unpleasant dysesthesiae in the right upper extremity were worse. Urgent repeat MRI was obtained on December 15, 1999. This revealed an adequate decompression with CSF both in front of and behind the spinal cord at the decompressed levels. The amount of room for the cord at the C4/5 level, above the decompression, was marginal but felt adequate. The area of cord injury documented pre-operatively at C5/6 was again seen. Conservative treatment, including physiotherapy was undertaken.

Through the course of the year 2000, Mrs. Perrozzino-Monks' clinical status again insidiously worsened and by the end of November:

- Her neck and back pain improved very incompletely with a variety of analgesics, pain clinic techniques and physiotherapy techniques (she had a long course of outpatient physiotherapy from January to December 2000).
- Her range of cervical motion remained moderately restricted in all directions.
- Her walking tolerance deteriorated; she was limited to about 5 minutes of walking with the aid of a walker before she was compelled to stop and rest.
- Her sitting, standing and walking balance deteriorated, she walked with a wide based gait, required the walker for stability and was subject to multiple falls.

MRI images were obtained on March 31, 2000 and again on October 16, 2000. My review of the latter was delayed by my own extended illness and I did not see Mrs. Perrozzino-Monks until November 28, 2000.

- Clinical exam revealed:
 - Right-sided weakness in both the upper and lower extremities (grade 4/5 proximally and 4+/5 distally with a giving way component due to induced pain).
 - Diffuse subjective alteration to pin and touch below the elbows and knees bilaterally but worse on the right.
 - Diffusely brisk deep tendon reflexes with bilaterally positive Hoffman reflexes but flexor plantar responses.
 - Somewhat unsteady wide-based gait.
- There had been very slow but clear functional deterioration despite a variety of conservative measures.
- The MRI showed:
 - At C4/5, continuing presence of a spondylitic bar, which coupled with related degenerative changes, resulted in decreased AP canal diameter, flattening of the spinal cord, and moderate stenosis of the neural foramina.
 - At C5/6, a flattened cord (although no evidence of extrinsic compression was present, this having been relieved by the previous decompression) and some foraminal stenosis.
 - At C6/7, residual spondylitic bar formation laterally with flattening of the spinal cord, and moderate stenosis of the neural foramina.
 - Continuing signal change in the spinal cord tissue, possibly more diffuse than previously.

Mrs. Perrozzino-Monks was understandably desperate with her clinical status worsening despite the best efforts regarding conservative therapies. There was some residual neural element compromise displayed on the MRI although this was not severe. The possibility that relief of this might at least arrest symptom progression was raised although a high probability of success could not be forecast. After considerable discussion Mrs. Perrozzino-Monks and I concluded that we would proceed with the decompression from a posterior approach.

On January 11, 2001 I carried out the posterior decompression, which involved: partial laminectomy at C3, full laminectomies at C4-C7 and bilateral foramenotomies at C4/5, C5/6 and C6/7. Subsequent to the surgery:

- She experienced a severe exacerbation of her cervical pain and muscular spasm that took many weeks to settle. This likely arose from the muscle dissection attendant to a posterior decompression in the setting of her chronic pain syndrome.

- A prolonged period of inpatient rehabilitation was undertaken, first in the alternate care unit at the Civic campus, then at the rehabilitation centre.
- Her clinical status has remained stable in recent months without substantial improvement or deterioration.

With regard to her functional status, I would consider her to be completely disabled from any form of gainful employment. The factors are:

- Chronic severe pain in the neck and upper extremities, markedly exacerbated by any attempt to increase activity level beyond sedentary.
- Cervical myeloradiculopathy with:
 - Loss of strength and dexterity in the upper extremities, worse on the right. This markedly impairs her ability to write or keyboard and dramatically increases the time and effort required for most basic activities of daily living.
 - Gait ataxia with decreased balance and stability requiring a walker for mobility involving even short distances.
 - Easy fatigueability with markedly decreased walking tolerance, even with the walker.
 - Urinary retention requiring routine bladder catheterization.
- GI tract dysfunction causing recurrent nausea and vomiting and requiring limitation of solid food intake and liquid nutritional supplements.

I believe these limitations constitute a catastrophic impairment for her.

With regard to her stage of recovery:

- It has been over four months since her surgical intervention and there has been no subsequent trend to improvement.
- She has participated in and been discharged from a subsequent inpatient rehabilitation program at the rehabilitation center, having been felt by the rehabilitation team to have reached the point of maximum benefit from the program.

There is not therefore any reason to expect further substantive improvements in her clinical status and I believe she should now be classified as having achieved maximum medical recovery.

[260] During cross-examination, Dr. Lesiuk was asked if he thought Ms. Monks' case was a case where he should defer to Dr. Hansebout's opinion. Dr. Hansebout, a neurosurgeon, has

prepared a defence medical report and testified at trial. He disagreed with that suggestion, stating that he had several advantages over Dr. Hansebout, as Dr. Hansebout had never met Ms. Monks and had never viewed the films of her MRIs. On the other hand, he had followed Ms. Monks through the years and had discussions with other doctors, including her family doctor.

[261] When asked what he thought of Ms. Monks' difficulty while shopping in 1998, he dismissed the incident, saying he himself has a hard time following his wife when they go shopping. He agreed that Ms. Monks' irritable bladder correlated with neck pain. He reminded defence counsel that when she saw orthopaedic surgeon, Dr. Donald Chow, on April 1, 1999 (Exhibit 1, Tab D-2), she had stated that "her arm symptoms are tenfold more than they ever were" from the 1995 accident.

[262] Dr. Lesiuk was told that Dr. Hansebout testified Ms. Monks should have had surgery prior to the 1998 motor vehicle accident. Dr. Lesiuk maintained that there is room for different opinions, but he remained resolute in saying that the classic indicators were not there. That, he explained, is why he had Ms. Monks visit Dr. Bourque, a neurologist, for confirmation. Dr. Lesiuk expressed the view that Dr. Hansebout's opinion looks good with hindsight.

[263] Dr. Lesiuk observed during cross-examination that a Zurich Canada payment for surgery was not a welcome indicator for surgery. When asked what critical role does reporting immediate numbness and weakness play in assessing a patient, he replied that symptoms go to future management of the patient. The fact that symptoms do not appear right away does not preclude the development of symptoms later. He disagreed with Dr. Hansebout, stating that it is impossible for the accident to lead to future neurologic injury without the symptoms appearing at the time of the accident. (This factor was critical for Dr. Hansebout, but not for Dr. Lesiuk.) Dr. Lesiuk disagreed also with the suggestion that neurologic symptoms do not necessarily generate pain. He cited the presence of cervical strain and soft tissue injury as capable of causing pain.

[264] A collision, taking place at high speed, is more likely to cause spinal injury than a collision at low speed, but Dr. Lesiuk observed that both types of collision cause spinal cord

injury. He gave the example of a hockey player sliding into the boards at walking speed and producing a spinal cord injury.

[265] When Dr. Lesiuk was referred to a CT Scan (Exhibit 6, Tab 8), he observed that such an investigation is inadequate as a case study of a signal change, as you can hardly see the spinal cord on the scan. On Exhibit 6, Tab 5, the MRI shows the signal change in the spinal cord ... the signal change is subtle; it is a change in water content – edema – a swelling that can be reversible, but usually it results in some degree of permanent injury.

Dr. Hansebout and the 1998 Motor Vehicle Accident

[266] His evidence at trial is similar to his comments contained in his written report, found in Exhibit 1, Tab G-9, page 38. His interpretation of events is as follows:

- **Role of 1998 MVA**

Dr. Lesiuk had the obvious advantage of having seen the patient both before and after the 1998 motor vehicle accident, which is the accident in question in this particular lawsuit.

*Comment: He ascribes most of the patient's difficulties to progressive dysfunction due to spinal stenosis [the precursor to spondylotic myelopathy]. He seems reluctant but felt it reasonable to ascribe some blame to the 1995 accident. He mentioned (May 21 2001) that the character and distribution of the symptoms was **unchanged after the December 23rd 1998 accident.** In fact, the exam is unchanged except for a questionable increase in weakness of the right finger flexors. Dr. Lesiuk also indicated that this problem had begun before the second (1995) accident.*

Comment: In a subsequent letter [2002] Dr. Lesiuk indicated that the 1998 motor vehicle accident had caused substantial problems in the progression of the patient's symptoms. This appears to be in contradiction to his feelings of May 29th, 2001.

Comment: If the 1998 accident caused the urogenic bladder, this should have occurred immediately after the accident and then gradually improved. Her course of progression is completely different and instead one of gradual worsening. In fact, she appeared to worsen after each decompressive surgery. This is unusual, since patients usually improve thereafter if there are no operative complications. None are mentioned in the chart but Dr. Lesiuk mentioned a

number of postoperative problems after the September 27, 1999 decompressive surgery, probably causing the prolonged hospitalization.

Comment: The patient was relatively well after the December 23rd, 1998 MVA. She was able to care for her family, walk to a motel for use of a phone, initially refuse treatment, continued her trip after the accident and in addition was able to work for many months after the December 23, 1998 MVA. After a traumatic incident the worst damage, dysfunction and pain occurs right after or within a few days after an accident, not months later.

On the other hand cervical spondylosis causes many of the signs symptoms and time course displayed by this patient's course of progression. In fact the MRI done about 6 months after the 1998 MVA does show some worsening of the C5-6 disc protrusion. This may have occurred even in the absence of the 1998 MVA.

Similarly, the bowel and bladder dysfunction is gradual in this case. This is the usual progression of spinal stenosis and spondylotic myelopathy. If an accident such as that of December 1998, had caused these problems, one would expect them to have occurred immediately.

The physician (Dr. Engel) at the hospital who saw her after the accident said the diagnosis was history of degenerative disk disease and acute cervical strain.

The patient was able to continue on her trip into the USA, in her van after the accident in 1998.

She was able to return to her usual work after return from vacation. After the 1998 accident the patient developed progressively more difficulty maintaining her responsibilities in the home. She did not take time off work for any significant period and tried to continue the maximum expected of her. In fact, she was often exhausted after a long drive at work. (Dr. Besemann).

Dr. Roussev pointed out that the 1998 accident did little to increase her objective problems. The problems she has are consistent with naturally progressing cervical spinal pathology.

Comment: I agree with his assessment and queries. However, I disagree that the 1998 accident was the most severe since for example she was off work 1 month after the 1993 MVA and did not lose any time (to the best of my knowledge) after the 1998 MVA when she returned from her holiday.

Taking everything into consideration, it seems that the accident of December 23rd, 1998 had a very minor role in causing the patient's current disability.

[267] During cross-examination, Dr. Hansebout agreed that he made an error in his written report, where at page 30 he had stated that Ms. Monks had reported numbness in her legs to Dr. Lesiuk on April 6, 1998.

[268] Looking at St-Vincent's Hospital report, found at Tab 5, and the recorded statement, "there was no weakness or numbness", Dr. Hansebout was asked to look at Exhibit 3, Tab 1, page 18; it contained Ms. Monks' statement to Dr. Vandewaetere that indicated "initially at the accident, both arms were numb and tingling, both arms and a heavy feeling for six hours". Dr. Hansebout was asked if he thought Ms. Monks would have gone to see Dr. Vandewaetere and told her something that wasn't true. Dr. Hansebout replied: "No". Dr. Hansebout was asked why, if numbness and tingling were key symptoms in his assessment, he had not mentioned it in his report.

[269] Dr. Hansebout stated that when he wrote his report, he had trouble reading Dr. Vandewaetere's notes, but agreed that the fact both arms were numb and tingling, could be an indication of potential involvement at the scene. He agreed that Ms. Monks might have been seen by the doctor, as late as six hours after the accident.

[270] Dr. Hansebout also agreed that Ms. Monks' reference to heaviness in the arms could be an indicator of potential nerve involvement, as well.

[271] Next, Dr. Hansebout was taken through the mechanism of the accident. He thought he was being referred to the police report, but defence counsel pointed out that the police report doesn't tell what had happened in the car. Ms. Monks in her evidence stated that on hitting the bridge abutment, she was thrown forward. Dr. Hansebout was reminded that Ms. Monks had bruises across her breasts that lasted six months, and Dr. Hansebout agreed it was a significant forward motion. Next, the witness was told that Ms. Monks' glasses flew to the back of the van, suggesting a flexion injury. Dr. Hansebout agreed that the going back caused an extension. Dr. Hansebout stated that it might not have been a big motion, as Ms. Monks is a big person and was "probably close to the steering wheel". "I think she was close to it." Dr. Hansebout was reminded that he was speculating.

[272] Ms. Monks had reported she had suffered numbness in her arms for 15 minutes after the 1995 motor vehicle accident, to Dr. Snook, several days after the accident. As nothing on numbness was recorded by the hospital doctor in 1998, Dr. Hansebout had stated, in his written report, that the 1995 motor vehicle accident was more severe. He was asked again, if Ms. Monks' later reporting of the same symptoms to Dr. Vanderwaetere, one month after the accident, did not matter. Dr. Hansebout agreed that it is crucial, to determine causation, to know what happened at the scene of the accident. He also agreed that, had he met with Ms. Monks, he could have clarified that matter. Then Dr. Hansebout argued with defence counsel that, because the statement about numbness had not been made in the emergency department in the Pennsylvania Hospital, and because Dr. Vandewaetere was only a family doctor and not a neurologist, he could not be sure the statement was made. Dr. Hansebout had to agree that Dr. Vandewaetere had to be trusted as being able to write it down.

[273] Again, Dr. Hansebout had to admit that he got his facts wrong in his report when he stated that Ms. Monks was off work for one month after the 1995 accident.

[274] Dr. Hansebout was then referred to the report of the triage nurse at the Pennsylvania Hospital, at page 3 of Exhibit 5. The report stated: "went down into embankment and patient walked and went to hotel". Dr. Hansebout was then told that Ms. Monks in her evidence had stated that she was taken to the motel by the police in a cruiser. Next, Dr. Hansebout was told that the trailer weighed 30 to 40 lbs. and that the road was icy. Dr. Hansebout agreed that he had diminished the role of Ms. Monks' 1998 accident because Ms. Monks had walked away from the scene of the accident and had helped her husband push a trailer, whose weight he had grossly exaggerated. He admitted that, had he interviewed Ms. Monks, he would have had his facts right.

[275] The witness' report at page 31 and again at page 38 made a bold and underlined citation from Dr. Lesiuk's May 2001 report: "the character and distribution of the symptoms were unchanged after the December 23, 1998 accident". Dr. Hansebout admitted that he cut out of the citation that, following the 1998 motor vehicle accident, Ms. Monks' symptoms had undergone "exacerbation magnitude change". Dr. Hansebout's excuse at trial was that his report would

have been lengthened and that it was the longest report he had ever written in his life. Again, he agreed with plaintiff's counsel that the exacerbation was relevant to his assessment and that he had left out the increased severity of Ms. Monks' symptoms. In the same vein, Dr. Hansebout did not mention that her left arm, in January 1999, was now in the same state as her right arm had been before the 1998 motor vehicle accident.

[276] Dr. Chow's report of April of 1999 (Exhibit 1, Tab D-1) had mentioned that Ms. Monks' arm symptoms had been ten times worse than they were after the 1995 accident. Dr. Hansebout was asked why his report only mentioned that Dr. Chow had sent Ms. Monks for physiotherapy, but had left out that the reference that the symptoms had increased ten times. Dr. Hansebout had to agree that it was a reference to magnitude and that it was relevant, and he had in fact left it out of his report.

[277] As Dr. Hansebout had questioned Ms. Monks' credibility, he was reminded that Dr. Lesiuk would testify and say that she was an excellent historian and very reliable. Dr. Bourque in his report (Exhibit 1, Tab G-2) noted that Ms. Monks was: "pleasant, cooperative, Caucasian woman in no acute distress. She appears to be a consistent and reliable woman". Dr. Gérin-Lajoie (Exhibit 12, Tab 17) stated in the physical examination, "she has a very cooperative manner and very talkative. Under cognition – on each assessment, she remains alert and well oriented; she provides excellent detail and attends well to the interviews; her concentration is very good and her memory for recent details is very good".

[278] Dr. Collins (Exhibit 12, Tab 18) described Ms. Monks as a "pleasant and cooperative individual". "She was alert and articulate and provided a highly detailed history of her medical condition that belied significant memory problems."

[279] Dr. Attack (Exhibit 1, Tab G-5, page 9) also reported that his "neurological examination revealed a pleasant, cooperative individual, who gave a factual and consistent history".

[280] Dr. Weinberg (Exhibit 1, Tab G-4, page 4) also noted, “an obese, alert, coherent person in no obvious distress. I reviewed her history with respect to activities and find it generally consistent with the report of Smith-Bradley, occupational therapy, dated 2001”.

[281] Dr. Hansebout had to agree Ms. Monks was a good historian. He also had to agree with plaintiff’s counsel that he had implicitly stated that Ms. Monks’ story was full of inconsistencies. He was reminded that he had cited the report of Dr. Roussev, who had been retained to prepare a report for Zurich Insurance, in Ms. Monks’ litigation for the 1995 motor vehicle accident and who had stated: “many of the findings of Ms. Monks are not consistent with the injuries she sustained”. Dr. Hansebout had underlined his citation from Dr. Roussev’s report to support his assessment that Ms. Monks’ story was full of inconsistencies and discrepancies. Dr. Hansebout had to agree that the discrepancies Dr. Roussev talked about had to do with Ms. Monks’ symptoms and had nothing to do with what she had to say. Dr. Roussev had not met Ms. Monks.

[282] Next, Dr. Hansebout had referred to the report of Dr. Weinberg, another defence doctor who had met Ms. Monks and had come to the conclusion that there was “a functional overlay”. Dr. Hansebout defined functional overlay, as having some signs and symptoms that you cannot explain based on objective findings. Dr. Hansebout agreed that Ms. Monks had not misled any doctors.

[283] Dr. Hansebout also had to admit that he overreacted to Dr. Attack’s statement that there is also some sudden-give weakness of upper extremity groups, which he believed could be overcome entirely by suggestion. Dr. Hansebout admitted that Dr. Attack was physically pushing on Ms. Monks during the examination and when the test was done over again, the result was fine.

[284] Dr. Hansebout had mentioned that Ms. Monks had a billing of \$280,000 for one year. Dr. Hansebout said he was astounded and did not believe that to be true. He had to correct himself at trial when plaintiff’s counsel pointed out that the Court had received evidence that Ms. Monks had a large amount of work that was billed prior to 1996, added in that year, and that the \$280,000 comprised not only 1996 work, but work done in previous years. It is commonly

called work in progress. Dr. Hansebout admitted he used this fact to question Ms. Monks' credibility. He also admitted that he had not, in all the reports and charts he reviewed, found any discrepancy. While two defence doctors referred to functional overlay, both Dr. Lesiuk and Dr. Vandewaetere had vehemently refuted that theory.

[285] In dealing with the cause of Ms. Monks' problems, Dr. Hansebout remained resolute in blaming her progressive degenerative disc disease problems as the cause for her deteriorating condition. Dr. Hansebout stated that if you have spinal myelopathy, it is too late for surgery ... a patient doesn't get better.

Dr. Bruce Stewart – 1998 Motor Vehicle Accident

[286] Dr. Stewart stated that there was a limited amount of change after the 1998 accident and there was a definite progression before that accident. He also stated that there could have been a good decision to operate before the 1998 accident. In his report, at page 11 in Exhibit 1, Tab G-7, Dr. Stewart states: "only a small portion of that impairment can be attributed to December 23, 1998 accident, if any. I would say that it is 10% or less."

[287] Dr. Lesiuk had said about the 1998 accident that "he felt it was a minor contribution, but significant". Dr. Stewart, replying to a question by defence counsel, admitted he agreed with Dr. Lesiuk, but only on the word "minor" and not on "significant".

[288] Dr. Stewart was asked to describe what Ms. Monks had told him about the 1998 motor vehicle accident. He said what follows:

A. As we know, she's on her way to visit a sister in Kentucky, she was driving this time with her seatbelt. There was an accident, which the vehicle struck an abutment on the right front. She said she didn't strike her head at all, but her glasses flew backwards.

She had chest pain, neck pain and shoulder pain; I gather those are the acute symptoms that she described. The airbags didn't go off. She admitted she was concerned about her mother who was in the vehicle.

An ambulance came to take the mother, not her, but she went to the hospital with the mother. And while she was there, she was examined. And she told me then that she thought after that accident there was some greater weakness in the right upper extremity. Those are the symptoms referred to.

Q. Now again under this heading "Accident of December 23, 1998" is this information that Ms. Monks would have provided to you?

A. Yes, and it's in my notes.

Q. Okay. Did she make any mention to you at all about experiencing numbness or tingling following the accident?

A. None at all.

Q. Now, if we accept - - and I'll have the registrar please put before you Exhibit No. 5, which is the records from the Pennsylvania hospital.

A. If [we] accept the commentary in the second page which indicates "has no weakness or numbness", and under "Neural" in the bottom left corner it says "No focal deficit". Does this play a factor in your opinion?

A. In?

Q. In your opinion? In your overall opinion?

A. Oh yes.

Q. How so?

A. How so?

Q. Yes.

A. Well she's told the doctor that she had previous neck pain and problems, so he knew about that. And I'm sure the Court has heard about the visual analogue scale of measuring pain, we try to get patients to use that; she's obviously familiar with it. She put her pain to this doctor as a level 4 - - this is pre-accident pain.

After this accident, in December '98, she said it was worse that day. There was no weakness or numbness reported to the doctor. And the right breast was painful. Those are the symptoms described.

Now, when this gentleman examined her, he found no focal - - that I believe refers to neurologic deficits, that's paralysis or increased weakness, or any weakness, any sensory deficit, any reflex change, or any new pathological signs. There are a number signs in neurology, Babinski signs that are high indicators of spinal cord injury or nerve injury.

So he found, essentially, no significant abnormality neurologically. He did find that the neck was tender in the lower posterior muscles, which one would expect even without the accident, but she's saying there's greater pain this day. And everything else was normal. It says "extremities: no sign of trauma". So he has diagnosed acute cervical strain. There was an x-ray obtained as well, which showed some narrowing at two disks levels, the lower two in the cervical spine, C5-6 and C6-7, with small osteophytes. There's no sign of fracture, no dislocation. So in my opinion, this is very helpful in determining the degree of trauma and what actually occurred at that time. The notes made by a physician or an experienced person, immediately or shortly after an incident of any type are probably the most valuable of all documents in determining the degree of trauma in an incident like this.

Q. Now, you made mention of this pain scale and you indicated the Court had probably heard about it. Could you please just provide us with a description of that pain scale and what a 4 would represent?

A. Yes, it's extremely simple. A visual analogue scale, which we refer to as VAS, runs from 0 to 10: 0 is what you and I have now, no pain at all [all]; 10 is the worst imaginable pain in the world. One can very easily put one's pain on that level.

Unfortunately some people say they have pain at 15 and 20 level, and they don't understand the thing. But what they're trying to say is this pain is terrible.

So 0 to 10 is the level. And she has, in my opinion, at 4, modest pain, that would be annoying through the day at 4.

Q. And in terms of that depiction of 4, is that something that the doctor would have depicted, or is that something - - -

A. No. That is her subjective assessment of it. It's very difficult to evaluate levels of pain other than seeing objective signs of limitation and discomfort and grimacing.

Q. Now if I could just get you to jump to page 8 of your report, and specifically to paragraph 4 of that report - - of that page. You are mentioning the

visit that Ms. Monks has to the family doctor on January 15, 1999. Is this something that you would have considered in reaching your opinion?

A. Yes.

Q. And how would it affect your opinion, given that it appears to say something different from the hospital records?

A. Well, there has been time for the patient to go over her observations. She is a nurse, and she would want to - - I would hope she would want to provide as much detail as possible. But it is different a couple of weeks after the accident. Weakness in the right arm, fatigue in that arm, heavily feeling in the arms.

There is no mention of actual numbness, but she does mention residual tingling over the deltoid, that's the upper arm, down to the elbow, and questionably - - it's plus or minus, that could be equally a question mark, down the arm. And she thought she was worse after this accident, although that I think refers to general things.

And in this regard, she thought the right arm was the same or 75 percent of - - I take that to be power; she thought that it might be a little weak or it was normal. These are her comments to the doctor.

Still some headache, constant headache, and the neck was a little tender. These I think are observations of the doctor. She found tenderness in the neck. There is no mention of objective findings by the doctor; I assume there weren't any if they're not there. I would have to go back to the report - - Dr. Vandewaetere's note to see if there's any physical sign - - I don't think there were.

So how does this affect my opinion? We have an individual with chronic neck pain, some right arm weakness, difficulties in using that hand functionally, who has another accident which appears to have possibly produced a cervical - - at least the doctor thought it was cervical strain, with no change in the x-rays at that time. And a week later she still thinks she's worse, but again there are no objective findings found. And the features seem to be very much the same at that time, except her opinion, and she is the one who's using the arm - - she felt that that arm was somewhat weaker. But it was to weaker - - it was weak before, we know that from the notes.

The numbness here described - - or the tingling she describes was not noted before, and it follows a pattern which is - - the deltoid to the elbow would be very high - - if it was coming from a nerve root in the neck it would be very high, it would be from fifth cervical root. And I'm jumping forward and

extrapolating the areas that were operated upon later did not - - are nowhere near that root. So this is an observation that was not helpful in defining what really happened at the C5-6 and C6-7 levels of the cervical spine in this or the other accident.

...

Q. Okay. Did you find her to have any difficulty with her memory or cognitive ability?

A. Not at all. She was bright, alert, and provided this information, to the best of my knowledge, without notes.

Q. If I could ask you to turn page 4 of your report. In the third paragraph you indicate that she has constant "neuropathic" pain "everywhere". Did she give you a time frame, or would you be able to tell us the time frame that - - -

A. Oh yes. This information was presented in chronological order to me, as you can see, it flows in time. And this followed her surgery, and I believe this is the first operation - - yes, it is. So this is after the surgical procedure of September 1999.

Q. Okay. And she had also indicated that after the surgery she experienced gastrointestinal abnormalities. What is your opinion with respect to the relationship to her spinal situation?

A. In my experience gastrointestinal problems, such as nausea and vomiting, do not occur with chronic cervical injury or spinal injury.

In the acute phases of severe spinal cord trauma one can get absence of bowel movement, inability to digest food, so there's gastric atony, the stomach becomes acutely dilated. But that's a transient thing, and it's after definite damage - - severe damage to the spinal cord, and that generally clears up.

Here we have a very prolonged period of nausea and vomiting requiring various anti-emetics. And in my experience, I've never seen that in the many cases of spinal cord injury I've seen, particularly those on a chronic basis. As I say, it may occur in the acute cases that come in quadriplegics, cannot not move arms and legs, and have that kind of problem occur on an acute basis. But that clears. This I have never seen, and I do not believe it's associated in any way with the accidents here.

I must allow that this happened - - this appears to have happened - - she didn't tell me at that time, rather soon after the surgical procedure. And without being critical of the surgeon, there may have been pressure on the cord during the

surgery, which, hopefully, would not be serious, and I would not think would produce the kind of gastric disturbance that I'm commenting on after acute spinal cord injury.

But it occurred at that - - roughly at that time. She went back into the hospital with it about a month later, I think, but - - and it has continued even to date. That's very - - I don't believe it's related at all to these accidents.

Q. You mentioned in your experience that you had never seen this. Can you give the Court an idea of how many cases or patients you would have seen over your experience that would have a diagnosis of incomplete quadriplegia?

A. Yes. Over the years I would have seen a new case roughly every two to three months, that's six per year, approximately. A number of those, of course, are in hospital - - in rehab. But I see that many, and have done so over a good portion of my practice.

[289] Dr. Stewart testified that there was a psychosomatic element both in Ms. Monks' function and in her use of many drugs. He explained what he meant by that comment:

A. Well, I indirectly indicated that that might be the case with the volume of drugs here that - - She's a nurse. She complains - - again I will reiterate how important it is for a physician to feel that he's helping his patient - - his or her patient. There is often an add-on sort of problem that develops.

Now pain is one of those things that is highly subjective. Pain may have no organic basis - - it often has no organic basis. When it's described as "worse", then sometimes the doctor raises the dose of medicine or changes it.

We have Fentanyl, Fentanyl patches, Gabapentin, Statex, and Lorazepam. All of those would help to reduce pain, but it's in my opinion perhaps over done here.

I don't think that's a conscious thing on any patient's part that symptoms are psychosomatic. She may feel depressed, she may feel the pain gets her worse, she can't do things, she gets discouraged. Those are psychosomatic symptoms. They may - - they usually do not have a strong organic basis, but some do. That's what psychosomatic condition is, it's a greater response to non or minor problems. Some of these problems of course were not minor, but there is growth of drugs here that makes me - - would make me as her physician very comfortable and I would work diligently to reduce them, for her benefit, not for anyone else's.

[290] Dr. Stewart agreed that there was a significant difference between the April 1998 MRI and the August 1999 MRI. He was then reminded that he had stated during his examination-in-chief that notes taken immediately following the accident were probably the most helpful. He was asked why his report made no mention of the Pennsylvania Hospital reports. Since Dr. Stewart had not mentioned any symptoms other than what Ms. Monks had told Dr. Vandewaetere in January 1999, the Pennsylvania report then became the report closest to the time of the accident. Dr. Stewart was reminded that he had left out of Dr. Vandewaetere's notes, the report of numbness at the scene of the accident. Dr. Stewart was reminded that he had accepted Ms. Monks' statement made to Dr. Snook, her family physician, one month after the 1995 motor vehicle accident. Yet, he would not accept a report to Ms. Monks' physician, Dr. Vandewaetere, also made within one month of the 1998 accident.

[291] Dr. Stewart admitted that, in terms of spinal cord injury, both accidents present the recurrence of the same extension mechanism of the spine. He was asked if looking at the MRI (Exhibit 6, Tab 5) if there was not more disc protrusion at all levels, but in particular C5-6, which had retro displaced and moderately compressed the spinal cord. He replied that it is true that the radiologist had reported the discs at C5-6 larger. He also stated that the description of the accident was consistent with both flexion and extension mechanism of the spine. (Ms. Monks had given a description of the accident to her insurance agent on her return to Ottawa, January 20, 1999, Exhibit 40.)

[292] Dr. Stewart was told that the testimony given at trial by individuals who worked with Ms. Monks just prior to the accident had stated that she was an entirely different person after the 1998 accident. He agreed then that the changes in Ms. Monks' symptoms could not be described as minor. Dr. Stewart conceded that he had not said in his report that he disagreed with Dr. Lesiuk's designation of "significant", in describing the role of the 1998 motor vehicle accident in Ms. Monks' condition.

[293] The cross-examination concluded with Dr. Stewart remaining resolute that Ms. Monks' vomiting and nausea were psychosomatic, despite the fact that the treating physician and neurosurgeon who constantly met with Ms. Monks, disagreed with him. Dr. Vandewaetere had

stated that (Exhibit 1, Tab E-2) the neurogenic bladder and bowel needed to be self-catheterized repeatedly throughout the day. Dr. Stewart was asked whether he implied that Ms. Monks would catheterize willingly unless it was truly necessary? Dr. Stewart was reminded that he saw Ms. Monks once, for a very short period of time, when she was sick and had edema.

The Evidence of Laurie Warren – 1998 Motor Vehicle Accident

[294] She stated that Ms. Monks had called her following her December 23, 1998 accident. She recalled Ms. Monks telling her she was in a lot of pain. She saw Ms. Monks at work in January 1999. She was wearing a soft collar, her movements were stiff and she could not turn freely. She was getting up and moving about. Ms. Monks had pain and Ms. Warren stated that the pain before the accident was a seven or an eight at worst, and that Ms. Monks had been able to manage the pain. After the accident, the pain was a 10 or beyond, and it affected Ms. Monks' attendance at work. Ms. Monks was using cold and hot packs from the Canadian Back Institute (their office was in the same building), and she lay down on mats on the floor. Prior to the 1998 accident, Ms. Warren recalled that Ms. Monks had used hot and cold packs about five times during all the months they had worked together. Following the September 1999 surgery, Ms. Warren testified that Ms. Monks could not cope with work at all. Before the 1998 accident, Ms. Monks had bad headaches and had not felt well at times, but Ms. Warren stated there were no phone calls from Ms. Monks in the morning to say she was not able to come to work. Following Ms. Monks' first surgery, Ms. Monks had a maximum of two hours endurance and then had to stop for the day. She could not make up the time. She recalled that before the 1998 motor vehicle accident, Ms. Monks had worked non-stop during the day, except for lunch or supper breaks, notwithstanding headaches.

[295] Ms. Warren recalled Ms. Monks' attempt to return to work after the 1999 surgery. Ms. Monks had recognized that she would have to go on medical disability. She needed a walker, she was wearing a Miami collar, and could not commit to come to work every day. She had nausea, was vomiting and could not keep her food down. As Ms. Monks was "a work identified type", she described her as tearful when she left work and worried about the financial impact, her inability to work would have on her and her family.

[296] Ms. Warren testified that her visits to Ms. Monks fell to twice per year after the 2001 surgery and then Ms. Monks cancelled her visits because she was too sick to see her.

The Evidence of Lynn Parker – 1998 Motor Vehicle Accident

[297] She described a trip to Las Vegas with Maxine Morrison and Ms. Monks in September 1999. She testified that Ms. Monks' friends knew that the surgery scheduled for the end of that month would be invasive and Ms. Monks could die. During the shopping, it was noticeable that Ms. Monks' endurance had deteriorated. She stated that after they had done a couple of stores, Ms. Monks would have to sit down. She was exhausted and only did 50% of the shopping, compared to what she would have had accomplished in November 1998, during their previous trip. Ms. Monks did not do any driving. At Wal-Mart and Cosco, no wheelchair had been available. Ms. Monks used a wheelchair at another store. Ms. Parker confirmed that an assistance device had never been used before. She added that she had given Ms. Monks Aeroplan points to make the trip, and testified that the difference in Ms. Monks from November 1998 to September 1999 was unbelievable.

[298] Lynn Parker also testified that she was attending university between December 1998 and Ms. Monks' first surgery. She had stopped at work to see Ms. Monks and had noticed a real decline in Ms. Monks' productivity. Before the 1998 motor vehicle accident, Ms. Parker stated she was giving Ms. Monks complex files to work on and she produced good work. After the 1998 motor vehicle accident, she stated she had to place competence and diligence first before friendship, and she had stopped sending files to Ms. Monks. At Christmas 2000, as Ms. Monks was facing another surgery in January 2001, Ms. Parker stated she bought plane tickets for Ms. Monks and her family, and with Mr. Parker made a trip to California. Ms. Monks brought her walker. There had been very little shopping done and Ms. Monks was falling asleep at the dinner table. They took the same trip at Christmas 2001 and Ms. Parker stated she only saw Ms. Monks one hour each day, as she was sick all night. She described Ms. Monks formerly as an above average person and now when she sees her, it's difficult to imagine Ms. Monks is the same person.

[299] During cross-examination, she testified that Ms. Monks was wrong when she testified that she had used a wheelchair when shopping with her friends, in November 1998. The wheelchair was used for the first time on the trip before the September 1999 surgery.

Dr. Maxine Morrison and the 1998 Motor Vehicle Accident

[300] She recalled the 1999 trip with Ms. Monks to Las Vegas, just prior to her surgery. Following the 1998 motor vehicle accident, she testified Ms. Monks could not do much though they got together to exchange gifts. Ms. Monks was cancelling a lot of meetings and she also stated that Ms. Monks had trouble lifting her legs in her car. She also said that Ms. Monks was taking a great deal of medication. Prior to the 1998 accident, Ms. Monks had not expressed problems with coping. Dr. Morrison stated that she could not have driven all over Las Vegas, had Ms. Monks not been able to cope. On November 1998, although they had not done as much as on previous trips, they stayed together and helped her carry parcels. She was independent and had not used a scooter. In 1999, she left Ms. Monks at the hotel and picked her up at the hotel, and Ms. Monks had used a scooter while shopping. At Christmas 2000, she added, Ms. Monks had not gone shopping and had spent 90% of her time in her hotel room.

[301] She testified that there is a difference of night and day when comparing what Ms. Monks could do before her accident and what she can do now.

Dr. Markus Besemann

[302] Dr. Besemann was qualified as an expert in physiatry. He performs catastrophic impairment assessments for a company called Work-Wise. There are two Designated Assessment Centers (“DAC”) in Ottawa and Work-Wise is one of them. The centres are set up by legislation passed by the Government of Ontario. Dr. Besemann stated that the system is “essentially there to act as an arbitrator when there are disputes between insurers and plaintiffs regarding treatment. So a medical rehabilitation DAC is designed specifically to answer questions as to the reasonableness and necessity of specific treatments relating to accident caused injuries.”

[303] The categories of boody impairments were explained by the witness, as follows:

A. So, again, based on the current legislation, which will be modified shortly as I understand it, insured accident victims are entitled to a certain lifetime benefit threshold. And patients that have sustained what are considered catastrophic lesions are entitled to a higher threshold of lifelong benefits.

In the case of CAT DACs, there are various categories going from A to G, including complete quadriplegia, complete paraplegia, loss of an arm and a leg, loss of vision; or under category F, which relates to whole person impairment, which is a quantification using the American Medical Association Guides to the impairment of the whole person, and those are specific percentages attributed to each body system or part. To quantify that, you need to add up all those various levels.

There is also a category G, which is due to mental and behavioural [behavioural] issues, and usually that's when patients develop psychiatric or psychologic problems impacting their daily lives that impair them from functioning. And finally there is also head injuries of various severities that fall into those classifications. So there are very distinct categories.

I've given lectures on the whole CAT DAC system to lawyers in Kingston just recently last June. It's a complex system and it's meant to be fairly well established. There is always grey zones, but certainly it's meant to be quite restrictive in terms of who actually qualifies for the designation of "catastrophic impairment", given that the implications are quite significant.

The guidelines to the assessment of catastrophic impairment gives categories from A to G and the Financial Services Commission Ontario ("FSCO") publishes them.

[304] Dr. Besemann indicated that his assessments are usually confrontational with patients, as large financial consequences arise from the results of his assessments. In Ms. Monks' case, Dr. Besemann wrote a report. Because Dr. Besemann attempted to give evidence outside his report (Volume 1, Tab G-6), I have set out the part of his report on Ms. Monks' 1995 accident and 1998 accident where he discussed whether or not Ms. Monks met the definition of catastrophic impairment:

In determining whether or not this patient fulfills the criteria for catastrophic impairment, two main questions need to be answered.

1. Whether or not any of the categories are fulfilled in the definition of catastrophic impairment.
2. Whether or not causation applies, and if so, which accident was primarily at fault.

1. With respect to the definition of catastrophic impairment, category (a) indicates paraplegia or quadriplegia as being catastrophic impairments. There is no quantification as to whether or not this is partial or complete. Certainly this patient is quadraparatic at best and not quadriplegic. She, from her own description, appears to deteriorate markedly with fatigue, however, strictly speaking quadriplegia implies complete paralysis which clearly does not apply in this situation.

The only other section which would apply in this patient would be category (f) any impairment or combination of impairments that in accordance with the AMA Guides results in a 55% or more impairment of whole person.

When calculating this impairment, based on Chapter 4 of the AMA Guides, and taking into account the recommendations in section 4.3 on spinal cord injuries, one is referred to tables 13-19. Therefore these tables will be used in calculating the percentage whole person impairment.

- (a) Table 13 – patient can rise to a standing position and can maintain it with difficulty, but cannot walk without assistance – percentage whole person impairment – 20
- (b) Table 13 – patient can use both upper extremities for self-care, grasping and holding, but has difficulty with digital dexterity – percentage whole person impairment – 19
- (c) Table 17 – criteria for neurologic impairment of bladder – patient has poor bladder reflex activity, intermittent dribbling and no voluntary control – percentage whole person impairment – 25
- (d) Table 18 – criteria for neurologic anorectal impairment – anorectum has reflex regulation, but only limited voluntary control – percentage whole person impairment – 10
- (e) Table 19 – sexual impairment criteria – sexual functioning is possible, but with difficulty, lack of awareness, excitement or lubrication – percentage whole person impairment – 9

Based on the combined values chart on page 322, one obtains a total whole person impairment of 60%. Taking into account the restricted range of motion of the cervical spine and the restricted motion in the shoulders, one would add the following percentages for whole person impairment.

Cervical spine Table 76, page 118 and Table 77, page 120 and Table 78 page 122

Flexion	15°	4%
Extension	15°	4%
Right lateral flexion	10°	2%
Left lateral flexion	15°	2%
Right lateral rotation	45°	2%
Left lateral rotation	45°	2%

Shoulder Table 38 and 41, page 43

Flexion	160° bilaterally	2x1%= 2%
Abduction	90° bilaterally	2x4%= 8%

This results in a whole person impairment of 70%.

Based on the above, it appears that this patient does satisfy the criteria for catastrophic impairment in that the whole person impairment falls well above 55%.

2. With respect to causation, as most evaluators appear to agree, and as Dr. Lesiuk eloquently attempts to clarify in his summary letter dated May 29, 2001, it appears to have been the 1995 accident which was largely responsible for the subsequent neurologic symptoms which this patient incurred. The patient herself is in full agreement with respect to this conclusion, and in fact, in her history reports that the deterioration of her balance, gait and lower extremity weakness was well underway by the time the accident of December 1998 took place. She was already using a wheelchair for long distances in November 1998.

As Dr. Lesiuk has explained, no one single feature can be in itself ascribed to either one of the MVA's. Degenerative changes of this type are seen frequently outside of the context of trauma. Usually however, they do not occur at such a young age, nor do they progress with such rapidity. The significant deterioration in the C5-6 disc herniation documented between May 1997 and June 1999 may have progressed regardless of the December 1998 accident. There is no absolute way to verify this with certainty, however the patient's clinical picture of progressive deterioration of symptoms would appear to attest to gradual progression which was well underway prior to the 1998 accident.

[305] Dr. Besemann testified that through both subjective findings and objective findings, he could not find neurologic involvement in Ms. Monks' case. He stressed the importance of doing an independent medical evaluation, and that it does not matter to him if it is the plaintiff or the

defendant who orders the assessment. In Ms. Monks' case, after he had seen her the first time, his administrative director asked him, not only to determine whether Ms. Monks was catastrophically impaired, but as a result of which accident, because there were two insurers. Then Dr. Besemann stated: "She (Ms. Monks) clearly indicated to me at that time that it was her impression that most of her symptoms were due to the 1995 accident and not the 1998 accident". And in his written report, the witness wrote: "From the patient's perspective, the majority of her symptoms were well underway prior to the 1998 accident, in fact, she feels that over 50% of her symptoms were due to the 1995 accident and 25% due to the 1998 accident".

[306] The definition the witness gave to the Court to distinguish quadriplegia and quadriparesis is as follows: "quadriplegia, anyone who is plegic is completely paralysed as opposed to quadriparesis which can be a varying degree of weakness".

[307] "If we are going to consider someone catastrophically impaired, and in this particular case, it could have been on a file review alone, as I mentioned, if in fact the patient was quadriplegic. So that, by definition, means no motor function before the level of the lesion". Dr. Besemann explained the terms: "usually, if a patient has a fracture or a dislocation at a specific level and the spinal cord is injured, then below that level, the patient will be completely quadriplegic, there is no function, no motor function".

[308] Dr. Besemann testified that there was no spinal cord injury in the 1998 accident and anyone who stated the contrary "is clearly wrong". I disallowed that evidence when the plaintiff's counsel objected, because that statement was not in the witness' written report. I also disallowed questions on video surveillance, as the video was taken in 1999 and many factors, including Ms. Monks' surgeries, intervened before the release of Dr. Besemann's report in July 2001. Dr. Besemann also had to agree that, his statement that Ms. Monks was using a wheelchair for long distances in 1998 was not factually correct.

[309] Under cross-examination, Dr. Besemann agreed that his neutrality in this case had been seriously compromised, because guideline 4 of the DAC legislation was not followed. Defence counsel had Dr. Besemann agree that he had received documents ... legal documents and

insurance documents that were not listed on Exhibit 48. Moreover, defence counsel met with Dr. Besemann to prepare him for trial, and the meeting lasted one to one and a half hours without the presence of the plaintiff or her counsel. I reproduce the exchange between Dr. Besemann and plaintiff's counsel.

MR. CRONYN:

Q. And if you look at the statement of intent (Exhibit 42), that the Designated Assessment Centres must not only be neutral, they must also be perceived to be neutral, and the guideline is there to promote the neutrality of the DAC system. So you're aware of the fact that it's intended to be neutral?

A. Fully. In fact I've recently written a letter to the Ministry regarding the fact that I think DAC centres shouldn't be abolished precisely because I strongly believe in that neutrality.

Q. I tell you what, we'll get out of here faster if you just answer the questions instead of adding things on.

If you look at the next page, under C, under Communications with Parties, it will indicate that except during the DAC assessment itself, when a claimant is present for it and involved in an examination, there should be no one-sided verbal communication between the DAC and either of the parties unless the DAC confirms the details of the discussion in writing and copied to the other party, includes all telephone communications before the assessment to set up appointment, verbal reminders to the parties when necessary, to provide necessary documentation, et cetera, and all other questions that are posed to a DAC should be in writing.

Then under written it says:

"When the DAC responds to written correspondence, the response must be written and copied to both parties. And if a party corresponds with DAC, and the other side has not received a copy of the correspondence, the DAC must copy both the original letter and their response to the other party, this includes covering letters from either the insurer or the claimant and their representatives. The DAC should not provide any type of report, verbal or written, to one side or the other in advance of the final written, and the final report must be sent to both parties."

And you would agree with me that it's intended that all communications should be going out to both sides?

A. I agree.

Q. It looks like in this case at least - - and we already know that there were communications between ING and Mr. Mertick that did not occur with Ms. Monks or her counsel prior to the assessment. And it looks as though you received documents that were not sent to Ms. Monks or her counsel either?

A. I'm not aware of that.

Q. All right.

A. It's the first time I've heard this.

Q. Okay. Now, if we want to stay with guideline 4, if you look under post-DAC treatment and examinations, if you look at the second paragraph in that section?

A. Uh'hmm.

Q. And it says:
"In order to maintain objectivity and a neutral portion once DAC assessment is completed, the DAC assessors are not to permitted to provide treatment or conduct a medical/legal assessment for either claimant or insured. If the matter is not resolved after the DAC assessment and the matters continues to arbitration or court, the DAC assessor may have to testify. This assessor must remain and appear to remain neutral."

Right?

"This objective cannot be met if the assessor has provided/performed post-DAC treatment, a medical/legal assessment, or an insurer examination at the request of one of the parties."

A. Right.

Q. Now when you became involved after your assessment, which I take it was sometime in the autumn or the fall of 2004?

A. That's when I received the Summons to Witness, the exact date I have somewhere here.

Q. Yes. And you have a bundle of documents beside it. I'm just going to show His Honour. All of these documents were provided to you by Ms. Orth after that letter in November or December?

A. That's correct, within the last month.

[310] Defence counsel established that Ms. Monks, when she attended for the catastrophic assessment, did not know that Dr. Besemann would make a determination on causation, or that the defendant had asked Dr. Besemann to determine causation.

[311] Plaintiff's counsel had this to say as to whether or not defence counsel knew about guideline 4 and neutrality:

As for the suggestion that my friend was not aware of guideline 4. I'm shocked. Because during the examination for discovery of Ms. Beehler*, guideline 4 was marked as an exhibit, and it's an exhibit in the discoveries. So for her to say she's not aware of it is astounding; she was there during the discoveries.

And I'm not being defamatory when I say that she's aware of it. And to try and suggest that I have an obligation to get in contact with her, when I'm presuming that the guidelines are being observed is absurd. I had a right to assume that, Your Honour. Without having to participate or make questions, I assumed that my friend was doing the same.

Anyway, Your Honour, I will carry on, it will be subject to arguments that I'll make on that. I frankly was taken by somewhat surprise by all of this.

* Joanne Beehler was the case manager for the defendant in assessing Ms. Monks' needs and finding solutions to those needs.

[312] Dr. Besemann stated that when it comes to spinal injury, neurosurgeons and neurologists do not have greater expertise. At first, he said he had to decide "every day" the causation between two accidents, then admitted later that Ms. Monks' case was the first quadriparesis case he had had, arising from a whiplash type of injury. He also agreed that he had worked in the past with Dr. Bourque and Dr. Lesiuk, and that they are capable neurologists.

[313] Under cross-examination, Dr. Besemann agreed that he had not reviewed the films for Ms. Monks' x-rays or MRIs. Dr. Besemann also told the Court that he had destroyed his notes taken from his two interviews with Ms. Monks. He placed Ms. Monks in the 40% category of patients who do not stabilize, once they have a spinal cord compression, as Ms. Monks had in

1998. He agreed that Dr. Lesiuk had seen more than he, in terms of interviews with Ms. Monks and of viewing Ms. Monks' x-ray films. He also had to agree that Dr. Vandewaetere had not seen a deterioration of Ms. Monks' symptoms in 1997 and 1998.

[314] Dr. Besemann conceded that he had not read Dr. Vandewaetere's notes prior to issuing his written report of July 2001.

[315] The following exchange took place between Dr. Besemann and plaintiff's counsel concerning a possible misinterpretation about what Ms. Monks said that related 75% to the 1995 accident and 25% to the 1998 accident:

Q. She also said that when you had talked about the 50 percent, 25 percent item, that's at page 9 of your report, at the very bottom of the page.

A. Uh'hmm.

Q. Now, again, you had said that your notes that you made at this time with her, you would have used those to dictate your report?

A. That's correct.

Q. And you discarded your notes?

A. That's correct, that was my practice at the time.

Q. She says that what she had said to you at that point was that when she was talking about the tasks around the house, that she had reduced her tasks around the house by 50 percent after the '95 accident and then a further 25 percent after the '98 accident, and that was the context. Is that a fair way of how that came out?

A. I don't recall that. Certainly for the importance of the question, I would have noted that under my other section, under self-reported functional abilities, I mean that's where that would that have fit in normally.

Q. If you look at page 10, the very next paragraph - - sorry, not the very next, but the next one after that, you start going into difficulty maintaining responsibilities in her house?

A. Uh'hmm.

Q. So it is in quite close proximity to that whole issue, isn't it?

A. I suppose it is. But, again, if it had been clearly described as such, it would have been transcribed in that way.

Q. We don't have your notes either at this point, do we?

A. That's correct.

Q. Now, I know you talked about the fact that you're physiatrist and you have a holistic approach, but you would agree with me that when it comes down to the role of trauma and degenerative diseases of the spinal cord and the spine itself, that neurologists a neurosurgeons have a greater expertise in that field than you would?

A. I would disagree totally with that statement.

[316] Dr. Besemann agreed that Ms. Monks could have had weakness and numbness in her arms, as stated to Dr. Vanderwaetere in January 1999. Following an exchange with plaintiff's counsel, Dr. Besemann also agreed, following an exchange of view with counsel for the plaintiff, that he was in no position to challenge the findings of Dr. Lesiuk and Dr. Vanderwaetere:

Q. In terms of what she reported, both before and after, the symptoms in her arm related to tingling and numbness and weakness in her right arm?

A. Uh'hmm.

Q. When she talked about the symptoms being 10 fold, that's what she talked, right? That's what she said in this courtroom, okay. Now that is a sign, is it not, of increased nerve involvement?

A. I can't agree with that. Because if that were the case, then all my patients presenting with tingling and numbness would be presenting with nerve involvement, which isn't the case. It could be; it's part of the differential diagnosis, of course it is. But you're trying to make me say that those signs and symptoms were clearly indicative that at that time they were caused by nerve damage or nerve involvement.

Q. Let me put it this way then, Dr. Besemann, Dr. Vanderwaetere and Dr. Lesiuk, who saw her both before and after, came here and testified that from what they saw they were satisfied that this accident exacerbated her preexisting

conditions, not just pain but the nerve conditions, right? And from their observations, and you're in no position to challenge that at this point because you weren't there?

A. No, I'm in no position to challenge their observations.

The end of *viva voce* evidence on 1998 motor vehicle accident.

Causation

[317] The following is a summary of what the medical witnesses said concerning which accident caused Ms. Monk's spinal cord injury. The summary is taken from Dr. Bruce Stewart's written report and I believe it is accurate.

Re: Suzanne Monks
June 11th, 2002

All of this is summarized by Dr. Lesiuk in a report May 29th, 2001. Here, he notes that she had had a whiplash injury in 1993. There was an onset of symptom associated with a 1995 accident and an accumulation of degenerative changes in the x-rays which he attributed mostly to the 1995 accident. He notes her December, 1998 accident with an exacerbation of symptoms but no change in signs. The new MRI along with consultative opinion with a partner, Dr. B. Guyot, encouraged him to perform an anterior approach surgery.

After this operation, she continued to have cervical pain now requiring parenteral narcotics, severe nausea and vomiting, intermittent abdominal pain, urinary retention, unsteadiness of gait and increased fatiguability. A post-op CT of the neck showed a good decompression at C5/6 and 6/7. She seemed to continue to deteriorate through 2000. MRI changes in March and October, 2000, showed no definite compression of the cord, but pre-existing flattening of the cord. When Dr. Lesiuk saw her on November 28th, 2000, he thought she was deteriorating further. He therefore performed a posterior surgical decompression of the cervical spine with laminectomies from C4/C7 and foraminotomies bilaterally at C4/5, 5/6 and 6/7.

Post-operatively, she was worse with severe cervical pain and muscular spasm, which took weeks to settle. She took a lot of in-patient rehab. He therefore considered her completely disabled after this surgical procedure. It was his opinion that there had been some degenerative cervical disease in 1993. The 1995 accident was associated with a progression of degenerative changes. He thought most of her clinical syndrome was due to that accident. The 1998 accident had a

minor role, albeit, significant in his opinion. Symptoms were similar after that accident, but he thought there was further clinical and structural deterioration.

Dr. R. Roussev, neurologist, examined the documents August 13th, 2001. He noted she had degenerative spinal disease before the 1993 accident. He thought the 1993 and 1995 accidents were limited in their effect (as he says, extent). He thought there was more symptomatology after the 1998 accident although it, too, was limited in extent. He thought she had a progressive, multi-level, naturally occurring, degenerative spinal pathology, which produced very limited neurological findings until the 1998 accident and beyond. He thought the decompression associated with the first surgical procedure was partial and insufficient. The second surgical procedure was adequate. There were some functional findings. He thought, for example, that the GI symptoms were out of keeping with involvement of the autonomic system and there was a questionable basis for her self-catheterization.

There was also an examination by Dr. D. Attack, another neurologist. This is dated June 6th, 2001. Dr. Attack agreed that she had mild degenerative changes in the 1993 cervical spine films, which were obtained after a cervical sprain injury. He thought she may have had a spinal cervical concussion and a stretch injury of the cervical roots or brachial plexus in the July, 1995 accident. He felt this accident contributed to her situation when he saw her. The 1998 accident aggravated the situation. He couldn't tell whether her cervical myelopathy would have developed in the absence of any of these injuries.

Dr. Lesiuk (October 9th, 2001) disagreed with Dr. Roussev's conclusions. He noted Dr. Attack's opinion was more in keeping with his own. He noted that he did not feel her life expectancy had been altered by her present medical difficulty.

Dr. A. Weinberg, an internist, reported June 5th, 2001 that he found a substantial functional overlay that seemed greater than the objective neurological findings. He felt she had degenerative disc disease since at least 1993. He thought the effect of the 1995 accident was no greater than the 1993 accident and considerably less than that of the 1998 accident. He noted that she worked long hours between the 1995 and 1998 accident. He actually thought the progression of her symptoms and signs from 1993 to 1999 was not unusual for significant degenerative disc disease, caused by the aging process.

Dr. M. Besemann, a physiatrist, examined her in a CAT DAC June 30th and July 6th, 2001. It was his opinion that her impairment of the whole person was 70%. Therefore, she met the criteria for Catastrophic Impairment. However, he pointed out that he was unable to reliably conclude that she was catastrophically impaired on the basis of the December 23rd, 1998 M.V.A.

[318] Dr. Bruce Stewart then stated:

... she definitely has had degenerative disc disease in the cervical spine with cord compression. It was appropriate to treat her surgically and prevent further deterioration. There are comments to suggest that she has deteriorated despite the best surgical approach. That cannot be explained on the basis of cord compression and there is no other disease process known to be active in the spinal cord.

She had degenerative disc disease in 1993. This was minor. She did have a cervical sprain in the accident of 1995 which, from the history and the available notes, produced the majority of her symptoms. The 1998 accident aggravated her symptoms and contributed only a minor element, if any, to her present disability.

Dr. Lesiuk, who is closest to the situation, I believe, has provided a reasonable explanation of the associations between these accidents. I agree with him.

To answer your questions, I believe she is seriously impaired in function. I do accept Dr. Besemann's opinion that she [is] catastrophically impaired now, at least at the 70% whole person impairment level.

Only a small portion of that impairment can be attributed to the December 23rd, 1998 accident, if any. I would say that it is 10% or less. Although it is almost impossible to give a percentage figure, I would stress that that accident added a minor element, if any, to her disability.

[319] Dr. Harold Becker, assistant professor, Department of Family and Community Medicine, University of Toronto, wrote a report (Exhibit 1, Tab G-8) on October 2, 2002 and had this to say on causation:

There actually is consensus on the causation issue with all consultants indicating that the 1998 accident did contribute to some extent to the worsening of her condition and was associated on a temporal basis, at least, in her need for two subsequent surgeries and subsequent neurogenic bladder, gastroparesis and spastic quadriparesis.

Additionally, I disagree with the Work Wise analysis that Ms. Monks' presentation was not defined by the (a) criterion under Part I, Section 2 of the Statutory Accident Benefits Schedule of Bill 59, namely, the quadriplegia "definition". Indeed, Dr. Besemann is correct in that the quantification of the term "quadriplegia" is not defined; however this is a case of incomplete quadriplegia, with cord injury and evidence of myelomalacia. Whether one

observes this to result in weakness or partial paralysis is, by the same name, cord injury.

There was no myelomalacia seen prior to the 1998 accident in spite of serial MRIs. She was not confined to a wheelchair/walker prior to the 1998 accident. It does not appear on the balance of probabilities that her condition would have deteriorated further (Dr. Lesiuk himself provided a “40% probability” that this may occur; otherwise, I believe neurosurgery would have been undertaken prior to the 1998 accident. By these points, I believe that her condition was not a “crumbling skull” condition such that she would have developed in any case the quadriparesis/quadriplegia that ultimately manifested. For this reason I believe that the motor vehicle [accident] in 1998 was the material cause of the subsequent events as it occurred (prophetically) on the substrate of an extremely vulnerable cervical spine and resulted in immediate increased symptomatology and subsequent rather profound difficulties.

[320] Dr. Becker quoted Dr. Lesiuk’s assessment of the causation issue as follows:

Dr. Lesiuk provided a comprehensive medical legal report to yourself, dated May 29, 2001, in which he has very thoroughly reviewed his involvement over time in Ms. Monks’ case. I direct the reader to this report as it is indeed comprehensive and, except for his conclusion, is not in need of repeat in my report.

Dr. Lesiuk continued to support the concept that the third accident in 1998 contributed significantly to Ms. Monks’ need for surgery and her subsequent clinical presentation which involved the development of urinary retention, gait unsteadiness and increasing symptoms in the upper extremities. He concluded

“Cervical spine radiographs taken in 1993 demonstrated only minimal degenerative changes. Her present clinical symptoms are explainable on the basis of the changes shown on the serial images. The changes are degenerative in nature and there is no one feature of the MR images that can be identified as being the obvious result of trauma.

However in view of the onset of symptoms in relation to the 1995 MVA and the rather brisk pace of accumulation of degenerative changes which have been documented since, I feel that on balance it is reasonable to ascribe the most significant roles to that trauma in the subsequent development of her present clinical syndrome.

Regarding the 1998 accident, I believe a more minor but still significant role can be ascribed to it in aggravating and hastening the progression of

her symptoms (the symptoms remained similar in nature and character but the pace of clinical and structural deterioration increased).”

It was at this point I believe, that Dr. Lesiuk completed an OCF-19 Certificate in which he indicated that Ms. Monks’ injuries resulted in

“...substantive spastic quadriparesis (incomplete quadriplegia). The lower extremities are much worse clinically than the upper extremities. By the attached definitions the patient would be classified as suffering an incomplete quadriplegia (1A). I believe the sensory motor and autonomic impairments result in at least 55% impairment of the whole person.”

[321] Dr. Hansebout testified on causation and he followed what he had set out in his written report under the caption of Attribution of Disability as follows:

The letter from Dr. A. Weinberg (June 5th, 2001) attributes much more significance to the 1998 MVA than to the 1993 and 1995 MVA's. He felt that Mrs. Monks suffered from degenerative disc disease since at least 1993.

Comment: He does not really give the reasoning for this attribution. How does he account for the fact she was off work 1 month after the 1993 accident and little immediately after the 1998 accident. Also after the 1995 MVA she reduced her work hours from 70 to about 35.

If one looks at the spreadsheet [Table one] there are numerous complaints that the patient has after the 1993 motor vehicle accident. Similarly, there are a number of complaints after the 1995 motor vehicle accident. However, she appears to have few of the complaints as described above after the 1998 motor vehicle accident.

It is quite clear that even without accidents, the patient would have eventually reached her current state of disability, due to the role of the cervical spondylosis, causing myelopathy. Accordingly, I attribute the 75% of Mrs. Monks’ problems to the damage to the spinal cord from the compression due to the cervical spondylotic myelopathy, especially at the C5-6 level.

She did not get better after the initial anterior decompressive operation, done by Dr. Lesiuk. In fact, she mentioned and he confirmed that the patient developed a number of problems, possibly as a result of the operation. I would attribute 5% of her current problems to the effects of the operation, which probably prevented her from becoming quadriplegic, but nevertheless, resulted in some damage to the inner aspect of the spinal cord with neurological dysfunction.

That leaves 20% of her problems to be accounted for: Although various consultants, mainly for the plaintiff, have indicated that the 1998 motor vehicle accident played a significant role in her current disability, the patient herself in a statement to the disability assessment center personnel in 2001 did not mention the 1993 accidents role in her disability but rather that of the accidents of 1995 and 1998. She herself acknowledged that the 1995 accident was responsible for 50 percent of her problems and the 1998 accident responsible for 25 percent of her problems (Besemann 2001). I would suggest that the other 25 percent of the unaccounted for disability may have been due to the 1993 motor vehicle accident. All of this totals 95% of the patient's current disability.

There is no doubt that the accidents caused soft tissue injury, damage and pain. However, I do not believe that the accidents worsened the patient's neurological function through a spinal cord injury. By the patient's own reckoning, we are left with 5 percent of recurrent problems being due to the 1998 motor vehicle accident. However, I thought that the effects of the 1998 motor vehicle accident where [were] quite insignificant and perhaps account for about 3% of the patient's current disability problems through increased neck pain and reduced range of neck motion. Accordingly, on the balance of probabilities, the 1998 motor vehicle accident probably caused from 3 to 5 percent of the patient's current disability.

ANALYSIS OF PARTIES' SUBMISSIONS ON CAUSATION AND THE LAW

1. CAUSATION

A. Plaintiff's submissions on the evidence

The role of the two previous accidents and the pre-existing medical condition

[322] The plaintiff submits that the 1993 accident has no significant bearing on her current condition or symptoms, and can be discounted in the analysis on the issue of causation. Counsel for the plaintiff has set out his client's position, as follows.

[323] The key evidence given by Ms. Monks herself was that the injuries sustained in the 1993 accident were temporal and lasted only 3 to 4 months. After that, they had no impact on her ability to work or indeed any of the rest of the activities in her life. This was supported by the evidence of her husband, Lynn Parker, Maxine Morrison and Sandra Guest.

[324] The only diagnostic imaging that followed this accident was x-rays. There can be no doubt that an x-ray does not offer anywhere close to the same level of information, as does an MRI, particularly for a case such as this. An x-ray is two-dimensional and only offers information about the bony structures in the image. An MRI is three-dimensional and offers information about the bones, nerves, ligaments and soft tissue.

[325] The x-ray report at Tab 1 of Exhibit 6 was prepared on March 15, 1993 following the 1993 accident. It shows “a little degenerative narrowing of the C-6, C-7 discs”. There was some disagreement between the doctors who testified at trial. Dr. Lesiuk felt this x-ray was entirely normal for someone of Ms. Monks’ age at the time. Dr. Hansebout thought that she was too young to disclose such changes.

[326] Interestingly, though an x-ray was taken following the 1995 accident (Exhibit 6, Tab 2) and the narrative report is remarkably the same as that at Tab 1. Furthermore, the x-ray taken following the 1998 accident in Pennsylvania (Exhibit 5, p. 4) is also similar in its description to the previous two x-ray reports. The doctors called by the defendant attempted to suggest that the plaintiff’s condition is due to an on-going degenerative condition which started earlier than 1993 and in doing so relied on the 1993 x-ray report. However, the three x-ray reports by themselves appear to suggest that there is no change between those years whatsoever when clearly changes had occurred as disclosed MRI reports. Accordingly, it is the submission of the plaintiff that the x-ray reports are of little assistance in this case.

[327] As a result, there is no evidence to suggest that the 1993 had any real significant impact on the plaintiff’s current condition.

1995 Accident

[328] Counsel for the plaintiff submitted that the 1995 accident may have played a role in causation, since Ms. Monks suffered fairly severe symptoms as a result, but these symptoms did not constitute a total debilitating condition, and did not have as significant an impact on her life, employment, health and lifestyle as did the 1998 accident.

[329] Ms. Monks was rear-ended while she was stopped at a traffic light near Montreal, Quebec. She was pushed back and then forward in her car. Her arms went numb for about 15 minutes. She had pain in her neck and across her shoulders and she developed stiffness and headaches as the day progressed. She attempted to go to a clinic in Quebec but there was no one there who could see her. As a result, she drove back to Ottawa.

[330] Her symptoms increased – neck and shoulder pain and stiffness. Ms. Monks went to her cottage for two weeks to see if her symptoms would resolve. They did not. Upon her return to the city she went to her then GP, Dr. Snook. He diagnosed a cervical strain.

[331] She thought initially that this would have been the same as the 1993 accident and with some massage therapy and time, the symptoms would resolve. But they did not. She continued to have neck and upper back pain. She also developed weakness in her right arm and hand. She is right-handed and her work required her to do a fair amount of writing. In terms of impact, she found that it was taking her more time to do tasks she had done prior to the accident. She had reorganized her workstation to be more ergonomic with the hope that it would enable her to be more productive.

[332] She found that it had an impact on her home and family life as well. For the first time in their lives John had to pitch in and do more of the work at home and for the family. Ms. Monks estimated that he took on about 50% of the tasks at home.

[333] She found following the 1995 accident that writing for lengthy periods became increasingly more difficult. Ultimately, she confined her report writing to use of a computer. Even then she found that typing was a problem, particularly with her right hand. As a result, she purchased voice-dictation hardware and software to facilitate her work.

[334] She found that her symptoms deteriorated subtly over the period prior the 1998 accident but not to the degree that it had any more of a significant impact on her life than it had originally. She was noticing that she was more clumsy with her right hand and that she had increasing right hand weakness. She also noted some increased fatigue in her legs. More will be said of the

advice given to her by Dr. Lesiuk and Dr. Bourque in 1997 but she was well aware that she needed to be on the look-out for a change in her symptoms and in particular any intractable pain or hard signs of myelopathy or radiculopathy (terms and signs of which she understood and would have principally have included increasing numbness and/or weakness in the limbs). Leading up to the accident of December 1998, she certainly had not encountered any intractable pain and she had not noticed any significant change in terms of the symptoms in her arms and legs other than subtle changes.

[335] Her own testimony was that if she had noticed any such change she would have gone back to her doctors. The last time she saw Dr. Vandewaetere was in June 1998 and Dr. Lesiuk was in April 1998. It is reasonable to assume that she indeed would have gone back to either of them if she had felt her condition was deteriorating. She had been more than diligent in that regard, when one considers the reports of Dr. Lesiuk. In his report of August 12, 1997 (Ex. 1, Tab A1), in spite of the advice from both Dr. Bourque and Dr. Lesiuk that she was not a classic case for surgery, she still had questions and concerns and wanted a further opinion. Furthermore, the April, 1998 report (Ex. 1, Tab A3) was generated as a result of the questions she put to him, all of which clearly demonstrate that this issue continued to be on her mind and she was still very much assessing her situation and options.

[336] Both Dr. Bourque and Dr. Lesiuk were of the opinion that Ms. Monks would have known if the signs they told her to look for had appeared. She herself testified that she knew what to look for. Given her conduct to that point, it is reasonable to assume that if she had encountered any such symptoms, she would have gone to the doctors. She did not. This issue is being assessed in light of the subsequent events, which we now know occurred. Ms. Monks did not know then that she would have an accident in 1998 that it would lead to the two surgeries and the ultimate condition she is in now. She was merely concerned about the condition she was in at that time and was clearly being diligent about what was happening. If her condition had deteriorated to any great degree, she would have acted on it.

[337] While she testified that she slowed down following the 1995 accident, it would appear that she moved from an extraordinary work week to what we might categorize as a 'normal'

work week. She had been putting in work weeks in of 75-80 hours prior to this accident. After the accident, her work week was reduced to 35-40 hours. These hours are taken from the report of Dr. Lesiuk dated April 6, 1998 (Ex. 1 Tab A3, p.2). Clearly, these were the hours that she was billing in 1998. Such hours are hardly part-time and are more typical of a normal person's full week.

[338] During the period from 1995 to 1998, Ms. Monks had worked for Renaud & Senecal and during that period, she was starting up a separate entity to handle rehabilitation clients (this, it is to be kept in mind, was at her suggestion in order to better service the clients). When the owner of Renaud & Senecal decided to sell his business, Ms. Monks then left to start her own business and she was in the process of getting this off the ground prior to the 1998 accident.

[339] In addition to all of this, she also had on-going marketing and education commitments and she was unable to estimate the hours she devoted to these but she continued to maintain these functions right up to the 1998 accident.

[340] A review of her resume (Exhibit 7, p. 4) discloses that between the 1995 accident and 1998 accident she completed her AIIC Certification courses except for the last one (and solely for the reason that they would not permit her to write it with a computer). She received her Level 1 Certification from the Ontario Brain Injury Association in 1996. She also received her Life Skills Certification. She also became an Accredited Rehabilitation Professional that year.

[341] Notably in 1998, the year of the accident crucial to this case, she had taken and completed two separate courses on Threat of Violence In the Work Place and Corporate Crisis Intervention. In addition to that, in 1998, she arranged for herself and a number of other nurses to take and complete the program to become a CRRN (Certified Rehabilitation Registered Nurse). She completed the last exam to obtain her CRRN just prior to the December accident.

[342] Throughout this period she was also extremely active in the Canadian Association of Rehabilitation Professionals ("CARP"). This was confirmed by Lynn Parker, Laurie Warren and Sandra Guest.

[343] The defence medicals and the defence argument has attempted to suggest that Ms. Monks was on a steady decline leading up to the 1998 accident and that she was becoming increasingly disabled. All of this activity hardly bespeaks of such a decline and frankly suggests the contrary – Ms. Monks was still very much a going concern and frankly doing more than the average person.

[344] Ms. Monks was quite candid about the impact the 1995 accident had on her. She did not try and diminish its impact. There is no doubt that Ms. Monks was upset about what had happened to her. This is reflected in the notes of Dr. Vandewaetere. In her first visit to Dr. Vandewaetere, her condition was discussed and there was no doubt it had had an impact on her psyche. She was essentially depressed about her loss of energy and strength. This is quite understandable for someone who is driven as she was; any loss of capacity would be mourned.

[345] However, a review of Dr. Vandewaetere's notes for 1997 and 1998 though disclose that there really is not much significant change in her condition during this period. The defence attempted to place significance on the June 19, 1998 entry in which it was noted that her life was restricted, she could not type and could not use the mouse, could not do sports and could not even cake decorate. But it was clear from the testimony of both Ms. Monks and Dr. Vandewaetere that this was not a new list of problems but rather a summary of where she was and the fact that it depressed her that she did not possess all of her previous energy and power. She herself testified that she was still able to do most of the sports she had done prior to the 1995 accident, it is just that Dr. Lesiuk had told her not to because of the risk of further injury. So when she stated that she could not do sports, it was not because she was physically unable to do so but rather that she had been told not to do so for fear of further injury. This inability to be active depressed her, along with the loss of her ability to be as effective in her work.

[346] Much was also made by the defence of the wheelchair episode. According to Ms. Monks, she had gone to Las Vegas with Lynn Parker and Maxine Morrison in December 1998. During one of their 'power shopping' sprees in one of the malls, Ms. Monks indicated that she was having a hard time keeping up with the other two and so she got a wheelchair. It was her testimony that this was the one and only time that she had needed the use of a wheelchair and it

was in unique circumstances. In the defence medical reports and even in Dr. Besemann's report, there is an attempt to extrapolate more from this event than logic would dictate.

[347] Even more significant is that Ms. Parker and Ms. Morrison were both adamant in their testimony that this event with the wheelchair did not, in fact, occur during the 1998 trip but rather was in September, 1999 when they took Ms. Monks to Las Vegas just prior to her first surgery. They both remembered it because they had taken her there as a present because the surgery was risky and none of them knew what the outcome would be. It was their recollection that Ms. Monks was as energetic as usual during the 1998 trip. By the time, Ms. Monks first began to speak of this incident it was well after the surgeries and so there is reason to believe that she may well have been mistaken about the timing of this event.

[348] Either way, this event clearly does not warrant the significance that the defence would have the Court attach to it. As with many aspects of this case though, the fact that the defence attempts to expand this one-time event (which may in fact be a non-event) into an all-encompassing conclusion about Ms. Monks mobility at that time speaks more about the credibility of the defence doctors than it does about Ms. Monks.

[349] The evidence of those around Ms. Monks about what she was like prior to the 1998 accident is consistent with her own. Lynn Parker worked extensively with her; she hired her to work on her most complex files. She was impressed with the breadth of her knowledge, her work ethic and quick turn-around time. Prior to the 1998 accident, she did not see any change in respect to any of these traits. She was aware that Ms. Monks had had an accident in 1995 and that she had some neck stiffness and soreness but from her perspective there was no real impact on her or her productivity.

[350] Maxine Morrison was not as involved on a day to day basis with Ms. Monks but she was aware that following the 1995 accident Ms. Monks had some chronic pain in her neck but it did not interfere with any of her activities.

[351] Laurie Warren started working with Ms. Monks in June 1998 and worked with her on a daily basis right up to the accident and also following it. She is an occupational therapist and so perhaps better positioned than the average person to notice any deficits Ms. Monks may have had. She noticed very early on in the relationship that Ms. Monks would have to pause from time to time when using the computer to flex and stretch her right hand and arm. She also noted that Ms. Monks took Advil or Motrin from time to time for headaches and neck pain. She had been informed by Ms. Monks that these conditions arose from the car accident in 1995. But throughout the period between June 1998 and December 1998 she did not notice any change or deterioration in her condition, energy or ability.

[352] John Perrozzino also confirmed that Ms. Monks was slowed by the 1995 accident. In particular, he noted that he had had to become more involved in taking care of the family and agreed with Ms. Monks' assessment that he had taken over about 50% of the family tasks. It was his testimony that it was and is not Ms. Monks' nature to complain. But he knew that if she was asking him to become more involved that she must have had some constraints. He observed that she was spending as much as time as she ever had at work but she did tell him that it was taking longer to achieve things than before the 1995 accident. But he recalled all of the effort she was putting in to start her new business and in terms of the various education courses she was taking. From his standpoint, she did not disclose any evidence of being fatigued.

[353] He recalled that she remained active in terms of the sports she liked until Dr. Lesiuk told her that she had to be careful for fear of further injury. The only sport he recalled was affected by her condition was that she could not drive a golf ball as far as she had before this accident. Although he himself did not see this because he did not play golf with her, this was something she had told him.

[354] The defence argued that Ms. Monks was on a steady decline leading up to the 1998 accident. The evidence of Ms. Monks and those around her is clearly to the contrary. The defence attempts to grab at what amounts to nothing more than straws to support its argument. The sheer weight of the evidence of Ms. Monks, her friends, work colleague and husband was unshaken throughout the trial. I will come to the medical evidence later in this section, but it is

significant that the evidence of Dr. Vandewaetere and Dr. Lesiuk is consistent with the portrait painted of the period both prior to and following the 1998 accident. They too saw her and spoke with her. And their observations of her and what she described were consistent with her testimony and those around her.

1998 Accident

[355] The plaintiff submitted that the latest accident in 1998 exacerbated her symptoms resulting from the first two accidents, as well as causing further damage to her cervical spine and spinal cord, which required surgery to stabilize her condition and prevent further decline. Ms. Monks' symptoms worsened (and new symptoms resulted), following decompressive surgery in September 1999 to prevent further decline, although her condition stabilized. A second round of decompressive surgery in January 2001 also prevented further decline in her overall condition, but had no change on Ms. Monks' worsening symptoms. Therefore, despite the presence of pre-existing injuries related to prior accidents, the plaintiff submitted that there is a causal connection between the "catastrophic impairments" of which Ms. Monks now suffers, and the 1998 accident. The plaintiff submits that the medical evidence on this point confirms this causal connection.

[356] The accident, which is critical to this action, occurred at approximately 5 a.m. near Erie Pennsylvania on December 23, 1998. Ms. Monks was driving a Ford Expedition. Her mother was in the front passenger seat; John and their son Jonathan were in the passenger seats immediately behind them and their son Shaughn was in the very rear passenger seat.

[357] They were towing a small trailer, which contained some of their bags and Christmas presents. The weather had turned bad and was very snowy. Ms. Monks estimated that their speed had been reduced to 50 km/hr. She noted that the trailer was moving awkwardly through the snow. The bridge where the accident happened was very icy. The trailer swung over to one side of the vehicle and Ms. Monks attempted to correct this and, in doing so, lost control of the vehicle, and it struck one of the abutments on the bridge.

[358] All the occupants were wearing their seat belts; only Ms. Monks and her mother were awake. The airbags did not deploy. Ms. Monks was thrown forward by the impact and then thrown back. The force of the forward motion was stopped by her chest hitting the steering wheel or being constrained by the seat belt. Either way, it resulted in significant and immediate bruising across her chest. Those bruises remained for almost 3 months. The backwards movement was of sufficient force to cause her eyeglasses to be thrown all the way into the very back of the SUV. In short, the mechanics of this impact subjected Ms. Monks to fairly significant forward and backward movements. These were the very types of mechanics that Dr. Lesiuk had warned against in 1997 and which were set out in his report of April 1998.

[359] It was Ms. Monks' testimony that she was most concerned at the scene for the welfare of her mother and family. This was confirmed by Mr. Perrozzino and is entirely consistent with the nature of Ms. Monks. The defence and defence doctors attempted to attach significance to much of what Ms. Monks did at the scene and at the hospital following this accident. It is the submission on behalf of the plaintiff that these actions probably do not warrant so much weight in the larger scheme of the entire body of evidence but to the extent any weight is attached at all, it must be tempered by the fact that Ms. Monks was not as concerned about herself as much as those around her.

[360] In her testimony at trial, she described her symptoms at the time of impact as not quite the same as in 1995; her arms were more numb in that accident but her arms still had some tingling and heaviness. They did not "feel right". She developed increased stiffness and soreness in her neck and shoulders, along with the chest pain, which was new.

[361] Ms. Monks, her mother and Shaughn were taken by the police officer to a nearby motel. When there, an ambulance was called for her mother because she was having pain and trouble breathing. When the ambulance attendants arrived they thought Ms. Monks should also go to hospital and so another ambulance was called.

[362] Ms. Monks was seen at the hospital, was x-rayed and released. Mr. Perrozzino rented another vehicle and the family went on to Lexington, Kentucky where they spent Christmas and New Years.

[363] In terms of her symptoms, Ms. Monks testified that she had increased neck and shoulder pain and headaches and that the pain seemed “ten times” worse than before. But she also noticed more neurogenic-type pain in her right arm and that for the first time she was experiencing this pain on the left side. She was able to differentiate this type of pain from myofascial-type pain. The latter is typical soft-tissue type pain whereas the former was more like “electric shocks” or being “bitten by bugs” or a “burning” sensation. She testified that this type of neurogenic pain was increased in her upper back, down her right arm and was “substantially more” than it had ever been and was now occurring on the left side for the first time.

[364] They stayed in Lexington longer than they had originally planned because Ms. Monks was not ready to face the trip back. As it was, she was mostly in her bed during the visit.

[365] When she got back to Canada, she saw Dr. Vandewaetere on January 15, 1999. Ms. Monks reported that her state had worsened “dramatically”. She had almost constant headaches and new symptoms in the left arm like fatigue of muscles. She had an increase in neck and thoracic back pain and some ‘numbness’ also, (Ex. 1 Tab E1, p. 7). Dr. Vandewaetere was extremely concerned about this development given the reduced spinal canal reserve and she arranged for her to see Dr. Lesiuk as soon as possible.

[366] Of note, Ms. Monks first saw Joanne Beehler in January 1999 as well. She had applied to ING for accident benefits and Ms. Beehler was assigned to the case. Ms. Beehler’s notes, dated January 21, 1999 (she testified that she saw Ms. Monks the day before) indicate in last paragraph on page 1 that Ms. Monks was experiencing numbness on the right-hand side. The right arm weakness was much more evident – “Prior to accident she felt she had a decrease of 20% now she feels it is 40%” - in other words twice as bad as before. Ms. Beehler noted she has difficulty buttoning things and anything that requires strength and repetitiveness. And that when

she wakes up “she feels like she has been in a boxing match – arms are very heavy”. She has “pins and needles down the top of the upper arm and goes down the back of the lower arm”.

[367] Ms. Monks found as she returned to work that her symptoms did not get better and in fact got worse. She tried massage therapy and it did not help. Dr. Vandewaetere had given her a prescription for Tylenol 3 and a muscle relaxant. This was the first time she recalled needing prescription drugs (although there is some evidence of her having used a prescription anti-inflammatory, naproxyn, before the 1998 accident).

[368] She described her symptoms as similar to those prior to the accident but more intense. In particular, she noted more burning and tingling in the upper arm down into the hands on the right side and for the first time she was regularly experiencing these same symptoms in the left arm and down into the hands. She also felt weaker than before the accident and was dropping things and more clumsy than she had been.

[369] She saw Dr. Chow in April of 1999 to see if she should undergo any physiotherapy. He noted in his April 1, 1999 report (Ex. 1, Tab D1) that “she states that the arm symptoms are ‘tenfold more than they ever were’ from the 1995 accident. It is mainly going to the right arm all the way down to the hand. The position varies from the first web space to the middle three fingers. This is in the C6 and C7 dermatome that she is pointing to. She also has some left arm pain going down the shoulder and to the lateral aspect of the upper arm which is in the C5 distribution.”

[370] Dr. Chow cleared her for physiotherapy and she continued it right through the summer. It had some improvement in terms of increasing the range of motion of her neck but it did not reduce any of the pain in her neck and shoulders or headaches and had no effect on the neurogenic-type pain she described in her arms.

[371] She was attempting to control the pain with over the counter medications and occasionally Tylenol 3 and Flexiril (as noted in Dr. Chow’s report). However, as the year progressed, Dr. Vandewaetere provided her with increasingly more powerful medications to

attempt to control the pain. She first tried percocet and then oxycontin. Neither was able to take the pain away completely.

[372] She also noted a significant increase in fatigue. Her arms and legs were now much weaker. She was not able to walk as far and her general endurance was greatly reduced. This had a significant impact on her ability to work and on her tasks at home. In regard to the latter, she estimates that she reduced her input by another 25% and was only able to perform 'light' tasks. Mr. Perrozzino did all of the rest. In terms of her work, she was looking to Ms. Warren to take on more and she was requiring many rests during the day. She was much less efficient than before this accident.

[373] Laurie Warren noticed a significant change in Ms. Monks' work habits immediately following the 1998 accident. She was never certain whether Ms. Monks was going to come into the office. When she did come in, she had to lie down and rest throughout the day. She was only able to work for a period of up to 2 hours and then would need a rest. None of this had happened prior to the 1998 accident; she worked through whatever symptoms she had. Ms. Warren noticed that Ms. Monks was progressively worsening as the year went on. She was more and more ill and less and less able to manage. Her attendance suffered and her missed time increased over the year leading up to the surgery.

[374] Ms. Warren also noticed a change in Ms. Monks' ability to walk. Her gait was more unstable and she stumbled and swayed when she walked. Her endurance for walking was reduced.

[375] Lynn Parker noticed the same deterioration following the 1998 accident. She had a habit of going to Ms. Monks' office to work with her in the evening. She noticed that Ms. Monks was 'dozing off' at the computer and that she did not have the ability or endurance to work the way she had before the accident. She saw a decrease in her productivity to the point that she would no longer refer to her the types of files she had in the past because she had no confidence that Ms. Monks could handle them in the manner she had.

[376] Her husband also noted that she was slowing down after the 1998 accident. She was taking much longer to perform tasks. She was 'dead tired' when she came home at night. She could not walk as far and she had less energy in general.

[377] There is no doubt on all of the evidence that the 1998 accident had an immediate and lasting impact on Ms. Monks. The defence attempted to suggest that there might have been other events, which might have contributed to the deterioration of her condition. However, the defence and its doctors simply seem inclined to ignore the patently obvious difference between Ms. Monks before and after this accident. The evidence summarized above was uncontradicted throughout the trial.

[378] The defence also attempted to argue at trial that the increased symptoms following the 1998 accident were merely increased symptoms of a soft-tissue whiplash-type injury or a cervical sprain. But in looking at the summary of the evidence, there is no doubt that there was an increase in the neurogenic-type pain Ms. Monks was experiencing along with increased weakness in her arms and legs. All of these symptoms are consistent with nerve injury or involvement. Again, the defence and its doctors simply ignore this evidence.

[379] Prior to the surgeries Ms. Monks was not catastrophically impaired as defined in the *SABS-96*. However, it is to be remembered the advice given to her in the summer of 1999 following the June MRI. Dr. Lesiuk told her that given the evidence of spinal cord edema or even possible cord damage, if she did not have surgery she would most certainly have become quadriplegic. So that if the two surgeries had not been performed, she would have gone on to become a quadriplegic. This was put to all of the doctors who testified at the trial and they all agreed that this would have been her prognosis in the absence of surgery. Prior to the 1998 accident, whether she had surgery or not was elective. Following the 1998 accident, she no longer had an election and she was required to have surgery or face certain quadriplegia.

[380] Clearly, if she had not had the surgeries, she would have developed a catastrophic impairment as defined by *SABS-96*. It was Dr. Lesiuk's evidence that both surgeries were a success in the sense that the desired decompression occurred but that the deteriorating symptoms

continued in spite of the decompression. As a result, even with the surgeries, Ms. Monks has gone on to develop conditions, which clearly can be characterized as catastrophic as defined in SABS-96. It is possible that some of the current problems, which confront Ms. Monks, were complications arising out of the surgery (and certainly there is no suggestion by anyone at this trial there was any medical misadventure during either of the surgeries; it was agreed by all that this is risky surgery rife with complications). But if she did not have the surgery, she was headed for certain quadriplegia.

[381] On the lay evidence, it is the submission of the plaintiff that it is more than obvious that the 1998 accident played a material role in the ultimate catastrophic condition.

The Test

[382] The test for causation, according to the plaintiff is whether an accident “contributed materially to the ultimate condition of the plaintiff”. Her counsel made the following submissions.

[383] It is accepted by both parties that Ms. Monks’ impairments meet the definition of ‘catastrophic’ in the SABS-96. The issue is whether there is a causal connection between the catastrophic impairments and the December 23, 1998 accident.

[384] In s. 2 (1.1) of SABS-96, the definitions of ‘catastrophic impairment’ for this case are:

(a) paraplegia or quadriplegia;

.....

(f) subject to subsections (2) and (3), an impairment or combination of impairments that, in accordance with the American Medical Association’s *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, results in 55 per cent or more impairment of the whole person;

[385] Dr. Lesiuk, Ms. Monks’ treating neurosurgeon, first provided the opinion that she met the test of ‘catastrophic’ in the Application for Determination of Catastrophic Impairment (Exhibit 1, Tab A9) dated December 15, 2000. He stated that she “has MR documented spinal

cord injury resulting in substantive spastic quadriparesis” and that she “would be classified as suffering an incomplete quadriplegia”.

[386] Dr. Besemann in his report dated July 20, 2001 (Exhibit 1, Tab G6, p. 15) felt that the definition of “quadriplegia” in the regulations implies complete paralysis which does not apply in this case. Dr. Besemann concluded, however, that Ms. Monks had a resulting whole person impairment of 70% (p. 16) which meets the definition contained in s. 2 (1.1)(f).

[387] Dr. Becker, in his report dated October 9, 2002, (Exhibit 1, Tab G8, p. 12) disagrees with this opinion. He agrees that the term ‘quadriplegia’ is not defined in the regulations, but he is of the opinion that this is a case of incomplete quadriplegia with cord injury and myelomalacia. He goes in the report (second last paragraph on p. 12) to refer to the manner in which this regulation should be interpreted – “in a manner that complies with the legislative text, promotes the legislative purpose and produces a reasonable and just meaning, with ambiguities being resolved in favour of the insured person”.

[388] Dr. Becker did not testify at the trial, but the plaintiff still relies on his report and opinion. Exhibit 33 is Dr. Becker’s resume. As can be seen on page 2, he has “extensive experience in auto insurance and in the assessment and treatment of patients suffering personal injury”. He has carried out “more than ten thousand assessments over his career spanning more than a quarter century” and he has done so for both plaintiffs and defendants.

[389] His clinic, Omega Medical Associates, is a multidisciplinary practice specializing in comprehensive, interdisciplinary forensic analysis of disability and catastrophic impairment within the context of Bill 59. He has been involved in the analysis of more than one thousand catastrophic impairment cases. He was also the Chair of the CAT DAC Guidelines Working Group under the Minister’s DAC Committee.

[390] As a result, Dr. Becker brings extensive experience and knowledge of both the medical and legal aspects of the determination of whether a patient has sustained a catastrophic impairment.

[391] It is arguable therefore that given the opinions of Dr. Lesiuk and Dr. Becker that Ms. Monks meets the definition of “catastrophic impairment” on the basis of being quadriplegic, even if incomplete.

[392] In the end, it really does not make a great deal of difference to the issue of causation in this case. The question for the court is what role did the December 23, 1998 accident play in the ultimate condition of Ms. Monks, be it 70% whole body impairment or incomplete quadriplegia?

[393] A review of the applicable law will follow but in brief the test is whether an accident contributed materially to the ultimate condition of the plaintiff. It is the submission on behalf of the plaintiff that when the facts and the medical evidence is considered as against the legal test for causation, there is no doubt that the 1998 accident materially contributed to the catastrophic condition sustained by Ms. Monks. It is something that is obvious and profound and should have been to ING from the outset.

B. Plaintiff's submissions on medical evidence

[394] It is the submission of the plaintiff that on the medical evidence, it is also obvious that the 1998 accident made a material contribution to Ms. Monks' catastrophic impairments.

[395] Ms. Monks, proud as she is, did everything in her power to prevent the Court from seeing the impact this process had on her.

[396] Frankly, given the tenor of her testimony, it often became easy to forget all of what she must endure throughout the day. She still had tremendous presence of mind and understanding of the process, medicine and law. One has to wonder what someone not of her ilk would have been like when coming to this court (if he or she would have even had the endurance to get to a trial in the first place, without having compromised the case prior to trial).

[397] She was described in nothing less than glowing terms throughout the trial and in the documents. The following are examples:

- a) Dr. Vandewaetere – “She is analytical and logical and by extension tends to be stoic. She loves to work. She is one of the most “normal” people I have known and has faced this catastrophe with more grace than I can imagine.” (Ex. 1, Tab E2, p. 3)
- b) Dr. Lesiuk – “She's a very bright lady. When I first encountered her, she seemed to be very much a type A personality: hard driving, working long hours.....In short, I found her to be of above average intelligence and above average insight into the nature and concerns regarding her condition.”
- c) John Berriault of ING - “The insured is extremely stoic and motivated. She continued to work for at least 6 months post motor vehicle accident, albeit with difficulty.” And then goes on the next page “In short there is no questioning either her character, motivation or the severity of her condition.” (Ex. 40, Tab 10)
- b) Dr. Stewart – “She is a remarkably intelligent woman.” (Ex. 1, Tab G7, p. 10)
- c) Laurie Warren – pre-accident: “animated, involved, dynamic”
- d) Maxine Morrison – pre-accident: “very active, very energetic; she worked phenomenal hours”
- e) Lynn Parker – “articulate, sharp, perceptive, incomparable work ethic”
- f) Sandra Guest – “dynamic, on top of things, empathetic, caring with clients”

[398] It is also to be noted that in their reports the following doctors described Ms. Monks as a reliable and consistent historian: Dr. Bourque, Dr. Gerin-Lajoie, Dr. Collins, Dr. Atack and Dr. Weinberg. Dr. Lesiuk also testified in court to that effect.

[399] As far as her treating physicians, Dr. Vandewaetere and Dr. Lesiuk are concerned there is no doubt of the role that the 1998 accident played in Ms. Monks ultimate condition. They both are of the view that it played a “significant” role, a “material” role. This was heard in their testimony and summarized in their reports.

[400] Both doctors were very fair and candid during their testimony. They were not argumentative nor did they appear to be advocating for their patient (as was suggested may be the case by Dr. Stewart who had the temerity to make such a suggestion without even having heard the testimony of either of them).

[401] On the other hand, the two doctors who were hired by the defence, Dr. Hansebout and Dr. Stewart were anything but fair and disinterested. There is no doubt from their reports and their evidence that they were predisposed to arriving at a conclusion consistent with the interests of the defendant. They were selective in terms of the evidence they chose to rely upon and dogmatic during cross-examination. Both were inclined to draw negative conclusions about Ms. Monks. In the case of Dr. Hansebout, he had not even met her and in the case of Dr. Stewart, he had only been with her for an hour or so. As already pointed out, the observations both have made about her are completely without substance. But their willingness to do this on so little evidence is indicative of their total lack of impartiality.

[402] It was apparent that they attempted to exaggerate and expand the symptoms before the 1998 accident and to downplay the ones following it. Both had to be informed of the evidence of Ms. Monks in respect to her conditions before and after. Both seemed surprised by this evidence. In the case of Dr. Hansebout, he never met Ms. Monks and there were numerous occasions during his cross-examination where it was demonstrated that if he had met with her he could have received the correct factual foundation for his opinion. There are numerous instances in his report and his testimony in chief, which demonstrate that the facts upon which he based his opinion were not in line with the evidence heard during the trial. A review of Dr. Hansebout's report demonstrates that he came to a desired conclusion first and then chose the facts, which would support that conclusion. In cross-examination the foundation for his conclusion was destroyed and as a result so was his conclusion.

[403] Probably the most troubling example of Dr. Hansebout's selective process was the quote he took from Dr. Lesiuk's May 29, 2001 report on pages 31 and 38 that "the character and distribution of the symptoms was unchanged after the December 23, 1998 accident" and he bolded and underlined that phrase to provide emphasis. And yet that was not the complete sentence in Dr. Lesiuk's report because the actual sentence in its entirety (Ex. 1, Tab A10, p. 2) was: "The character and distribution of symptoms was unchanged but the severity was significantly worse, particularly in regard to the weakness and clumsiness in the right upper extremity."

[404] Any objective observer would immediately come to the conclusion that the choice not to include the last part of the sentence gives an entirely different impression than the one intended by Dr. Lesiuk. The only explanation Dr. Hansebout had was that he wanted to keep his 49-page report short. This is an extraordinarily hollow excuse when one looks at the repetition contained in the report and the fact that he makes completely gratuitous comments throughout such as the one at the bottom of page 8 that voice dictation could work on a less expensive computer than the one she bought (something not even within his expertise).

[405] Furthermore, Dr. Hansebout admitted that a change in the magnitude of the symptoms would be one of the factors he would have considered important in determining causation. As such, it is incomprehensible that he would choose to exclude that part of Dr. Lesiuk's observations. Indeed in his own report, Dr. Hansebout supported his conclusion on page 40 by stating that in his view Ms. Monks had greater complaints after the 1993 and 1995 accidents than the 1998 accident (which, as an aside, was clearly not the case as the rest of the evidence demonstrated). This demonstrated at one and the same time that her symptoms were important to him and that he selectively chose the ones he needed to support his opinion.

[406] He also chose to exclude Dr. Vandewaetere's notation of Ms. Monks telling her that she had numbness and tingling in both arms at the scene of the accident in Pennsylvania. He relied instead on the notes from the ER at St. Vincent's Hospital. When pressed on cross-examination, he agreed that there would be no reason for Ms. Monks to lie about this or to fabricate what she told Dr. Vandewaetere. However, he still was not prepared to give it any credence.

[407] He also did not make reference to what Ms. Monks had told Dr. Chow in April of 1999 (Ex. 1 Tab D1) that her arm symptoms were "tenfold" what they had been before the 1998 accident.

[408] In short, it is the submission of the plaintiff that Dr. Hansebout's opinion should be given little weight, if any. His report clearly demonstrates that he did what he was hired to do – provide an opinion that supported the defence position. But by the time his cross-examination

was over, it was clear that his report and opinion simply did not bear any resemblance to the true facts of this case.

[409] In the case of Dr. Stewart, he only met with Ms. Monks for about an hour and clearly did not go through in detail the symptoms she had before and after the 1998 accident. This became obvious during the cross-examination when he appeared to learn of these facts for the first time.

[410] When all of the facts of Ms. Monks activities and symptoms both before and after the 1998 accident were put to him as summarized earlier in these submissions, he turned to the Court and asked if that was in fact the evidence. The Court responded that indeed it was and he then testified that it would have an impact on his opinion. There can be no doubt that once again, the fact that he was not there both before and after to get the true facts was fatal to his opinion. He only saw the patient briefly and he too did what he was hired to do – provide an opinion which would support the defence position. By the time, the cross-examination was over, he was prepared to recant his original opinion and it is submitted by the plaintiff that the Court should so find.

[411] It is interesting to note that in his report he stated (Ex. 1, Tab G7, p. 11): “Dr. Lesiuk, who is closest to the situation, I believe, has provided a reasonable explanation of the associations between these accidents. I agree with him.” At trial, Dr. Stewart attempted to diminish the effect of this very clear statement by saying that what he meant to say was that he agreed with Dr. Lesiuk when he said that the 1998 accident was more minor. Frankly, this explanation was quite lame; if he had intended to say that, he could have easily done so and should have done so in his report and he did not. It is the submission of the plaintiff that the Court should accept the statement in his report in its entirety, which is that Dr. Stewart considers Dr. Lesiuk to be in the best position to make this call and he agrees with his opinion (which was clear that the 1998 accident materially contributed to Ms Monks catastrophic condition).

[412] Dr. Besemann was supposed to be a neutral assessor in his role of performing the CAT DAC. While he did not know that the neutrality of his assessment had been breached at the time

of the assessment, he certainly was anything but neutral in the tenor of his testimony. He too was often argumentative and combative during his testimony (a good example was the lengthy exchange when I presented to him the many reports of clear neurological symptoms reported by Ms. Monks following the 1998 accident, Dr. Besemann continued to insist that they could be as a result of some other cause – he was clearly no longer being impartial and was dogmatic in his refusal to accept the obvious). He also appeared inclined to draw negative conclusions about Ms. Monks while hardly knowing her.

[413] Also by the time he came to testify, he had reviewed all of the subsequent medical reports and it was clear that his opinion was affected by this. The neutrality of the DAC assessor, which was supposed to be preserved, was clearly breached a second time. His testimony in chief and in cross-examination was littered with references to what other doctors had observed and the opinions they had reached (more of this below).

[414] As a physiatrist, Dr. Besemann may be able to perform the assessment of whether a patient has catastrophic impairments but it is the submission of the plaintiff that he is not equipped to determine from a neurological perspective the causal role of the 1998 accident. In his testimony, he indicated that he has been doing DAC assessments since 1999. This one was done in 2001. In his testimony, he said he did roughly 40 to 50 per year. And so by the time he did this one, he might have done 80 to 100 CAT DAC's in total. Most importantly, of all of those, the only one he could recall, which involved a patient with quadriparesis such as had been sustained by Ms. Monks, was her case alone. He was asked whether he was the right person to do this assessment and he responded that as the clinical coordinator, he had to find the best person to do the assessment but as he was only one doing them for Work Wise at the time, there was no other choice.

[415] He testified that he thought he was well equipped to do the assessment and provide an opinion on causation. But what else would you expect him to say? The facts, however, demonstrate that while he may have been equipped to discern whether Ms. Monks passed the threshold of a 55% whole body impairment, he was not well-equipped to provide an opinion on causation in this case. He testified that he thought he was as equally well-equipped to provide an

opinion as a neurologist or neurosurgeon, he candidly had to admit on a number of the key issues to the neurological aspect of this case that he would have to defer to Dr. Lesiuk and Dr. Bourque.

[416] His original opinion contained in his report is probably more reflective of his ability to determine the role of the 1998 accident when he said he could “not reliably conclude” that Ms. Monks was catastrophically impaired by the 1998 accident. In other words, he was not able to say one way or the other. This was consistent with what Ms. Monks and Mr. Perrozzino stated in their testimony that he told them during the assessment that he was unable to determine the roles of the two accidents in terms of her ultimate condition. It was only when he came to trial and after he had read all of the subsequent opinions that he appeared more fortified in his opinion. His testimony must be considered tainted.

[417] During the trial, we heard that Ms. Beehler had breached Guideline 4 (Ex. 42) – a guideline to ensure the neutrality of the Designated Assessment Centre System. The statement of intent of that guideline is that DAC’s “must not only *be* neutral, they must also be *perceived* to be neutral”. The guideline is intended “to *promote the neutrality* of the DAC system”. Under section C – Communications with Parties, it is made clear that there should be no one-sided communications either in writing or verbally. In Exhibit 40, there are a number notes made by Ms. Beehler of conversations she had with Alan Mertick of Work-Wise the DAC in this case. They are at Tabs 21, 22, 30 and 33. All of these conversations took place with Ms. Beehler alone and Ms. Monks and her then counsel, Giovanna Roccamo were never advised of the conversations or their contents. In fact, it was clear from the exchanges between Ms. Beehler and her superior, John Berriault that ING wanted to make sure that the CAT DAC protocol included taking into account prior medical conditions. They had heard that this was not being done and they wanted to ensure that it would be done in this case. Clearly, this was important to ING because even before the case was submitted for a CAT DAC, both Ms. Beehler and Mr. Berriault were well aware that Ms. Monks would be declared catastrophic. Their only hope remained a determination that the 1998 accident did not cause the condition.

[418] Dr. Besemann testified that the assessment of Ms. Monks took two days and that between the two days Mr. Mertick had come to him and told him that he would be required to determine the issue of causation. ING's wishes were being fulfilled.

[419] Ms. Beehler testified that she was unaware of Guideline 4 until the examinations for discovery in this case. Whether she unwittingly breached the guideline or not is really not important. The fact is the neutrality to which Ms. Monks was entitled was clearly not honoured. One has to wonder what might have occurred if Ms. Roccamo had been made aware of ING's arguments as to the protocol in place at that time and whether prior medical conditions were to be considered or not. Also, the fact that Dr. Besemann was required to determine the issue of causation was dropped on Ms. Monks at the time of the second assessment. She and her counsel should have been made aware of the context in which she was being assessed, particularly when it was so apparently important to ING. If nothing else, plaintiff's counsel may have been able to have input as to the question to be answered by Dr. Besemann on the issue of causation. As will be seen below, it is the plaintiff's submission that Dr. Besemann did not truly understand the test for whether an accident has a causal connection and perhaps if this had been explained to him, he might have given a different response.

[420] For these reasons, the breach of Guideline 4 prior to and during the CAT DAC has in fact resulted in prejudice to the plaintiff. The DAC itself was tainted and as a result, it is the submissions of the plaintiff that the weight attached to it should be considered in that light.

[421] The defence breached the obligation to preserve neutrality contained in Guideline 4 again prior to trial. In Section A – Post-DAC Treatment and Examinations, it is provided that “if the matter is not resolved after the DAC assessment and the matter continues to arbitration or court, the DAC assessor may have to testify. This assessor must remain and appear to remain neutral. This objective cannot be met if the assessor has provided/performed post-DAC treatment, a medical-legal assessment or an Insurer Examination at the request of one of the parties. Please note that this applies to all post-DAC medical-legal assessments whether the request is to conduct a paper review and/or an actual examination.”

[422] Ms. Orth sent voluminous material to Dr. Besemann in a number of letters written to him prior to the trial, none of which was copied to plaintiff's counsel. She also met with Dr. Besemann for an hour and a half prior to the trial commencing and again on the day he testified. He also spoke with her on the phone during the trial, for approximately 15 minutes. None of these communications took place in the company of the plaintiff's counsel and indeed we were unaware of the communications at all.

[423] During the trial, the Court was referred to the case of *Lee v. State Farm*, [2003] O.F.S.C.D. No. 165, which was a FSCO decision, which touched on this very point. In that case, counsel for State Farm had wanted the DAC assessor to testify beyond what was contained in the DAC report. He delivered a further report and then included his opposing counsel in all communications thereafter. The arbitrator held that by taking all of these steps, the neutrality intended in Guideline 4 was preserved.

[424] Ms. Orth suggested that because counsel for the plaintiff knew she intended to call Dr. Besemann, it was somehow incumbent on him to contact her to be included in any communications she had with him. Counsel for the plaintiff pointed out that it is an extraordinary suggestion. He argued that counsel in his office all know that if one is doing SABS work that counsel cannot communicate with the DAC assessors prior to trial (or arbitration) for the very reason of Guideline 4 and in effect assessors must be called to testify. Counsel for the plaintiff also argued that he was entitled to assume that Ms. Orth knew this and assumed that she was calling Dr. Besemann "cold". Otherwise, in order for him to have done what Ms. Orth suggested, he would have had to assume that both she and Dr. Besemann intended to breach Guideline 4. Counsel for the plaintiff maintained that he had a right to assume that the Guideline was not being breached not the contrary. Counsel for the plaintiff argued that it became very rapidly clear during the examination-in-chief that the neutrality had been breached when Dr. Besemann attempted to expand his original opinion considerably.

[425] Ms. Orth also stated at the trial when this issue came up that she was not aware of Guideline 4. However, Guideline 4 was made an exhibit during the examination for discovery of Ms. Beehler and Ms. Orth was present at that time. As a result, she must have been aware of the

Guideline. In any event, as stated above, she should have been aware of it and ignorance of the guideline can be no excuse when Ms. Monks' right to neutrality was breached. Whether it was intended or not, the damage was already done and as the expression goes "the genie could not be put back in the bottle".

[426] There is no doubt that Dr. Besemann was aware of further information by virtue of all of what Ms. Orth had sent to him and discussed with him. He made reference in his testimony to the incident involving the golf cart. He could not have been aware of this without Ms. Orth having told him.

[427] He also indicated in his testimony that he "understood" Ms. Monks later changed her evidence on whether she experienced neurological symptoms at the scene of the 1998 accident. In point of fact, Ms. Monks did not change her evidence at all. She told Dr. Vandewaetere on January 15, 1999 that she had experienced numbness and tingling in both arms immediately at the scene in Pennsylvania. Dr. Besemann admitted that while he had originally been provided Dr. Vandewaetere's clinical notes, he had not read them (and he also admitted this in his report – Ex. 1, Tab G6, p. 3 second last para). If he had, he would have seen that in fact Ms. Monks had reported shortly after the accident that she had experienced neurological symptoms at the scene. However, it is clear that Dr. Besemann came to the trial under the impression that Ms. Monks had changed her version and he could not have formed this impression unless Ms. Orth had informed him. Whatever the exchanges were between the two of them, Dr. Besemann's view of Ms. Monks was 'poisoned' now to the point that he thought she had changed her version of the events. Any hope of neutrality was long sacrificed.

[428] He also made reference to the fact that certain doctors had reviewed the video surveillance and had questioned his percentages. Leaving aside the fact that the video was taken before the surgeries and the CAT DAC was performed after the surgeries, Dr. Besemann could not have known what these other doctors were saying without having read the reports and/or spoken with Ms. Orth.

[429] At one point, he questioned whether Dr. Lesiuk had changed his opinion between his report of May 2001 and November 2002. The only difference between the two reports was that he described the 1998 accident as being “very significant” whereas he had not used the word “very” in his earlier report. Clearly, Dr. Besemann had taken this from Dr. Hansebout’s report; it is too subtle a difference – a difference, which Dr. Lesiuk declined to accept, was a difference at all.

[430] These are just a few examples of the new information Dr. Besemann had by the time he came to trial. As submitted above, Dr. Besemann had the appearance of being much stronger in his views when he testified at trial. In fact, there were numerous occasions during his examination-in-chief when I had to object to evidence Ms. Orth was attempting to lead from Dr. Besemann that was simply not contained in his original report. As a result, again it is the submission of the plaintiff that the breach of the neutrality guaranteed by Guideline 4 taints the evidence given by Dr. Besemann and the weight given it, if any, should properly reflect this.

[431] Also important, it would appear from the evidence of Dr. Besemann that he did not fully understand the nature of the test to be applied in this case. It was his opinion that the 1995 accident was the more significant cause of her condition. This was clear from his testimony and also the second last paragraph on page 16 of his report (Ex. 1, Tab G6). But the question for the court is not whether it was one or the other or indeed which one was more. When you are considering pre-existing conditions, it really does not matter what they are or how they happened or indeed whether they or more or less than the accident in question. The real issue is whether the accident in question was a material cause and that question was not asked of Dr. Besemann and he did not provide an answer to that question. However, in both his report on page 16 and his testimony, he accepts the analysis provided by Dr. Lesiuk as to the causal connection of the two accidents. And if that is the case, then it is the submission of the plaintiff that the test of materiality has been met even on Dr. Besemann’s evidence.

[432] In cross-examination, all three doctors called by the defence were taken through the evidence relating to the pre-1998 accident medical situation. I took them through the reports of Dr. Lesiuk at Exhibit 1, Tabs A1, A2 and A3, the report of Dr. Bourque at Exhibit 1, Tab G2 and

the MRI reports at Exhibit 6 Tabs 3 and 4. They all understood that Dr. Lesiuk and Dr. Bourque had come to the conclusion that a conservative approach should be taken and that she did not present as a “classic” case for surgery at that time.

[433] Dr. Hansebout and Dr. Stewart, in their testimony suggested that in their opinion the surgery should have been done before the 1998 accident. Dr. Besemann deferred to Dr. Lesiuk and Dr. Bourque on this as being more within their expertise.

[434] It is interesting to note that in what Dr. Hansebout himself characterized as his lengthy report, he makes no comment anywhere in it that the surgery should have been done prior to the 1998 accident. When Ms. Orth attempted to lead this evidence from him, the best she could point to in the report was the quote on page 47 which stated that surgery would have been required even if she had not had the 1998 accident. That quote suggests that surgery would have been inevitable at some point after the 1998 accident. Dr. Hansebout clearly saw the reports of Dr. Lesiuk and Dr. Bourque and the MRI reports. If he were of the view that the surgery should have been performed before the 1998 accident, one would have presumed that opinion would have been clearly stated in his report. There was no reason for him not to do so but he did not. One has to question the strength of the opinion given this failing.

[435] In Dr. Stewart’s case, he too did not clearly state this opinion in his report and in fact a close reading of his report demonstrates an opinion opposite that which he tried to give at trial. Ms. Orth attempted to justify his giving the opinion that the surgery should have been done prior to the 1998 accident from the quotes on page 11 of his report where he states that he thinks “she would likely have come to surgery in a reasonable time, even if she had not suffered this December 23, 1998 accident”. He goes on to say that once the abnormalities were seen in the MRI taken following the 1998 accident were identified, then “a reasonable opinion would be to recommend decompression, even if there was no change in symptomatology at that time”. These words make it clear that it was only then – following the 1999 MRI - that surgery became necessary. This is exactly what Dr. Lesiuk had said at the time.

[436] The issue of whether the surgery should have been performed before the 1998 accident does not provide any relief to ING, in any event. As Dr. Lesiuk testified, having the surgery would not necessarily have protected Ms. Monks from the injuries she received in the accident. She may well have been exposed to equal or even greater risks of injury than she sustained. It was his opinion that you do not do this type of surgery on a prophylactic basis; i.e. to prevent the kind of injury he warned about in the April 1998 report.

[437] The fact is, as well, the surgery was not done and ING must take its insured as she was. It cannot lie in their mouth to say that she should have been in some other condition than that in which she was in at the time.

[438] In any event, all three doctors agreed that the situation prior to the 1998 accident was one which could be characterized as “elective”. I then took them through the post 1998 accident MRI taken in June 1999 and the opinion given to Ms. Monks by Dr. Lesiuk at that time which was that she had to have decompressive surgery or she was sure to become quadriplegic. They all agreed with the observations made by and advice given by Dr. Lesiuk. They all also agreed that what had been an elective situation had now become a mandatory one. And they all agreed that this was a significant change.

[439] When one looks at the evidence, the one significant event, which took place between those two periods, was the 1998 accident. Not coincidentally, it was also the very kind of event Dr. Lesiuk warned against in his report of April 1998 (Ex. 1, Tab A3). As was discussed above, the doctors called by the defence at trial all tried to diminish this event but the weight of the evidence is against them and frankly all of the evidence points quite obviously to the 1998 accident. Their dogmatic refusal to accept this speaks more about their lack of impartiality than anything else.

[440] As stated at the outset of this section, the evidence of Dr. Vandewaetere and Dr. Lesiuk is to be preferred on this issue. For a primary reason, they were present and communicating with Ms. Monks during that critical period. They knew what her symptoms were both before and after the accident. They saw her, spoke with her and examined her both before and after. They

were both clear and unhesitant in giving their opinion about the role of the 1998 accident. It was a material contributing factor to the ultimate catastrophic condition Ms. Monks now unfortunately finds herself in.

[441] They were not hired by one party or the other to provide an opinion. They were forthright in giving their evidence and were unshaken on cross-examination.

[442] In Dr. Vandewaetere's case, she had seen Ms. Monks through the whole process of receiving the initial opinions from Dr. Bourque and Dr. Lesiuk. She assisted Ms. Monks by telling her to get answers from Dr. Lesiuk in regard to issues that were bothering her about her difficult situation. That resulted in the April 1998 letter (Ex. 1, Tab A3). Dr. Vandewaetere followed her symptoms leading up to the 1998 accident. She saw her shortly after the accident and Dr. Vandewaetere's: "initial reaction to this was horror. This was the worst possible scenario for her. The last thing she needed was a further injury to her neck. To be involved in another motor vehicle accident with her spinal canal reserve being so reduced meant potentially catastrophic results." In other words, Dr. Vandewaetere knew immediately upon seeing her, the implications of the 1998 accident.

[443] She followed her throughout 1999 and recorded her increased symptoms. When she got the June MRI, she observed that the increased symptoms of which Ms. Monks was complaining matched up with the areas of increased damage on the MRI.

[444] When Dr. Vandewaetere was asked what her opinion was in regard to the causal connection of the 1998 accident, she responded that it was purely a matter of common sense. You have a woman with disc herniations at 3 levels of the cervical spine, she has been warned not to have any further trauma to that part of the spine, she comes in reporting of a further accident with increased symptomatology which then matches the MRI which discloses further disc protrusions to the point of retrodisplacing the cord and some spinal cord damage. She was required to undergo surgery or face being a quadriplegic. It was obvious to her that the 1998 had a significant impact on her condition.

[445] As for Dr. Lesiuk, his reports provide his opinions very clearly. And when he was asked for his opinion at trial, he turned to those opinions to express it yet again.

[446] In his report of May 29, 2001 (Ex. 1, Tab A10), he provides a very fair and thorough summary of the salient facts leading up to and following the 1998 accident, including his observations, analysis and conclusions and those of Dr. Bourque. He then concludes on the last page of the report:

- Cervical spine radiographs taken in 1993 demonstrated only minimal degenerative changes. Her present clinical symptoms are explainable on the basis of the changes shown on the serial MR images. These changes are degenerative in nature and there is no one feature on the MR images that can be identified as being the obvious result of trauma.
- However in view of the onset of symptoms in relation to the 1995 motor vehicle accident, and the rather brisk pace of accumulation of degenerative changes which has been documented since, I felt that on balance it is reasonable to ascribe the most significant role to that trauma in the subsequent development of her present clinical syndrome.
- Regarding the 1998 accident, I believe a more minor but still significant role can be ascribed to it in aggravating and hastening the progression of her symptoms (the symptoms remained similar in nature and character but the pace of clinical and structural deterioration increased).

[447] He repeated this opinion again in his report of October 9, 2001 (Ex. 1, Tab A11), when he responds to the report of Dr. Roussev (who was hired by Zurich in the litigation arising out of the 1995 accident).

[448] In his report of November 11, 2002 (Ex. 1, Tab A12), he expands on his opinion in regard to the 1998 accident and makes the following additional comments in regard to the 1998 accident:

- Regarding the 1998 accident, I believe it too played a very significant role in exacerbating her condition. MR imaging in the spring of 1998 (sic) did not show spinal cord injury. In correspondence from April 1998 I had noted the size of the cervical spinal canal was significantly reduced leaving “absolutely no reserve room”. I warned that further trauma; particularly if it resulted in

neck extension could lead to “sudden and potentially irreversible spinal cord injury”. Unfortunately, in December 1998 Mrs. Perrozzino-Monks was involved in such a further trauma and her subsequent rapid deterioration both clinically and radiologically has been documented in subsequent consultation notes.

[449] Dr. Hansebout tried to suggest that this was a change in his opinion between the two letters. This was put to Dr. Lesiuk during his testimony and he denied that there was any difference in his opinion. He simply expanded on it in the second letter and he stood by what he expressed in both letters.

[450] In the opinion of November 11, 2002, he went on to say that he rejected the notion that Ms. Monks spinal cord injury and consequent impairments are simply the result of natural progression as had been suggested by Dr. Roussev and Dr. Stewart. He goes on to say “I believe that both accidents contributed materially to the development and progression of her disability.”

[451] At trial, he stated at trial directly in response to the question what role the 1998 accident played, that it played a material role in her ultimate condition.

[452] Dr. Lesiuk testified at trial that if he had seen the MR image taken in June 1999, without having seen and spoken to Ms. Monks, he would have asked “Did something happen?” Because the progression of change was much greater than he would have expected to see. He also provided the opinion that the evidence of change disclosed on the MRI was consistent with the forces at play in the December 1998 accident. In addition to this, he also knew that Ms. Monks had increased symptomatology, which was also consistent with the MRI. All of this supports his opinion on materiality.

[453] Dr. Lesiuk, in his April 1998 report (Ex. 1, Tab A3) responded to Ms. Monks’ question as to what he would anticipate if she did not have surgery. He indicated that disease progression in the setting of spinal stenosis is difficult to predict with certainty. In the absence of clear myelopathic features, clinical symptom progression may occur in approximately 40% of individuals. He explained in his testimony that as of April 1998 when he wrote that report, Ms. Monks did not have any clear myelopathic features and as a result, she had a 60% chance of

remaining stable. So prior to the 1998 accident, Ms. Monks still had a 60% chance of her condition not worsening any further than it already had. If on the other hand, she did start to have myelopathic features, then there was a much greater chance of the symptoms progressing.

[454] No one called to testify at the trial disagreed with these statistics. Dr. Hansebout though testified that he thought Ms. Monks fit into the 40% category as of April 1998. Frankly, again this tends to say more about Dr. Hansebout than Ms. Monks. As Dr. Lesiuk pointed out in his testimony, he finds it hard for another doctor to give such an opinion not having seen her at the time and also with the benefit of hindsight.

[455] As for the question of whether the surgery should have been done prior to 1998, Dr. Lesiuk questioned how any other doctor might be prepared to offer such an opinion without having seen and examined the patient and without having seen the actual MR images, both of which Dr. Hansebout and Dr. Stewart did not do in this case. However, in keeping with the fair demeanour of his testimony, in the end all he said was that this was a difficult situation and one open to differing opinions. He simply did not agree with the opinion that surgery should have been done prior to the 1998 accident.

[456] In any event, he also testified that in his training and experience one does not usually perform prophylactic surgery on the spine. He described prophylactic surgery as performing surgery to try and avoid some further injury because the spinal canal is narrowed even though the patient is asymptomatic. Furthermore, he went on to testify that even with prophylactic surgery there would be no guarantee that Ms. Monks would not have sustained the same or worse injury from the 1998 accident.

[457] Dr. Hansebout, Dr. Stewart and Dr. Besemann all seemed to give much significance to the fact that there does not appear to be any evidence of Ms. Monks reporting any neurological symptoms immediately following the 1998 accident. Of course, it is the plaintiff's submission that she did report such symptoms shortly after the accident to Dr. Vandewaetere on January 15, 1999. Also the ER reports from the hospital in Pennsylvania were shown to be incorrect on several points (they had Ms. Monks driving down an embankment and walking to the motel). In

addition to that, Ms. Monks testified that she more concerned about her mother than herself and her husband confirmed this.

[458] It is important to note that all of the doctors accepted the fact that she had reported “numbness and tingling” following the 1995 accident. The accident was first reported to Dr. Snook in August of 1995 and there is no mention of these symptoms in those notes (Ex. 2, Tab 1, p.12). And yet they are not prepared to accept the fact that Ms. Monks reported numbness and tingling in both arms to Dr. Vandewaetere on January 15, 1999. It seems again that the doctors are prepared to accept what it is helpful to their arguments and reject that which is not and that this speaks more about their lack of impartiality than anything else.

[459] In any event, Dr. Lesiuk dealt with the issue of the presence or lack of neurological symptoms at the scene of the 1998 accident during his cross-examination. It was his evidence that “when we are going to the issue of the role of the accident in the development of subsequent symptoms, I don't think whether or not she has immediate symptoms or whether they develop subsequently down the road is crucial in that determination”.

[460] In the end, it is the submission of the plaintiff that the opinions of Dr. Vandewaetere and Dr. Lesiuk were the most impartial, fair and even-handed of those presented at trial and also the most well-informed in terms of the facts. Both doctors also had the benefit of seeing and examining Ms. Monks both before and after the 1998 accident. Both doctors were able to question her about her symptoms. Dr. Lesiuk saw the actual MRI and x-ray radiographs both before and after the accident and none of the defence doctors did. And both Dr. Lesiuk and Dr. Vandewaetere independently came to the same conclusion that the 1998 accident had a material contribution to the ultimate condition in which Ms. Monks finds herself – catastrophically impaired.

[461] The plaintiff did not call Dr. Harold Becker but his report was admitted into evidence (Ex. 1, Tab G8) and his resume was also made an exhibit (Ex. 33). His resume is referred to above but what is most important to his credibility is his extensive experience in performing CAT DAC's and his understanding of the test to be applied when determining causation. At the

bottom of page 11 of his report, he reviews the evidence of Dr. Attack and Dr. Stewart. He then says: “My understanding of the Athey principle is that if an accident materially contributes to an individual’s clinical presentation, then that insurer is fully responsible for the outcome even if there are pre-existing vulnerabilities associated with the individual.” As the Court will see when the law is reviewed this is a correct understanding of the test to be considered (and frankly was the issue Dr. Besemann should have been trying to determine).

[462] Dr. Becker goes on to provide the opinion that on all of the evidence the 1998 motor vehicle accident did in fact contribute materially to Ms. Monks’ clinical presentation and subsequent need for surgeries. He points out that the vulnerability with Ms. Monks came to that accident was “prophetically reviewed by Dr. Lesiuk in his last report” prior to the 1998 accident.

[463] Included in Exhibit 1 are also the reports of a number of other doctors who provided opinions in the Zurich action. It is interesting to note the views of Dr. Attack, neurologist, (who was retained by counsel for Zurich Insurance in that action) which are consistent with those of Dr. Lesiuk:

“In addition, given the natural course of this patient’s cervical spondylosis and degenerative disease one cannot say with certainty whether or not she would have gone on to develop cervical myelopathy at some point in the future even in the absence of motor vehicle accidents with cervical sprain injuries. Having said this, it is the widely held belief of many Neurologists and Neurosurgeons that these types of injuries probably accelerate the process of degenerative resulting in myelopathy and/or radiculopathy.

I believe that the 1995 accident significantly contributed to her present situation and that the 1998 accident further served to aggravate the situation.” (Ex. 1, Tab G5, last page)

[464] Dr. Roussev, neurologist, (who was also retained by Zurich for an opinion) came to the conclusion that: “The consequences of the 1993 and 1995 accidents were limited in their extent” and that the 1998 accident “was clearly the most significant of the three in terms of symptomatology”. (Ex. 1, Tab F1, p. 29)

[465] Dr. Weinberg, (who was also retained by Zurich) came to the conclusion that the “consequences of the 1995 accident were probably no greater than those of the 1993 accident and considerably less than those of the 1998 accident which materially disabled her. I note that she worked full time, long hours, during the interval between the 1995 and 1998 accidents.” (Ex. 1, Tab G4, p. 5)

[466] There can be no doubt that the weight of the medical evidence strongly and obviously supports the conclusion that the 1998 accident materially contributed to the catastrophic condition which has ultimately developed for Ms. Monks.

2. THE LAW

[467] The plaintiff relies on a number of cases to support the existence and application of this test (“contributed materially to the ultimate condition of the plaintiff”) in the present case. I will set out the plaintiff’s submissions on the law and then briefly summarize each case and comment on its relevance to the plaintiff’s submissions.

[468] The principle case in the law of causation is *Athey v. Leonati*, [1996] 3 S.C.R. 458. It is a tort case but as will be seen, the principles established in that context have been imported into the accident benefit regime as well and this case is considered as the leading authority in that arena too.

[469] The basic facts and the issues in the *Athey* case were briefly summarized by Justice Major who wrote the decision for the Court:

“The appellant suffered back injuries in two successive motor vehicle accidents, and soon after experienced a disc herniation during a mild stretching exercise. The herniation was caused by a combination of the injuries sustained in the two motor vehicle accidents and a pre-existing disposition. The issue in this appeal is whether the loss should be apportioned between tortious and non-tortious causes where both were necessary to create the injury.”

[470] The trial judge held that although the accidents were "not the sole cause" of the disc herniation, they played "some causative role" and she ascribed 25% to the accidents. This was upheld on appeal to the British Columbia Court of Appeal.

[471] Justice Major reviewed and summarized the general principles of causation as follows:

- Causation is established where the plaintiff proves to the civil standard on a balance of probabilities that the defendant caused or contributed to the injury
- The general, but not conclusive, test for causation is the "but for" test, which requires the plaintiff to show that the injury would not have occurred but for the negligence of the defendant
- The "but for" test is unworkable in some circumstances, so the courts have recognized that causation is established where the defendant's negligence "materially contributed" to the occurrence of the injury
- The causation test is not to be applied too rigidly. Causation need not be determined by scientific precision. It is essentially a practical question of fact which can best be answered by ordinary common sense. Although the burden of proof remains with the plaintiff, in some circumstances an inference of causation may be drawn from the evidence without positive scientific proof.
- It is not now necessary, nor has it ever been, for the plaintiff to establish that the defendant's negligence was the sole cause of the injury. There will frequently be a myriad of other background events which were necessary preconditions to the injury occurring. To borrow an example from Professor Fleming (*The Law of Torts* (8th ed. 1992) at p. 193), a "fire ignited in a wastepaper basket is . . . caused not only by the dropping of a lighted match, but also by the presence of combustible material and oxygen, a failure of the cleaner to empty the basket and so forth". As long as a defendant is part of the cause of an injury, the defendant is liable, even though his act alone was not enough to create the injury. There is no basis for a reduction of liability because of the existence of other preconditions: defendants remain liable for all injuries caused or contributed to by their negligence.
- The law does not excuse a defendant from liability merely because other causal factors for which he is not responsible also helped produce the harm
- This position is entrenched in our law and there is no reason at present to depart from it. If the law permitted apportionment between tortious causes and non-tortious causes, a plaintiff could recover 100 percent of his or her loss only when the defendant's negligence was the sole cause of the injuries. Since

most events are the result of a complex set of causes, there will frequently be non-tortious causes contributing to the injury. Defendants could frequently and easily identify non-tortious contributing causes, so plaintiffs would rarely receive full compensation even after proving that the defendant caused the injury. This would be contrary to established principles and the essential purpose of tort law, which is to restore the plaintiff to the position he or she would have enjoyed but for the negligence of the defendant.

[472] The respondents in the appeal attempted to argue amongst other things that apportionment is permitted where injuries caused by two defendants are divisible (for example, one injuring the plaintiff's foot and the other the plaintiff's arm). However, Justice Major observed that separation of distinct and divisible injuries is not truly apportionment; it is simply making each defendant liable only for the injury he or she has caused, according to the usual rule. In the *Athey* case, there was a single indivisible injury, the disc herniation, so division was neither possible nor appropriate. The disc herniation and its consequences are one injury, and any defendant found to have negligently caused or contributed to the injury was held to be fully liable for it.

[473] That reasoning is particularly applicable to this case.

[474] The injuries to Ms. Monks involved injuries to her cervical spine, which occurred in both the 1995 and 1998 accidents. Even if the 1993 had made some contribution to the pre-existing condition prior to the 1995 and 1998 accidents, according to the principles in *Athey*, it would not relieve the insurers from being fully liable to Ms. Monks. And certainly, the suggestion that she was pre-disposed to injury by virtue of any condition not related to any of the accidents would also be of no assistance to the defendant.

[475] Justice Major, in analyzing the case summarized what the trial judge found in *Athey*:

“The findings of the trial judge indicate that it was necessary to have both the pre-existing condition and the injuries from the accidents to cause the disc herniation in this case. She made a positive finding that the accidents contributed to the injury, but that the injuries suffered in the two accidents were "not the sole cause" of the herniation. She expressly found that "the herniation was not unrelated to the accidents" and that the accidents "contributed to some degree" to the subsequent herniation. She concluded that the injuries in the accidents "played some

causative role, albeit a minor one". These findings indicate that it was the combination of the pre-existing condition and the injuries sustained in the accidents which caused the herniation. Although the accidents played a lesser role than the pre-existing problems, the accidents were nevertheless a necessary ingredient in bringing about the herniation."

[476] Justice Major then concluded:

"The trial judge's conclusion on the evidence was that "[i]n my view, the plaintiff has proven, on a balance of probabilities, that the injuries suffered in the two earlier accidents contributed to some degree to the subsequent disc herniation". She assessed this contribution at 25 percent. This falls outside the *de minimis* range and is therefore a material contribution: *Bonnington Castings, Ltd. v. Wardlaw*, *supra*. This finding of material contribution was sufficient to render the defendant fully liable for the damages flowing from the disc herniation."

[477] This conclusion stands for the proposition that as long as there is a material contribution which is "outside the *de minimus* range" then liability will follow. Justice Major was prepared to find the defendant 100% liable for the plaintiff's damages even though the trial judge had found the accidents "played some causative role, albeit a minor one".

[478] It is the plaintiff's submission that the evidence more than clearly establishes in this case that the 1998 contributed materially and well outside the *de minimus* range to the catastrophic condition in which she has ultimately found herself.

[479] The Court in *Athey* also discussed the "thin skull" versus "crumbling skull" theories. Justice Major, in fact found that the case before the Court was in fact one of a "thin skull". He described the "crumbling skull" theory as follows:

"The so-called "crumbling skull" rule simply recognizes that the pre-existing condition was inherent in the plaintiff's "original position". The defendant need not put the plaintiff in a position better than his or her original position. The defendant is liable for the injuries caused, even if they are extreme, but need not compensate the plaintiff for any debilitating effects of the pre-existing condition which the plaintiff would have experienced anyway. The defendant is liable for the additional damage but not the pre-existing damage."

[480] It is the argument of the plaintiff in this case that there is no room for the concept of the “crumbling skull” in accident benefits cases. The essence of the theory arises out of tort law, which attempts to establish what damage a wrong-doer must pay to a party he or she injured by his or her negligence. As stated by Justice Major, a tortfeasor need only place a plaintiff in his or her original position. However, Justice Major stated that would involve making a deduction from the damage award for some percentage attributable to the pre-existing inevitable condition. There is no mechanism to do that in the context of accident benefits – either the insurer is required to pay benefits or not. There is no mechanism to reduce the benefits to some degree.

[481] In accident benefits cases, it is a matter of a contract of insurance. The test is not whether the insurer need only place its insured in his or her original position. The test is whether the insurer is required to fulfill its contract with the insured and pay him or her the required benefits. Whether the insured would have experienced certain effects in any event is not relevant. The fact is that the insured experienced those effects when he or she was an insured of the insurer and as long as the insured event made a material contribution to the condition, then the insurer must fulfill its contract.

[482] It is to be remembered that an insurer, unlike a tortfeasor, has the choice to accept an insured or not and has the ability to draft its insurance to exclude certain conditions or not. Albeit in the context of auto insurance, the policy wording and entire regime is as a result of a consensus reached by the various stakeholders, nonetheless, the insurers have always been one of the stakeholders participating in that process. In the end, insurance is the business of accepting and insuring risks and one of the risks accepted by all auto insurers is the pre-existing medical condition of each of its insureds. It is no answer for an insurer to say that its insured would have experienced these effects in any event; the fact is that they occurred during the period of time that it insured that individual.

[483] This theory was expressed in the case of *Z. v. Dominion Insurance*, [2000] O.F.S.C.I.D. No. 51. In that case Z. was involved in an accident in which she was struck from the rear. The evidence disclosed that Z. had a lengthy and complicated pre-existing psychological condition.

But the arbitrator nonetheless found that the accident materially contributed to Z.'s on-going condition after the accident.

[484] Dominion had argued, based on *Athey*, that Z.'s benefits should be reduced because there was a measurable risk that her pre-existing condition would have deteriorated even if the accident had not happened. In response to this submission the arbitrator expressed the following:

“While this submission may apply to tort compensation, which allows greater flexibility in fine-tuning appropriate compensation, the issues I must decide arise from a statutorily mandated contract of insurance. I find that the particular provisions in issue in this hearing do not allow benefits to be reduced by the percentage that non-accident factors contribute to any subsequent disability or expense. When assessing causation, the provisions in question require an "all or nothing" approach.”

[485] The decision was appealed and the arbitrator who heard the appeal found that there was no error in law in the original decision.

[486] As a result, it is the contention of the plaintiff that the concept of “crumbling skull” has no application to this case.

[487] But even if it is found to apply, the facts in this case do not suggest that it should have any application. The evidence of Dr. Lesiuk was that prior to the 1998 accident, Ms. Monks did not have any clear myelopathic features and as a result, she had a 60% chance of remaining stable. In other words, on the balance of probabilities she had a greater chance than not of remaining in the condition she was in as of April 1998, had the December 1998 accident not occurred.

[488] While Dr. Bourque did not actually make reference to these statistics, it can be implicitly taken that he was of a similar view given the advice he gave to Ms. Monks. When he saw and examined her, his findings were that there was “no significant objective neurologic deficit”. He told her that a conservative approach was the most appropriate and that she should be on the watch for “progressive neurologic deficit, either a cervical radiculopathy or progressive

myelopathy”. This implicitly places her in the category of not having any clear myelopathic features at that time and as a result, in the 60% category.

[489] Dr. Besemann deferred to Dr. Lesiuk on this issue. Dr. Hansebout and Dr. Stewart were not prepared to agree with this assessment of Ms. Monks prior to the accident. But Dr. Hansebout never met with the plaintiff at any time and Dr. Stewart met with her only for a brief period in 2002 long after the 1998 accident and two subsequent surgeries. Furthermore, as has already been argued, neither could be characterized as being impartial. It is practically unthinkable that the opinion of the treating neurosurgeon and consulting neurologist who both reached their conclusion at the relevant time based on their examinations, observations and discussions with Ms. Monks should be rejected in favour of the views of two doctors who were not present at the time, one of whom never saw her at all and the other who saw her briefly years later and who are clearly not impartial. Accordingly, it is the submission of the plaintiff that the opinions of Dr. Lesiuk and Dr. Bourque be accepted and on that basis on the balance of probabilities, the plaintiff in fact had a greater chance than not of remaining stable. That certainly does not meet the test for a “crumbling skull”.

[490] Furthermore, even if Ms. Monks were to deteriorate in the future, there is no clear evidence that in the absence of the 1998 accident, her course would have been the same as what has occurred; i.e. that she would end up as an incomplete quadriplegic. She may well have had some increased symptomatology but how that may have occurred and when and what might have happened if it did is nothing more than speculation.

[491] The only opinion elicited by the defence as to what the future might have held in the absence of the 1998 accident was that Ms. Monks was most likely to have surgery at some point. Even assuming that (which of course, the plaintiff argues should not be - the evidence of Dr. Lesiuk and Dr. Bourque just referred to militates against that argument; she had a 60% chance of remaining stable and not requiring surgery), there is absolutely no opinion before the court that this would have resulted in Ms. Monks being catastrophically impaired as she now is.

[492] Even if she did require surgery at some point in the future, there was no evidence heard at trial that the nature and circumstances that would give rise to that need would be the same as the ones, which actually occurred in this case. In this case, the MRI of June 1999 disclosed significantly increased disc protrusion at 3 levels and cord swelling and/or damage. It was Dr. Lesiuk's evidence that even without having seen Ms. Monks and if he saw this MRI in conjunction with the previous two, he would have been compelled to ask "What happened?" because the deterioration was simply too rapid.

[493] As Ms. Monks was told by both Dr. Bourque and Dr. Lesiuk, she was to watch for intractable pain or signs of radiculopathy (numbness or weakness in the arms) or myelopathy (numbness or weakness in the arms and/or legs). There is no doubt that the forces of the 1998 accident were significant. Ms. Monks had been told by Dr. Lesiuk to avoid such circumstances. She had done a number of things to respond to that warning. She ceased doing sports or activities, which would give rise to the risk of further injury to her neck. She also leased the Ford Expedition she was driving at the time of the accident under the belief that it would afford her greater protection (one only has to wonder what the result of this accident might have been if she had not taken that step). She had taken steps to reduce the chance of injury but poor luck still intervened.

[494] At this stage, it would only be sheer speculation as to what might have happened in the absence of this accident. Ms. Monks may have encountered one of the signs she was told to watch for and intervention would have occurred in less severe circumstances as occurred following the 1998 accident; e.g. there would not have been retrodisplacement of the cord, such increased disc protrusion and also most importantly no cord damage. Following the 1998 accident, Ms. Monks was told that she must have surgery or she most certainly would go on to become quadriplegic. There is no certainty that the same dire circumstances would present themselves when the need for some hypothetical future surgery might arise. As a result, the ultimate result of that surgery would by no means be the same as what occurred in this case.

[495] As a result, it is the submission of the plaintiff that on the balance of probabilities there is a greater likelihood than not that in the absence of the 1998 accident surgery would not have

been inevitable. Furthermore, there was no evidence that even if surgery were to have become inevitable, it would have occurred in the same circumstances as occurred following the 1998 accident and thus there was no evidence that Ms. Monks' ultimate condition would result in the same catastrophic impairments she now has. Simply put, those possibilities are simply too speculative to have any impact on the decision in this case. As a result, it is the submission of the plaintiff that even if the Court accepts the notion that the "crumbling skull" theory could be applied in an accident benefit case, there is simply no evidence to support the conclusion that it should be applied.

[496] As indicated above, the principles of the *Athey* case as it regards the "material contribution" test have been adopted in the accident benefits context. There are numerous cases which could be cited for this proposition but the case which appears to be referred to the most is *Levey v. Traders General Insurance Co.*, [1998] O.I.C.D. No. 113.

[497] In that case, Ms. Levey, an Air Canada flight attendant, suffered from a condition called spasmodic torticollis (an involuntary repetitive movement disorder) in her neck. At issue was whether or not an MVA accident caused or materially contributed to this condition. The accident was a light impact from the rear, which resulted in pain and stiffness on the left side of her neck radiating into her shoulder and arms. It was not until over a year later that the condition of torticollis was diagnosed.

[498] The arbitrator concluded as follows:

"I do not have to find that the accident was the sole or even the principal cause of Ms. Levey's condition. Although Ms. Levey's pre-existing disposition may have aggravated her injuries, it is sufficient if the contribution by the accident was more than minimal and thereby made a material contribution to the development of the torticollis: *Athey v. Leonati et al.*, [1996] S.C.R. 458. Dr. Paulseth testified that the progression of the condition did not follow its normal path, suggesting a link between the accident and the subsequent exacerbation of the condition. In light of this evidence, I find that the accident made a material contribution to the development of Ms. Levey's spasmodic torticollis."

[499] Perhaps the most compelling case is that of *Saliba v. Allstate Insurance Co. of Canada*, [1999] O.F.S.C.I.D. No. 170 because the facts are strikingly similar to this case.

[500] In the Saliba case, the applicant was injured in motor vehicle accidents on January 28, 1992, and on May 3, 1992. He applied for and received statutory accident benefits after each of the accidents. Allstate Insurance Company of Canada paid Mr. Saliba weekly income benefits until July 13, 1992. Progressive Casualty Insurance Company paid Mr. Saliba weekly income benefits until January 25, 1995. In this arbitration Mr. Saliba claimed ongoing entitlement to weekly income benefits. Both insurers disputed such entitlement. At the time of the accidents, Mr. Saliba had some undiagnosed conditions. He argued that either or both accidents caused these conditions to become symptomatic, and directly or indirectly caused him to require surgery. Unfortunately, following the surgery, Mr. Saliba developed incomplete quadriplegia (sadly similar to this case)

[501] The relevant pre-existing conditions consisted of degenerative disc disease, fused C2-3 vertebrae, spinal stenosis, and intermittent back problems. On some occasions his back pain was treated with chiropractic manipulations and back strengthening exercises, and appears to have resolved a few years before the relevant accidents. The arbitrator found that before the January 1992 accident, Mr. Saliba had a number of significant impairments. However, based on his performance appraisals, also found that despite his impairments, Mr. Saliba was able to do his job. He was a willing employee, with an average degree of productivity (also similar to this case).

[502] In the first accident, Mr. Saliba sustained injuries to his neck, lower back and right shoulder. He complained of neck pain, headaches in his forehead, at the back and occipital regions of his head, and was tender over his C4-C7 vertebrae. He had pain radiating from his neck into his right shoulder and right upper back, but had a good range of shoulder movement. He had a painful lower back, and was tender over the L4-S1 area. Mr. Saliba had problems sitting or standing for long periods. His doctor diagnosed strained cervical and lumbar muscles and a strained right shoulder, and referred Mr. Saliba for physiotherapy. Mr. Saliba made slow

but steady progress at physiotherapy. He was at the point of graduating to a more active component of his physiotherapy program, when he was injured in the second accident.

[503] In the second accident, Mr. Saliba's injuries consisted of a painful aggravation of the previous injury to neck, right shoulder and back with no change in his previous physical status except for increased pain. X-rays showed a congenital fusion of his C2-3 vertebrae, degenerative disc disease at the C6-7 levels of his cervical spine and at L5-S1 of his lumbar spine. The medical practitioners agreed that these were pre-existing conditions.

[504] The orthopaedic surgeon who examined Mr. Saliba at Allstate's request agreed that Mr. Saliba sustained a combination of soft tissue injuries to his cervical and lumbar spine. He anticipated that Mr. Saliba would be able to return to work within the six to eight weeks. Mr. Saliba did return to work as advised. He attempted a return to work for 11 months but had continuing pain throughout.

[505] Mr. Saliba was treated with hydrotherapy, active physiotherapy, a work hardening program, a lumbosacral brace, adjustable canes, medication and supportive counselling. He continued to complain of headaches, neck and low back pain radiating into his left leg and great toe, paresthesia, reduced grip strength in his left arm, sensory loss in his fingers, numbness and tingling in his left arm radiating into his fourth and fifth fingers, recurrent temporary paralysis in his arms and legs, and L'Hermitte's sign, or shock like symptoms when he flexed his neck. These complaints prompted referrals to an orthopaedic surgeon, a neurologist, and two neurosurgeons.

[506] Mr. Saliba had two further car accidents in 1995. The arbitrator made the following observations about their potential role:

“There is little medical opinion evidence with respect to the role of these accidents. Dr. deVilliers testified that these 1995 accidents probably did play a role in Mr. Saliba's symptoms. However, he could not deduce from this that the two 1992 accidents were not causally related, since by history, Mr. Saliba's symptoms started after the first of his 1992 accidents. I make no finding with respect to the role of these two 1995 accidents and the extent to which they may cause or contribute to any disability. While it is conceivable that the subsequent

accidents might create fresh entitlement to statutory accident benefits, the evidence does not suggest that having become symptomatic, Mr. Saliba would again become asymptomatic. The 1995 accidents would not therefore relieve either Allstate or Progressive from their obligations under the Schedule.”

[507] The arbitrator then went on to describe the issue as it related to causation:

“Mr. Saliba alleges that his injuries and consequent pain and disability are caused by either or both of the motor vehicle accidents. This is disputed by Allstate and Progressive. Mr. Saliba has the burden of adducing credible evidence to establish that his pain "probably," or more likely than not, resulted from the automobile accident or accidents in question. [See Note 11 below] Arbitrators have stated that the accident(s) need not be the sole cause but must significantly or materially contribute to the disability.

Note 11: Hamelin and Alpina Insurance Company, Limited, (OIC A-003506, July 19, 1994); Koch and AXA Insurance (Canada), (OIC A-951417, June 6, 1996), confirmed on appeal (OIC P96-00058, May 8, 1997) and many others. “

[508] The arbitrator accepted the treating doctor’s opinion that the motor vehicle accidents aggravated or accelerated Mr. Saliba's condition and concluded that Mr. Saliba's largely asymptomatic pre-existing conditions were rendered symptomatic by the motor vehicle accidents, causing him to suffer the disabling neck and back pain of which he complained following the accidents.

[509] In 1997 Mr. Saliba had a cervical laminectomy to relieve the compression to his cervical spinal cord. Mr. Saliba alleged that there was a causal relationship between either or both of the motor vehicle accidents, and his cervical laminectomy. Both insurers disputed that such a causal relationship exists.

[510] The MRI showed severe compression of Mr. Saliba's spinal cord, and his symptoms were consistent with that compression. Yet his neurological examination continued to be normal. Based on the data he had, the neurologist could recommend one of three courses of action to Mr. Saliba. He could continue to observe him, with the possibility that no further compression of his spinal cord would occur; he could continue to observe him and hope that if further compression did occur, surgery would take place before significant and irreversible paralysis occurred; or he

could proceed with surgery to avoid the risk of paralysis. The surgery itself carried a two per cent chance of paralysis. Timing of the surgery could be a critical factor.

[511] In the neurosurgeon's judgment, the surgery should proceed, given Mr. Saliba's symptoms of L'Hermitte's sign, intermittent quadriparesis and his complaint of chest pain. Mr. Saliba consented to the cervical laminectomy, which took place on September 23, 1997. The neurosurgeon testified that he removed the laminae of the C2-C6 vertebrae, and the C2-3 fused vertebrae, as well as the yellow ligament. In his opinion, satisfactory decompression was achieved, and nothing abnormal happened during the surgery. Unfortunately, Mr. Saliba awoke with quadriplegia.

[512] The arbitrator found as follows:

“I find that Mr. Saliba sought medical treatment for his post-accident pain and symptoms and followed the advice of his treating practitioners. Medicine remains an inexact science and it would appear that there is a great deal of room for differing opinions and judgment. There is no suggestion that Dr. deVilliers failed to disclose the risks of surgery, or that he carried out the surgery in an improper manner. I find no new intervening act which would relieve Allstate and Progressive from responsibility for Mr. Saliba's statutory accident benefits. Although Mr. Saliba's disability has worsened following the surgery, this also does not relieve Allstate and Progressive from responsibility for Mr. Saliba's statutory accident benefits. [See Note 12 below]

Note 12: *Alderson v. Callaghan* [1998] 40. O. R. 3d, Ont. C. A. applying *Athey v. Leonati*, [1996] 3 S.C.R. 458, S.C.C.

....

Dr. Wismer and Dr. deVilliers could not tell which of the accidents had a more significant impact on Mr. Saliba. Dr. deVilliers felt both were probably involved and there was no way of determining which was more severe or long lasting. The physiotherapist who was treating Mr. Saliba before the second accident, noted some setback in his recovery. Dr. A.K. Kapoor opined that Mr. Saliba's injuries got worse after the second accident. Dr. Dolan proposed a 75-25% split between the first and second accidents, and Dr. Howell adopted this apportionment.

Drs. Cameron, Urovitz, Ameis and Gray opined that the second accident produced little or no ill effects, at most caused a very minor neck and back strain, which should have resolved somewhere between June 1992 and December 1992. All relied on Dr. Wismer's examination, and/or on the physiotherapist's statements in

reaching their conclusion. However, those health practitioners felt it impossible to apportion responsibility to one accident or another. I reject the opinions that the effects of the second motor vehicle accident were trivial. Of the remaining opinions, two suggest the first accident is 75% responsible and the second 25% responsible. Even a 25% amount is more than minimal or trivial and is therefore material or significant. [See Note 13 below] I find it probable that both accidents significantly contributed to Mr. Saliba's disability.

Note13: *Athey v. Leonati*, [\[1996\] 3 S.C.R. 458](#), S.C.C.

In my view, neither the Act nor the Schedule contemplate apportionment of statutory accident benefits in such circumstances. Rather, each accident gives rise to entitlement to a fresh set of statutory accident benefits. Each insurer remains fully responsible for Mr. Saliba's statutory accident benefits since each 1992 accident was a cause of his impairment.”

[513] As can be seen the principles in *Athey* are applied in accident benefit cases with the necessary modifications. Consistent with the argument raised above that the “crumbling skull” concept does not apply to accident benefits cases, the quote above confirms that while in tort cases apportionment will occur, in accident benefit cases, it does not as it is not contemplated by the Act or the Schedule.

[514] As a result, it is the submission on behalf of the plaintiff that on the facts and law, the test for causation; i.e. material contribution to the catastrophic impairments Ms. Monks suffers has been more than satisfied. It is the contention of the plaintiff that this has been obvious from the very beginning and that she should not have been put through the ordeal of having to prove what is so self-evident. It will form part of the claim for aggravated damages that ING has been unreasonable in taking the position it has.

Directly Caused

[515] It was suggested by counsel for ING during the trial that the *SABS –96* provides benefits only for impairments which are caused ‘directly’ by an automobile accident. It is the contention on behalf of the plaintiff that this is not a correct interpretation of the regulation and that ING is

required to provide benefits in relation to all of the impairments, which Ms. Monks has sustained since the accident of December 23, 1998.

[516] The key provisions of SABS-96 on this issue are:

2. (1) In this Regulation,

“accident” means an incident in which the use or operation of an automobile directly causes an impairment or directly causes damage to any prescription eyewear, denture, hearing aid, prosthesis or other medical or dental device; (“accident”)

.....

14. (1) The insurer shall pay an insured person who sustains an impairment as a result of an accident a medical benefit. O. Reg. 403/96, s. 14 (1).

.....

15. (1) The insurer shall pay an insured person who sustains an impairment as a result of an accident a rehabilitation benefit. O. Reg. 403/96, s. 15 (1).

.....

16. (1) The insurer shall pay an insured person who sustains an impairment as a result of an accident an attendant care benefit. O. Reg. 403/96, s. 16 (1).

[517] While Ms. Orth has not yet articulated her argument on this issue, I assume that she proposes that the definition of “accident” means that an impairment must be “directly” caused by an automobile accident and if the impairment does not “directly” flow from the accident then the insurer should not be liable for benefits.

[518] I make this assumption because Ms. Orth asked a number of questions of the doctors who testified whether a particular impairment which Ms. Monks suffers is directly related to a spinal cord injury. With all due respect to Ms. Orth, her interpretation is without merit and in fact, has been rejected in the already decided case law.

[519] In fact, the word “directly” is intended to be a descriptor of whether the impairment arises directly out of the “use or operation” of an automobile, not whether the impairment flows directly from the accident. All of the cases interpreting the phrase focus on the issue of whether

the incident giving rise to the impairment flows directly from the 'use or operation' of an automobile.

[520] In *Chisholm v. Liberty Mutual Group*, 60 O.R. (3d) 776 (Ont. C.A.), the plaintiff was rendered a paraplegic when an unknown assailant fired gunshots at his car. The defendant insurer refused to pay rehabilitation or other statutory accident benefits on the basis that the plaintiff was not in an "accident" as defined under s. 2(1) of the Statutory Accident Benefits Schedule -- Accidents on or after November 1, 1996, as the "use or operation of an automobile" did not "directly cause" his injuries.

[521] Justice Laskin identified the issues succinctly: "The question of law on this appeal is whether a motorist, who was severely injured in a drive-by shooting, is entitled to rehabilitation and other statutory accident benefits from his own insurer. The answer turns on whether "the use or operation of an automobile directly cause[d]" his injuries."

[522] The Court of Appeal held that the plaintiff's argument that the phrase "directly causes" in the 1996 Schedule should be interpreted very broadly and equated in substance to "directly or indirectly" in order to avoid an alleged gap or inconsistency between the Insurance Act, R.S.O. 1990, c. I.8 and the Regulation, flew in the face of the government's clear intent. The 1996 Schedule limits coverage to incidents in which the use or operation of an automobile directly causes an injury.

[523] In paragraphs [7] to [11] of Justice Laskin's decision, he provides an excellent brief summary of the history of the exchanges of rights, which have occurred in the context of our automobile insurance regime since 1990. In the versions of the SABS regulations under the OMPP and Bill 164, the definition of accident in the SABS regulations was whether an impairment was caused 'directly or indirectly' by an automobile accident. Under SABS-96 that changed to an impairment caused 'directly' by the use or operation of an automobile.

[524] The leading case on this issue prior to the change in the legislation was *Amos v. Insurance Corp. of British Columbia*, [1995] 3 S.C.R. 405. In that case, the insured Amos was

driving his van in California. As he pulled away from an intersection, he was attacked by a gang of people. With his van surrounded by six men, he locked the doors of the van and kept it moving slowly ahead. The assailants commenced pounding on the door windows on both sides of the van. The glass on the driver's door shattered but was held in place by its frame. Another man then walked out in front of the van and pointed a gun at Amos. As he tried to duck from the line of fire, he was shot, the bullet striking his spinal cord. He was able to keep the van moving. When he had distanced himself from the assailants by several blocks he brought the van to a stop. He obtained assistance and was transported to a hospital. He sustained serious, disabling and permanent injuries. He submitted a claim for accident benefits and the insurer refused coverage.

[525] The applicable wording of the B.C. regulation was: “[T]he corporation shall pay benefits to an insured in respect of death or injury caused by an accident that arises out of the ownership, use or operation of a vehicle ...”

[526] The Supreme Court of Canada decision was rendered by Major J. who stated that the test for determining whether the event fell within the definition was:

- (1) Did the accident result from the ordinary and well-known activities to which automobiles are put?
- (2) Is there some nexus or causal relationship (not necessarily a direct or proximate causal relationship) between the appellant's injuries and the ownership, use or operation of his vehicle, or is the connection between the injuries and the ownership, use or operation of the vehicle merely incidental or fortuitous?

[527] The Court referred to the two parts as the "purpose" and "causation" branches of the test.

[528] With regard to the purpose test, Major J. held that the accident clearly resulted from the ordinary and well-known activities to which automobiles are put. In using this language, Major J. was not speaking of causation. He addressed causation in the second branch of the test. Rather,

the phrase intended to indicate whether the accident flowed out of or arose out of the ordinary and well-known activities to which automobiles are put. It cannot be said that the insured's injuries in the *Amos* case "resulted from" the regular activity to which his automobile was being put, in the causal sense of these words. Rather, it meant simply that his injuries flowed out of the regular activity to which his automobile was being put.

[529] When it came to the causation test, Major J. noted that the triggering words in the B.C. legislation was "arises out of the ownership, use or operation of a vehicle". He found that these words are broader than "caused by" and so the required nexus did not need to be a direct or proximate cause. And so he went on to find that shooting appeared to have been the direct result of the assailants' failed attempt to gain entry to the van. It was important to Major J. that the shooting was not random but a shooting that arose out of the ownership, use and operation of his vehicle.

[530] In *Chisholm*, Laskin J. noted that the Ontario Court of Appeal had applied the twofold test in *Amos* to two cases under Bill 164 because it was felt that the language of the SABS regulations at that time and the B.C. legislation was similar enough to warrant the same treatment.

[531] But he went on the find that the more stringent wording of the SABS-96 definition "directly" caused by meant that the causation portion of the *Amos* test could not apply. He even felt that *Chisholm* would not meet the *Amos* test in any event because his injuries were caused by what appeared to be a random shooting and not in an attempt to gain entry to the vehicle, as was the case in *Amos*.

[532] In articulating the causation test under SABS-96, Laskin J. stated the following:

[27] A direct causation requirement conjures up memories of the famous English tort case of *In Re Polemis & Furness, Withy & Co. Ltd.*, [1921] All E.R. Rep. 40, [1921] 3 K.B. 560, where recovery was allowed for damages that were not a foreseeable result of the defendant's negligence but were directly caused by it. When one thinks of direct causation one thinks of something knocking over the

first in a row of blocks, after which the rest falls down without the assistance of any other act.

[28] In his text *Handbook of the Law of Torts*, 4th ed. (St. Paul: West Publishing Co., 1971) at pp. 263-64, Dean Prosser defined "consequences" directly caused as "those which follow in sequence from the effect of the defendant's act upon conditions existing and forces already in operation at the time, without the intervention of any external forces which come into active operation later". Here an external force, the gun shots, came "into active operation later". Thus, in Prosser's terms, Chisholm's impairment was not a consequence directly caused by the use or operation of his car.

[533] This issue was considered by the Court of Appeal again in the case of *Greenhalgh v. ING Halifax Insurance Co.*, [2004] O.J. No. 3485. In that case, Greenhalgh's car became lodged upon a rock on a country road. She could not dislodge it and attempted to use her cellular phone to call for assistance, however, the battery was dead. Greenhalgh and her friend who was with her decided to leave the vehicle and walk back to the main road. They became disoriented and walked for nine or ten hours. Along the way they fell through ice into a river. Greenhalgh lost her shoes. As a result of exposure to the cold she suffered extreme frostbite, which necessitated the amputation of her fingers and legs below the knees. Greenhalgh then submitted a claim for accident benefits. ING Halifax refused coverage and Greenhalgh brought an action. They asked the Court to resolve an issue of law as to whether Greenhalgh suffered an impairment as a result of the accident. The motions judge held that the plaintiff had suffered an impairment as a result of the accident. ING Halifax appealed that decision.

[534] The Court of Appeal allowed ING's appeal. The decision was rendered by Labrosse J., who provided an excellent summary of the law as it has developed and articulated the two-pronged test which should apply to cases under SABS-96:

1. Did the incident arise out of the use or operation of an automobile?
2. Did such use or operation of an automobile directly cause the impairment?

[535] The first was the same 'purpose' test taken from *Amos* and the second is the 'causation' test as determined by the wording of the definition. Labrosse J. then went on to ask two further questions which are relevant to the 'causation' test:

1. Was the use or operation of the vehicle a cause of the injuries?
2. If the use or operation of a vehicle was a cause of the injuries, was there an intervening act or intervening acts that resulted in the injuries that cannot be said to be part of the "ordinary course of things"? In that sense, can it be said that the use or operation of the vehicle was a "direct cause" of the injuries?

[536] Labrosse J. observed that in *Chisholm*, Laskin J. found three inquiries helpful in answering the "direct cause" question: the "but for" inquiry; the "intervening act" inquiry; and the "dominant feature" inquiry.

[537] In the end, he found that while Greenhalgh may have been able to prove that the accident resulted from a well-known and ordinary activity, namely that she was operating a motor vehicle when she took a wrong turn, she was not able to meet the causation test, which was determinative of her claim. Although she would not have been wandering through the woods at night, fallen into the river and suffered severe frostbite if she had not become stuck on a rock, there were numerous intervening acts between the time that her car got stuck and when she sustained her injuries. Those intervening acts supported the conclusion that the use of the car was not a direct cause of the injuries. The dominant feature of Greenhalgh's injuries was exposure to the cold. The use of the motor vehicle was ancillary. The injuries she suffered were not a direct result of the automobile accident within the meaning of the legislation.

[538] I have probably gone into far more detail than is necessary for this case because there are no extraordinary circumstances here such as in *Amos*, *Chisholm* or *Greenhalgh*. But the point of doing so for the Court is to clearly demonstrate that the term 'directly causes' relates to whether the activity causing the impairment has a direct causal connection to the use or operation of the vehicle. In Ms. Monks' case, her injuries arose immediately following the impact of the vehicle with the bridge abutment. The use of the car was the direct cause of her injuries. There were no "intervening acts". "But for" the impact with the bridge, she would not have had the injuries. The "dominant feature" of the incident was the impact while driving the car. As a result, there really is no need to make the kind of inquiries made in *Chisholm* and *Greenhalgh*.

But I have made this review to demonstrate to the Court that the issue of “directly causing” is not as Ms. Orth would suggest that the impairment must be directly caused by the accident, rather it is the accident giving rise to the impairment must be directly caused by the use or operation of an automobile.

[539] What Ms. Orth appears to be suggesting is that following the accident, there were two surgeries and following those surgeries there have been numerous complications and that these are not ‘directly’ connected to the accident. However, she has confused the definition of “accident” with the consequences of the accident. In each of sections 14, 15 and 16 set out above, the wording of the regulation is: “The insurer shall pay an insured person who *sustains an impairment as a result of an accident*” (emphasis mine) medical benefits, rehabilitation benefits and attendant care benefits accordingly. It is important to note that for the entitlement to these benefits the impairment does not have to be a ‘direct’ result of the accident but simply ‘as a result of’.

[540] This issue was dealt with directly in the case of *Correia v. TTC Insurance Co.*, [2000] O.F.S.C.I.D. No. 192. In that case, Ms. Correia was injured as she exited a TTC bus. She underwent physiotherapy for her injuries and then was asked to attend an FCE (functional capacity evaluation) at the behest of TTC. During the course of the FCE, she was injured.

[541] The TTC submitted that the bus incident did not “directly cause” Ms. Correia's injuries sustained in the FCE, and therefore she failed to satisfy the causal nexus requirement set out *SABS-1996* (definition of “accident”).

[542] The arbitrator however, observed the following:

¶ 71 It can be argued, in support of the TTC's position, that section 2 requires each impairment giving rise to benefit entitlement to be directly caused by the accident. However, the definition of “accident” in section 2 uses the indefinite article: it provides coverage where the use or operation of an automobile “directly causes an impairment.” The preamble to section 4, the IRB entitlement section, sets out the pre-condition for entitlement: “an insured person who sustains an impairment as a result of an accident.” Nothing in the wording of these sections limits benefits to those impairments directly caused by the accident. Moreover, in

enacting the Bill 59 amendments to the SABS, the legislature chose not to amend the provisions setting out the entitlement criteria for various types of benefits, all of which retain the "as a result of" test. This supports the Applicant's view that the "as a result of" test applies in determining the extent of an insurer's liability for the consequences of an accident. Reading subsections 2(1) and 4(1) together, I find that once the insured person proves that she sustained an impairment in an "accident," the test for entitlement to benefits is whether the impairments giving rise to the claim for benefits were sustained "as a result of" the accident.

[543] The arbitrator went on paragraphs 72 to 77 to provide a review of a number of decisions, which support her conclusion in the context of accident benefit cases. She then went on to review the issue at common law and find that it applies equally to accident benefit cases. It is the submission on behalf of Ms. Monks that this is the applicable law to her situation. The arbitrator stated:

¶ 78 The general rule is that a tortfeasor is responsible for all the reasonably foreseeable consequences of his wrongdoing. [See Note 26 below] That a subsequent event contributed to the victim's losses does not necessarily limit recovery:

The intervening conscious act of a third party will break the chain of causation and relieve the party who may be otherwise negligent of liability, unless to a reasonable man in the same circumstances that conscious act would have been foreseeable. [See Note 27 below]

Note 26: A good review of the cases is found in *Cotic v. Gray* (1981), [33 O.R. \(2d\) 356](#) (Ont. C.A.), affirmed by the Supreme Court of Canada, [\[1983\] 2 S.C.R. 2](#).

Note 27: *Jones v. Shafer*, [\[1948\] S.C.R. 166](#), at 170-1.

¶ 79 Where the intervening act is that of a health care provider, the law is as stated in *Mercer v. Gray*, [See Note 28 below] a decision of the Ontario Court of Appeal:

... if reasonable care is used to employ a competent physician or surgeon to treat personal injuries wrongfully inflicted, the results of the treatment, even though by an error of treatment the treatment is unsuccessful, will be a proper head of damages. ...

There may be cases where the medical or surgical treatment is so negligent as to be actionable. This would be in effect novus actus interveniens and the plaintiff would have his remedy against the physician or surgeon. But this the defendant would have to prove ...

Note28: [\[1941\] OR 127](#), [\[1941\] 3 DLR 564](#).

¶ 80 The Court reaffirmed this principle in *Papp v. LeClerc*, (1977) 16 O.R. (2d) 158 (Ont. C.A.):

Every tortfeasor causing injury to a person placing him in the position of seeking medical or hospital help, must assume the inherent risks of complications, bona fide medical error or misadventure, and they are reasonable and not too remote. ... It is for the defendant to prove that some new act rendering another person liable has broken the chain of causation.

[544] The arbitrator then also went on to consider what has occurred in the context of the Worker's Compensation scheme, where these common law concepts of foreseeability and remoteness have long been accepted as still applicable to those cases. The arbitrator concluded:

¶ 88 Like workers' compensation, the statutory accident benefits scheme represents a legislative policy decision about the appropriate allocation of certain costs of pervasive economic activities. It also involves an exchange of tort rights for statutory benefits. Both schemes aim to provide comprehensive benefits for injured persons while containing costs by restricting litigation. I find that the workers' compensation and accident benefits schemes are statutes in pari materia and should be presumed to take a consistent and coherent view of accident benefits. I can think of no reason why the common law principles of causation and remoteness governing the workers' compensation scheme would not apply to the very similar language of the SABS-1996 entitlement provisions.

[545] The arbitrator then goes on to provide an excellent review of the history of the insurance regime in Ontario and the guidelines for statutory interpretation at paragraphs 89 to 95 of her decision. She then concludes:

¶ 96 To the extent the SABS is ambiguous as to the extent of coverage for the consequences of an accident, I find that it should be given an interpretation that is consistent with the legislative objectives underlying the statutory accident benefit scheme, analogous common-law principles and related (workers' compensation) jurisprudence. I conclude that the extent of coverage for the consequences of an accident is governed by the "as a result of" test, which requires proof that the accident materially or significantly contributed to the disability or impairment that gives rise to the claim for benefits. I find that treatment and assessment-related impairments come within this rule where it is reasonably foreseeable that the insured person would seek treatment as a result of the accident and that treatment might result in further injuries or aggravation of the original injury.

¶ 97 Important policy considerations also favour this approach. Accident benefits are intended to be payable on a non-adversarial, expedited basis. Requiring an insured person to trace the "chain of causation" with precision is inconsistent with this policy. Early rehabilitation is another important objective of the accident benefits scheme. [See Note 43 below] For this reason, the SABS places special emphasis on prompt payment of medical and rehabilitation benefits, and requires insurers to pay certain benefits pending resolution of any dispute about entitlement. Moreover, the SABS-1996 explicitly requires an insured person to "obtain such treatment and participate in such rehabilitation as is reasonable, available and necessary" to permit her to engage in suitable employment. [See Note 44 below] Visiting the costs of therapeutic mishaps on insured persons is likely to discourage participation in therapy, contrary to the policy set out in the SABS-1996.

[546] This decision and its underlying reasons were upheld on appeal in *Correia v. TTC Insurance Co.*, [2001] O.F.S.C.I.D. No. 106. There were some interesting observations made in that decision as well:

¶ 36 Under the principles of proximate cause, absent explicit language in a policy, if injury or death results from treatment necessitated by an accident, providing there is no intervening cause, the accident remains the direct or proximate cause.

.....

¶ 41 New injuries clearly may be related to an impairment sustained in the accident. The arbitrator gave examples of cases in which someone fell down the stairs due to dizziness related to, or a knee weakened from, the effects of an accident and sustained new injuries. Those new injuries can be viewed as a direct consequence of the accident and a result of it. Likewise, if someone undergoes surgery for accident-related problems and, through no one's fault, suffers new

impairment in the course of the operation, such consequences are directly traceable to the accident.

¶ 42 The same applies to injuries sustained in rehabilitation assessments. As the arbitrator notes, participation in reasonable treatment and rehabilitation is not only to be encouraged, but is mandatory under section 55 of SABS-1996 as a condition of receiving full benefits.

[547] I would also remind the Court of the decision in the *Saliba* case referred to above. You may recall that this was the SABS case which was extraordinarily similar to this one where Mr. Saliba suffered from pre-existing degenerative conditions of the spine and had 4 accidents and ultimately surgery which resulted in him becoming an incomplete quadriplegic. In that case, his when he was confronted with a situation similar to Ms. Monks prior to the 1998 accident, his neurosurgeon advised that he should undergo the decompressive surgery and it did not go well. He awoke with incomplete quadriplegia and the arbitrator found:

¶ 66 I find that Mr. Saliba sought medical treatment for his post-accident pain and symptoms and followed the advice of his treating practitioners. Medicine remains an inexact science and it would appear that there is a great deal of room for differing opinions and judgment. There is no suggestion that Dr. deVilliers failed to disclose the risks of surgery, or that he carried out the surgery in an improper manner. I find no new intervening act which would relieve Allstate and Progressive from responsibility for Mr. Saliba's statutory accident benefits. Although Mr. Saliba's disability has worsened following the surgery, this also does not relieve Allstate and Progressive from responsibility for Mr. Saliba's statutory accident benefits. [See Note 12 below]

Note 12: *Alderson v. Callaghan* [1998] 40. O. R. 3d, Ont. C. A. applying *Athey v. Leonati*, [1996] 3 S.C.R. 458, S.C.C.

[548] In Ms. Monks's case, the injuries she received in the 1998 accident were directly caused by the use or operation of an automobile. She was told by her treating neurosurgeon that if she did not have surgery, she would certainly go on to become a quadriplegic. Therefore, the impairments she would have directly sustained from the injuries she received in the accident in the absence of surgery were surely even worse than she has sustained to date with surgical

intervention. She would have been classified as suffering from a catastrophic impairment, i.e. 'quadriplegia' if she had not undergone the treatment she did.

[549] But she underwent surgery for her accident-related problems and the result has also been that she has ended up catastrophically impaired (be it 'quadriplegia' as defined by Dr. Becker or 55% whole body impaired). There is absolutely no evidence that the surgery was anything other than properly performed and there is no evidence of any intervening act. As a result, the consequences are directly and properly traceable to the accident. And frankly, the line to the 1998 accident is much clearer than the line to the two accidents in the *Saliba* case.

[550] A review of the evidence of Dr. Vandewaetere and Dr. Lesiuk discloses that all of the symptoms and problems Ms. Monks currently suffers from have their root in the original injury or the surgeries. Clearly, the condition of quadriparesis has its roots in the injury and the surgeries. All of the resulting pain, weakness, numbness flow from this. As a result, all of the mobility needs and problems flow from this.

[551] The bowel and bladder dysfunction occurred immediately following the first surgery and flow directly from it. While the cause of the nausea and vomiting has not been clearly determined, Dr. Lesiuk has certainly provided a strong argument in his letters (Ex. 1, Tabs A11 and A12) and testimony that the gastrointestinal problems she has relate to neurological causes. But it is the submission of the plaintiff that it is not incumbent on her to establish this. As stated in the *Correia* decision, if a new impairment is caused by surgery, which was necessitated by the accident, then it flows from the accident. The same can be said of the septicemia she had, the edema, the sleep apnea and the narcolepsy. If she had not had this accident, she would not have required the surgery and she would therefore, not had any of these conditions.

[552] One part of the evidence of Dr. Lesiuk cannot be emphasized too much – the surgeries were intended not to provide an improvement of her condition, but to prevent any further deterioration, which in the absence of taking that step would certainly have occurred. The surgery was successful in the sense that the desired decompression occurred in both cases, but

the process he was attempting to halt appears not to have halted (although it is arguable it has been slowed).

[553] Both Dr. Lesiuk and Dr. Vandewaetere gave the opinion that Ms. Monks' weight problems flow from the accident. She is unable to be as active and has gained significant weight, which cannot be controlled by diet alone. This weight problem has led to problems with her knees to the extent that she may require an operation. This still has its root in the accident.

[554] Both doctors have also testified that the many powerful medications she must take have also contributed to her condition and may be the source of her sleep difficulties. Again this flows from the conditions she must medicate which flow from the accident.

[555] Ms. Monks is understandably depressed by virtue of all that she has suffered. Whether she may or may not have had pre-existing depression is irrelevant, there can be no doubt and this was more than foreseeable that someone such as Ms. Monks would suffer from depression and perhaps other psychological difficulties and problems as a result of the injuries and impairments she has sustained due to this accident.

[556] In short, once the law is properly understood, there can be no doubt that all of the conditions and impairments Ms. Monks suffers from at this time have their roots in the two accidents. And of course, once it is concluded that the 1998 accident was a material cause, then it gives rise to entitlement to all of the benefits under SABS-96.

[557] And if there can be any doubt about this, one should have consideration of the case of *Sohi v. ING Insurance Co. of Canada*, [2004] O.F.S.C.D. No. 106 (a case of which ING and its counsel should obviously be aware). In that case, Mr. Sohi was run down by a motor vehicle in a parking lot. Following that incident, he was taken by ambulance to a hospital where he was treated and released. He received some further treatment for the sequelae of the accident over the next few days. Some three weeks later, in his home, in the presence of his family, he poured gasoline over himself, and set himself on fire.

[558] Mr. Sohi claimed that the fire that precipitated his serious injuries was linked directly to the motor vehicle accident and that the fire-related injuries constituted an impairment as a result of an accident as defined by the regulations. ING saw the matter differently. While accepting that an "accident" did likely happen and that Mr. Sohi was seriously injured by the gasoline fire, it took the position that the suicide by fire was a discrete event due to other tragic factors in Mr. Sohi's life, and was not directly related to the earlier motor vehicle accident.

[559] The arbitrator after an analysis of the case law, which included *Chisholm* and *Greenhalgh*, concluded: "that Mr. Sohi's suicide attempt, and the injuries arising from it, was triggered by the motor vehicle accident, which was a direct cause of his burn injuries. The motor vehicle accident was the direct trigger setting a compromised Mr. Sohi on the road to self destruction."

[560] Certainly, if this can be seen to meet the test of causation, all of what Ms. Monks has gone through surely does. Frankly, it is the submission of the plaintiff that this case does not warrant such a lengthy discussion of the issue of 'direct' cause. The law has been clearly decided on the point (and indeed one of the leading cases involves ING) and the facts of this case do not justify this issue being raised. I have dealt with it because Ms. Orth continued to insist during the trial that this would be a real issue – it is not and should not have been and frankly, it is patently unreasonable for ING to even raise it. It is contrary to the obligations owed to its insured to provide peace of mind.

My Analysis of Plaintiff's Case Law

[561] *Athey v. Leonati*, [1996] 3 S.C.R. 458

- FACTS / ISSUES:
 - The Appellant suffered back injuries in two successive motor vehicle accidents, and then suffered a disc herniation during a mild stretching exercise; the herniation was caused by a combination of the injuries from the two accidents and a pre-existing disposition.
 - Should the Appellant's losses and damages be apportioned between tortious and non-tortious causes, where both types of causes were necessary to create the injury?
- OUTCOME:

- The SCC allowed the appeal and found the tortious defendant fully liable for the totality of the plaintiff's damages, even though the trial judge had found the accidents had played a "causative role, albeit a minor one."
- **RATIO:**
 - A tortious defendant is liable for any injuries caused or contributed to by his or her negligence; the presence of other non-tortious contributing causes does not reduce the extent of that liability, and loss cannot be apportioned according to the degree of causation where the loss is created by both tortious and non-tortious causes.
 - Causation will be found where an event "materially contributed" to a plaintiff's injuries, in circumstances where the more conventional "but-for" test is unworkable (see para. 15).
 - So long as there is a "material contribution" to the plaintiff's injuries which is "outside the *de minimis* range," then liability will follow.
- **ANALYSIS:**
 - At first glance, the *Athey* case appears to be of limited applicability, since it is a tort case and not an "accident benefit" case involving an insurance company.
 - However, the SCC's statements of legal principles concerning causation are broad enough to be applicable in almost any context, including the present case.
 - In finding that the cause of an injury need not be the sole or primary cause of the injury, the SCC established a broad threshold test for causation; so long as an event was *part* of the cause of an injury, that event "caused" the injury even though it may not have been sufficient by itself to cause the injury.
 - Therefore, in the present case, the 1998 accident caused the plaintiff's injuries, since this accident materially contributed to her condition, despite the role of prior accidents and an alleged pre-existing condition.
 - Furthermore, the Plaintiff is correct in pointing out that in *Athey*, as in this case, there is a "single indivisible injury" or a single condition, such that a defendant could not escape full liability by arguing that he or she was only responsible for one localized injury (such as a foot or an arm) as opposed to all injuries.
 - Finally, the Plaintiff argues that the SCC's notion of a "crumbling skull theory" (reducing a damages award to reflect a pre-existing injury or condition) cannot apply to insurance cases, since there is no scheme to reduce the amount of benefits to be awarded – it is a black/white determination of full liability.
 - The *Athey* case has been upheld and followed numerous times.

[562] *Z. v. Dominion Insurance*, [2000] O.F.S.C.I.D. No. 51 (Ontario Financial Services Commission)

- **FACTS / ISSUES:**
 - The plaintiff was in a car accident, and applied for and received accident benefits from an insurer, including amounts for rehabilitation and treatment of physical and psychological injuries.

- However, the plaintiff already had a lengthy and complicated pre-existing psychological condition; the insurer terminated the benefits; the plaintiff applied for arbitration of the matter.
- OUTCOME:
 - The arbitrator found that the accident materially contributed to the plaintiff's ongoing condition, despite the pre-existing condition, and ordered the payment of additional accident benefits.
- RATIO:
 - The statutory scheme for the payment of accident benefits does not allow for partial payment, or for a reduction in amount; it is an "all or nothing" approach to liability and payment.
- ANALYSIS:
 - This case confirms the Plaintiff's contention that in an insurance benefits case, it is no defence to argue that benefits should be reduced in order to reflect an apportionment of causation.
 - However, I note that the arbitrator's findings on causation were heavily influenced by the evidence concerning the plaintiff's immediate physical condition following the accident, the severity of the accident, and the pre-existing medical reports; the situation is different here.
 - There has been no subsequent judicial treatment of this case.

[563] *Levey v. Traders General Insurance Co.*, [1998] O.I.C.D. No. 113 (Ontario Financial Services Commission)

- FACTS / ISSUES:
 - The applicant was injured in a motor vehicle accident, and diagnosed with spasmodic torticollis in her neck; she received accident benefits from her insurer, which were then terminated because the insurer discovered the applicant had a pre-existing disposition; the matter was put to arbitration.
 - The insurer argued that the accident did not "cause" the applicant's injury, because her pre-existing disposition would have resulted in the injury eventually.
- OUTCOME:
 - The arbitrator ruled that the applicant's statutory benefits should be reinstated.
- RATIO:
 - Although a pre-existing condition may aggravate certain injuries suffered in an accident, it is sufficient if the contribution by the accident was more than minimal and thereby made a material contribution to the injury or condition, in order for the accident to have "caused" the injury.
- ANALYSIS:
 - This case suggests that a pre-existing condition may not only be a contributing cause, but also an aggravating cause of an injury or condition, without affecting "legal" causation.

- *Levey* also supports the Plaintiff's arguments in the present case, since the arbitrator in *Levey* indicated that the applicant's pre-existing medical condition did not follow its "normal path of progression," arguing that it was exacerbated by the accident, as was the case here.
- The decision in *Levey* was affirmed on appeal by the Financial Services Tribunal – see 1999 CarswellOnt 5547. The Tribunal ruled that the arbitrator did not err in his analysis of the causation of the injury, that the pre-existing condition did not affect the causation role played by the accident, and that the "material contribution" test for causation is a legitimate substitute for the "but for" test in many circumstances.
- *Levey* has been discussed in one subsequent case:
 - In *Todd v. State Farm Mutual Automobile Insurance Co.*, 2003 CarswellOnt 5823, the Financial Services Tribunal affirmed that in order to find a causal relationship between an accident and an injury, the accident need not be the sole or principal cause. The Tribunal affirmed that the accident need only be a material contributing cause, citing *Levey*.

[564] *Saliba v. Allstate Insurance Co. of Canada*, [1999] O.F.S.C.I.D. No. 170 (Ontario Financial Services Commission)

- FACTS / ISSUES:
 - The applicant was injured in two motor vehicle accidents, and received statutory accident benefits, including disability benefits, following each accident.
 - The two insurance companies providing benefits terminated payments after a few years, arguing that the applicant had pre-existing medical conditions which the accidents did not cause.
 - The applicant argued that he had three undiagnosed medical conditions at the time of the accidents, and that the accidents exacerbated the conditions and caused them to become symptomatic, requiring him to undergo surgery; following the surgery, the applicant became quadriplegic.
- OUTCOME:
 - The applicant was entitled to receive ongoing weekly income benefits, and the insurers were required to respond to his claim for accident benefits, because there was a causal connection between the accidents and his condition following his surgery which was caused by the accidents.
- RATIO:
 - The applicant's largely non-symptomatic pre-existing conditions were rendered symptomatic by the accidents, which resulted in tremendous pain, and led to surgery; this surgery, brought on by the accidents, ultimately caused the incomplete quadriplegia.
 - It is not necessary for an accident to be the sole or primary cause of an illness or injury for an injured person to be entitled to accident benefits, so long as the accident was a material contributing cause.

- ANALYSIS:
 - This case expands the range of liability relating to accidents and pre-existing conditions, since in *Saliba* the applicant's doctor stated that it was "possible" that the pre-existing condition could have resulted in surgery eventually, and the quadriplegia might still have developed even without the accidents. Nevertheless, the accidents probably hastened the need for surgery and aggravated the applicant's overall injuries and symptoms, enough to establish causation.
 - Therefore, causation also involves temporal elements in addition to considerations of "materiality."
 - In subsequent proceedings, the applicant was awarded an amount for renovations to his home to meet his long-term housing disability needs .
 - The *Saliba* case was discussed in a subsequent case:
 - In *Argirovski v. Allstate Insurance Co. of Canada*, 2000 CarswellOnt 3147, the Financial Services Tribunal ruled that causation need not be established on a standard of "certainty," but rather on a "balance of probabilities" that the accident was a "significant factor" or "material contribution" in causing an applicant's injuries or condition, citing *Saliba*. The Tribunal also concluded that once an accident has been found to be a major contributing factor to an injury, an intervening accident does not negate an insurer's liability to pay, citing *Saliba* again.

The interpretation of the phrase "directly causes" in the SABS

[565] There is a "sub-issue" in the discussion of causation in this case, relating to the proper interpretation of the *Statutory Accident Benefits Schedule*, O. Reg. 403/96 ("SABS") and whether benefits should only be provided for injuries and losses caused "directly" by an automobile accident. The plaintiff submitted that the use of the word "directly" in the SABS definition of "accident" at s. 2(1) "is intended to be a descriptor of whether the impairment arises directly out of the 'use or operation' of an automobile, not whether the impairment flows directly from the accident". The plaintiff further submitted that this interpretation has been well-established in the jurisprudence, and that the opposite interpretation (i.e., if the impairment does not "directly" flow from the accident then an insurer is not liable to pay benefits) has been rejected in the case law. I will briefly summarize the cases on which the plaintiff relied, and comment on the relevance of each case to the plaintiff's submissions.

[566] *Chisholm v. Liberty Mutual Group* (2002), 60 O.R. (3d) 776 (C.A.)

- FACTS / ISSUES:

- The plaintiff was rendered a paraplegic when an unknown assailant fired gunshots at his car; the insurer refused to pay any benefits since the plaintiff was not in an “accident.”
 - The plaintiff argued that the phrase “directly caused” in the definition of “accident” should be broadly and liberally interpreted so as to permit both direct and indirect causation.
 - The plaintiff’s claim was dismissed by a motions judge; the plaintiff appealed
- OUTCOME:
 - The plaintiff’s appeal was dismissed, since the plaintiff’s injuries did not arise out of any motor vehicle accident; rather, the gunshots were the cause of his injuries, not the use of an automobile.
- RATIO:
 - Under the legislation, injured persons are entitled to accident benefits only if their injuries or conditions are “directly caused by the use or operation of an automobile” (para. 11).
 - To interpret the phrase “directly causes” to mean either direct or indirect causation would “fly in the face of the government’s intent” (see para. 13).
 - The phrase “directly caused” should be interpreted to mean a sequential order of causation where “direct consequences” are those which follow from a defendant’s acts upon conditions and forces already existing and in operation, without the intervention of external forces which come into operation at a later point in time (see paras. 28-29).
 - The use or operation of a motor vehicle must not be incidental or ancillary to the impairment or injury; it must be a direct cause of the impairment.
- ANALYSIS:
 - At first glance, this case appears to undermine the Plaintiff’s position, since it establishes that the legislation does not permit recovery where the impairment was “indirectly caused” by an accident.
 - However, a careful reading of the case indicates that the Ontario Court of Appeal merely concluded that the “use or operation” of a motor vehicle must “directly cause” an impairment, not whether the accident itself directly caused the impairment to the exclusion of all other material contributing causes.
 - The Court of Appeal’s interpretation does not exclude or negate the possibility that there may be other contributing causes to an impairment without affecting a finding that the use or operation of a motor vehicle “directly caused” the impairment, sufficient to ground an insurer’s liability. This is especially true given that the “material contribution” test (discussed above) can be applied in situations where the “but for” causation test is not appropriate or effective.
 - The phrase “directly caused” is therefore meant to distinguish between situations where there is an unbroken chain of events versus a broken chain of events, with the latter type of situations being non-compensable through the insurance legislation.
 - The *Chisholm* case has been followed in two recent Ontario cases:

- In *Greenhalgh v. ING Halifax Insurance Co.*, [2004] O.J. No. 3485 (C.A.) (discussed in more detail below), the plaintiff suffered severe frostbite when she fell in icy water after becoming lost and disoriented, having abandoned her car which was stuck on the side of the road. The Ontario Court of Appeal ruled that the causation requirements established in *Chisholm* were correct, and overturned the trial judge's finding that the plaintiff's injuries were "directly caused" by the use or operation of a motor vehicle; there were simply too many "intervening acts" between the role played by the car and the ultimate injury.
- In *Kumar v. Coachman Insurance Co.*, [2004] O.J. No. 2494 (Sup. Ct. Just.), the Court upheld an arbitrator's approval and application of the causation test from *Chisholm*, and ruled that the plaintiff could not obtain accident benefits where there was no evidence to establish that his injuries were in any way caused by his head striking anything in the automobile when he lost control of the vehicle.

[567] *Greenhalgh v. ING Halifax Insurance Co.*, [2004] O.J. No. 3485 (C.A.)

- FACTS / ISSUES:
 - The plaintiff's car became lodged on a rock on a country road. The plaintiff decided to abandon the vehicle and walk back to a main road. She became disoriented and walked for nine or ten hours, eventually falling through ice into a river and losing her shoes; as a result of exposure to the cold, she suffered extreme frostbite and her fingers and legs below the knees were amputated.
 - The insurer refused to pay any benefits, arguing that the injuries were not caused by the use or operation of a motor vehicle; the motions judge agreed with the plaintiff, and the insurer appealed.
- OUTCOME:
 - The insurer's appeal was allowed; the plaintiff's injuries were not directly caused by the use or operation of a motor vehicle as required by the SABS legislation.
- RATIO:
 - Following *Chisholm*, an impairment will not be "directly caused" by the use or operation of an automobile where there are too many intervening events arising after the use of the automobile.
 - In this case, it may be true that were it not for her car getting stuck on the road, the plaintiff would not have been wandering in the woods towards the main road; however, there were too many intervening events between the use of the car and the ultimate injury to establish causation.
- ANALYSIS:

- This case affirms the interpretation of the phrase “directly caused” favoured by the Plaintiff, namely, that the restriction implied in the word “directly” refers to whether or not the impairment was caused directly by the use or operation of an automobile, notwithstanding that there may be additional or other causes of the impairment. Causation will not be established where there are too many subsequent intervening events or forces between the use/operation of an automobile and the claimant’s injury; otherwise, the causation link should stand.
- There has been no relevant subsequent judicial treatment of this case on the issue of causation.

[568] *Correia v. TTC Insurance Co.*, [2000] O.F.S.C.I.D. No. 192 (Ontario Financial Services Commission)

- FACTS / ISSUES:
 - The applicant was injured as she exited a bus; she underwent physiotherapy for her injuries and was asked to attend a “functional capacity evaluation” at the behest of the defendant; the applicant was seriously injured during the evaluation; the issue was whether the bus incident “directly caused” the injuries sustained during the evaluation.
- OUTCOME:
 - The arbitrator awarded benefits to the applicant, ruling that the accident was causally connected to the applicant’s injuries, since the evaluation only occurred as a result of the accident.
- RATIO:
 - Where a person is injured during medical treatment or evaluation following an accident in which the person was originally injured, that person will be entitled to accident benefits even where the benefits are intended to address injuries resulting from the subsequent medical treatment or evaluation, so long as there is a sufficient causal connection to the original accident. The “later” injuries are to be treated as the “consequences” of the original injuries which arose from the accident – such consequences are eligible for accident benefits.
 - Once an insured person proves that she sustained “an impairment” in an accident, the test for entitlement to benefits is whether the impairments giving rise to the claim for benefits were sustained “as a result of” the accident; the test will be met where the impairments giving rise to the claim for benefits are the consequences of the impairments directly caused by the accident.
 - “Accident benefits are intended to be payable on a non-adversarial, expedited basis. Requiring an insured person to trace the ‘chain of causation’ with precision is inconsistent with this policy” (para. 97).
- ANALYSIS:

- This case deals more with entitlement to income replacement benefits under s. 4 (for which the test is a less stringent phrase: “as a result of”) – which is analogous to the entitlement to accident benefits under s. 14 – than with the issue of whether or not the use or operation of an automobile directly caused an injury or impairment under s. 2 (for which the test is a more stringent phrase: “directly causes”).
- Nevertheless, this case provides additional support for the Plaintiff’s position in that it establishes a clear distinction between the definition of an accident and the consequences of an accident. Entitlement to benefits under s. 14 merely requires that the impairments were sustained “as a result of” the accident. Whether or not the claimant will be eligible for this entitlement depends on whether there was an “accident” – i.e., whether the use or operation of an automobile “directly caused” an impairment (even if that impairment is not the impairment for which benefits are now being claimed).
- This case supports the Plaintiff’s position since it establishes the notion that causation in insurance cases may still be flexible despite the apparent specificity of the wording in the legislation.
- The arbitrator’s decision was upheld on appeal in [2001] O.F.S.C.I.D. (Ontario Financial Services Commission, Office of the Director of Arbitrations) (Tab 9 of the Plaintiff’s Book of Authorities):
 - The Director’s Delegate dismissed the insurer’s appeal, affirming that s. 2(1) determines whether or not an “accident” has occurred, while s. 4 (or s. 14) determines entitlement to benefits based on impairments which were sustained “as a result of” the accident. The key observation to be made is that s. 2(1) uses the indeterminate article “an” impairment, such that a person may bring a claim for impairments which were sustained “as a result of” an accident even though, in that accident, the use or operation of an automobile did not “directly cause” the specific impairment for which benefits are being claimed. It is enough that the use or operation of any automobile directly caused “an” impairment, whether or not this is the impairment that is the subject of the claim.
- In my opinion, a strong analogy can be drawn to the present case, where the use of an automobile directly caused “an” injury to Ms. Monks, even though the impairments which are the subject of her claim are not necessarily the same injuries sustained directly in the accident. In other words, there is scope for multiple injuries, with multiple causes, so long as there is a sufficient causal connection, expressed in the words “sustained as a result of.”
- *Correia* was followed (on relevant issues of causation) in one subsequent case:
 - In *Belous v Scottish & York Insurance Co.*, 2002 CarswellOnt 5322, the Ontario Financial Services Tribunal applied the same test and reasoning from *Correia*, but ultimately denied the applicant’s claim because the injury in question (the death of the applicant’s father) was not sustained “as a result of” the prior motor vehicle accident in which the applicant’s father injured his foot; rather, his death was caused by an injury resulting from alcohol impairment.

[569] *Sohi v. ING Insurance Co. of Canada*, [2004] O.F.S.C.D. No. 106 (Ontario Financial Services Commission)

- **FACTS / ISSUES:**
 - The applicant was injured in a motor vehicle accident, and subsequently attempted to commit suicide by setting himself on fire, which resulted in severe burns; the applicant applied for and was denied accident benefits from his insurer, and the matter proceeded to arbitration.
 - The key issue was whether or not the burns caused by the suicide attempt were sustained “as a result of” the motor vehicle accident.
- **OUTCOME:**
 - The applicant’s claim for accident benefits was allowed, following a finding that the burns were sustained as a result of the accident.
- **RATIO:**
 - A claimant will be entitled to benefits for certain impairments if those impairments were sustained “as a result of” an accident, even if those impairments for which benefits are claimed were not caused “directly” by the use or operation of an automobile, so long as the use of an automobile did cause some other impairment which is causally connected to the impairment(s) for which benefits are being claimed.
 - The fact that a claimant is seeking benefits for injuries sustained *after* an accident should not disentitle that claimant to benefits, so long as those injuries are themselves the consequences of an original injury directly caused by a motor vehicle accident (in this case, the burns were caused by severe psychological distress following the original injuries resulting from the accident). All that is required is a sufficient chain of causation from the accident to the original injury to the injury for which benefits are claimed.
- **ANALYSIS:**
 - This case strongly supports the Plaintiff’s claim in the present case, since it establishes a fairly broad test for entitlement to benefits even where the injuries for which benefits are being claimed were not “directly caused” by the use or operation of an automobile.
 - The *Sohi* case has not been referred to in subsequent jurisprudence.

DEFENDANT’S SUBMISSIONS ON CAUSATION AND THE LAW
(taken from the defendant’s written submissions)

1. CAUSATION

A. Evidence – including medical evidence

[570] The defendant submits that Ms. Monks had been experiencing progressive degeneration of her spinal cord, prior to the 1998 accident, following the neurological symptoms as a result of the 1995 accident and the rapid degenerative changes, which occurred thereafter. There is evidence from medical experts that there was already a need for serious surgical consideration in July 1997, at the latest. The DAC assessor's opinion was also that, prior to the 1998 accident, it was apparent that Ms. Monks would require surgery. When Ms. Monks finally did undergo surgery in 1999 and 2001, her health deteriorated rapidly after each surgery and, eventually, resulted in her becoming catastrophically impaired.

[571] If the plaintiff's cervical spondylotic myelopathy had progressed to the point that Ms. Monks required surgery prior to the 1998 accident, then the resulting deterioration after the two surgeries cannot be said to have been caused by the 1998 accident – rather, the deterioration and catastrophic impairment was caused by a combination of the two previous accidents, and by a pre-existing medical condition.

[572] Counsel for the defendant submitted the following argument.

It is an uncontroverted fact in this case that the Plaintiff was experiencing progressive encroachment on the spinal cord degenerative spinal stenosis prior to the 1998 accident. It is uncontested that she experienced neurological symptoms of numbness and tingling following the 1995 accident and that there was a brisk pace of degenerative changes which took place following the 1995 accident onwards up to the 1998 accident. The evidence was consistent that there had been no radiological testing done for over 19 months prior to the 1998 accident and that the treating family doctor was not aware of the progression of her symptoms for the six month period prior to the 1998 accident and her treating neurosurgeon was not aware of the progression of her symptoms for the nine month period before the 1998 accident. It was agreed by all the experts who testified that the radiological findings of July 1997 wherein the spinal canal had narrowed to 6mm warranted serious surgical consideration at that early stage. It was uncontested that since the 1997 MRI her symptoms increased to include symptomatology in both arms, weakness of the lower extremities and irritable bladder which are all neurological symptoms. Unquestionably, it was the opinion of two leading experts in the field of neurology and neurosurgery that surgery was needed prior to the 1998 accident. It was also the opinion of the neutral DAC assessor that prior to the 1998 accident that she was going to need the surgery. We know that

the Plaintiff stated under oath in the Zurich action that she would have undergone the surgery before the 1998 accident had Zurich agreed to accept its responsibility to pay for the expenses associated with having the surgery.

It is also clear from the facts of this case that her health took a turn for the worse immediately following the first surgery in the fall of 1999, and that it continued to deteriorate after the second surgery in January of 2001. The CAT DAC in assessing her after the second surgery, found that she had deteriorated to the point where she was found to be catastrophic.

Therefore, ING submits, that if it is found that the progression of her cervical spondylotic myelopathy had progressed to the point that she required surgery prior to the 1998 accident, that the resulting deterioration after the surgery causing the catastrophic injury was not caused by the 1998 accident. Therefore the responsibility for the payment of accident benefits should not fall to ING but rather to Zurich, for which the Plaintiff has received \$1.54 million dollars as settlement.

i) History of Cervical Spondylotic Myelopathy up to December 23, 1998

[573] In following the history of Ms. Monks' cervical spondylosis (degenerative condition of the spine relating to the neck), we see the following progression.

[574] In early 1993, the plaintiff was involved in a motor vehicle accident wherein she suffered injury to her neck. At this point, it was already apparent from the x-ray taken of her cervical spine that she has degenerative disc narrowing at C6-C7 (Exhibit 6, Tab 1). Dr. Lesiuk testified that the cervical lordosis noted in that x-ray of March 1993, is less common, but is indicative of someone with a neck strain or having neck pain. Dr. Hansebout concurred saying that "it shows a straightening of the spine. Usually that's because the muscles in the neck are tight and they straighten the spine [and] it usually indicates that there is neck pain. In this case that's obviously what she had, neck pain, that's what straightened it." (Page 14, transcript of Dr. Hansebout.)

[575] Counsel for the defendant argued that the 1993 motor vehicle accident was at least the second motor vehicle accident the plaintiff had been in, as she testified that she had been in a previous accident in the 1980s, whereby the vehicle she was driving went out of control into the

ditch. Further, there was a Worker's Compensation notice filed at trial that showed that in January 1993, the plaintiff had slipped, wrenching her neck in the process.

[576] Notwithstanding that the plaintiff testified that her February 1993 neck injury had resolved in three-four months, we see in her family doctor's notes, Dr. Snook, that as of:

a) April 29, 1993, she is still undergoing treatment, that she is off work driving the bus until September and that it is a "slowly resolving whiplash" (Exhibit 2, Tab 1, page 8). The Ontario Automobile Insurance Medical or Psychological report, dated May 4, 1993, says "has been off work and will not return to driving the bus until approximately September 1, 1993 (Exhibit 2, Tab 3, page 30).

b) June 21, 1993, that over four months later, she "still has upper mid and now LBP [low back pain] which goes to buttock – see previous entries". She is diagnosed with a back/neck strain. So clearly, it appears that it is getting worse and not completely resolved as the plaintiff had testified (Exhibit 2, Tab 1, page 8).

c) The September 1993 note indicates "right shoulder a little sore at times and so is neck from previous motor vehicle accident (see previous entries)" (Exhibit 2, Tab 1, page 10).

[577] The plaintiff was in a motor vehicle accident on July 21, 1995, in which she was rear-ended and suffered immediate numbness and tingling in both arms, and an injury to her neck, mid-scapular and lower back. Dr. Hansebout testified as follows on that issue:

That accident was – her car was hit from behind. And according to the records, the person whose car struck the back of hers says it was not a high impact accident. Nevertheless, with problems in the spine, her neck would have gone backwards, in other words extended.

And the stretching, and possibly the stretching of the spinal cord over an osteophyte in the back, caused damage, a little bit of damage, possibly in the spinal cord, but probably in the nerve roots going out from that level, and that caused tingling to occur in her arms and numbness. And that lasted for about 15 minutes. And it just means that accident did probably cause some damage to nervous structures, notably the nerves coming off the spinal cord. (Page 14, transcript of Dr. Hansebout)

[578] When asked whether this tells him anything with respect to the narrowness of the spinal cord, Dr. Hansebout replied:

Well, yes. Because if the spinal canal is not narrow and the neck extends, there is nothing for the spinal cord to stretch up against, so that you wouldn't get this situation normally, in a normal person. Usually it means that there is degenerative change already in the spine and some narrowing. (Page 15, transcript of Dr. Hansebout)

[579] Accordingly, in Dr. Hansebout's opinion, the plaintiff had already undergone narrowing of her spinal canal before the July 1995 accident had even occurred.

[580] The Accident Benefit application to Zurich indicates that she also sustained an injury to the base of her skull. She was prescribed Fiorinal and was also taking Robaxacet and Advil (Exhibit 12, tab 2). We note that, two years later, when she saw Dr. Lesiuk in May 1997, she continued to be on Fiorinal, which is a prescriptive pain medication and Robaxacet, which is non-prescriptive, but has a pain medication component to it. This is reflected in the New Patient Registration Form completed at Dr. Lesiuk's office (Exhibit 4, Tab 5, page 95). It further notes that she was on Anaprox, Altace, Robaxacet and Fiorinal, which indicate that her prescriptive medication consumption has increased since the date of the 1995 accident.

[581] The clinical note of Dr. Gagnon, dated August 3, 1995, confirmed that she was having headaches and neck grinding. The plaintiff testified that she still gets neck grinding when she turns her head and that she will get a sensation and will hear a noise like a crunch. She noted that it is a very rough movement, which is noisy.

[582] The progression of her symptoms was obviously having a major effect on her as on October 10, 1995. The plaintiff wrote to Zurich and stated:

I am currently having symptomatology that is affecting my ability to perform my occupation effectively.

I am attempting to maintain my caseload, however, at this time I am having considerable difficulty performing tasks requiring fine motor control.

...

My inability to write is seriously affecting my ability to complete assignments and class tests. With mid-terms approaching, I have some concerns about successfully completing my course.

As an independent adjuster, my license is dependant on my successfully completing four courses before May 1996. As such, my current limitation poses serious employment threat.

...

The physician has recommended that I take time off work, however, at this time, this option is one that I would prefer to avoid, however, with increasing headaches and neurological symptoms, I may have no other choice. (Exhibit 16, Tab 8)

[583] The first radiological finding after the July 1995 accident, shows that in September of 1995, which is a little over two years after the first x-ray in 1993, that there has now been progression in that two disc levels were now being affected, the C6-7 level, which was previously noted, and now a new finding at the C5-C6 level. Dr. Hansebout commented that there is progression not only with the different levels, but he also commented that the previous x-ray did not show osteophytes and therefore, she was now starting to get bony spurs sticking out, which is a degenerative change. (Page 15, transcript of Dr. Hansebout)

[584] This report therefore reflects that the new finding at C5-6 notes the changes as being related to minimal marginal osteophytes, which we know from both Dr. Lesiuk's and Dr. Hansebout's definition do not arise as a result of trauma, but are as a result of natural progression.

[585] The report indicates that C6-7 continues to have mild narrowing, which was noted as being degenerative in the previous report. Therefore, changes that have been noted as of September 1995 are as a result of the natural progression of her condition.

[586] The next radiological testing is the MRI taken only ten months later, on July 27, 1996. It shows that she has now been affected at three levels, C4-5, C5-6 and C6-7.

[587] Within those levels, we note that the C4-5 level is a new finding and is a disc bulge, which is associated with an osteophytic bar, which is a natural progression not trauma related. Dr. Hansebout testified that the report reflects that the bar of bone is already starting to press on the plaintiff's spinal cord. We also see that there is a new finding, in that the neural foramina has been affected, noting mild left and mild to moderate right foramina stenosis from osteophytes. Dr. Hansebout described this as the "foramina, that is the little holes through which the nerves go out to the muscles and skin, that they're already getting smaller due to the bony growth degenerating and making them smaller, which means that the nerves may be that they're going to get squeezed soon or are getting squeezed already". (Page 16, transcript of Dr. Hansebout) Again Dr. Hansebout's definitions of stenosis and osteophytes reveal that this progression is a natural progression. Dr. Lesiuk testified that an osteophytic bar reflects part of the broader nature of degenerative spondylosis, as it implies a bony change, which doesn't occur suddenly. It gets built up over some period of time, generally over a period of months to years. Dr. Lesiuk described the changes at C4-5, as some significant anatomic developments that are not normal.

[588] Progression in the C4-5 level has increased from mild degenerative narrowing to moderate left paracentral disc herniation, producing moderate central and left sided cord compression. We also know that the neural foramina is not affected, thereby indicating that there is no radiculopathy.

[589] The overall impression noted on this report is multi-level degenerative disc disease with the most significant stenosis from a disc herniation at C5-6. Dr. Lesiuk described these changes as an indicator of more degenerative change, and indicated that it raises the issue, in comparison with the x-rays, that things have progressed and the possibility that they have progressed rather more quickly than one would typically anticipate. Overall, Dr. Lesiuk describes that there is moderate degenerative change affecting the lower cervical segments with disc space narrowing, disc bulging at C4-5, C5-6 and C6-7 associated with osteophyte formation, or osteophytosis and spondylotic bar formation.

[590] Dr. Hansebout indicated that there was a significant change in that now three levels were involved with osteophytes at two levels representing progression. He also noted that at C4-

5, there was an osteophytic bar, which represents a growth in the osteophyte since the last x-ray in September 1995.

[591] However, in terms of the rate of progression, Dr. Hansebout, who has significantly more years of experience than Dr. Lesiuk, went on to state, “that’s quite a lot of problems for a woman I think who’s, what 40, 41, that’s a lot of problems. That’s pretty significant stenosis and spinal degeneration already.”

[592] Dr. Stewart, when asked to comment on the x-rays, commented that they show degenerative changes beyond those, which he would expect in a woman in her late 30s, early 40s. He states, “they represent bones of a much older woman”.

[593] Dr. Lesiuk was not prepared to agree to this assessment. An example, we submit, of his “advocating” for his patient.

[594] Again, contrary to the evidence given by the plaintiff, the massage therapist’s report of July 7, 1996, reveals that the plaintiff continued to experience discomfort and headaches, and a referral to a physiatrist was scheduled to follow-up on the possibility of neurogenic component to her injury (Exhibit 12, Tab 7). The plaintiff testified that she did not continue to have headaches between the 1995 accident and the 1998 accident.

[595] Further, the Health Practitioner’s Statement, prepared by Dr. Snook (Exhibit 2, Tab 4, page 47), reveals that again, contrary to the plaintiff’s evidence in chief, she had to modify her duties due to the pain she was in, and that by August 1996, one year after the accident, she was still experiencing a decreased range of motion and headaches. Dr. Snook diagnoses her with whiplash/myofascial neck pain and degenerative disc disease.

[596] In January of 1997, the plaintiff undergoes x-rays of the lumbar region (Exhibit 12, Tab 9), which are significant, in that, they show that:

- i) there was minimal bony spurring (which is another name for osteophytes) in the anterior aspects of L1-2, 2-3, 3-4 which, as Dr. Hansebout testified, is also a natural phenomena;

ii) there are more prominent osteophytes at D-11-12, which again is showing that the degeneration is taking place in not only the cervical area, but as well in the lower areas of the spine. This again supports Dr. Hansebout's position that her situation is not consistent with her stated age, but that the progression reflects a much older person.

[597] Dr. Hansebout testified that this x-ray revealed that the spine was straightening, which could mean that there is some muscle spasm in the back. Of specific note, he stated:

What's also very unusual, extremely unusual, is that she's got an osteophyte at the D11-12 level, that's thoracic spine. That's very, very unusual. Maybe you see one of those in a neurosurgical practice every two or three years. It's very unusual. She's got one of those too. What that means, since there is still spinal cord up there, that could also be pressing on the spinal cord, that is at the D11-12 level. (Page 18, transcript of Dr. Hansebout)

[598] A review of Addendum "A" reveals an alarming increase of radiological findings, which exhibit her naturally progressing degenerative spondylosis, not only at the cervical level, but as well as the thoracic level. The chart shows that, starting as early as 1993, the natural progression had gone from one level to seven levels over a four-year period (C4-5, C5-6, C6-7, L1-2, L2-3, L3-4, D11-12) and the levels themselves have gotten progressively worst.

[599] She then began seeing Dr. Vandewaetere as her family physician, in January 1997. On her first visit, Dr. Vandewaetere notes her two motor vehicle accidents in 1993 and 1995, and the results of the 1996 MRI. The plaintiff's symptoms are recorded as being numbness and weakness in her right hand. It specifically documents that she had woken up with numbness in her right hand for one year and that her hand is more clumsy. She is also noted as being fatigued and suffering from decreased motivation. Dr. Vandewaetere testified that her weight range was in the range of 200-250 pounds.

[600] Her next visit to the family doctor, in late January 1997, records increased weakness through the thenar, which Dr. Vandewaetere described as being the thumb area. Further, she is described as having depression, poor energy and poor motivation. Obviously, the plaintiff was becoming very concerned about the increasing symptoms of weakness and clumsiness, as she

requested that Dr. Vandewaetere provide her with a referral to Dr. Lesiuk, which Dr. Vandewaetere testified that she arranged.

[601] It is important to comment here that the plaintiff specifically requested to see Dr. Lesiuk, who is a neurosurgeon, rather than requesting to see a neurologist. Dr. Lesiuk testified that the difference between a neurologist was that their training is different, in that neurologists train to diagnose and treat diseases of the nervous system by medical means, but neurosurgeons are surgical doctors trained to do operations. Knowing that the plaintiff is trained as a nurse and is well versed in medical knowledge, it is not insignificant that she chose to see the expert who is responsible for surgery rather than diagnosis. It is our submission that she was already contemplating, given her rapidly progressing symptoms, that she will need to undergo surgery – why else would she specifically request to see a neurosurgeon?

[602] The March 27, 1997 note of the family doctor reveals that her complaints are in both arms and this is consistent with Dr. MacGregor's report of November 14, 1996 (Exhibit 1, Tab G-1).

[603] It is apparent that her clumsiness in her right hand is progressively getting worse as, in December of 1995, she was able to complete the two insurance courses, in April of 1996, she was able to successfully complete another insurance course, but by April of 1997, she was unable to complete her course, because her ability to write an essay examination had deteriorated to the point that she could not do it. These courses were very important to the plaintiff in her career development and, accordingly, her condition must have been deteriorating to such an extreme level for her to have given up on proceeding with them, particularly when she was only one or two courses away from getting her designation (Exhibit 12, Tab 13).

[604] She is also having increasing difficulty with work and, in May of 1997, she incorporated her own business and left Renaud and Senecal to set up her own rehabilitation company, which allowed her to control her own hours. This is reflected in Dr. Vandewaetere's note of June 10, 1997, when it indicates that she has to make a decision about her lifestyle.

[605] Given her significant concerns over her deteriorating situation, the plaintiff met with Dr. Lesiuk on May 13, 1997, at which time her symptoms are noted as being “fluctuating neck pain and headaches. More concerning, since the fall of 1995, she has had increasingly fatigability of the right upper extremity and to a lesser extent the left upper extremity ... The right hand becomes progressively clumsy until she can hardly hold the pen. She has noted that the clumsiness in the hands is more noticeable with less activity now.” At that point, the lower extremities and the sphincters were not affected. Specifically, Dr. Lesiuk notes that there are slight, but definite weakness of the finger extensors of the right hand which, he testified, implies some neurological involvement (test of motor strength) and that there are equivocally positive Hoffman reflexes more prominently on the right than the left which, he testified, a normal person would not have. Dr. Lesiuk testified that when a Hoffman reflex is seen, it raises the concern that there may be spinal cord involvement.

[606] In the May 13, 1997 report, Dr. Lesiuk comments on the 1996 MRI, noting that it reveals moderate degenerative spondylolytic change affecting the lower cervical segments. There is disc space narrowing, disc bulging at the C4-5, C5-6 and C6-7 levels associated with osteophytosis and spondylolytic bar formation, as well as facet and ligamentous hypertrophy. Dr. Hansebout explained that the spine has an upper and a lower facet forming a joint, and he notes that these facets are getting arthritic and hypertrophied, he explained, means enlargement therefore, the arthritic joints are getting larger, which reflects that these joints are showing increasing arthritis. He went on to explain that at the back of the spinal cord, there is a ligament, which defined the back of the spinal canal. This ligament has also gotten a lot bigger, which is a change, he testified, which comes with aging. Dr. Hansebout opined that what this means is that it makes the spinal canal even smaller and, therefore, the spinal stenosis is increased as a consequence of those changes.

[607] This is what the plaintiff’s status was as of July 1996, over two and a half years before the 1998 accident. Dr. Hansebout testified that these findings mean that her spinal cord is getting early compression already, it is getting squeezed, which means that the blood supply to the spinal cord is reduced and it is not working as well as it should be.

[608] At this point Dr. Lesiuk only has the 1996 MRI to review and he noted specifically that her symptoms have become more prominent since then, obviously denoting a progression of her symptoms. He therefore sent her for another MRI, because he wanted to see if there was a corresponding anatomic progression. Of note was the fact that his referral for the MRI indicated “F/U spinal stenosis with worsening myelopathic symptoms” (Exhibit 4, Tab 5, page 98). Spinal stenosis is the natural narrowing of the spinal cord. Clearly, Dr. Lesiuk is contemplating a further course of treatment and felt that a second opinion was warranted and he sent her to Dr. Bourque.

[609] A review of Dr. Bourque’s note of the meeting in July 1997, reflects a diagnosis of cervical myelopathy. In the report generated after meeting with the plaintiff, Dr. Bourque indicated that, “since that time (referring to the 1995 accident), she has had symptoms, which may have been gradually worsening. The most prominent is a sensation of fatigue, stiffness and loss of fine motor control, affecting the arms, right more than left. For example, she has difficulty writing a long exam, swimming, holding heavy objects in her right hand.” He pointed out that she seemed to have less endurance in her legs and that she has mild urinary urgency and is prone to some stress incontinence.

[610] Dr. Bourque did not see the 1997 MRI, but had the report, which indicated that “there has been no significant interval change”. It did not say that there has been no change, but that it was not a significant change. It is also important to note that he never did become aware that the plaintiff’s symptoms had progressed before the 1998 accident to the point where she had urinary retention (he is told about mild urinary urgency and some stress incontinence, which is completely different) and which is a significant neurological symptom, or that she had been diagnosed with an irritable bladder (before the 1998 accident), which is also a neurological symptom. Nor did he ever become aware that the deterioration of her endurance of the lower extremities had reached the point, by November of 1998, where she was required to use a wheelchair for ambulating long distance. That, obviously, would have been very significant to his opinion, given the commentary he had made about her walking ability. ING submits that Dr. Bourque’s opinion is applicable only to that snap shot in time in July of 1997, and by that we

mean that his opinion should be given no weight in making a determination as to her status as of December 23, 1998, as 17 months had gone by the time the December 1998 accident occurred, and he had been provided with no update on the progression of her symptoms, and we argue that there had been significant progression during that subsequent period of time.

[611] The plaintiff was cautioned to watch for progressive myelopathy, being increasing numbness or tingling in the arms or legs, and who noted that if it was in the legs, that would certainly imply a spinal cord problem, weakness or clumsiness, increasing fatigability or loss of balance and difficulty walking.

[612] Dr. Hansebout, in cross-examination, notes that he does not agree with the conclusion reached by Dr. Bourque that the MRI findings largely explained her symptoms, and stated specifically that the symptoms reported by the patient of arm and leg fatigue and urinary urgency are in keeping with the MRI findings pointing to spinal cord compression. He also noted that that was not radiculopathy, that it was more.

[613] The plaintiff then re-attended to see Dr. Lesiuk in August 1997, at which time it is noted that the more recent MRI reflected that the AP diameter of her spinal canal was reduced to 6 millimeters. This was a very significant factor. In Dr. Hansebout's words, he stated:

This is, to a neurosurgeon, I would describe it as shocking. It is shocking, because when the spinal cord is compressed to an anteroposterior, from front to back, diameter of 11 millimeters, we consider it already compressed sufficiently to consider surgery.

This woman has a compression of – she's got a narrowing of the spinal canal to only 6 millimeters, which means her spinal cord is getting flattened, it's really suffering. This is – with her complaints of clumsiness in her right hand and beginning problems with the left, this to me is a very good indication that surgery needs to be considered. (Page 22, transcript Dr. Hansebout)

[614] When cross-examined on the fact that although this may be alarming from a radiological point of view, but that the corresponding symptomatology was not present (as you operate on symptoms and not MRIs), Dr. Hansebout disagreed, stating that she had weakness of

the arms and legs already and urinary problems, which he opined, was quite a lot (page 60, transcript of Dr. Hansebout).

[615] Dr. Stewart, in his cross-examination, also described this as being very shocking and in chief stated:

Six millimeters space is, in my opinion, a dangerous situation ... And by itself, in my opinion, would make me call a neurosurgeon for an urgent neurosurgical opinion regard decompression. That's a very narrow distance and probably – as I've said, puts the spinal cord at risk of injury.

[616] In explaining his statement in his report that in his opinion “she would likely have come to surgery in a reasonable time, even if she had not suffered this December 23, 1998 accident,” he stated that:

Well, in my last statement I pointed out what I believed was a high risk situation, as noted in the MRI we've just been discussing. I believe that she was at great risk of spinal cord damage prior to December 1998 accident, and it would have been wise for her to have a decompression at that time.

[617] In support of this opinion, he advised the Court that:

There is a pattern of progression of not only radiological changes but physical symptoms, which in my opinion indicate that she needed surgery regardless of the accident. And let me outline them, if I can.

As you have pointed out, the x-rays, beginning in 1993, show degenerative disease much ahead of what one usually expects in a woman. These progress. She also had lumbar disc disease, it was much broader than cervical as osteoarthritis degenerative disc disease.

There are two MRIs before the 1998 accident and they show cord compression at three levels, mild at C4-5 with a central cord disc bulge; moderate at C5-6, with stenosis of the canal and cord compression; and mild stenosis at 6-7, but also with cord compression at that level.

Now the cord signal was normal at that time. So there's no evidence that the cord was actually damaged at that time.

The plain film taken at the early hospital December 23, 1998 shows narrowing at the same levels. There is really no change at that time. There is no fracture and no real change. There are osteophytes, that's all. There are osteophytes (The Court will recall that osteophytes are a naturally occurring phenomena which occur over a long period of time)

[618] Dr. Stewart then, in his testimony, went on to give a lengthy history of the progression of her symptoms prior to the 1998 accident.

[619] In cross-examination, he again reiterated his opinion on numerous occasions that she should have had surgery before the 1998 accident, and explained this on the basis that "this woman has some myelopathic features and she's deteriorating, and she has a remarkably tight canal, which we've seen in the MRIs is compressing the spinal cord. This is a high risk situation."

[620] When cross-examined as to whether prophylactic surgery should have been done for the prophylactic reason, Dr. Stewart indicated that:

He did prophylactic surgery when he did it, and the prophylactic surgery should have been done earlier.

[621] Any surgery done is a preventative measure to stop the progression of symptoms and signs. Dr. Stewart felt that it should have been conducted earlier than when it was done, and specifically, before the 1998 accident.

[622] In attempting to try to get Dr. Stewart to agree that there was a significant difference between Dr. Lesiuk's report of April 1998 and August 1999, Dr. Stewart disagreed, indicating that "there was a high risk situation before, and it was a high probability that it was going to happen".

[623] Dr. Hansebout was very clear in his evidence that, given that the spinal cord had been reduced to 6 millimeters by August of 1997, it would not have been a significant change for there to be signal cord change. He stated that:

The signal cord change just indicates that it's being compressed even more, that's all it means. And since she was progressing anyhow, it's not that big a deal, because I know that eventually she would get it, whether she had it before (the 1998 accident) I mean she did have it after. When she got it before is uncertain. She may have had it, you know, it could have been before the accident, it could have been some time after the accident, we're not 100 percent sure. But all I do know is that she's gradually getting more and more problems with her spinal cord. (Page 26, transcript of Dr. Hansebout)

[624] Dr. Hansebout was unequivocal in his opinion that the plaintiff required spinal surgery before the 1998 accident occurred. (Page 26, transcript of Dr. Hansebout)

[625] Dr. Stewart was unequivocal in his opinion that the plaintiff required spinal surgery before the 1998 accident occurred.

[626] Both Dr. Lesiuk and Dr. Hansebout agreed that surgery cannot fix the problem of progressive cervical spondylotic myelopathy, but rather the purpose of surgery was to stop it from progressing further. Dr. Lesiuk testified that surgery would be mandatory when there are three things, intractable pain, signal cord change on the MRI, and hard myelopathy or radiculopathy. He did not say that it would not be needed without any of these three findings. Dr. Hansebout quite frankly agreed that surgery is mandatory in those circumstances. However, Dr. Hansebout was clear in his evidence that you don't wait for the permanent damage to occur before operating, you operate to ensure that the permanent damage is prevented. Counsel for the plaintiff tried to suggest in cross-examination that the surgery then became elective because it had not reached a point that it was mandatory. However, as Dr. Hansebout testified every surgery conducted is elective. The only elective issue was when she was going to have the surgery, not if she was going to have the surgery. Given the rate of the progression and the signs which had been exhibited prior to the 1998 accident, i.e., the equivocal Hoffman reflex, Dr. Hansebout, Dr. Stewart and Dr. Besemann all found that the plaintiff was in the category of the 40% of people whose clinical symptoms would progress. And we know from Dr. Lesiuk's April 1998 report that the progression could take the form of pain and/or sensory motor loss, the latter of which is what we argue was in the plaintiff's situation before the 1998 accident. Dr.

Hansebout also agreed on this point that the progression could continue to take place without the plaintiff necessarily experiencing pain.

[627] In his report (page 46) Dr. Hansebout specifically states that:

The accident of December 1998 has little, if any, bearing on her current disability, which is mainly the result of her progressive cervical spondylosis. This was progressive before the 1998 accident, progressive encroachment on the spinal cord spondylosis would have required surgery even if she had not been involved in the 1998 motor vehicle accident.

[628] We know from Dr. Bourque's report that he felt that if the plaintiff started seeing a progression of neurological deficits that she would likely report any significant deterioration. Although this deterioration happened, it is clear that she did not report it to either Dr. Lesiuk or Dr. Bourque. The following took place after seeing Dr. Bourque in July 1997 and Dr. Lesiuk in April 1998, and the plaintiff never reported to either of them the following:

- She did not report the rear-end collision, wherein she broke her foot while in a golf cart. The Court may recall that we did not have an opportunity to cross-examine the plaintiff on this issue, as she, notwithstanding the accolades she received for being a great historian, said that she did nothing to break her foot. It was apparent throughout her testimony that where her testimony related to something detrimental to her case, her ability to remember was compromised, (for example, moving the trailer off the road after the accident). Here again, the plaintiff knows that any minor trauma could cause her injury and she neglects to advise the Court of this incident; we only learn of it from Lynn Parker. Notwithstanding that Lynn Parker stated that it had been a minor contact – it was significant enough to break her foot and this would fall within the very circumstances she was warned to be careful about a minor trauma. Here we have a situation where there would be no seatbelt – no headrest to protect from an extension injury. This happened after the July 1997 MRI and there wasn't another MRI done until after the 1998 accident in June 1999.
- We know that Dr. Lesiuk thought that after the 1998 accident:

She finds that she has great difficulty continuing with her work, which involves considerable keyboarding. This represents a functional change from prior to the second accident. (Taken from his January 28, 1999 report)

However, upon being confronted in cross-examination with the note of Dr. Vandewaetere, dated June 19, 1998, which states:

Her life is restricted – can't type, difficult with mouse, can't do sports – can't even cake decorate. (Exhibit 3, Tab 1, page 14)

Dr. Lesiuk saw that this functional change, which he obviously saw as being significant enough to comment on his report, had taken place before the 1998 accident.

- The clinical note of Dr. Vandewaetere, dated June 19, 1998, indicated that the plaintiff experienced the feeling that her bladder is full and can't release – which is a neurological symptom, plus she is continuing, for over a year now, to have some urge and stress incontinence (Exhibit 3, Tab 1, page 15). This is significant enough that it warranted a referral to a urologist, who diagnoses her with irritable bladder, which is a neurological symptom.
- Before the 1998 accident, the same note further indicated that she had experienced vertigo in May of 1998 and since then, when she extended her neck, she got more vertigo. As of June, she was continuing to experience vertigo when she lied down and her head falls off the pillow or with sudden movements. She recounted one episode where she was waiting for the train and actually fell down. One would have thought, given the increased risk of spinal injury with minor trauma noted back in 1997, that she would have reported this to her neurosurgeon. Dr. Lesiuk testified that, depending upon the nature of the fall, it could have caused the suggestion of signal cord change.
- The plaintiff testified that the endurance in her arms and legs had deteriorated to the extent that she could not swim the lake at the cottage like she used to, and now all she could do is

go from the dock to the shore. As well, she testified that, by the fall of 1998, she was no longer swimming with her friend at the pool, as she had done previously.

- The plaintiff testified that she would always use a golf cart rather than walking a golf course.
- In addition, there were a number of other injuries producing events that she did not advise Dr. Lesiuk about. He testified that he only needed to know the symptoms not the causes, as his role was only management of the symptoms. Much of her history would be relevant, we submit, to the causation issue, and Dr. Lesiuk had very little in the way of information, when he rendered his opinion in May 2001.
- And finally, we know that her level of endurance regarding her legs had deteriorated to the point that, in the month before the accident when she was in Las Vegas, she was unable to keep up with her friends on their shopping expedition and had to be in a wheelchair. The plaintiff testified that this happened, she told Dr. Besemann and he reported it in his CAT DAC report, she reported it to Dr. Stewart at the IME and this is reflected in his report, and Mr. Perrozzino testified that this occurred. We would suggest that the evidence of her close personal friends was incorrect, in that the plaintiff had a specific recollection of which plaza it took place at (Bartolo), and although both Ms. Parker and Ms. Morrison testified that she was in a wheelchair on their 1999 shopping trip, both testified that they didn't attend that specific plaza (Bartolo) in 1999.

[629] Dr. Lesiuk, when asked why, when he had two concurring opinions on Ms. Monks' condition, he was seeking another opinion from an internationally known spinal surgeon, he replied that he thought there was room for disagreement about management here. Clearly, Dr. Lesiuk never testified that the plaintiff didn't need surgery before the 1998 accident. The best he could say was at the time he was managing it conservatively, but he felt that that might not be the right answer, so he was sending her off to an internationally known spinal surgeon, for his opinion on this. He was troubled by the case and was thinking that maybe surgery should be conducted for what he stated, "amounts to non-classical indications". All Dr. Lesiuk could say was that, "I hadn't formulated in my mind that she had a definite need for surgery".

[630] Dr. Hansebout testified that, clearly, Dr. Lesiuk was concerned about the need for surgery and the fact that it was complicated surgery, because he sent her to Dr. Fehlings and you don't send a patient from Ottawa to Toronto unless you are worried. Dr. Hansebout was unequivocal in his opinion that the plaintiff was in the 40% category where her symptoms will continue to clinically progress. He stated that he was in a position to make this finding, based on the information available to him, by way of radiological documents and medical documents.

[631] It is apparent that her treating neurosurgeon felt that she was not a classic case for surgical intervention and that Dr. Hansebout is of an opposing opinion, in that he believes that she required the surgery before the 1998 accident. A very critical piece of evidence that relates to this difference in opinion is the fact that, once signal cord change occurs, then permanent damage has occurred, which cannot be fixed by way of surgery. Dr. Hansebout's opinion, therefore, is to conduct the surgery before the permanent damage is done, knowing that any damage cannot be repaired. Whereas, Dr. Lesiuk told her that if she had an MRI in the future and it showed signal change, (meaning the permanent damage had already taken place), then it was time to conduct the surgery. He gave that opinion to Ms. Monks after warning her that an extension injury could cause her to be a quadriplegic (page 70, transcript of Dr. Hansebout). One would expect that the reasonable avenue to follow is to conduct the surgery prior to the permanent damage being done, (when that damage could in fact be that you are a quadriplegic), rather than wait to take your chances that it won't happen or that the permanent damage done by way of the signal cord change is not to debilitating.

[632] Although plaintiff's counsel attempted to get Dr. Hansebout to admit that something significant happened between April 1998 and January 1999, relating to the signal cord change, Dr. Hansebout did not agree. He replied that given that the last radiological finding before the June 10, 1999 MRI was in 1997, it was apparent that a change had occurred over that two-year period; however, there is no connection whatsoever to the change occurring at the time of the accident. In fact, the evidence supports that it is as a natural progression.

[633] Plaintiff counsel was also unsuccessful in attempting to discredit Dr. Hansebout's evidence and report by attacking the factual underpinnings. Particularly, plaintiff counsel tried

to get Dr. Hansebout to admit that the urinary symptoms are rarely seen with spinal stenosis. In fact, Dr. Hansebout confirmed that Dr. Walsh had diagnosed irritable bladder just weeks before the 1998 accident occurred (his report is dated December 9, 1998 (Exhibit 1, Tab G-3)), and that meant it could certainly be indicative of spinal stenosis (pages 95-96, transcript Dr. Hansebout). Contrary to plaintiff counsel's suggestion that there are no urinary problems between December 9, 1998 and the September 1999 surgery, it is a fact that in April 1999, Ms. Monks had raised complaints to Dr. Chow of "having a bladder problem she felt was an irritable bladder and that the irritability of the bladder seemed to correlate with neck pain". (Exhibit 1, Tab D-1, page 1)

[634] Although plaintiff counsel was attempting to lead Dr. Hansebout into believing that the plaintiff had attended a number of educational programs in 1998, he neglected to advise him that the few programs she attended were one or two-day seminars and were not full time programs. The evidence is clear that she could not and did not continue with the Insurance Institute courses, which were very important to her career development.

[635] The plaintiff's counsel successfully pointed out that the plaintiff was not off work from both her jobs (only the bus job), therefore attacking a factual underpinning of his attributing a percentage of her disability to the 1993 accident. The plaintiff did not succeed in proving that there was no basis for that percentage attributed to the 1993 accident, as clearly Dr. Hansebout found that there was spinal narrowing before the 1995 accident, thereby conforming the correctness of the assessment done by him.

ii) The Extent of Injury Sustained in the 1998 Accident

[636] The defendant argues that the plaintiff did not sustain a neurological injury at the time of the 1998 accident.

[637] The plaintiff testified that the motor vehicle accident occurred at 5:00 a.m. on December 23, 1998. The plaintiff testified to the events of the accident, but noted that her memory was fuzzy for one short period of time, which was just when she exited the vehicle. However, on cross-examination, her recall of that period of time came back and she admitted that she had assisted with moving the trailer off the road. She had not experienced a significant injury and

declined an offer for medical help, and it was only as a result of concern for her mother that an ambulance had attended at the hotel.

[638] The fact is that the hospital record (Exhibit 5, pages 2 and 5) indicates that the plaintiff attended at the St. Vincent Trauma Centre at 8:46 that morning. The emergency department (page 3) noted her injury as right breast and neck pain. The Trauma Centre record reveals that she was asked specifically whether she had weakness or numbness and she denied having any weakness or numbness. She underwent neurological testing and it was determined that she had no focal deficits. Finally, her discharge instructions were as follows: “F/U [follow-up] with your MD in Can. Consider further evaluations if parathesia or pain”.

[639] Her admitting diagnosis was acute cervical strain (page 5). The Daily Order Summary (page 6) reflects that her symptoms are pain only and there is no mention of any neurological symptoms. The Interdisciplinary Note (page 7) indicates that she was discharged at 11:00 a.m. The Final Coding record (page 8) reflects a diagnosis of sprain of the neck. Again, the Radiology Consultation report (page 4) reflects that her only symptom is pain with no neurological findings.

[640] A very important factor which supports ING’s position is that the results of the x-ray taken at the time of the accident show nothing different than what had been shown through the previous radiological findings. Although plaintiff’s counsel tried to suggest that an MRI would have been a better diagnostic tool, Dr. Stewart denied this, as an x-ray will show arthritis, which is the plaintiff’s problem. He noted that the only advantage that would have resulted in undergoing an MRI would be that it shows soft tissue. Accordingly, the increased disc protrusion, which ultimately appeared on the June 1999 MRI, which caused the suggestion of signal cord change, would have had to have been reflected on the December 23, 1998 x-ray, if it had occurred as a result of the accident. That disc protrusion was not found on that x-ray and, therefore, the only reasonable conclusion could be that the 1998 accident did not cause signal cord change, nor the increased protrusion. This is consistent with Dr. Stewart’s opinion (and the radiologist’s comment on the report) that the suggestion of signal cord change was as a result of

swelling (edema), which Dr. Stewart testified could not have been caused by the 1998 accident and would have occurred in the short period (week) prior to the testing in June 1999.

[641] Prior to this accident, Ms. Monks was a woman who had spent the last three and a half years being concerned about her weakness and clumsiness in her arms with neurological symptoms, which were deteriorating so much so that she had insisted on seeing a neurosurgeon and was going to undergo surgery. Both the plaintiff and Dr. Lesiuk testified that, prior to the 1998 accident, the plaintiff had been told to monitor her own symptoms and to watch specifically for myelopathic symptoms, which she knew included numbness and weakness. As a nurse, she was well aware of the importance of reporting all symptoms, particularly when she had been specifically advised that trauma, particularly a motor vehicle accident, could render her a quadriplegic. If the plaintiff had been experiencing numbness and weakness, she would have reported it to the doctor and this would have been reflected in the hospital records. Instead, it clearly notes that she denied these symptoms. Her excuse to justify not reporting it was that she had not experienced the symptoms while at the hospital. That could not be true. She said the symptoms had lasted six hours from the time of the accident and the accident occurred at 5:00 a.m.; she had been admitted at 8:46 a.m. and had been discharged at 11:00 a.m.; accordingly, if it were true, she would have had to have experienced those symptoms for the entire period she was there. She then changed her evidence and indicated that she did not report it because she was more concerned about her mother. In essence, what she would have this Court believe is that when she was examined on her own and she answered a specific question about numbness and weakness, she gave an incorrect answer because she was concerned about her mother? We argue that, given all that she had endured in the last three and a half years, and given her background as a nurse, if she had truly experienced neurological symptoms, she would have reported those neurological symptoms if she had experienced them at the time.

[642] Even if the Court were to accept this explanation, it does not explain why the plaintiff did not report these symptoms to the other doctors she had seen subsequently. It is interesting to note that every account and recitation about the 1995 accident clearly includes annotation about her immediate numbness and tingling. However, this woman has seen upwards of 50 doctors

subsequent to her surgery, yet the only doctor who has any note of neurological problems, immediately following the 1998 accident, is Dr. Vandewaetere. Of note is the fact that if she did experience neurological symptoms, given the warnings given to her by Dr. Lesiuk, one would anticipate that she would have been very concerned about the symptoms and would have sought medical treatment immediately upon her return to Canada, yet she only sees the family doctor ten days after her return to Canada.

[643] It is readily apparent from reading the thousands of pages of medical documents that whenever the plaintiff retold the story of her 1995 accident, she always mentioned that she suffered immediately numbness and tingling in both arms, which lasted only 15-20 minutes. This indicates that she obviously was aware of the importance of reporting this symptom and is clearly indicative that this is what occurred at the time of the 1995 accident.

[644] However, when it comes to the 1998 accident, she is now trying to convince the Court that she experienced immediate neurological symptoms of numbness and tingling, which lasted six hours after the accident, but that she does not mention it to, nor is it recorded in the records dated:

- December 23, 1998 – the ambulance attendant who brought her to the hospital; the attending physician and/or triage nurse at the hospital; the radiologist at the hospital.
- January 28, 1999 – Dr. Lesiuk on her first visit to him after the accident nor at any subsequent visits after that.
- April 1, 1999 – although she specifically tells Dr. Chow about the numbness in her arms after the 1995 accident and she advises him of symptoms after the 1998 accident, she does not mention any numbness or tingling after the 1998 accident. Her only comment is on her present state at the time of the visit, which was that at that time (being April 1999) her arm symptoms were tenfold (note particularly the use of different tenses in his language).

- June 5, 2001 – there is no mention of any numbness or tingling immediately after the 1998 accident in Dr. Weinberg’s report, although it is apparent that he met her and one would expect that she would be telling an IME doctor about all of her symptoms and particularly with her knowledge about the significance of this type of symptom to her injury (Exhibit 1, Tab G-4).
- June 6, 2001 – the plaintiff again attended in person for an independent medical examination with Dr. Attack. Although there is an extensive description of the details of the 1998 accident, her immediate symptoms and her subsequent attendance at the hospital, there is no mention of numbness and tingling following the 1998 accident (Exhibit 1, Tab G-5, page 6).
- July 20, 2001 – CAT DAC – again this is a situation when Dr. Besemann met with the plaintiff twice in person and she was aware that he was assessing her as to what role the 1998 accident played in her overall condition. Here, he specifically notes in his detailed description of the 1998 accident and the resulting symptoms that “there were no immediate neurological symptoms, however. She tried to lie down when they arrived at the hotel, but she had too much neck pain.” The report goes on to note, “It was difficult for her to tell if this was significantly different from previous”. (Exhibit 1, Tab G-6, page 9)
- June 11, 2002 – The plaintiff met in person with Dr. Stewart. At this point, her action with Zurich is settled and she is aware that she is attending an independent medical examination. In describing the accident and her symptoms following the accident, she stated that, “she remembers experiencing chest pain, neck pain and shoulder pain ... She thought there might be some further weakness in the right upper extremity.” There is absolutely no mention of any immediate neurological symptoms and, in fact, when she is recounting her experiences to both Dr. Besemann and Dr. Stewart, she tells them that she is not even sure whether there was any change in the weakness that she was experiencing in her right upper extremity. Dr. Stewart comments that the physician found no neurological signs and commented that she had a stiff neck during the holiday. That latter piece of information had to have come from the plaintiff herself (Exhibit 1, Tab G-7, page 3).

- October 19, 2002 – The plaintiff provides another detailed description to the neuro-psychologist, Dr. Collins, yet the report reflects that she “was not immediately aware of any injury or symptoms”. The only symptom mentioned was neck pain.
- November 5, 2002 – again in the report of Dr. Gerin-Lajoie, there is no mention of any neurological symptoms immediately following the 1998 accident (Exhibit 12, Tab 17).
- February 4, 2003 – In Dr. Hansebout’s extensive review of the existing medical documents, there is no reference to a complaint to any doctor or treating medical specialist of immediate numbness and tingling after the 1998 accident.

[645] When asked about the relevance of there being no numbness or tingling, Dr. Hansebout testified that:

Now if she would have had injury to the spinal cord, a severe injury to the spinal cord, one would have expected immediate problems, which could have ranged anywhere, you know, increased numbness, increased weakness, to complete paralysis.

But just like in her 1995 accident, she complained of immediate numbness in her arms for 15 minutes. She didn’t have anything like that. Therefore, I conclude that this injury of December 1998 did not damage the nervous system. It did some soft tissue injuries, meaning muscles and ligaments in the neck and around her chest. (Page 34, transcript of Dr. Hansebout)

[646] Dr. Hansebout also testified that an increase in neck pain does not mean an injury to the spinal cord. It could be an indication that the neck muscles and ligaments are stretched or hurt (which is consistent with a diagnosis of cervical strain given by the attending doctor at the time of the 1998 accident). Dr. Hansebout also went on to testify that the prescription for Percocet, which the plaintiff eventually progressed to, some time after the 1998 accident, is consistent with the diagnosis of cervical strain and, in fact, Dr. Stewart testified that if the plaintiff had the neurogenic pain that she alleges that she had immediately following the accident, Percocet is not the appropriate medication for such symptoms.

[647] Dr. Hansebout testified that the fact that the plaintiff could get out of the vehicle, help move the trailer off the road and pick up luggage from the road (all of which he learned from his review of the file), he concludes that she had been moving pretty well.

[648] As we know, the plaintiff testified that she was an experienced accident benefit adjuster and knew that any expense associated with conducting an MRI would be covered either by OHIP or by the insurer. If the plaintiff had experienced immediate numbness and tingling, as is recorded in Dr. Vandewaetere's notes, given her previous knowledge about these symptoms and the concern that she had about minor trauma affecting her spinal cord and the knowledge that the cost would have been covered, she would have insisted on an MRI being performed. The fact that she did not proceed with an MRI supports the facts as set out in the emergency record that she did not experience numbness or weakness.

[649] Plaintiff counsel spent some time trying to suggest that there was an extension injury in the 1998 accident. However, Dr. Hansebout testified that if you go forward and back, the going back is probably not the same intensity as if you were hit from the rear, and he noted that her head would have hit the headrest. He also testified that, given the angle it was hit, it is not a significant extension. Common sense tells us that the amount of bruising to her breasts was likely due to her size behind the wheel, in that there was likely very little room between her and the steering wheel.

[650] Plaintiff counsel went to great lengths in his cross-examination of Dr. Hansebout to comment on the fact that the plaintiff is a good historian and reliable. As a reliable, good historian, she would have advised the hospital doctor of any numbness, tingling and/or weakness at the time of the accident. She did not do so.

[651] The plaintiff did testify to a heaviness in her right arm, which is not a neurological symptom.

[652] It is the position of ING that the extent of the injury sustained in the motor vehicle accident of 1998 was a minor cervical strain. ING argues that it is confirmed by the existing documentation.

iii) The History of the Progression following the 1998 Accident

[653] The plaintiff testified that she experienced neurogenic pain, immediately following the 1998 accident.

[654] Notwithstanding the alleged significant increase in pain, the plaintiff was not prescribed any pain medication by the doctor at the hospital, and she continued on her vacation and remained in Kentucky for a two-week period.

[655] Although she testified that they had stayed longer in Kentucky due to her injury, the fact is the reason they stayed longer is that they left once they got the call that their vehicle was ready to be picked up.

[656] During the time she was in Kentucky, she did not receive any prescriptive medication and most telling of all is the fact that, if she was in such pain, why, when she returned to Canada on January 5, 1999, did she not attend on any doctor until ten days later when she saw her family physician on January 15, 1999.

[657] Of particular note was Dr. Stewart's comment that, "if one injures the cord, one has acute permanent problems". That is not seen here.

[658] She saw her family doctor on January 15, 1999, and she had not seen her doctor since June 19, 1998. The family doctor testified that she made no inquiries of the plaintiff of her progression in her symptoms from June 19, 1998 up to the date of the accident in December 1998 and, therefore, would have had no knowledge of the extent that her symptoms progressed. Specifically, she would have had no knowledge as to the deterioration in the endurance in her legs, which we know from the evidence is a very significant neurological symptom or myelopathic finding.

[659] The plaintiff testified that for the period between the December 1998 accident and the surgery in September 1999, she experienced neurogenic pain starting immediately after the accident. However, notwithstanding that she had seen her family doctor on five occasions during that time, there is no mention of neurogenic pain in any of those appointments, nor does she report neurogenic pain to any doctor until sometime the year after the September 1999 surgery. Her own doctor, Dr. Vandewaetere, testified that, between January 15, 1999 and her surgery in September 1999, the plaintiff does not present with any complaints of neurogenic pain, but that the first mention of same is sometime after the surgery in September 1999. Again, this is another instance where the plaintiff's evidence is clearly not supported by the records or even by her own doctor.

[660] Although the plaintiff testified that she was in significant pain, she only saw her family doctor four times after the initial appointment in January 1999; one of those visits was for a prescription renewal. The strongest medication, which she was prescribed, was Percocet. Percocet is the same pain medication the plaintiff was prescribed when she injured her coccyx and, therefore, prescribing Percocet would be consistent with treating pain for a minor cervical strain. Further, Dr. Hansebout testified that Percocet would be the appropriate medication for treating pain arising from a cervical strain, and Dr. Stewart testified that Percocet would not be an appropriate prescription if she, in fact, were experiencing neurogenic pain, as she alleged she was during that time.

[661] She saw Dr. Chow in April of 1999 and it is evident that her urinary symptoms had continued, as she noted having a bladder problem, which she felt was an irritable bladder and that the irritability of the bladder seemed to correlate with the neck pain. This symptom was diagnosed on December 9, 1998, by Dr. Walsh and clearly is a neurological problem that had existed prior to the 1998 accident.

[662] ING argues that most telling of all was the video surveillance. This video was taken in May of 1999, five months after the 1998 accident. The video clearly showed the plaintiff showing no sign of pain or discomfort. Clearly, this surveillance does not reflect the physical state, which the plaintiff testified that she was experiencing after the 1998 accident. She was not

suffering the reduced endurance she testified to, given that she alleged putting in an 18.5-hour workday in the first part of the video. The plaintiff was then viewed the following day, bending into the back of her Expedition vehicle and lifting different sized brief cases into the back of the vehicle. Her movements were smooth and fluent and again there was no sign of any pain or discomfort exhibited by the plaintiff. The plaintiff's sworn evidence concerning her health is clearly inconsistent with what the video shows. This video surveillance clearly reflects a situation, which is also inconsistent with what Laurie Warren, her personal friend, testified to and clearly discredits her testimony as to the plaintiff's physical status during that intervening period between the 1998 accident and the surgery. What deterioration may have taken place during that nine-month period of time was due to, we submit, the natural progression of her cervical spondylotic condition.

[663] Upon her return to Canada on January 5th, the plaintiff testified that she immediately returned to work and continued to work right up until the surgery in September 1999.

[664] Dr. Lesiuk testified that when he saw the plaintiff after the 1998 accident, she had not talked about the progression of her symptoms over the eight months prior to the accident. Therefore, he had no knowledge about the motor vehicle accident with the golf cart, nor any details relating to her fall from the vertigo, both being incidents which Dr. Lesiuk testified could cause a spinal cord injury. He further noted that her inability to keep up with her friends walking and requiring the use of a wheelchair was certainly something that she should have told him.

[665] Of particular note is that Dr. Lesiuk, in his report dated January 28, 1999, indicated that the plaintiff had remained clinically stable and he stated specifically in that report that "the overall distribution and character of her symptoms is the same as has been described in previous correspondence, but she feels they're more severe". Dr. Hansebout noted that the report revealed that she had complained of slightly more weakness of the right hand and when Dr. Lesiuk had examined her, he indicated that the finger flexors on the right might be more weak than they had been in the past. Dr. Hansebout, in commenting on this, noted that it had been many months since Dr. Lesiuk had seen her previously (from April 1998 – January 1999) and he would have

no knowledge of how much her natural cervical spondylosis had caused further deterioration in the meantime.

[666] Dr. Hansebout was clearly of the opinion that if there was any increase in weakness in the right hand, it was due to the natural progressive history of the cervical spondylosis (page 36, transcript of Dr. Hansebout).

[667] In the same vein, Dr. Stewart commented that the findings by Dr. Lesiuk regarding her right hand weakness were equivocal and therefore, Dr. Stewart's impression from the treating neurosurgeon is that overall things had not changed by this period in time, being the first examination after the 1998 accident. Dr. Stewart confirmed that there were no changes in the neurological examination conducted by Dr. Lesiuk after the accident.

[668] It is important to point out that there are no objective findings that support her subjective reporting that her symptoms are more severe. It is also very interesting to note that she did not consistently tell the same story to different doctors about the increase in her symptoms:

- December 23, 1998 – It is the plaintiff's own evidence at trial that the attending doctor had noticed that her right arm was weaker and the plaintiff had advised the doctor that it was pre-existing. She testified that the doctor had asked her if it was weaker and she had replied: "I was not able to judge".
- July 20, 2001 – CAT DAC – Dr. Besemann specifically noted in his detailed description of the 1998 accident and the resulting symptoms that "there were no immediate neurological symptoms, however. She tried to lie down when they arrived at the hotel, but she had too much neck pain." The report goes on to note, "It was difficult for her to tell if this was significantly different from previous". (Exhibit 1, Tab G-6, page 9)
- June 1, 2002 – The plaintiff met in person with Dr. Stewart. At this point, her action with Zurich was settled and she was aware that she is attending an independent medical examination. In describing the accident and her symptoms following the accident, she stated

that “she remembers experiencing chest pain, neck pain and shoulder pain ... She thought there might be some further weakness in the right upper extremity.”

- October 19, 2002 – The plaintiff provided another detailed description to the neuropsychologist, Dr. Collins, yet the report reflected that she “was not immediately aware of any injury or symptoms”. The only symptom mentioned was neck pain.

[669] Even if the Court were to accept that there was a change in the power of her right arm, Dr. Stewart noted that weakness in the right arm (which we know pre-existed the 1998 accident) is assessed in total, pursuant to the American Academy Standards, as 7% and therefore, the change in this weakness would reflect some marginal portion of that 7%.

[670] The August 11, 1999 report of Dr. Lesiuk indicated that, clinically, the plaintiff felt that she was continuing to insidiously worsen which, Dr. Hansebout testified, meant that her cervical spondylosis was progressing in an extremely subtle way, in a gradual downhill progression. This, he testified, is very much consistent with her ongoing natural progressive degenerative spinal stenosis, which predated the 1998 accident. Further, he commented that the indication in the report that she continues to manifest predominantly cervical pain, reflected neck pain, which usually is due to soft tissue stretching or ligaments, but is not indicative of a spinal cord injury (page 37, transcript of Dr. Hansebout). When you take into consideration the evidence, which suggested that she is unsure of the extent of the change in weakness, if any, the fact that there are no objective findings to support a finding that there has been a change in weakness and her own evidence that any change has been subtle or insidious, again supports Dr. Hansebout’s medical conclusion that her symptoms are as a result of the naturally progressing degenerative disc disease.

[671] Dr. Lesiuk’s report, dated May 29, 2001, concluded that a significant role can be ascribed to it (the 1998 accident) in aggravating and hastening the progression of her symptoms. However, a careful review of the evidence relating to the post accident symptoms will not support the conclusion that the 1998 accident aggravated or hastened the symptoms.

[672] From the point of view of the clinical symptoms, ING argued that the plaintiff herself told a number of doctors, including the doctor at the hospital, that she was not able to judge if there was any difference in the weakness in her right arm. The objective findings tell us that there may very well not have been any increase in the weakness of her arm and, in essence, all we have is an increase in pain, which we know could very well relate to the cervical strain and have nothing to do with the spine at all. Both Dr. Lesiuk and Dr. Hansebout testified that signal cord change could occur without the patient experiencing pain.

[673] The MRI taken in June of 1999 revealed that disc protrusion at C4-5 is relatively the same as the May 1997 MRI. The increase in the disc protrusion at C5-6 indicated that, since the last MRI (10 months earlier), there had been some increase in the cord compression to the point that it had increased to suggest a signal cord change. Dr. Hansebout testified, specifically, that this was not a significant change in the compression itself; that it just meant that there was slight more pressure, which now reflected as a possibility of signal cord change. We know from Dr. Lesiuk's and Dr. Hansebout's testimony that if there was signal cord change that this would be permanent and therefore, because signal cord change does not show up on the subsequent MRI, the fact that the report said there was a "suggestion" was obviously not a confirmed finding. Therefore, there was no signal cord change in June of 1999, otherwise it would have had to have appeared on the December 1999 MRI (because we know that surgery cannot repair the problem, only attempt to stop the progression). Further, Dr. Stewart in his report states:

I don't believe that the progression of signs seen in the MRI after the 1998 accident were, in any significant way, caused by that accident.

[674] Dr. Stewart explained this statement in his testimony when he indicated that the MRI reflected a small zone of increased T2 and decreased T1 signal in the spinal cord at the C5-6 level, suggesting a small area of cord edema. He explained that this change in the T2 and T1 level appeared to be as a result of edema, which meant swelling. He also testified that that swelling could not have been there for the six-month period, between the time of the accident and the time of the MRI. He stated specifically that the edema, at best, would have been as a result of something, which occurred within a week of the testing.

[675] In commenting on the June 10, 1999 MRI, Dr. Hansebout indicated that the osteophytes that had increased could not have done so since the 1998 accident, as it is a gradual thing. He stated that the osteophytes are formed because the discs get narrower, the annulus bulges out and becomes calcified and, therefore, the resulting disc protrusion is the product of a gradual process. He stated that when the spinal cord is narrowed to six millimeters by osteophytes, the osteophytes are narrowing the cord. He noted that either way, even if it was the disc protruding that retro-displaced the cord, it was, in his opinion, due to the cervical spondylosis. (Page 146, transcript Dr. Hansebout)

[676] When asked by the plaintiff counsel after a long cross-examination: “Isn’t it the most consistent explanation for that change was what happened in that 1998 accident?” Dr. Hansebout replied:

I don’t think so. I think it’s due to progressive degenerative problems and I think that is her whole problem. And I have dealt with spines for many years and I am a specialist in this and I have done it and I have seen this happen in many cases.

He went on to say unequivocally that:

I don’t think that this accident caused all those disc protrusions, that would be just impossible almost. (Page 147, transcript Dr. Hansebout)

Further, Dr. Hansebout adamantly stated that:

...and the reason I said that one should have considered surgery more forcefully initially that if there is deterioration of the spinal cord due to spondylotic myelopathy you don’t get normal anymore, it’s too late then. ...with an accident or not, it still progresses.

Plaintiff counsel questioned, “could have?” and Dr. Hansebout emphatically replied, “it does”.

[677] In reviewing the report prepared by Dr. Lesiuk, dated May 29, 2001, Dr. Lesiuk commented about his clinical examination of the plaintiff in 2000 (after the first surgery) “there had been very slow, but clear function deterioration, despite a variety of conservative measures”. This, Dr. Hansebout testified, was again further evidence that is consistent with progressive

degenerative disc disease for which the 1998 accident played no role. Dr. Hansebout stated that sometimes people don't get better from the first surgery with cervical spondylosis, they don't always get better. This reflects continuing compression of the spinal cord (page 39, transcript of Dr. Hansebout).

[678] Of particular significance is the fact that Dr. Lesiuk didn't even have the critical records from the hospital, in Pennsylvania, which indicated that the plaintiff had not experienced any numbness or weakness, and that the neurological testing had shown no focal points, when he wrote this opinion. Dr. Hansebout also indicated that Dr. Lesiuk would not have had the family doctor's records.

[679] Initially, Dr. Lesiuk, in his first opinion rendered on May 29, 2001, had indicated "regarding the 1998 accident, I believe a more minor but still significant role can be ascribed to it in aggravating and hastening the progression of her symptoms". (The symptoms remain similar in nature and character, but the pace of clinical and structural deterioration increased.) His next report, dated November 11, 2002, had noted his opinion as being "regarding the 1998 accident, I believe it too played a very significant role in exacerbating her condition". Dr. Hansebout commented that he saw this as a change in the opinion and indicated that there was no medical documentation generated on this file, which would warrant a change in the opinion. Here again is another example of Dr. Lesiuk advocating for his patient.

[680] Initially, the May 29, 2001 opinion was written with a view to supporting the plaintiff's position in her action against Zurich, however, the second report was generated at the request of the plaintiff, for no apparent reason, after the Zurich settlement had taken place and the action was now only against ING.

[681] This opinion, as to the contribution of the 1998 accident, is in contrast to every other doctor who testified and is also in contrast with Dr. Desforbes' diagnosis. Dr. Desforbes was the plaintiff's treating doctor at the Rehabilitation Centre, after her second surgery (Exhibit 26). In Dr. Desforbes' discharge summary, he noted that the plaintiff's diagnosis is quadriparesis from spinal stenosis. This, Dr. Hansebout testified, meant "weakness of the arms and legs due to

smallness of the spinal canal, which puts pressure on the spinal cord, in this case, it's an acquired stenosis". (Page 40, transcript of Dr. Hansebout). This, Dr. Hansebout clearly testified, is suggestive of something naturally progressive (page 41, transcript of Dr. Hansebout). No evidence was given by Dr. Lesiuk to contradict this.

[682] Dr. Hansebout also had the benefit of reviewing Dr. Stewart's report who had examined the plaintiff personally. In that report, it is noted that she is unsteady on her feet, which means it's spinal cord dysfunction, and that her muscle power in the upper extremities was reduced, which indicated there had been more progression despite two surgeries. There is evidence of clonus, which is a repetitive jerking of a muscle due to stiffness, which is an abnormal sign usually resulting from a dysfunction of the spinal cord. It is her naturally progressing spondylosis and her increased weight to which Dr. Hansebout attributes her falls.

[683] In providing his opinion as to the relationship between the plaintiff's physical condition and the 1998 accident, Dr. Hansebout was clear in his opinion that the accident caused soft tissue injuries causing further neck pain, but that the 1998 accident was not a big problem in her life. His diagnosis is that she has degenerative disc disease in the neck, and cervical spinal spondylotic myelopathy with myelomalacia and a cervical muscle and ligament distrain injury.

[684] Counsel for the defendant devotes 12 pages of her submissions on the credibility of the witnesses, as her submissions on the evidence and the medical evidence has already shown her position on the credibility of witnesses. I will simply add her comments on matters affecting credibility and not already discussed. Credibility is usually a matter best left to the trial judge.

[685] Counsel for the defendant argued that, given his over 40-year history in working extensively in the field of neurosurgery with the focus in the last 25 years on the spinal aspect, clearly Dr. Hansebout was the most qualified and experienced neurosurgeon who testified. His evidence, we submit, cannot be diminished by the fact that he did not personally meet with the plaintiff given that, as he testified, most diagnoses are based on history which either the patient recounts or which the documents reveal. Given the thousands of pages of medical material and other documentation, which was produced to Dr. Hansebout to review, and the fact that he had

the opinions of a lot of doctors who had actually met with her and had an opportunity to hear from the plaintiff directly, he was definitely in a position to provide his opinion regarding her need for surgery prior to the accident, and what effect the 1998 accident had on the plaintiff's condition.

[686] What was most telling about Dr. Vandewaetere's testimony was the open hostility, which she exhibited the minute that the cross-examination began. The light camaraderie immediately became confrontational upon being questioned in cross-examination. Clearly, the family doctor felt that she had to advocate on the part of her patient and this was evident, not only in her testimony, but as well in the summary, which she prepared of her clinical notes. Critical comments referring to neurological symptoms prior to the 1998 accident were noticeably absent from her summary, which she candidly stated that, in retrospect, she should have included. Further, the particulars of the symptoms she was reporting in June 19, 1998 were also left out.

[687] Therefore, it is the position of ING that Dr. Vandewaetere's notes and evidence cannot be relied upon, relating to the plaintiff's symptoms, given her lack of medical documentation, the fact that the plaintiff did not always report her symptoms to her, the fact that she made assumptions about what was reported to her, which were inaccurate, i.e., left arm a new symptom, and her obvious bias to favour the plaintiff.

[688] The only benefit of Dr. Vandewaetere's evidence and notes might have been to provide a factual underpinning for the neurologists and neurosurgeons to form their opinion in this matter. Clearly, her records and evidence provide little assistance for those purposes. There is no question and she testified herself that she would have to defer to the opinion of the experts, regarding the issue of contribution, if any, of the 1998 accident. Although it is interesting that after avoiding the question three times, Dr. Vandewaetere finally answered the question and stated clearly that she felt that the plaintiff should consider the surgery before the 1998 accident occurred.

[689] It is the submission of ING that no weight be given to the report of Dr. Becker, as he is a well-known advocate for plaintiffs. We refer you to *Elkaim v. State Farm Mutual Automobile Insurance Co.*, [1997] O.I.C.D. No. 113, wherein the arbitrator, Joyce Miller, repeatedly makes comments about his advocacy for the insured, and the fact that no weight should be given to his evidence. The arbitrator goes so far as to find that the insured, with the assistance of Dr. Becker, misled the insurer about his physical condition. Although Dr. Becker applied for judicial review of his treatment as a witness, in other words that the arbitrator failed to advise him that his veracity and *bona fides* as a witness and as a doctor who submitted medical information would be under attack, he was unsuccessful, as s. 11 of the *Powers and Procedures Act* did not protect him from adverse findings as to his credibility or criticism, based on his evidence or the medical reports he submitted. (*Becker v. Ontario (Financial Services Commission)*, [2000] O.J. No. 210)

[690] ING argued that no weight should be given to the opinion of Dr. Becker, as he too tried to usurp the role of the judge.

[691] ING argued the effect of a CAT DAC assessment and the credibility of Dr. Besemann, as follows:

DAC Assessment

[692] We set out our analysis of the CAT DAC in a separate section, given the uniqueness of this report and the evidence of Dr. Besemann. Truly, this is a neutral report generated by a neutral evaluator and should be given a significant amount of weight.

[693] Section 2(1.1) of the Statutory Accident Benefit Schedule defines catastrophic impairment and identifies the potential categories under which a person could be considered. In the plaintiff's case, she was considered under two potential categories: quadriplegia and where an impairment, or combination of impairments, result in 55 percent or more impairment of the whole person.

[694] Where there is a question as to whether an insured is catastrophic, the SABS allows for a CAT DAC to be conducted to make that determination. A DAC is an assessment centre

appointed by the Province of Ontario. The statutory process stipulates that insured person will attend at the DAC location, which is closest to where that person lives. Therefore, neither the insurer nor the insured has any input into which facility or which assessor conducts a DAC assessment.

[695] David Draper from the Office of the Director of Arbitrations commented on the concept of a DAC in the Appeal decision of *M.D. v. Halifax Insurance Company* (Appeal P00-00049). He noted that:

Unlike certificates, treatment plans and insurer examinations, neither party chooses the DAC. The assessment is done by the closest DAC with the appropriate expertise. ... The DAC provides an impartial assessment where the parties are unable to agree on the insured person's entitlement to accident benefits.

The DAC report is not just another opinion. Although either party can challenge it through the dispute resolution system, the DAC's conclusion governs the payment of benefits in the meantime. If it finds that the insured person meets the test for IRBs, or that the medical and rehabilitation expenses are reasonable and necessary, the insurer must pay. If, as in this case, the DAC reaches the opposite conclusion, the insurer does not have to pay. The 'losing' party's option is to apply for mediation and, if the dispute is not resolved, proceed to Court or arbitration.

Despite his suggestions to the contrary, they [DACs] are not simply another insurer examination. Their function is to take the dispute out of the back-and-forth of competing partisan reports by providing an impartial assessment.

[696] The process of a DAC is completely neutral and this DAC was no different.

[697] Dr. Besemann testified that the appropriate conflict of interest letter was sent out to the parties and it was determined that no conflict of interest existed. This is to ensure that the report is to be unbiased as possible. He then would have read the complete file, upon receiving it, to determine whether or not there was a requirement for the insured to attend for an examination. We know from the documents entered as an exhibit that all medical documentation, including Dr. Lesiuk's opinion of May 29, 2001, was forwarded to the DAC for review. In this case, it was determined that the plaintiff's attendance was required and the appointment was scheduled. Due

to the lack of time, the plaintiff was required to come back for a second day and the examination and report was completed.

[698] In terms of qualifications, it is clearly apparent that Dr. Besemann was well qualified to conduct such an assessment, having an expertise in physiatry, which included training in the field of neurology, orthopaedics, rheumatology. He testified that he was the Chief of Physical Medicine and Rehabilitation at the Hull Hospital, as well as the Rehabilitation Centre adjacent to the hospital. This unit, he indicated, treated some spinal cord injury cases, and specifically cases very similar to the plaintiff's. He testified to the breadth of his experience in treating patients with quadriplegia and indicated that he would have, on average, 3-6 cases per year. In addition, he testified that he had conducted at least 40-50 CAT DACs a year, of which six CAT DAC involved quadriplegia patients. Also, he conducts independent medical examinations, on average 40-50 per year, involving whiplash type injuries. Dr. Besemann began conducting CAT DACs in 1998 and was the only doctor providing this assessment, however, he now shares that responsibility with one other doctor.

[699] The plaintiff, out of great concern over the damaging content of Dr. Besemann's report and subsequent evidence, has tried to introduce a red-herring in this case.

[700] There is absolutely no evidence that the neutrality of the CAT DAC report was ever compromised. By virtue of the fact that the ING adjuster requested that the CAT DAC make a determination of the causation issue does not compromise the report. The adjuster would have had no contact with the assessor whatsoever. Clearly, the plaintiff was aware that this assessment was going on, as Dr. Besemann asked her point blankly what her opinion was as to the contribution of the 1998 accident.

[701] In terms of any allegations that the contact with ING counsel compromised the report is ludicrous given that the report was long written before the contact was made. Therefore, the evidence of the plaintiff's history given and the opinion expressed by the assessor therein should, we submit, be given a very significant amount of weight, given its neutrality.

[702] Thereafter, Dr. Besemann was very specific in saying that the opinions expressed in Court represented the opinions that he had when he generated the report in 2001. Only Dr. Besemann knows what his thought processes were in reaching the conclusion in 2001 that he did in that assessment. When cross-examined with the suggestion that the report could have contained the wording he was using in the Court, Dr. Besemann was confident in his response that the wording in the report conveyed the same meaning as what he testified to, however, he was careful to ensure that it was written in a neutral manner.

[703] To suggest that Dr. Besemann did anything but to tell his opinion as it was in 2001 is flat out wrong. It is insulting to his integrity and quite frankly, Dr. Besemann presented as a very knowledgeable and credible witness. This is particularly insulting to him, given his evidence of how important it is to get to the truth. On numerous occasions, during his testimony, he referred to the fact that it was his role to get to the truth and to tell the truth – a role, which he obviously takes very seriously. It is our submission that the evidence given by Dr. Besemann should be given the greatest weight, given his neutrality in reviewing, assessing and preparing his opinion.

[704] A careful review of Guideline no. 4, to which the plaintiff referred to in Court as being the provision which was allegedly breached, indicates that after a DAC is completed, the DAC assessor is not to provide treatment or conduct a medical/legal assessment for either the claimant or the insured. No such treatment or assessment was done by Dr. Besemann. There clearly has been no breach of Guideline no. 4, as alleged by the plaintiff.

[705] Plaintiff counsel raised the issue that he was not copied with notice that Dr. Besemann was receiving all of the updated medicals. These were all documents, which the plaintiff would have provided to ING and, therefore, the plaintiff was in possession of these documents. We note that plaintiff counsel provided no argument as to why the provision of the plaintiff's medicals compromised the neutrality of the witness. However, we submit that we failed to see how ensuring that the doctor has the plaintiff's complete medical history, somehow makes him something other than neutral. How can having all of her neurosurgeon's reports detrimentally affect the neutrality of his testimony (which the plaintiff was clearly suggesting happened, when Dr. Besemann referred to the November 2002 report)? Clearly, the provision in the guidelines,

and we emphasize that they are guidelines only, is meant to ensure that the assessing doctor is not influenced by either side, prior to doing his or her assessment. In our situation, not only had the assessment been done but, as well, the opinion rendered. Therefore, there can be no suggestion that the thought processes and the written opinion have been anything other than neutral. Due to lack of knowledge of the existence of this specific provision relating to contact with the DAC doctor, contact was made to have him attend at Court and to send him the updated medicals. This was done in order to provide Dr. Besemann with the plaintiff's complete history, which, obviously, is in the plaintiff's best interest. The conduct of counsel, in knowing that we had to have contacted the doctor in order to get him to attend at Court, and failing to bring to our attention the lack of including him in on those communications, seems unprofessional in our submission.

[706] The test for causation, according to the defendant, can be explained as a two-part inquiry in the present case:

1) Did Ms. Monks need surgery prior to the 1998 accident?

- If the answer to this question is "yes," then the 1998 accident cannot be said to have caused her catastrophic injuries.
- It is clear that the answer to this question is "yes":
 - Ms. Monks' condition had already progressed to such a degree that surgery was required prior to 1998, and should have taken place prior to the 1998 accident. I have reproduced the defendant's summary of the medical and non-medical evidence on this point. Key points include:
 - The Plaintiff herself testified under oath in the Zurich action that she required surgery prior to the 1998 accident, that she requested that Zurich cover her expenses for surgery, and that she would have proceeded with the surgery had Zurich agreed to cover her expenses.
 - The Plaintiff suffered from a pre-existing medical condition even prior to the 1995 accident, which played a significant role in the progression of her symptoms, arguably even to the point of requiring surgery.
 - There is expert evidence suggesting that the Plaintiff's condition had rapidly deteriorated prior to the 1998 accident, and that surgery was certainly required as of July 1997.

- Because her catastrophic injuries arose after the surgery, then having the surgery prior to the 1998 accident would likely have resulted in Ms. Monks already becoming catastrophically impaired prior to 1998.
- Therefore, it cannot be said that the 1998 accident materially contributed to the deterioration of Ms. Monks to the point of resulting in a catastrophic injury.

2) Even if Ms. Monks did not need surgery prior to the 1998 accident, did the 1998 accident materially contribute to the progression of her condition to the point of needing surgery in 1999, or would her condition have progressed to the point of surgery notwithstanding the 1998 accident?

- In the alternative, even if the Court finds that Ms. Monks did not need surgery prior to the 1998 accident, the Plaintiff must still prove that the 1998 accident materially contributed to the progression of her condition to the point of surgery (which ultimately resulted in the catastrophic injuries), in order to establish causation. If, instead, her condition would have progressed to the point of requiring surgery notwithstanding the 1998 accident, then it cannot be said that the 1998 accident materially contributed to the progression of her condition.
- It is clear that in this case, even if Ms. Monks did not require surgery prior to the 1998 accident, this need for surgery did not occur as a result of the 1998 accident, and would have transpired even in the absence of the 1998 accident. The surgery would therefore have occurred in any case, and Ms. Monks would still have become catastrophically impaired, even if the 1998 accident had never occurred. I have already reproduced the evidence and medical evidence the defendant relied upon. Key points include:
 - At the time of the 1998 accident, the Plaintiff denied having any neurological-related symptoms, and no neurological-related findings by any doctor.
 - Immediately following the 1998 accident, the Plaintiff returned to work and did not see her family doctor until 10 days later; and there was no record of any neurological pain.
 - There is no objective medical expert evidence that the Plaintiff's symptoms became worse following the 1998 accident.
 - The progression of the Plaintiff's symptoms following the 1998 accident is consistent with the natural and ongoing progression of her pre-existing condition.
 - Any worsening of pain or symptoms following the 1998 accident relate to the Plaintiff's diagnosis of cervical strain and do not relate to her condition of spinal stenosis.
 - Any progression of the Plaintiff's cervical condition was minimal and minute, and far below the standard of material or significant contribution required by the test for causation.

- Therefore, the 1998 accident did not materially contribute to the Plaintiff's catastrophic injuries.

2. THE LAW

[707] Counsel for ING pleaded the following cases as the law that should apply in this case.

[708] The Supreme Court of Canada in *Athey v. Leonati*, [1996] 3 S.C.R. 458*, set out the principal of the thin skull theory as follows:

This appeal involved a straightforward application of the thin skull rule. The pre-existing disposition may have aggravated the injuries, but the defendant must take the plaintiff as he finds him. If the defendant's negligence exacerbated the existing condition and caused it to manifest in a disc herniation, then the defendant is a cause of the disc herniation and is fully liable.

...

The so called "crumbling skull" rule simply recognizes that the pre-existing condition was inherent in the plaintiff's "original position". The defendant need not put the plaintiff in a position better than his or her original position. The defendant is liable for the injuries caused, even if they are extreme, but need not compensate the plaintiff for any debilitating effects of the pre-existing condition which the plaintiff would have experienced anyway. The defendant is liable for the additional damage, but not the pre-existing damage.

* This case was reviewed in my analysis of the plaintiff's case law.

[709] The defendant need not compensate the plaintiff for any debilitating effects of the pre-existing condition, which the plaintiff would have experienced anyway.

[710] The case law relating to accident benefit claims is clear – that the principals enunciated in the Supreme Court of Canada decision of *Athey* are equally applicable in the context of an accident benefit claim. We have a classic, unequivocal case of a crumbling skull plaintiff. When we apply this principle, then ING should not have to compensate the plaintiff for any debilitating effects of the pre-existing condition, which the plaintiff would have experienced

anyway. We submit that ING then is only obligated to pay for those expenses for those injuries caused by our accident, being a cervical strain. Those expenses have long been paid for.

[711] In the context of an accident benefit claim, the arbitrators have handled this situation whereby an applicant is injured as a result of two different accidents in which two different insurers were responsible for paying benefits by making a determination that one of the insurers must pay the benefit. This principle is clearly stated in the appeal decision of *Z v. Dominion of Canada General Insurance Co.*, [2003] O.F.S.C.I.D. No. 16, where the arbitrator noted that when assessing causation, the provisions in question require an “all of nothing” approach. This makes sense as it ensures that the concept of double recovery is avoided.

[712] In this case, the plaintiff has already been compensated by Zurich.

[713] In *Pisani v. Simcoe & Erie General Insurance Co.*, [1994] O.I.C.D. No. 129, upheld [1995] O.I.C.D. No. 201*, the applicant was injured as a result of two accidents in which two different accident benefit insurers were responsible for payment benefits. In that case, the arbitrator held, despite the fact that the insured testified that the second accident was the more severe of the two accidents, that the first insurer was responsible for payment of them because the second accident was not believed to have caused additional injuries. The arbitrator held that it was very difficult to determine which accident or accidents significantly contributed to the applicant’s fibromyalgia. The arbitrator accepted that both accidents aggravated the applicant’s symptoms and contributed to the subsequent development of fibromyalgia, but was not satisfied that they significantly contributed to the severity or duration of the applicant’s symptoms. This decision was upheld on appeal.

* My analysis of the cases in the next two pages is set out at the conclusion of the defendant’s submissions.

[714] The burden of proof of proving the causal connection between the accident and the continuing symptoms rests on the plaintiff. As noted by Osler J. in *Rothwell v. Raes*, [1990] O.J. No. 2298 (C.A.) (action no. 768188):

It is important to remember that the plaintiffs must prove their case and in medical and scientific matters, it is not sufficient to show that a cause and effect sequence is theoretically possible. For the plaintiffs to discharge their onus, they must show, on the balance of probabilities that a cause and effect relationship does exist.

[715] The Court of Appeal agreed with Osler J. that the standard of proof of causation was not met simply by demonstrating a possibility of some causal connection (*Rothwell v. Raes, supra*) as cited in *Pickering v. Liberty Mutual Fire Insurance Co.*, [1996] O.I.C.D. No. 76.

[716] The plaintiff must establish on a balance of probabilities that the injury was as a result of the accident. This requires more than some contribution. The contribution of the accident must be “significant” or “material” to the development or continuation of the disability claimed. *Oleiro v. Commercial Union Assurance Co.*, [1996] O.I.C.D. No. 152.

[717] It is well settled law that the accident related injuries need not be the sole cause of an applicant’s disability they must be the principal or primary cause of her disability (*Sarson v. Pilot Insurance Co.*, [1996] O.I.C.D. No. 202).

[718] In *Aguilar v. Allstate Insurance Co. of Canada*, [1995] O.I.C.D. No. 39, the facts were that the insured had been involved in several motor vehicle accidents, he was driving slowly (just as plaintiff in this action testified she was doing) when someone pulled out in front of him and the left front portion of his vehicle hit the rear quarter panel of a van. He was wearing a seatbelt, but was thrown forward and then back. The insured saw a family doctor a few weeks after the accident. He continued to work for a short period, but his symptoms had continued to worsen over the four weeks after the accident to the point where he was unable to work or walk any distance. He was described in reports as in obvious distress and was unable to walk more than a few steps. His lumbar spine was markedly rigid and he had marked paraspinal lumbar muscle spasms. He gradually appeared to progress and attempted to return to work in the day care his wife ran in their home. Although his condition was expected to resolve soon, his back continued with a slow deterioration, which pain gradually increased over the next year. A CT scan of his back was done and it was noted that there was marked bulging of the disc at L4-5 associated with

hypertrophic changes producing a degenerative segmental spinal stenosis. A posterior decompression at L4-5 was performed and a gradual improvement was noted after that.

[719] One of the experts who testified indicated that the widespread nature of the insured's disc protrusion was more indicative of a disease process rather than trauma induced injury to a specific section of the spine from an accident. The judge placed weight on the opinion that the CT scan showed bulging of the disc and not a frank herniation, which is what is usually expected with sudden trauma or sudden onset of pain. Bulging usually occurs gradually and is a result of slow degeneration of a disc and not a sudden traumatic event. The same doctor testified that it was reasonable to say that the subsequent motor vehicle accident exacerbated a pre-existing condition and that the motor vehicle accident had the effect of causing symptoms earlier than they may have otherwise occurred naturally. The arbitrator found that, once an individual has been injured and painful symptoms are present, it is very difficult to apportion residual pain between the progression of the pre-existing degenerative disc disease alone and any additional damaged caused by a motor vehicle accident. The arbitrator went on to say that, at the time of the accident, she found that the insured probably had a significant weakness of the spine, which would have likely caused him pain and disability irrespective of the motor vehicle accident. It was accepted that the accident affected Mr. Aguilar's condition to some extent, however, on the balance of probabilities, the arbitrator found that the other factors set out in the doctor's evidence, far outweigh the contribution made by the accident. The arbitrator found that the schedule is intended to compensate only those whose injuries arise as a result of a motor vehicle accident and not as a result of a disease and the insured therefore was found not to be entitled to benefits. This decision was upheld on appeal.

[720] Therefore, if the opinions of Doctors Hansebout, Stewart and Besemann are accepted, and a finding is made that she had progressive degenerative cervical spondylosis, which inevitably was going to require surgery, the Court should make a finding that she is not catastrophically impaired due to the 1998 accident.

[721] Further, Dr. Vandewaetere felt she should have proceeded with the surgery, Dr. Hansebout said she should have had the surgery before the 1998 accident, Dr. Stewart testified

that she should have had the surgery before the 1998 accident, and Dr. Besemann concurred with this opinion as well. Dr. Lesiuk was not definitive in his position on this issue, given the difficulty of the case, and the fact that he was awaiting another opinion from a neurosurgeon. Therefore, we submit that there is no evidence to support the position that she did not need the surgery before the 1998 accident and, therefore, applying the *Athey* principal, the claim against ING should be dismissed.

[722] We also submit that the evidence is overwhelming in favour of ING and that, notwithstanding the 1998 accident, Ms. Monks' cervical spondylotic myelopathy had progressed to a point that she would have required surgery in September 1999. Not only does Dr. Hansebout indicate this, but as well Dr. Stewart's, Dr. Besemann's and Dr. Desforbes' (her own treating doctor at the Rehab Centre) diagnoses reflect this as well.

[723] The plaintiff had the onus to show that the injuries sustained in the 1998 accident did cause the injuries, which required her to undergo surgery. It is not sufficient for her to have just established that it was theoretically possible. Although plaintiff counsel repeatedly put to the experts testifying that it was theoretically possible that the accident could have caused her to undergo the surgery, establishing this is not sufficient for him to have established his case. He must establish that on the balance of probabilities that the accident did cause the injuries, which precipitated the requirement for surgery. The evidence from the perspective of the radiological findings, the reported symptoms, the lack of neurological findings and signs, and the continued progression of her spondylosis both before and after the surgery, repeatedly showed that it was not the 1998 accident which caused the suggestion of signal cord change.

[724] If the plaintiff can show that it was not a natural progression of her pre 1990 spandylosis that caused the signal cord change, she then must prove that the injuries sustained in the 1998 accident were the primary or principal cause of her disability (*Sarson, supra*). That just cannot be so in these circumstances. The plaintiff's own evidence is an admission that it was the 1995 accident which was the major cause of all her problems, and even her own doctor says that the 1995 accident was the primary or principal contributor. All parties testified and agreed that the principal or primary cause of her progresing spondylosis was the 1995 accident.

[725] Even if the plaintiff can establish that the injuries sustained in the accident warranted the change in the MRI, she still has to establish that such a change was significant. The only doctor to testify on that issue was Dr. Hansebout who said that the increased pressure, which showed the signal cord change, was not a significant difference from compression that was evidence on the MRIs before the 1998 motor vehicle accident.

[726] Accordingly, it is the position of ING that even if the Court finds that she did not need the surgery prior to the 1998 accident, that she:

- Cannot meet the onus to establish that there is a cause and effect relationship between the injuries sustained in the 1998 accident and the requirement for surgery;
- Cannot show that the injuries sustained in the 1998 accident were the principal or primary cause of her requirement for surgery – we know that if the cause of the suggestion of the signal cord change was the osteophyte, then it was the natural progression which caused the change not the accident – we know that if the cause of the suggestion of the signal cord change was the protrusion, that the protrusion was not evident on the x-ray right after the accident and if the disc had protruded at the time of the accident, Dr. Besemann said she would have been on a stretcher, thereby indicating that the protrusion was as a result of the natural progression.
- Finally, she cannot show that the contribution of the injuries in the 1998 accident significantly affected the degree of compression – this is critical in that Dr. Hansebout testified that the suggestion of signal cord change did not mean that there was a significant increase in the pressure, in fact, he said just the opposite, her situation had deteriorated so badly that the indication of a suggestion of signal cord change was only a very slight difference in pressure.

[727] Accordingly, it is the submission of ING that the evidence overwhelmingly supports a finding that the 1998 accident did not materially or significantly contributed to her catastrophic injury, and an Order dismissing the action should be granted.

My Analysis of the Defendant's Case Law

[728] The Defendant relied on a number of cases to support its theories and arguments on the issue of causation. I will review the defendant's cited cases and analyze them.

[729] *Rothwell et al. v. Raes and Connaught Laboratories Ltd.*, [1990] O.J. No. 2298 (C.A.)

(Tab 1 of ING's Book of Authorities, vol. 1)

- **FACTS / ISSUES:**
 - The infant plaintiff was one of two twins; the other twin was stillborn.
 - When the infant plaintiff he was three, four and five months old, he received immunization doses of a vaccine to protect him against various diseases.
 - Shortly after the third immunization, it was discovered that the infant plaintiff suffered from a developmental abnormality; he was blind, almost deaf and severely retarded.
 - The plaintiff through his litigation guardian commenced an action against the family doctor, the manufacturer of the vaccine, and the Crown as distributor of the vaccine; causation was in issue.
- **OUTCOME:**
 - At trial, the action was dismissed; the trial judge held that the plaintiffs failed to prove on a balance of probabilities that the vaccine could cause the type of damage suffered by the infant plaintiff and that the damage was actually caused by the vaccine.
 - The Court of Appeal dismissed the plaintiffs' appeal.
- **RATIO:**
 - The trial judge did not err in his findings of fact concerning the causation link (or lack thereof) between the vaccine and the developmental abnormality.
 - The plaintiffs needed to discharge two burdens: first, that the vaccine was capable of causing such an injury, and second, that the vaccine did in fact cause such an injury.
 - The question of causation is essentially a question of fact to be determined by the trial judge; in this case, there was ample evidence before the trial judge to sustain his findings of fact on causation.
 - The Plaintiff bears the onus of demonstrating on a balance of probabilities (i.e., whether it is more likely than not) that causation has been established.
 - This onus is not satisfied merely by finding that a "possibility of some causal connection" exists, or that causation is "theoretically possible."
- **ANALYSIS:**
 - The Defendant relies on this case for two points:
 - The burden of proof lies on the Plaintiff to prove a causal connection between an accident and the presence of continuing symptoms.
 - In a medical context, it is not sufficient to show that causation is theoretically possible; the Plaintiff must show that on a balance of probabilities, causation has been established.
 - This case has been cited and followed in numerous subsequent cases.

[730] *Pisani v. Simcoe & Erie General Insurance Co.*, [1994] O.I.C.D. No. 129

(Tab 2 of ING's Book of Authorities, vol. 1)

[upheld on appeal: *Pisani v. Simcoe & Erie General Insurance Co.*, [1995] O.I.C.D. No. 201]

[(Tab 11 of ING's Book of Authorities, vol. 1)]

- **FACTS / ISSUES:**

- The applicant was severely injured in a motor vehicle accident in 1990; she applied for and received accident benefits from the respondent insurer.
- The applicant was injured in a second accident in April 1992; a second insurer was responsible for any benefits arising from this second accident. This second insurer refused to pay any benefits, claiming that the applicant's condition was still attributable to the 1990 accident.
- In October 1992, the first insurer was informed about the April 1992 accident and the fact that the second insurer might be liable for benefits; the first insurer therefore terminated all benefits to the applicant. The applicant sought to have the matter resolved through arbitration.

- **OUTCOME:**

- The arbitrator found that the 1990 accident did not materially contribute to the applicant's inability to work after October 1992; the first insurer was therefore not liable to pay weekly income replacement benefits past the date when these were terminated (i.e., October 1992).
- However, the arbitrator did find that the applicant was entitled to income replacement benefits until October 1992, including the months following the second accident of April 1992; this second accident did not materially contribute to the applicant's disability.
- On appeal, the arbitrator's ruling was upheld.

- **RATIO:**

- A person's pre-existing conditions may play a role in determining causation.
- An insurer's obligation to pay statutory income replacement benefits to a person who sustains an injury following a motor vehicle accident is not reduced or interrupted by an intervening injury.
- An injured person must prove that an intervening or second injury or accident significantly contributed to the severity of duration of the injuries or conditions.

- **ANALYSIS:**

- o The Defendant relies on this case for the principle that the Plaintiff must prove that a second accident significantly contributed to the severity or duration of symptoms. Drawing an analogy to the present case, it is arguable that this case does support ING's position as the "second insurer" following a second accident. The onus is indeed on Ms. Monks to show that the 1998 accident materially contributed to the severity or duration of her symptoms; if she cannot, the first insurer would continue to be liable (however, in this case, the 2002 settlement would prevent that result).
- o The appeal decision in this case has been commented on or referred to in a few subsequent cases:
- o *Thompson v. Peel Mutual Insurance Co.*, 1999 CarswellOnt 4499, [1999] O.F.S.C.I.D. No. 226
 - The decision in *Pisani* was referred to with approval for the principle that an arbitrator is entitled to consider all the evidence and reach a conclusion which is fair and sensible in the particular circumstances, even if this means choosing an apparently arbitrary cut-off date for the payment of statutory benefits.
- o *Elkaim v. State Farm Mutual Automobile Insurance Co.*, 1997 CarswellOnt 5183, [1997] O.I.C.D. No. 113
 - ING relies on this case more for the notion that Dr. Becker may be a biased advocate for injured insured persons such as Ms. Monks – *Becker v. Ontario (Financial Services Commission)*, [2000] O.J. No. 210 (Tab 6 of ING's Book of Authorities, vol.1).
 - The appeal decision in *Pisani* was referred to with approval for the principle that the test for causation is whether an accident "significantly" or "materially" contributed to an applicant's disability.
- o *M. (S.) v. Markel Insurance Co. of Canada*, 1997 CarswellOnt 3991
 - The appeal decision in *Pisani* was referred to with approval for the principle that the test for causation is whether an accident "significantly" or "materially" contributed to an applicant's disability; the test for causation need not require that the accident be the sole cause of a disability.

[731] *Oleiro v. Commercial Union Assurance Co.*, [1996] O.I.C.D. No. 152

(Tab 3 of ING's Book of Authorities, vol. 1)

- FACTS / ISSUES:
 - o The applicant experienced certain painful symptoms and a pre-existing condition in her back following work-related injuries.
 - o The applicant was then severely injured in a motor vehicle accident in 1992; she applied for and received accident benefits from the respondent insurer.
 - o The insurer terminated the applicant's benefits after 1993, arguing that she no longer suffered a substantial inability to perform the essential tasks in which she would normally engage.

- The applicant claimed she was entitled to additional benefits past the date on which they were terminated.
- OUTCOME:
 - The arbitrator ruled in favour of the insurer, finding that the applicant was no longer entitled to statutory income replacement benefits as of late 1992.
- RATIO:
 - For weekly income replacement benefits, the onus is on the applicant to show that as a result of the injuries she sustained in a motor vehicle accident, she continued to suffer a substantial inability to perform the essential tasks in which she would normally engage.
 - The applicant need not show that the disability was caused solely by the accident. However, she must establish on a balance of probabilities that the disability arose as a result of the accident; the accident must have "significantly" or "materially" contributed to the development or continuation of the disability. The applicant did not discharge this burden here.
 - Where there are pre-existing injuries or conditions and then a subsequent accident, the applicant must show that the subsequent accident materially or significantly contributed to the continuation or deterioration of a disability or condition.
- ANALYSIS:
 - The Defendant relies on this case for the principle that the test for causation includes the requirement that the Plaintiff must prove that in cases of pre-existing injuries or conditions, the contribution of a subsequent accident was significant or material to the development or continuation of a disability, in order to establish causation.
 - However, it should be noted that the arbitrator's decision in *Oleiro* was highly influenced by a finding that the applicant was not a credible witness (see paras. 42-54), which is not the same as in the present case.
 - No subsequent cases appear to have discussed this case.

[732] *Sarson v. Pilot Insurance Co.*, [1996] O.I.C.D. No. 202

(Tab 4 of ING's Book of Authorities, vol. 1)

- FACTS / ISSUES:
 - The applicant was injured in a motor vehicle accident in 1993; she applied for statutory accident benefits from the insurer after 1995, when her injuries prevented her from returning to work.
 - The insurer denied benefits on the basis that any disabilities suffered by the applicant arose from a previous work-related injury, for which she was already receiving Workers Compensation Board (WCB) benefits.
 - The applicant argued that although she had pre-existing injuries, it was the injuries resulting from the accident that were the primary cause of her inability to return to work.
- OUTCOME:

- The arbitrator refused to award income replacement benefits to the applicant, finding that her ongoing disability was not caused by or exacerbated by the accident.
- **RATIO:**
 - To establish causation, an accident must be the primary or principal cause of an injury; in this case, the evidence does not support such a causal link, since the applicant's injuries were mostly caused by pre-existing injuries and conditions arising from a prior incident.
 - Although accident-related injuries need not be the sole cause of an applicant's inability to work, they must be the principal or primary cause of her disability.
 - Alternatively, even if the accident in this case was the primary cause of the applicant's disability, the applicant still has not discharged the onus to prove that the disability resulted in her inability to work.
- **ANALYSIS:**
 - The Defendant relies on this case for the principle that accident-related injuries need not be the sole cause of the Plaintiff's disability; however, such accident-related injuries must be the principal or primary cause of the Plaintiff's disability, in order for causation to be established.
 - However, this case may be distinguished from Ms. Monks' claim, since the medical evidence seems much more solid and convincing than in the *Sarson* case.
 - No subsequent cases appear to have discussed this case.

Defendant's Interpretation of

a) Directly related to the Accident (Overview)

[733] ING submits that with respect to the issue of entitlement that the benefits claimed do not arise directly as a result of the 1998 accident and, therefore, the plaintiff is not entitled to payment for same from ING. In other words, the accident did not cause her to have the surgery, but rather she only incurred a cervical strain and, therefore, no benefits are owing.

[734] Alternatively, ING submits that many of the claims being submitted relate to injuries or health problems that are completely unrelated to the problems with her spine and, therefore, she is not entitled to be indemnified for them. In other words, even if this Court finds that the 1998 accident did cause her to have the surgery, she is not entitled to claim for expenses which are not related to the injuries sustained in the accident, for example, expenses arising out of her diagnosis of narcolepsy. It is the position of ING that the following medical problems do not relate to her injury sustained in the motor vehicle accident: narcolepsy, sleep apnea, colitis,

hiatus hernia, removal of her gall bladder, edema, nausea and vomiting, congestive heart failure, pituitary tumour, ulcers on her breasts, septicemia, bilateral knee replacement and, accordingly, all treatments, recommendations and equipment required as a result of these ailments are not compensable under the SABS.

[735] It is the position of ING that the expense must be incurred before an award can be made by the Court. Accordingly, it is the position of ING that the Court cannot make an order into the future and, therefore, the plaintiff's request for same must be denied. ING acknowledges that an order for declaratory relief for entitlement can be granted, however, the order cannot incorporate a quantification of the benefit into the future.

[736] ING submits that all expenses must be reasonable and necessary in order for the plaintiff to be entitled to be indemnified. The most substantial item, which ING takes issue with, is the pool in that it is not reasonable or necessary for the plaintiff, and this is discussed in detail later.

[737] It is the submission of ING that the 1998 motor vehicle accident did not cause the plaintiff to have the surgery, but rather the plaintiff only sustained a cervical strain. The evidence heard at trial shows that the plaintiff required the surgery prior to the 1998 motor vehicle accident. The plaintiff, by her own admission at examinations for discovery in the Zurich action, said that the surgery was required prior to the 1998 accident. The plaintiff had an AP Diameter of the spinal cord that was reduced to approximately 6mm. Medical experts agreed that such a narrowing of the spinal cord was shocking, when the normal individual should have one over 12mm. The plaintiff was suffering from disc degeneration disease and the medical evidence overwhelmingly concludes that the plaintiff was on a quick decline. It cannot be said that the use of her vehicle was a direct cause of her injuries. The plaintiff's impairment did not arise immediately after the accident, but rather following the surgery.

[738] In the alternative, ING submits that many of the plaintiff's claims relate to injuries or health problems that are not directly to the problems with her spine and, therefore, she is not entitled to be indemnified for them. Even if the Court finds that the 1998 motor vehicle accident

did cause the plaintiff to have surgery, we submit that the plaintiff's injuries are not related to injuries sustained in the accident. For example, the plaintiff is claiming that her sleep apnea arose as a result of the accident, but the evidence is clear that it relates to the thickening of the neck in people who are usually overweight. Hence, no causal connection to the accident.

The Law

[739] Entitlement to accident benefits turns on the definition of what is an accident. It is the definition of "accident", as defined by the SABS, which acts as the gatekeeper to all entitlement lying behind the Schedules. Subsection 2(1) of the SABS-1996 defines "accident" as follows:

2(1) In this Regulation,

"accident" means an incident in which the use or operation of an automobile directly causes an impairment or directly causes damage to any prescription eyewear, denture, hearing aid, prosthesis or other medical or dental device.

[740] The predecessor regulation defined "accident" differently:

1. In this Regulation,

"accident" means an incident in which, directly or indirectly, the use of operation of an automobile causes an impairment or causes damage to any prescription eyewear, denture, hearing aid, prosthesis or other medical or dental device.

[741] The definition in s. 2 of the SABS-1990 was substantially similar to the SABS-1994 definition:

"accident" means an incident in which the use or operation of an automobile causes, directly or indirectly, physical, psychological or mental injury or causes damage to any prosthesis, denture, prescription eyewear, hearing aid or other medical or dental device.

[742] Unlike its predecessor statutes, s. 2(1) of the Statutory Accident Benefits Schedule – Accidents on or after November 1, 1996, O.Reg. 403/96 defines the term accident as an incident in which the use or operation of an automobile directly causes an impairment ... Accordingly, the provision itself sets out the applicable test:

1. Did the incident arise out of the use or operation of an automobile?
2. Did such use or operation of an automobile directly cause the impairment?

[743] The first question deals with what is defined as the purpose test, meaning (1) for what purpose was the automobile being used or operated at the relevant time, or (2) did the incident arise out of the ordinary and well-known activities to which automobiles are put. The purpose test is common to the interpretation of all automobile insurance legislation.

The second question deals with the direct causation, rather than indirect, as seen in the previous legislation. It now requires a direct link of causation. Direct causation is factually driven and will depend on the circumstances of each case.

[744] The purpose test was enunciated in *Amos v. Insurance Corporation of British Columbia*, [1995] 3 S.C.R. 405. In this case, the insured Amos was driving his van in California. As he pulled away from an intersection, he was attacked by a gang of people. With his van surrounded by six men, the insured locked the doors of the van and kept it moving slowly ahead. The assailants commenced pounding on the door windows on both sides of the van. The glass on the driver's door shattered, but was held in place by its frame. Another man then walked out in front of the van and pointed a gun at the insured. As he tried to duck from the line of fire, he was shot, the bullet striking his spinal cord. The insured was able to keep the van moving. When he had distanced himself from the assailants by several blocks, he brought the van to a stop. He obtained assistance and was transported to a hospital. He sustained serious, disabling and permanent injuries. The insured submitted a claim for accident benefits and the insurer refused coverage.

[745] Major J., speaking for the Court, at paragraph 17, referred to the two parts, as the "purpose" and "causation" branches of the test and stated that this was the test for determining whether the event fell within the definition:

- (1) Did the accident result from the ordinary and well-known activities to which automobiles are put?
- (2) Is there some nexus or causal relationship (not necessarily a direct or proximate causal relationship) between the appellant's injuries and the ownership,

use or operation of his vehicle, or is the connection between the injuries and the ownership, use or operation of the vehicle merely incidental or fortuitous?

[746] With regard to the purpose test, Major J. held that the insured met this branch. Noting that the insured was driving his van down the street, Major J. held at paragraph 18 that “the accident clearly resulted ‘from the ordinary and well-known activities to which automobiles are put’”. He then went on to review the causation branch of the test and stated at paragraph 21:

The question is whether the requisite nexus or causal link exists between the shooting and the appellant’s ownership, use or operation of the van. With respect to causation, it is clear that a direct or proximate causal connection is not required between the injuries suffered and the ownership, use or operation of a vehicle. The phrase “arising out of” is broader than “caused by”, and must be interpreted in a more liberal manner.

[747] On the meaning of the phrase “arising out of”, Major J. noted at paragraph 21 of the *Amos* decision that:

The words “arising out of” have been viewed as words of much broader significance than “caused by”, and have been said to mean “originating from”, “having its origin in”, “growing out of” or “flowing from”, or, in short, “incident to” or “having connection with” the use of the automobile.

[748] Major J. found that there was the requisite connection and concluded at paragraph 25 that:

The shooting appears to have been the direct result of the assailants’ failed attempt to gain entry to the appellant’s van ... It is important that the shooting was not random, but a shooting that arose out of the appellant’s ownership, use and operation of his vehicle.

[749] The Court therefore concluded that the insured was to be entitled to coverage.

[750] Another case of importance is *Chisholm v. Liberty Mutual Group*, [2002] O.J. No. 3135, which enunciated the “causation test”. This is a case where Mr. Chisholm was an insured person under a car insurance policy issued by Liberty Mutual to his wife. He was driving his wife’s car when an unknown assailant fired gunshots at it. The shots rendered him a paraplegic. Liberty Mutual refused to pay Mr. Chisholm benefits on the ground that, although he suffered an

impairment, he was not in an “accident” within the meaning of the current legislation, because the use or operation of a motor vehicle did not directly cause his impairment.

[751] When Mr. Chisholm submitted that the *Amos* test applied to this case, Laskin J.A. rejected this argument, stating at paragraph 18 that: “In my view, the *Amos* test does not apply, and even if it did, I am dubious whether Chisholm could satisfy it”. Laskin J.A. recognized at paragraph 22 that: “He meets the first part of the test, the purpose test, but not the second part, the causation test”.

[752] Laskin J.A. described the causation test in *Chisholm*, at paragraph 27:

A direct causation requirement conjures up memories of the famous English tort case of *In Re Polemis & Furness, Withy & Co. Ltd.*, [1921] All E.R. Rep. 40, [1921] 3 K.B. 560, where recovery was allowed for damages that were not a foreseeable result of the defendant’s negligence, but were directly caused by it. When one thinks of direct causation one thinks of something knocking over the first in a row of blocks, after which the rest falls down without the assistance of any other act.

Laskin J.A. then went on to hold, at paragraph 29 that:

Even accepting that the use of Chisholm’s car was a cause of his impairment, a later intervening act occurred. He was shot. An intervening act may not absolve an insurer of liability for no-fault benefits if it can fairly be considered a normal incident of the risk created by the use or operation of the car – if it is “part of the ordinary course of things”. ... Gun shots from an unknown assailant can hardly be considered an intervening act in the “ordinary course of things”.

[753] A more recent decision of the Ontario Court of Appeal provides a good review of the state of the law pertaining to the purpose and causation test. In *Greenhalgh v. ING Halifax Insurance Co.*, [2004] O.J. No. 3485, Ms. Greenhalgh got lost while driving in the country with a friend. Her car got stuck on a rock on a private road, and eventually stalled. After failing in her attempts to get the car moving and to call for help (her cell phone battery died), she and her friend left the vehicle to look for help. They got lost, and walked through the night for nine or ten hours. Along the way, they fell into an ice-covered river, and Ms. Greenhalgh lost her boots.

She suffered severe frostbite, requiring the amputation of her fingers and her legs below the knees. Her insurer denied her claim for accident benefits.

[754] Labrosse J.A., for the majority, first analyzed the purpose test of *Amos* and stated, at paragraph 17:

In using this language, Major J. was not speaking of causation. He addressed causation in the second branch of the test. Rather, the phrase intended to indicate whether the accident flowed out of or arose out of the ordinary and well-known activities to which automobiles are put. It cannot be said that the insured's injuries in the *Amos* case "resulted from" the regular activity to which his automobile was being put, in the causal sense of these words. Rather, it meant that the insured's injuries flowed out of the regular activity to which his automobile was being put.

[755] The second branch of the test, namely causation, was then analyzed by Labrosse J.A. He found that what will amount to direct causation will depend much on the circumstances of each case. Labrosse J.A. opined that the following considerations may provide useful in determining whether or not the test has been met: (1) the "but for" test can be a useful screening; (2) intervening acts may actually break the link of causation; and (3) whether the use of the automobile was the dominant feature. If not, the Court may find that the link between the use of operation of the vehicle and the impairment are imply too remote to be called "direct".

[756] Labrosse J.A. concluded that the "but for" test only serves to eliminate from consideration factually irrelevant causes and it does not conclusively establish legal causation. He then, at paragraphs 38-41, continued his review of the second part of the test, namely, the "intervening act":

The next question is whether it can be said that the use or operation of the motor vehicle was "a direct cause" of the injuries ...

In *Chisholm*, it was clear that there was an intervening act that could not fairly be considered a normal incident of the risk created by the use or operation of the car – the gunshots. Therefore, the use or operation of the car could not be said to be a direct cause of the injury.

I find the following cases helpful in applying this consideration.

In *Hanlon v. Guarantee Co. of North America*, [1997] O.I.C.D. No. 43 (Ont. Ins. Comm.), affirming [1995] O.I.C.D. No. 172, Hanlon was injured when, after a car accident with another vehicle, the driver of the other vehicle got out of the vehicle and hit Hanlon in the head with a cellphone. Hanlon was no longer in his vehicle at the time he was hit. The Director Delegate of the Ontario Insurance Commission concluded that this injury could not be said to be a result of an “accident”. Noting that “at some point, the causal relationship, or nexus, between the use or operation of the automobile and the accident is too incidental or remote to be covered by automobile insurance”, the Director’s Delegate held that he was not persuaded that “there [was] a causal relationship, even an indirect one, between the use or operation of an automobile and Mr. Hanlon’s injuries”.

The Director’s Delegate concluded as follows:

“Cause” has an instrumental aspect that is lacking here. The incident involving the use or operation of an automobile ended with no one being injured. No further consequences were inevitable or linked to any ongoing use or operation of an automobile. Sometime later, in a location removed from the “incident,” there was a verbal confrontation between the two men outside their vehicles. At that point, Mr. Daly attacked Mr. Hanlon. No automobile was the target of the attack, was used in the attack, or contributed to Mr. Hanlon’s injuries. I conclude that ... the connection between the use or operation of the automobile and the injury is not sufficient to be covered by automobile insurance.

[757] Labrosse J.A. determined that the critical factors, for the Court, were that use or operation of the car had ended by the time Ms. Greenhalgh suffered her injuries. The insured had physically left her car, no other automobile contributed to the insured’s injuries, and there was a temporal distance between the end of the use of the car and the injuries.

[758] The last part of the test was the “dominant feature” consideration, where Labrosse J.A. stated, at paragraph 47, as follows:

As stated earlier, in some cases it may be useful to ask if the use or operation of the automobile was the dominant feature of the accident; if not, the link between the use and operation and the impairment may be too remote to be called “direct”. In *Heredi*, the Supreme Court was asked to interpret the phrase “damages occasioned by a motor vehicle” in legislation from Saskatchewan. Ms. Heredi had been injured while riding on a paratransit bus designed to accommodate persons with physical disabilities. Mr. Fensom was driving the bus. Mr. Fensom helped Ms. Heredi place her crutches under her right shoulder, and then

proceeded to drive the bus “in such a manner as to cause the plaintiff’s crutches to jar her right shoulder, thereby causing injury”. The Supreme Court held at para. 34, that if “the dominant feature of the damages is their relation to a motor vehicle accident,” then the legislation would apply. The Supreme Court held at para. 41 that “it [was] clear that the direct cause of the injury was the operation of a motor vehicle”.

[759] Labrosse J.A. concluded that in this case, the dominant feature of the insured’s injuries could best be characterized as exposure to the elements, and that the use of the motor vehicle was ancillary to the injury. Therefore, the use was not a direct cause of the impairment.

[760] As stated in the foregoing case law, in order for the Court to find that the 1998 accident caused the plaintiff to have surgery, there must be, it is submitted, a direct causal link between the use of the vehicle and the impairment. ING submits that the evidence does not support that the use of her vehicle caused the plaintiff’s injuries.

Pre-accident condition

[761] The plaintiff sustained serious injuries as a result of the 1995 accident. Immediately following the 1995 accident, the plaintiff experienced immediate numbness in both her arms. Dr. Hansebout explained that the significance of this particular fact is that the numbness, which lasted for about 15 minutes, means that the accident caused some damage to the nervous structures, notably the nerves coming off the spinal cord. He further opined that this would indicate that there is a degenerative change already in the spine and some narrowing.

[762] It is undisputed that the plaintiff had degenerative narrowing of the discs prior to the 1998 accident, which means that the discs were thinner. She was experiencing clumsiness in her right hand, she had difficulty writing and her left hand was being affected. The medical documentation revealed that she had pain and a positive Hoffman sign. But what is more telling is that her spinal cord was being compressed from 11-12 millimeters down to 6 millimeters. Dr. Hansebout testified that, in his opinion, there were clinical and radiologic findings.

[763] Prior to the 1998 accident, the defendant submitted that the evidence disclosed that the plaintiff needed the surgery. Mr. Cronyn spent a considerable amount of time at trial, attempting to illicit from the various experts that the plaintiff did not need the surgery, but rather it was

elective. This was only agreed upon by his own expert. What is most telling is the evidence given by the plaintiff during the examination for discovery in the Zurich action. “He told me to have the surgery. He told me that I need it” were her exact words.

[764] ING submits that, immediately following the 1998 accident, the plaintiff did not, although this is highly disputed by her, experience immediate numbness in her arms.

[765] The plaintiff contends that her injuries arose immediately following the impact with the bridge abutment in 1998. If that were truly the case, one would have expected the plaintiff to be significantly worse than what the evidence revealed. We reviewed the video surveillance taken on May 28, 1999. The images seen were extremely revealing and certainly did not show the plaintiff experiencing any significant impairment, as a result of the 1998 accident.

[766] It is obviously not disputed that the plaintiff had an accident, while driving her vehicle. The facts revealed that she did hit a bridge abutment. Therefore, in applying the “purpose test”, as explained in *Amos and Greenhalgh, supra*, the defendant submits that the plaintiff meets same. However, it is settled law that, in order to be entitled to accident benefits, the plaintiff will also have to satisfy the “causation test”, which had three components, namely, the “but for”, the “intervening act” and the “dominant feature”. In this case, we submit that, as described above, “but for” the accident, the plaintiff would have nonetheless sustained the injuries. They were in existence prior to the 1998 accident. Surgery was inevitable. With regards to the “intervening act”, it is submitted that the most consequential act is the surgery. Most of her injuries occurred following the surgery, and Dr. Lesiuk alluded in his testimony that, although no surgical misadventure transpired, it arose as a result of the surgery. Lastly, regarding the “dominant feature”, it cannot be said that it was the impact of the vehicle. Rather, the plaintiff had serious pre-existing injuries and did not suffer immediate impairment, as a result of the accident.

[767] Therefore, the defendant submits that the plaintiff has failed to meet the “causation test”, as enunciated in the *Chisholm* case and, as such, would not be entitled to the accident benefits, as a result of the 1998 motor vehicle accident.

[768] In the event that the Court concludes that the plaintiff has established the direct causal connection, whereby the 1998 accident did cause her to have the surgery, the defendant submits that the plaintiff's claim for various expenses must also fail, because most of her injuries (symptoms) are not directly related to her cervical spondylosis myelopathy.

Symptoms not directly related

[769] ING submits that once we look carefully at the medical evidence and the symptoms alleged, the only reasonable conclusion is that it cannot be causally related. The following list, in the defendant's submission, has not been established to be causally related. It was not challenged during trial.

Sleep Apnea

[770] It is undisputed that sleep apnea is not related to the spinal cord. Both Dr. Lesiuk (page 110 of his transcripts) and Dr. Hansebout testified that is not a spinal cord dysfunction. Dr. Hansebout further explained that it is related to the thickening of the neck and is usually found in overweight people (page 44 of his transcript).

[771] Dr. Snooks' notes reveal that the plaintiff, who measures in height 5 feet 8 inches, has always been extremely overweight. In April 1989, she weighed 195 pounds (Exhibit 2, Tab 1, page 4), in April 1990, 197 pounds (Exhibit 2, Tab 1, page 5), in September 1993, 204 pounds (Exhibit 2, Tab 1, page 10). Dr. Hansebout further opined that, in his view, she was on the verge of morbid obesity (page 47 of his transcript) and that the C-Pap, which is a respirator used by the plaintiff, is not associated, in this case, with spinal cord injury (page 45 of his transcript).

Narcolepsy

[772] Dr. Hansebout testified that narcolepsy is not related to the spine whatsoever, nor would any of the treatment requirements be related to the spine (page 45 of his transcript).

Hiatus Hernia

[773] Dr. Hansebout testified that it is not related to the plaintiff's spinal cord problems (see page 45 of his transcripts).

Hip Bursitis

[774] Dr. Hansebout commented that it is not related to the spine (page 45 of his transcript).

Septicemia

[775] Dr. Hansebout opined that it is not related to the spine (page 45 of his transcript).

Nausea and vomiting

[776] Dr. Hansebout stated that it is not related to the spine (page 45 of his transcript).

Edema

[777] Dr. Hansebout has never seen edema related to the spinal problem. His testimony, which can be found at page 45 of his transcript was as follows:

Q. She's also had edema?

A. There was one cardiologist, Dr. Chandler [sic], who said the edema might be due to a spinal problem, but I have never seen such a thing. It's usually edema. In this case, a large weight gain usually is related to a heart problem. Now he didn't find any evidence of a heart problem, Dr. Chandler [sic] in his report, as I recall.

I mean, if your spinal cord is really damaged and you can't move your legs, they can swell, okay, they can swell then. But in this case, I think she was still moving her legs enough, that I think it's unlikely that that was related.

Colitis

[778] Dr. Hansebout stated that it is not related to the spine (page 46 of his transcript).

Heart Failure

[779] Dr. Hansebout indicated that this is not related to the spine, but it is interesting because it can cause leg swelling (page 46 of his transcript).

Neurogenic Bowel and Bladder

[780] Although Dr. Hansebout testified that this could be related to the spine, he did state that, in his opinion, although the plaintiff does have a neurogenic bladder, this was evident prior to the 1998 accident (as a result of the naturally progressing spondylosis) and, therefore, it not causally related to the 1998 accident. The following excerpt of his testimony can be found at page 47 of his transcript:

She had problems with her bladder, you know, for a long time before the 1998 accident, and headaches. But she does have a neurogenic bladder, which means that the control coming from her spinal cord to her bladder has been compromised by dysfunction of the spinal cord.

Q. Did the 1998 accident contribute in any which to that in your opinion?

A. To the bladder problem, I don't think so; she had it before.

Pituitary Tumour

[781] Dr. Hansebout confirmed that it was not related to the spine (page 46 of his transcript).

Bilateral Knee Replacement

[782] The plaintiff testified that she had torn ligaments at different times in both knees, sometime in the 1980s. Dr. Hansebout testified that it is usually a function of the joints, and certainly not the spine (page 46 of his transcript). The plaintiff's knee problems continued, as seen below (Exhibit 2, Tab 1, pages 6, 7 and 9):

- Dr. Snook noted on January 12, 1993 that her right knee was sore. The plaintiff admitted during cross-examination that it might be residual from when she tore the ligaments playing slow pitch;

- In the February 1993 accident, the plaintiff hit her right knee on the console under the steering wheel;
- Dr. Snook noted on February 16, 1993 that Dr. Laflèche diagnosed the plaintiff with a right knee collateral strain;
- In August 1993, the plaintiff broke her left fibula, tibia and ankle. She was required to use crutches, which was hard on her knees, given her weight at the time;
- On July 17, 2001, Dr. Vandewaetere reports a knee injury while in physiotherapy (Exhibit 3, Tab 1, page 62). Dr. Johnson has not indicated causation.

Blood Pressure

[783] Although the plaintiff alleges that her blood pressure was as a result of the 1998 accident, it is clearly evident that, as early as 1990, she was having problems with her blood pressure requiring her to be medicated with Renedil (Exhibit 2, Tab 1, page 7).

Hemorrhoids

[784] The plaintiff was diagnosed with bleeding hemorrhoids in April 1994.

Psoriasis

[785] Psoriasis is an immune-mediated chronic skin disease. In 1988, the plaintiff was diagnosed with this condition and was prescribed Dermovate (Exhibit 2, Tab 1, page 2).

[786] In conclusion, ING submits that the foregoing review of the evidence clearly demonstrates to the Court that the plaintiff's numerous injuries (symptoms) pre-existed or were not directly related to her spinal cord injury. There is absolutely no causal connection established between these injuries and the spinal cord. The plaintiff has not advanced any evidence to the contrary.

b) Incurred

[787] Turning now to the issue of whether the Court can make an award into the future, it is our submission that the provisions of the SABS are clear and unambiguous. The Court does not have the jurisdiction to make an award into the future. The Court grants a declaration to the plaintiff that she is entitled to benefits under the SABS, then, the defendant submits that the declaration should be limited to simply making a declaration. The plaintiff's request that an amount be attributed is contrary to the legislation and must be denied.

Awarding an amount into the future

[788] Sections 14(1)(2), 16(1)(2) and 22(1)(2) of the SABS provides the mechanism as to which and when the expense is to be paid under the headings of Medical Benefit, Attendant Care or Benefit and Housekeeping and Home Maintenance, respectively:

Medical Benefit

14.(1) The insurer shall pay an insured person who sustains an impairment as a result of an accident a medical benefit.

(2) The medical benefit shall pay for all reasonable and necessary expenses incurred by or on behalf of the insured person as a result of the accident for,

- (a) medical, surgical, dental, optometric, hospital, nursing, ambulance, audiometric and speech-language pathology services;
- (b) chiropractic, psychological, occupational therapy and physiotherapy services;
- (c) medication;
- (d) prescription eyewear;
- (e) dentures and other dental devices;
- (f) hearing aids, wheelchairs or other mobility devices, prostheses, orthotics and other assistive devices;
- (g) transportation for the insured person to and from treatment sessions, including transportation for an aide or attendant;

(h) other goods and services of a medical nature that the insured person requires.

Attendant Care of Benefit

16.(1) The insurer shall pay an insured person who sustains an impairment as a result of an accident an attendant care benefit.

(1.1) Despite subsection (1), no attendant care benefit is payable to an insured person whose impairment comes within the Grade I Whiplash Guideline or the Grade II Whiplash Guideline if the accident occurred after April 14, 2004.

(2) The attendant care benefit shall pay for all reasonable and necessary expenses incurred by or on behalf of the insured person as a result of the accident for,

(a) services provided by an aide or attendant; or

(b) services provided by a long-term care facility, including a nursing home, home for the aged or chronic care hospital.

Housekeeping and Home Maintenance

22.(1) The insurer shall pay for reasonable and necessary additional expenses incurred by or on behalf of an insured person as a result of an accident for housekeeping and home maintenance services if, as a result of the accident, the insured person sustains an impairment that results in a substantial inability to perform the housekeeping and home maintenance services that he or she normally performed before the accident.

[789] The case of *Monachino v. Liberty Mutual Fire Insurance Co.*, [1997] O.J. No. 2571, was an action where the plaintiff who was involved in a serious motor vehicle accident sustained brain damage. The Court was asked to determine whether care benefits were reasonable expenses under section 7 of the Schedule. The Court reviewed, at page 10, paragraph 76, the significant differences between tort actions and the no-fault regime:

The No Fault regime is quite different than the traditional law governing motor vehicle actions in tort. In tort actions, the injured Plaintiff must establish liability, causation and damages. When the Plaintiff has done so, the tortfeasor will be ordered to pay as damages an amount equal to the present value of the losses and costs suffered and to be suffered by the Plaintiff. Determination of that amount

involves a process of calculations, projections, probabilities, reasonable possibilities, gross-ups, discounts and contingencies. That amount is calculated and awarded today and is to be paid today regardless of whether those losses turn out to be greater or less in the future.

[790] The no-fault scheme provides for treatment and rehabilitation of persons injured in motor vehicle accidents, regardless of fault. The losses and costs are paid when they are actually incurred. The Court commented as follows, at page 20, paragraph 77:

That whole area of actuarial calculations and projections was removed with the expectation that the huge awards of lump sums based on present value of future losses would be removed or reduced thereby stabilizing the automobile insurance industry and the escalating rates for automobile insurance paid by policy owners.

[791] This case was appealed (*Monachino v. Liberty Mutual Fire Insurance Company et al*, [2000] O.J. No. 865 (Ont. Ct. Appeal)) and at page 4, paragraph 10, in dealing with caregiver benefits, the Court concluded that the language of the no-fault schedule is clear and unambiguous.

There are three pre-conditions to entitlement: a cost must be incurred, it must be reasonable and it must be for a professional caregiver. To accept the appellant's interpretation would be to re-write the statute as opposed to applying its clear language. The trial judge correctly interpreted this provision and his conclusion was sound.

[792] It is submitted that, in the event the Court finds that the 1998 accident materially contributed to the plaintiff's current condition and make a declaration regarding future benefits, ING must then have the opportunity to consider and approve any specific proposals made by the plaintiff for intended treatment and services, in accordance with SABS. For example, with respect to Medical and Rehabilitation Expenses, should the Court made a declaration that she is entitled to this benefit, the plaintiff will need to follow the normal process, as outlined in the SABS. The plaintiff would submit an expense that she has incurred and, in accordance with section 14(2) of the SABS, and then the insurer would determine whether it is reasonable and necessary. The SABS provides the opportunity for the insurer to respond. The legislation, as drafted, does not provide for the ability to award a payment into the future. It would contradict

its intent and remove the insurer's right to properly investigate a claim and respond to same. We submit that any order providing for payment of an amount of benefit would be contrary to legislation and the current state of law.

ING's Entitlement to a Deduction

[793] As stated above in paragraphs 1 and 2, it is ING's position that it is entitled to deduct any amounts that are available to the plaintiff under any benefit plan. Section 60(1) and (2) of the SABS provides the following:

60.(1) The insurer may deduct the following amounts from the amount payable to an insured person as an income replacement or non-earner benefit:

1. Any temporary disability benefits being received by the insured person in respect of a period following the accident and in respect of an impairment that occurred before the accident.

2. Any other periodic benefit being received by the insured person in respect of a period following the accident and in respect of an impairment that occurred before the accident, if the insured person was receiving the other periodic benefit at the time he or she first qualified for the income replacement or non-earner benefit, and, at that time, the other periodic benefit was a temporary disability benefit. O. Reg. 403/96, s. 60(1).

(2) Payment of a medical, rehabilitation or attendant care benefit or a benefit under Part VI is not required for that portion of an expense for which payment is reasonably available to the insured person under any insurance plan or law or under any other plan or law. O. Reg. 403/96, s. 60(2).

[794] Section 60(2) does not specifically say that the amounts must be paid, but rather that payment of this expense is "reasonably available" to the plaintiff under another insurance plan. It is submitted that the Zurich plan falls squarely upon this requirement.

[795] In the decision of *Orchover et al v. Wright et al* (1996), 28 O.R. (3d) 263, the Court was faced with the issue of deciding whether the defendant was entitled to have any damages recovered by the plaintiff against him, reduced by the value of the accident benefits she would have been entitled to, rather than the amount of the accident benefits she received. The facts of

this case were that the plaintiff was involved in a motor vehicle accident with the defendant. The plaintiff negotiated a settlement or cash-out with the accident benefit insurance company, whereby she received a lump sum on account of future care costs and for future loss of income. In dealing with section 267 of the *Insurance Act*, Justice Forget concluded, at page 7, that the full present value of the benefits is deductible.

To my mind, the case reflects the intent of the legislature in passing s. 267, namely, that the tortfeasor, rather than the plaintiff, is to benefit from the availability of other insurance. If the plaintiff had made an improvident settlement with its own no-fault insurer, that fact should not be visited on the tortfeasor, thereby curtailing his rights pursuant to s. 267 of the *Insurance Act*. The defendant, Grogan, is entitled to the full present value of those benefits irrespective of what might have been in the mind of Zurich and the plaintiff when they negotiated the cash-out.

[796] Although this case dealt with a tort matter, it stands for the proposition that a defendant is entitled to a deduction of the full present value of the benefits to which a person is entitled, rather than just a deduction for the amount of the benefits received. The same principle applies to the wording in the SABS. If the drafters of the statute wished to limit the amount of the deduction to the actual amount received, it would have said so, rather, the drafters chose to make the wording broader, so as to protect against this very type of situation where a person does not follow-up on receiving their full entitlement. This shortfall should not be visited upon the other insurer. At the time of the settlement with Zurich, for example, had the plaintiff believed that she would require attendant care benefits for her lifetime, it begs the question as to why she would have settled for only \$300,000. The plaintiff had unlimited exhaustive attendant care resources available to her and significantly higher amounts for Medical and Rehabilitation benefits. ING submitted that, in accordance with section 60(2) of the SABS, and in applying the principles enunciated in the *Orchover* case, it is entitled to deduct the full present value of the benefits that she was entitled to from Zurich, not just the amount that was received, thereby resulting in a nil amount duly owing to the plaintiff.

[797] ING submits that the benefits claimed by Ms. Monks do not arise “directly” as a result of the 1998 accident (as required by the SABS provisions) and, therefore, ING is not liable to

pay the plaintiff any statutory accident benefits. Simply put, the 1998 accident either played no role or only indirectly resulted in the need for surgery, which, ultimately, led to the plaintiff's condition.

[798] The defendant focuses on the use of the words "directly causes" when considering the role of the use or operation of a motor vehicle in causing an impairment under the SABS. ING submits that the case law clearly supports its argument that this focus on "direct" causation clearly precludes recovery in a case such as the present one, where the surgery, which resulted in the impairment, would have occurred, even in the absence of the accident; the accident, therefore, cannot be said to have "directly" caused the impairment.

[799] ING relies on a number of cases to support this conclusion, including many of the cases cited by the plaintiff to support the opposite conclusion (I.e., that direct causation need not exclude the possibility that the accident played some causal role even though there were other causes).

My Analysis of the Law submitted by the Defendant

[800] *Amos v. Insurance Corp. of British Columbia*, [1995] 3 S.C.R. 405

(Tab 17 of the Defendant's Book of Authorities, vol. 1)

- FACTS / ISSUES:
 - The appellant was seriously injured when he was shot by a gang of assailants while driving his van; the assailants demanded that he leave the van, but he locked the doors and continued driving; he was then shot and suffered severe injuries.
 - The appellant applied for statutory accident benefits; the insurer refused to pay benefits, arguing that the impairments were not "directly caused" by the use or operation of a vehicle.
- OUTCOME:
 - The insurer was held to be liable to pay the statutory accident benefits owing to the appellant.
- RATIO:
 - The test for causation and liability involves two parts:
 - Did the accident result from the ordinary and well-known activities to which automobiles are put?

- Is there some nexus or causal relationship between the injuries and the use or operation of a vehicle, or is the connection merely incidental or fortuitous?

[801] *Chisholm v. Liberty Mutual Group* (2002), 60 O.R. (3d) 776 (C.A.)

(Tab 18 of the Defendant's Book of Authorities, vol. 1)

- I have discussed this case at page 201, *supra*.
- Justice Laskin ruled that the *Amos* test did not apply in this case, and even if it did, the applicant had failed to satisfy the second branch of the test (i.e., causation).
- This case appears to support ING's position in that it confirms that the legislation does not permit recovery where injuries are only "indirectly" caused by the use or operation of a motor vehicle. However, as noted in my analysis of the Plaintiff's submissions, the Court of Appeal merely affirmed that the "use or operation" of a motor vehicle must "directly cause" an impairment; the Court did not conclude that the accident itself must directly cause the impairment to the exclusion of all other material contributing causes.

[802] *Greenhalgh v. ING Halifax Insurance Co.*, [2003] O.J. No. 2740 (Sup. Ct. Just.)

(Tab 19 of the Defendant's Book of Authorities, vol. 1)

[reversed on appeal: *Greenhalgh v. ING Halifax Insurance Co.*, [2004] O.J. No. 3485 (C.A.)]
[(Tab 20 of the Defendant's Book of Authorities, vol. 1)]

- I analysed this case at page 202 of this decision.
- This case does seem to support ING's position, since the Court of Appeal concluded that an impairment cannot be said to have been "directly caused" by the use or operation of a motor vehicle if there are too many intervening events, actions or causes. ING argues that, in the present case, there were too many intervening and outside causes and forces acting on the Plaintiff's health condition to conclude that her catastrophic impairment was "directly caused" by the accident. While the Defendant's interpretation of the case law may be correct, I do not agree that there were too many "intervening causes" to establish causation. As I stated earlier, the Plaintiff convincingly argues that there is a sufficient causal link between the 1998 accident and Ms. Monks' current condition to satisfy the requirements of the legislation, despite the presence of additional outside forces and causes.

[803] *Athey v. Leonati*, [1996] 3 S.C.R. 458

(Tab 8 of ING's Book of Authorities, vol. 1)

- I analysed this case at page 197 of this decision.
- It is unclear to me why ING would be relying on this case, since it seems to fully support the Plaintiff's position on causation, as noted EARLIER; the Supreme Court in *Athey* establishes a fairly broad threshold test for causation.

- However, *Athey* does contain some discussion of the “thin skull” and “crumbling skull” rules; the Supreme Court noted that a defendant need not put a plaintiff in a position better than his or her original position where the plaintiff’s original position included a pre-existing medical condition. The defendant is not liable to compensate the plaintiff for any effects of the pre-existing condition which the plaintiff would have experienced anyway, even if the defendant’s tortious act had never occurred: “The defendant is liable for the additional damage, but not the pre-existing damage.” ING submits that the present case is a “classic, unequivocal case of a crumbling skull Plaintiff”. Therefore, ING argues that it is only obligated to pay for expenses for injuries caused by the 1998 accident; ING argues that these expenses have already been paid for.
- ING also submits that the principles in *Athey* are equally applicable in the context of an accident benefit claim.

[804] *Z. v. Dominion Insurance*, [2003] O.F.S.C.I.D. No. 16

(Tab 9 of ING’s Book of Authorities, vol. 1)

- I analysed this case at page 198 of this decision.
 - The Arbitrator had ruled that the accident materially contributed to the plaintiff’s condition, but refused to award additional weekly income replacement benefits, finding that the accident did not materially contribute to the plaintiff’s inability to continue her employment.
- The Office of the Director of Arbitrations of the OFSC dismissed the plaintiff’s appeal.
- It is unclear to me why ING would be relying on this case; although the plaintiff lost her appeal, the Director’s Delegate upheld the original Arbitrator’s findings of fact on the issue of causation, including a ruling that the accident significantly contributed to the plaintiff’s condition. The plaintiff only lost on the issue of income replacement benefits, but causation was not at issue.
- ING seems to rely on this case for the notion that the arbitrator’s comment that accident benefit recovery is an “all or nothing” scheme means that insured persons should not be awarded double recovery. This is a misinterpretation of the arbitrator’s comment; the fact that accident benefits are governed by an “all or nothing” approach simply means that if an insurer is liable to pay accident benefits, these benefits may not be reduced because there are other contributing causes. If causation exists, the insurer is liable to pay all benefits to which the insured is entitled; if no causation exists, the insurer is not liable for any benefits.

[805] *M.D. v. Halifax Insurance Company*, [2003] O.F.S.C.I.D. No. 71 [QL]

(Tab 7 of ING’s Book of Authorities, vol. 1)

- FACTS / ISSUES:
 - The applicant was injured in a motor vehicle accident; her insurer initially accepted the claim for income replacement benefits.

- However, approximately three months after the accident, the insurer determined that the applicant was no longer disabled and announced that it would terminate the benefits.
 - The applicant then asked for a DAC assessment; the insurer continued paying the benefits pending the results of the assessment.
 - The DAC assessment concluded that the applicant was not suffering a substantial inability to perform the essential tasks of her pre-accident employment; as a result, the insurer notified the applicant it would be terminating her benefits.
 - The DAC also referred the applicant to a “multi-disciplinary” assessment; in contrast with the DAC assessment, this second assessment concluded that the applicant’s injuries resulting from the accident prevented her from returning to work. The insurer did not reinstate benefits, and the applicant did not pursue the matter.
 - The only issue at arbitration was the insurer’s obligation to pay for the cost of the two assessments.
- OUTCOME:
 - The arbitrator refused to hold the insurer liable for the cost of the second assessment.
- RATIO:
 - Insured persons do not have an unfettered right to seek a second assessment after an unfavourable first assessment, and expect that it be funded by the insurer.
 - A DAC report is not just another opinion; although either party can challenge it through the dispute resolution system, the DAC’s conclusion governs the payment of benefits in the meantime.
 - The protection provided in the legislation is for an independent assessment at a DAC, not a DAC assessment and then a second opinion by someone of the insured person’s own choosing if the DAC’s opinion is not helpful; a second assessment arranged for the purpose of challenging the DAC through the dispute resolution process is better viewed as a litigation expense, recoverable through negotiation or as arbitration or court costs.
- ANALYSIS:
 - This case is not useful to ING in the present dispute, since the cost of the DAC assessment and other assessments is not the major issue in dispute between ING and Ms. Monks.

[806] *Pickering v. Liberty Mutual Fire Insurance Company Co.*, [1996] O.I.C.D. No. 76

(Tab 11 of ING’s Book of Authorities, vol. 1)

- FACTS / ISSUES:
 - The applicant was injured in a motor vehicle accident, and applied for and received statutory accident benefits from her insurer; weekly income benefits were then terminated by the insurer on the basis that the applicant’s continuing kidney problems did not result from the accident but were the result of pre-existing conditions.
- OUTCOME:

- The arbitrator found that the applicant was not entitled to receive weekly income replacement benefits following the termination of these benefits, as her ongoing inability to work was more properly caused by pre-existing conditions and not by her injuries resulting from the accident.
- **RATIO:**
 - The applicant bears the burden of proving the causal connection between her current condition and the injuries she sustained in the accident; here, the burden was not discharged. Demonstrating that causation was “theoretically possible” does not meet the standard of proof.
 - Although the applicant need not show that the accident was the sole cause, the accident must have materially or significantly contributed to the applicant’s condition.
- **ANALYSIS:**
 - This case is really just another application of the principles developed in *Rothwell, Pisani and Oleiro*.

[807] *Aguilar v. Allstate Insurance Company Co.*, [1995] O.I.C.D. No. 39

(Tab 12 of ING’s Book of Authorities, vol. 1)

[upheld on appeal: *Aguilar v. Allstate Insurance Company Co.*, [1996] O.I.C.D. No. 64]

[(Tab 13 of ING’s Book of Authorities, vol. 1)]

- **FACTS / ISSUES:**
 - The applicant was injured in a motor vehicle accident and received statutory benefits from the insurer; the insurer then terminated the income replacement benefits, on the basis that he was no longer substantially unable to perform the essential tasks of his pre-accident occupation.
 - Following an unsuccessful mediation, the applicant had back surgery, and then applied for arbitration, claiming additional income replacement benefits and rehabilitation benefits.
- **OUTCOME:**
 - The arbitrator concluded that by the time the weekly income benefits were cancelled, the applicant was no longer eligible because he was substantially able to perform the essential tasks of his pre-accident occupation; the arbitrator also found that although the accident played some role in the applicant’s ongoing back problems, other factors were of greater importance – the back problems were therefore not caused by the accident.
 - On appeal, the arbitrator’s ruling was upheld; no additional benefits were awarded to the applicant.
- **RATIO:**
 - Although the applicant need not show that the accident was the sole cause, the accident must have materially or significantly contributed to the applicant’s condition.

- In this case, there was conflicting medical evidence concerning the role of the accident; in cases where the accident appears to have played a “minimal” role in an applicant’s condition, especially where there are prior accidents and pre-existing conditions, this will be insufficient to establish a causal connection between the accident in question and the applicant’s current condition.
- Where there are pre-existing medical conditions that likely would have resulted in an applicant’s current injuries and conditions even without the accident, it will be difficult to find that the accident made a significant or material contribution to the applicant’s condition; in many cases, these pre-existing conditions and other factors may far outweigh the contribution made by the accident.
- ANALYSIS:
 - This case is also just another application of the principles developed in *Pisani* and *Oleiro*.
 - ING appears to rely on this case to show that the natural progression of a pre-existing medical condition makes it unlikely to find that an accident materially contributed to a current condition or disability; in most cases, the current condition would still have occurred even without the accident.

DECISION

My Decision on Causation and the Interpretation of “directly related to the accident”

[808] I agree with counsel for the plaintiff that there are many instances where counsel for the defendant refers to the evidence and then goes on to make representations that amount to a supposition unsupported by the facts and also by expert opinion. I will set out some examples of this and will cite other examples of facts, taken out of context, and of arguments made on evidence I disallowed at trial. As outlined in my analysis of the case law, I also find that counsel for the defendant has taken quotes out of context, in the case law, and cited cases for a proposition she did not establish.

[809] I will deal with the emphasis on the type of law applicable, the evidence that I did accept, and the case law that I find applicable in arriving at the above result.

Type of Law Applicable

[810] Eventhough defence counsel presented arguments about what I could, and could not do, in an accident benefit case at the opening of the trial, she confused the distinction between being a defendant in a tort case, and being a defendant in an accident benefits case, throughout the trial.

I issued a written ruling, containing several pages, at the beginning of this trial, outlining what I could do and could not do in a declaratory judgment. I also addressed the question of what was meant by benefits accruing for injuries directly caused by the accident, and the test used to determine whether such expenses were reasonable and necessary. The defendant chose to ignore my ruling and instead to argue these points all over again, at the end of the trial. My ruling is attached as appendix “A”.

[811] In a tort case, the court’s attention is taken up in deciding what duties were owed by the parties to one another and if those duties were breached, how the damages should be apportioned between the parties bearing the loss. In an accident benefit case, the court is dealing with a contract within the context of how an insurance company has to fulfill its obligations. The jurisprudence quoted earlier in the plaintiff’s submissions, underlined that such obligations must be fulfilled with the utmost good faith. Evidence was also given at trial that Ms. Monks’ contract was a contract for “peace of mind”. I agree with counsel for the plaintiff that certain circumstances and principles enunciated in the context of tort law are imported into the context of accident benefit cases. However, it must be done with discipline and the ING argument has ignored that discipline.

[812] ING’s argument, that it is unfair that it should have to pay Ms. Monks, completely ignores the accident benefit scheme and the fact that ING has a contract with Ms. Monks and ING is the insurer. The ING attitude reminds me of a Family Court case where the father and mother of children are so caught up in their vendetta towards one another, on separation, that they forget their children’s best interests. ING is a contracting party in this situation and has obligations to fulfill. In the analysis that follows, I find that ING was so intent in denying liability to Ms. Monks, that it breached its “peace of mind contract” by ignoring the needs of its insured.

The Evidence on Causation

[813] I do not agree that there was a progression in Ms. Monks’ symptoms, for the six months prior to the 1998 accident. I accept that Ms. Monks had consulted Dr. Vanderwaetere on many

occasions, in 1997, and again in 1998, and that, had she suffered aggravated symptoms, between April 1998 and January 1999, she would have consulted her family doctor. Dr. Lesiuk, in his report of January 18, 1999, makes exactly that point, namely, that nothing noteworthy had occurred during those six months prior to the December 1998 motor vehicle accident: “she remained relatively clinically stable until she was involved in the December 1998 accident”. During cross-examination, Dr. Lesiuk told the Court he would not have written this down, had he not received that information from Ms. Monks.

[814] The defendant anchors its case on the claim that it was the pre-existing conditions, that caused Ms. Monks’ injuries, as well as the need for the 1999 and 2001 surgeries, which in turn rendered her an incomplete paraplegic. Counsel for the defendant argued, based on the expert opinions of Dr. Hansebout and Dr. Stewart, that the natural progression of Ms. Monks’ degenerative disc disease had progressed to such a point that Mr. Monks required surgery, prior to her 1998 accident.

[815] Both Dr. Hansebout and Dr. Stewart agreed that, before the 1998 accident, the question of whether surgery should have been performed, or not, was a matter of opinion, and it was simply their opinion that surgery was needed. Dr. Lesiuk, in his evidence, stated that, it was hard for him to accept the opinion that surgery was needed, from two doctors who gave their opinions after the fact, without having treated the patient, and without having viewed the films of the MR Investigations. I accept his opinion, and the opinion of Dr. Bourque, over the opinion of Dr. Hansebout and Dr. Stewart. I will outline the credibility issues later, as I want to deal here with the argument that surgery was needed, before the 1998 accident.

[816] When testifying, experts have to stick to the opinions given in their reports. Presumably, such opinions are given when they have reviewed the plaintiff’s complete file, and have had time to think, in the quiet surrounding of their studies. The defendant will rely on the report of its experts and counsel for the plaintiff will consult his own experts, and prepare for trial, on the basis of the conclusion reached by each expert called by the defendant.

[817] In the case of both, Dr. Hansebout and Dr. Stewart, there was simply no reference in their reports that surgery was needed, before the 1998 accident. Counsel for the defendant had specifically asked Dr. Stewart whether Ms. Monks would have required surgery, regardless of the 1998 accident, and if so, when. A reading of Dr. Stewart's report at page 11, Exhibit 45, makes it clear that his opinion, at the time of writing his report, was that, once the abnormalities were seen in the MRI, taken after the 1998 accident and identified, then "a reasonable opinion would be to recommend surgery". This opinion, from the defendant's own expert, does not advance the defendant's theory that surgery was required, prior to the 1998 accident. At trial, Dr. Stewart argued over and over again that this was no longer his opinion. I restrict his evidence to the opinion given in his report.

[818] The same change of opinion emerged in Dr. Hansebout's evidence. He, too, did not wish to be restricted to his very long, and elaborate, report that did not say when surgery, in Ms. Monks' case, would have been required. He does say Ms. Monks would have required surgery without having been involved in the 1998 accident, but he does not say when. He certainly did not say in his report that Ms. Monks should have had surgery before the 1998 accident. At trial, he expressed shock at reading that Ms. Monks' spinal canal space was reduced to 6mm. and said it would have been imperative for a neurosurgeon to operate.

[819] I accept, without hesitation, the evidence of Dr. Lesiuk, and Dr. Bourque, over the evidence of Dr. Hansebout and Dr. Stewart. While true that Ms. Monks had the compelling feature of a 6mm. spinal canal, the analysis should not stop there. Dr. Lesiuk stated that she was not a classic candidate for surgery, and Dr. Bourque called for a conservative approach, knowing that:

- (a) spinal cord surgery had its own very severe risks;
- (b) Ms. Monks belonged to a group that stood a 60% chance of remaining stable, if she was careful.

[820] Ms. Monks explained at trial that she stopped all activities that could have caused problems for her, immediately after Dr. Lesiuk made that recommendation to her. I accept Dr.

Lesiuk's views that prophylactic surgery was not appropriate, as it might have brought trauma or injury that could have rendered Ms. Monks quadriplegic. Dr. Hansebout and Dr. Stewart both agreed that, if Ms. Monks did not undergo surgery, after the 1998 motor vehicle accident, she would become a quadriplegic. This was not the case prior to the accident, as no one argued that, if Ms. Monks did not have an operation, before the 1998 motor vehicle accident, she would become a quadriplegic. All doctors agreed that the evidence, shown on Ms. Monks' 1999 MRI, indicated that, if she did not have surgery, she would become quadriplegic. Thus, the surgery did not cause her condition; the surgery was undertaken to prevent the condition from deteriorating.

[821] Counsel for the defendant reviewed Ms. Monks' past medical history, dating back to the 1980s, with all of the medical expert witnesses. I find that none of the medical doctors stated that such medical problems had any influence on Ms. Monks' problems. Dr. Vanderwaetere and Dr. Lesiuk both stated that, if there were *sequelae*, following the occasion where she had put her car in a ditch, or when she had suffered rheumatoid arthritis, that turned out to be wrongly diagnosed, or had twisted her neck in a slip and fall at work, missed a year of school because of illness, none endured long enough to require long-term treatment. None of the past injuries would have necessitated Ms. Monks' 1999 surgery. I find that counsel for the defendant was wrong in her submission that the evidence supported that conclusion.

[822] Counsel for the defendant erred also when she submitted that there was no evidence supporting the claim that Ms. Monks did not need surgery, before the 1998 accident. Ms. Monks had been placed in the 60% category of patients that could remain stable, if nothing untoward happened. Doctors Lesiuk and Bourque both noted that she was not a classic candidate for surgery. It is also erroneous that Dr. Lesiuk agreed with everything Dr. Hansebout said at trial, as intimated by counsel for the defendant. Dr. Hansebout had placed Ms. Monks in the 40% category of patients who would not remain stable and required surgery, whereas Dr. Lesiuk maintained that she was in the 60% category of persons who remained stable.

[823] I disagree with the representations made by counsel for the defendant that Dr. Lesiuk would only operate if there were a signal change, rather than gradual determination in the future. I am aware that Dr. Hansebout formed this opinion, but it does not conform to the evidence in

this trial. It is somewhat disrespectful to an experienced surgeon, such as Dr. Lesiuk, to think he would let matters ride and leave Ms. Monks to have surgery only after permanent damage had occurred. In his report of April 1998, Dr. Lesiuk had asked Ms. Monks to monitor symptoms of intractable pain, or hard radiculopathic, or myelopathic symptoms (the three signs) indicating spinal involvement. All doctors testified that they treat symptoms and not MRIs. If intractable pain or radiculopathic or myelopathic symptoms appear, Dr. Lesiuk indicated that it does not mean permanent damage has set in, but that there was potential spinal cord involvement. Because Ms. Monks is a nurse, both Dr. Lesiuk and Dr. Bourque were confident she could monitor any spinal cord change.

[824] In the meantime, Ms. Monks was told to avoid activities that could bring about trauma to her spinal cord condition, as she was placed in the 60% category of persons who had a chance of remaining stable. It was reasonable for Dr. Lesiuk to think that further trauma to Ms. Monks' spinal cord would not occur. Chance did not favour Ms. Monks however and, if the 1998 accident had not occurred, it is difficult to say whether the three signs, referred to by Dr. Lesiuk, would ever have manifested themselves or not.

[825] The defendant argued also that the 1998 accident did not cause Ms. Monks' present condition, because no neurological signs appeared while she was in the emergency department, in Erie, U.S.A., following that accident. The defendant stated that Ms. Monks was asked specifically if she had weakness or numbness in the upper extremities. I find that there is no evidence as to what questions were asked of Ms. Monks, when she was at St-Vincent's Hospital. Furthermore, Ms. Monks testified that her neurological exam had been very perfunctory, at the hospital.

[826] Several points militate against placing too much emphasis, as the defendant did, on Ms. Monks' visit to St-Vincent's Hospital. Ms. Monks was very concerned about her mother who was not feeling well after the accident. Next, the examination of Ms. Monks took place several hours after the accident. Only an x-ray was taken and Dr. Lesiuk testified that not much concerning discs is seen with an x-ray, as opposed to an MRI. I agree with counsel for the plaintiff that there is no evidence that Ms. Monks' disc protrusion could have been or would

have been disclosed on the x-ray. It is misleading for counsel for the defendant to have submitted that, since the disc protrusion did not show up on the x-ray, the 1998 accident could not have caused the damage.

[827] The defendant ING attempts to further confuse the issues in this case, by arguing that Ms. Monks did not report tingling in her hands and arms, following the accident. The doctors testifying for the defence did not raise this matter as an issue in their reports. I find as a fact that, in the persistent increase in symptomology that followed the 1998 accident, continuing through 1999, the tingling problem is only a very small part in the vast array of other evidence on increased symptomology, following this accident.

[828] Contrary to what is alleged by the defendant, there is evidence that Ms. Monks suffered neurogenic pain, and it is found in Dr. Chow's report of April 1999, and in the evidence of John Perrozzino, who said that his wife referred to her pain "like fire ants biting her". There is more evidence on this subject, but I find the two references given will suffice.

[829] I viewed the surveillance video during trial and I cannot ascribe to it the conclusions reached by counsel for the defendant. There is no evidence that Ms. Monks was working 18-hour days at the time the video was taken (May 1999, five months after the accident), and I recall the evidence of Lynn Parker and Laurie Warren that Ms. Monks often slept at work, because she was in a bad physical condition. These two persons are well qualified to make their observations and they testified as to what they saw with their own eyes and they saw an immediate change in Ms. Monks. I recall that the video depicted a Ms. Monks leaning on a car when talking to her client and that she walked slowly. I find the video did not advance the theory of the defendant that she was in relatively good physical condition following the 1998 accident, and thus, whatever deterioration took place, between the December 1998 accident and the September 1999 surgery, was due to the material progression of her cervical spondylotic condition (degenerative condition of the spine).

[830] The defendant incorrectly advanced in its submissions that there was no signal change in the December 1999 MRI. There was no evidence given by the experts to that effect. Counsel

for the defendant attempted to have Dr. Stewart testify about Ms. Monks' edema condition in the June 1999 MRI. I disallowed that question, as Dr. Stewart had nothing in his report on this matter. Nevertheless, counsel for the defendant improperly made submissions to counter my disallowance of her question at trial. She pointed out, in her submissions, a broad sentence in Dr. Stewart's report, to the effect that the progression of signs seen in the MRI after the accident was not, in any significant way, caused by that accident. Dr. Stewart had seen Dr. Lesiuk's report and, if he held the opinion that the signal change was a transitory change, he should have said so specifically in his report. Dr. Lesiuk, once again, had an advantage over Dr. Stewart, as he had seen the MRI and it was his evidence from the MRI film that the signal change was evidence of either edema or cord damage (demyelination).

[831] I fail to see how counsel for the defendant can submit, as she did, that Dr. Lesiuk tailored his reports, first for the Zurich action and then for the ING action (Exhibit 1, Tabs A-10 and a-12). She never cross-examined Dr. Lesiuk at trial on that point. There is no evidence that Dr. Lesiuk knew of the Zurich settlement or of the nature of Ms. Monks' claim against ING, so that he could tailor his reports. Counsel for the defendant errs in stating that Dr. Lesiuk's second report, at Tab a-12, was written for no apparent reason, when the report makes it clear Dr. Lesiuk made the report to respond to the report of Dr. Stewart.

[832] The submission that Dr. Hansebout had more documentation and more experience than Dr. Lesiuk is unhelpful. Despite Dr. Hansebout's impressive credentials, he made crucial errors in his analysis of Ms. Monks' situation. I am mindful of the quote: "to err is human" and Dr. Hansebout showed he is human. He is now retired from an active practice in surgery and he does consultant's work for plaintiffs and defendants. Because he gives opinions on patients he has not seen, nor followed through the years, mistakes are bound to happen. Indeed, counsel for the defendant cited the case of *Becker v. Ontario (Financial Services Commission)*, [2000] O.J. No. 210 (Tab 6, Book 1 of ING's Book of Authorities), for the notion that Dr. Becker, (who gave medical opinion in this case), may be a biased advocate for injured, insured persons, such as Ms. Monks. Dr. Hansebout's cross-examination elicited several *lacunae* that place the usefulness of his report in doubt. As mentioned previously, he relied upon facts that were not the same as

those heard at trial. Emphasis was incorrectly placed on pushing the small trailer at the scene of the accident, on Ms. Monks walking down an embankment to the motel following the accident, or on Ms. Monks using a wheelchair on her shopping trips, a few examples that come to mind. It also seems clear that he selected, from the whole body of facts, only those supporting his conclusion, ignoring the weight of the evidence, which stood against it. He questioned Ms. Monks' credibility, based on a reported income, which he failed to understand, included work in progress from another year. He ignored the fact that other doctors, in their reports, had reported Ms. Monks was a credible patient. (See list at page 110.)

[833] Dr. Hansebout showed he was an advocate for the defendant when he claimed that, although Ms. Monks had severe bruising on her breasts, that lasted for months following the accident, Ms. Monks' forward motion could not have been that great, because she was a big person. This was speculation on the part of a doctor, one who has an "international reputation", as I was reminded by counsel for the defendant. The rather offensive advocacy on Dr. Hansebout's part, on behalf of the defendant, was clearly demonstrated when counsel for the plaintiff was able to show him that he had altered Dr. Lesiuk's report by putting in his report the following bold and underlined citation of Dr. Lesiuk's May 2001 report: "the character and distribution of the symptoms were unchanged after the December 23, 1998 accident ...", leaving out of the sentence, following the 1998 accident, Ms. Monks' symptoms had undergone "exacerbation magnitude change".

[834] I was impressed by the demeanour of Dr. Lesiuk, compared to the demeanour of Dr. Hansebout and Dr. Stewart. Dr. Lesiuk had no trouble conceding that, in April 1998, there was room for a different opinion, as to whether Ms. Monks needed surgery or not. Dr. Lesiuk was fair and he pointed out that, while he considered Dr. Hansebout to have a good reputation, he would not be inclined to accept his opinion on whether surgery was required in the absence of seeing the patient and reviewing the MRI films, neither of which Dr. Hansebout did. Dr. Lesiuk was quick to admit that Ms. Monks' case was a difficult one and he was looking to Dr. Fehlings for another opinion. Counsel for the defendant is wrong in arguing that Dr. Hansebout had stated

that surgery was required prior to the 1998 accident. Dr. Hansebout said that at trial, but when challenged to find it in his report during cross-examination, he was unable to do so.

[835] Dr. Stewart advanced the idea that treating physicians were not as objective in giving their opinions as consulting doctors were, but he also proved to be quite an advocate for the defendant. For instance, he accepted, from Ms. Monks, a statement she had made to her family physician, Dr. Snook, one month after the 1995 motor vehicle accident, but he would not accept a statement made by Ms. Monks to Dr. Vanderwaetere, one month after the 1998 accident. Dr. Stewart admitted he left out of Dr. Vanderwaetere's notes, the reference for numbness Ms. Monks felt after the 1998 accident. Dr. Stewart, who was paid close to \$10,000 to prepare his report, prepare for trial, and give evidence at trial, was not in a position to give an opinion that treating physicians lack objectivity, when he has never met Dr. Lesiuk or Dr. Vanderwaetere, nor has he seen them testify. His attempt to tarnish their reputation discloses, I feel, a lack of impartiality on his part. I have reviewed, earlier, Dr. Stewart's examination and cross-examination, and I am not impressed by Dr. Stewart's one on one, half hour, examination of Ms. Monks. It is not surprising that he could not remember that he had examined Ms. Monks without removing her collar or that he could not remember that Ms. Monks was not in a wheelchair, but had used a wheeled walker on the visit to his office in Toronto.

[836] I am not able to accept the evidence of Dr. Markus Besemann, the designated centre assessor for catastrophic conditions, as neutral evidence. The breach of section 4 of the Guidelines is not a red herring. The defendant's submissions on this matter are unacceptable and illustrate the point that the defendant chooses to see only what it wants to see ... in an attempt to escape liability. Firstly, Dr. Besemann testified on matters not contained in his report and it did not matter how often I attempted to stop him from doing so, he persisted in giving evidence the way he wanted, whether it was in his report or not. His opinions came from reports fed to Dr. Besemann by counsel for the defendant, and influenced by these medical reports, Dr. Besemann wanted to change his opinion at trial. In his report, Dr. Besemann had stated that he could "not reliably conclude that this patient is catastrophically impaired on the basis of the December 23, 1998 motor vehicle accident". Counsel for the defendant submitted that the last quoted words

were used by Dr. Besemann to stay neutral. But, I agree with counsel for the plaintiff that, at trial, Dr. Besemann made no attempt to stay neutral.

[837] The commission guidelines number 4 was filed, at trial, as Exhibit 42. The guidelines are made by the Ministry of Financial Institution's committee, pursuant to s. 7 of the *Insurance Act* and empowered by section 52 of *SABS-96*. The test of Guideline 4 reads:

The assessor must remain and appear to remain neutral. This objective cannot be met if the assessor has provided/performed post-DAC treatment, a medical legal assessment or an insurer examination at the request of one of the parties.

[838] Further, the same text continues:

C. Communications with Parties

Verbal

Except during the DAC assessment itself (when the claimant is present for and involved in an examination), there should be no one-sided verbal communication between the DAC and either of the parties *unless* the DAC confirms the details of the discussion in writing and copied to the other party. This includes all telephone communications before the assessment to set up the appointment, verbal reminders to the parties to provide necessary documentation, etc. All other questions or concerns that are posed to a DAC, by either party, should be in writing.

Written

When a DAC responds to written correspondence, the response must be written and copied to both parties. If a party corresponds with the DAC, and the other side has not received a copy of the correspondence, the DAC must copy both the original letter, and their response, to the other party. This includes the covering letters from either the insurer or the claimant and their representatives. The DAC should not provide any type of report, verbal or written, to one side or the other, in advance of the final written report. The completed and signed OCF-11 (Referral and Summary Report) may be sent to both parties in advance of the final report, if necessary.

[839] Dr. Besemann agreed that he became involved with the defendant's counsel in the fall of 2004. One month prior to trial, counsel for the defendant provided Dr. Besemann with all of

the defence experts' written opinion reports. Dr. Besemann did not have his original file when he testified, to tell the Court what exactly his file contained. Dr. Besemann stated that he had met privately with counsel for the defendant and then had a brief 15-minute conversation with her. Counsel for the plaintiff had not participated in the meeting or the telephone conversation.

[840] Counsel for the plaintiff referred to the decision of *Lee v. State Farm Mutual Automobile Insurance Co.*, [2003] O.F.S.C.D. No. 165, in which arbitrator S. Sapin states:

Should Dr. Hirshberg testify in Ms. Lee's arbitration, it would be perfectly appropriate for him to be asked about and provide his opinion from Dr. Vanreikam's [ph] report. When an expert is asked to testify about specific matters, the opposing party is entitled to notice of the substance of the expert's testimony in order to have the opportunity to prepare an effective cross-examination. This fundamental principle informs all of the early disclosure rule that underlie the entire arbitration process at the Financial Services Commission. For this reason, it is appropriate for Dr. Hirshberg to provide a summary of his opinion to the parties in the form of a supplementary report.

[841] In this case, there was no supplementary report filed by Dr. Besemann.

[842] When Dr. Besemann attempted to testify at trial that there was no spinal cord injury, caused by the 1998 accident, I rejected that evidence, because it was not contained in his report. Prior to trial, all Dr. Besemann expressed in his report was uncertainty. At trial, he testified he was now certain. I have to conclude that the change in opinion came about due to the materials provided and the communications made by counsel for the defendant. Counsel for the defendant urges me to accept that it was Dr. Besemann's role to get to the truth and that I should accept that. This submission would mean that there would be no necessity to bring this matter before a Court for a decision.

[843] I agree with counsel for the plaintiff that the leading of many new points at trial, by counsel for the defendant, points to pre-trial discussions between Dr. Besemann and counsel for the defendant. The guidelines clearly state that, not only must there be neutrality in the DAC assessor's report, but there has to be a perception of neutrality.

[844] It is not “only a guideline” as pleaded by counsel for the defendant. There was no way counsel for the plaintiff could anticipate that counsel for the defendant would breach the Guideline. All counsel for the defendant had to do, following the Guideline, was to deliver a subpoena to Dr. Besemann to appear and testify at trial, and call him, to advise him on what date she would require him to testify. That was all that needed to be done. All counsel had to do with Dr. Besemann was to follow the steps outlined in *Lee v. State Farm, supra*. The factual releases would have to be approved by counsel for the plaintiff, but not the reports of doctors who, although independent, are actually hired by the defendant. Had counsel for the defendant wished to have Dr. Besemann give additional opinions, she had to request a further report. The plaintiff would then have an opportunity to respond and also to prepare for cross-examination. How counsel for the defendant can argue that no damage was done by not following the Guideline is beyond my comprehension.

[845] I also disagree with counsel for the defendant, who claimed that Dr. Besemann had applied the correct test to determine causation. While Dr. Besemann was of the view that the issue was, which of the two accidents caused Ms. Monks condition, the issue should be whether the 1998 accident materially contributed to the condition. I agree with the submissions of counsel for the plaintiff that, if Dr. Besemann relied on Ms. Monks’ evidence that the 1998 accident was 25% responsible for her injuries, then the accident would be material, as that was the percentage ascribed in *Athey v. Leonati, supra*, which led to a finding that it was material in that case.

Further Analysis of the Case Law

[846] In *Athey v. Leonati, supra* (page 168), the Supreme Court of Canada’s statements of legal principles concerning causation are broad enough to be applicable in any context, including the present case. The Court found that the cause of an injury need not be the sole or primary cause of the injury and the Court established a broad threshold test for causation. So long as an event was part of the cause of an injury, that event “caused” the injury, even though it may not have been sufficient by itself to cause the injury. In the present case, the 1998 accident caused

the plaintiff's injuries since this accident materially contributed to Ms. Monks' condition, despite the role of prior accidents and alleged pre-existing conditions.

[847] I dismiss the effect of the 1993 accident, as there were no sequelae to the injuries received. There was the beginning of degenerative disc disease and Dr. Lesiuk stated it was normal for a person of Ms. Monks' age in 1993. I accept his evidence over Dr. Hansebout's evidence because Dr. Lesiuk treated Ms. Monks and he saw the films, x-rays, or MRI. Lynn Parker continued to refer difficult motor vehicle accident files to Ms. Monks. Ms. Monks continued to do her housework and her sporting activities. Her friend, Maxine Morrison, testified that she had a hard time keeping up to Ms. Monks, because Ms. Monks had boundless energy.

[848] The 1995 accident had caused Ms. Monks trouble in her right hand and she could not write for long periods. This brought about a slowdown at work and John Perrozzino took over some of Ms. Monks' heavier household chores. Ms. Monks carried out a full work week with reduced hours, still managed 50% of her household chores, looked after her aging mother and completed short continuing education courses in the rehabilitation therapy field. She could get in and out of her car and made several trips to the United States without mobility aids. I rely on the evidence of Maxine Morrison, Lynn Parker and Laurie Warren to the effect that Ms. Monks did not need mobility assistive devices prior to her 1998 motor vehicle accident. The three, last named witnesses, were impressive in their evidence that they gave separately, and convincingly. I believe Ms. Monks made a mistake when she recalled the use of a wheelchair on a trip, not long prior to her last accident. I accept the evidence of her friends that it occurred on the eve of Ms. Monks' September 1999 surgery. As that surgery had to be an invasive type of surgery with potentially disastrous consequences, Ms. Monks' friends decided to take her to Las Vegas. During that trip, on the eve of her surgery, Ms. Monks had used an assistive device. This is the evidence concerning assistive walking devices that I accept. Following her two surgeries, I accept that Ms. Monks' memory is not what it used to be. As Ms. Monks testified, she can no longer read, as when she arrives at the bottom of a page she has just read, she loses her

concentration to the point of not remembering what she has read. I am not surprised she did not remember her August 1999 trip to Las Vegas, accurately.

[849] The plaintiff wrote to Zurich Insurance in an attempt to have Zurich commit to pay her surgery expenses, in the event she became unable to work. Ms. Monks had experience in the field of disabled insurers in the insurance industry. She also received thoughtful advice from her friend, Laurie Warren, another experienced adjuster who knew that a claim has to be made prior to the expiry of the two-year limitation period, otherwise all her rights to sue would be lost. As Ms. Monks had been the principal wage earner in her household, the loss of her income was worrisome for her. What she did to protect her financial interests did not foreclose a claim against ING in the future.

[850] I conclude that, as I am faced with conflicting medical opinions, I attribute greater weight to the opinions of treating physicians, based on their clinical findings and the evolution of Ms. Monks' symptoms after each accident.

Analysis of the Case Law

[851] Both parties agree that the principles outlined in *Athey v. Leonati*, *supra*, apply in the context of an accident benefit claim. That case contains discussions on thick skull and crumbling skull rules. The defendant claims that Ms. Monks should not be placed in a position better than her original position, since she had a pre-existing medical condition. ING argues that this case is a typical crumbling skull plaintiff, and ING argued that it need only pay for injuries caused by the 1998 accident and such expenses have already been paid for.

[852] I agree with the plaintiff that there is no room for the crumbling skull theory in accident benefit cases. There is no scheme to reduce the amount of benefits to be awarded – it is a black and white determination of full liability.

[853] In arguing *Z v. Dominion of Canada*, *supra*, the defendant used the case out of context. The case does not stand for the proposition that when confronted by with two accidents, an arbitrator must decide which insurer is responsible to pay benefits. In the *Z* case, there weren't

two accidents and two insurers fighting over which one had to pay. The arbitrator was pondering whether pre-existing medical conditions, unrelated to any car accident, were to be taken into account in the context of a crumbling skull theory. This case, I reiterate, confirms the plaintiff's contention that, in an insurance benefit case, it is no defence to argue that benefits should be reduced in order to reflect an apportionment of causation.

[854] I agree with the plaintiff that the most compelling case is that of *Saliba v. Allstate Insurance Co. of Canada*, [1999] O.F.S.C.I.D. No. 170, because the facts are strikingly similar to this case (see pages 197-198). That case also stands for the proposition that the crumbling skull theory has no application to accident benefit cases. It also stands for the proposition that, in circumstances where there is more than one accident, more than one insurer can be liable to pay benefits. In any event, both Dr. Lesiuk and Dr. Bourque had found that Ms. Monks was in the 60% category of patients who remained stable, barring any unfortunate accidents.

[855] The defendant is very concerned that Ms. Monks is seeking double recovery. Ms. Monks' needs have not been adequately covered by the settlement received for the 1995 accident. Ms. Monks has accounted for all monies received from all sources, including Zurich Insurance. The analysis in the second issue in this judgment will show that monies received from all sources are being accounted for and ING Insurance will meet the needs not covered by Zurich.

[856] The *Pisani v. Simcoe and Erie General Insurance Co.*, *supra*, does not stand for a novel proposition. It is cited in subsequent cases, as outlined earlier (page 252), for the principle that the test for causation is whether an accident "significantly" or "materially" contributed to an applicant's disability; the test for causation need not require that the accident be the sole cause of disability. It is also noted that the *Pisani* case was decided before the *Athey* decision in the Supreme Court of Canada.

[857] In the same vein, the defendant cites *Sarson v. Pilot Insurance Co.*, *supra*, as containing the correct test. The *Sarson* case was decided the same year as the *Athey* case, but there is no reference to the *Athey* case in the *Sarson* decision. It is clear that I follow the Supreme Court of

Canada's enunciation of the test. Also, the facts in the *Sarson* decision have no similarity to the Monks case. Ms. Monks' medical evidence is more solid and convincing than what is found in the *Sarson* case.

[858] Finally, in *Aquilan v. Allstate Insurance Company, supra*, the decision was based on its particular facts dissimilar to those of Ms. Monks. In Ms. Monks' case, I find that, following the evidence of Dr. Lesiuk and Dr. Vandewaetere, her pre-existing conditions was not a factor and, as Dr. Lesiuk and Dr. Bourque testified, Ms. Monks was in the 60% category of persons who remained stable.

[859] I find that so long as the accident is found to have made a significant and material contribution to the plaintiff's injuries, then it would seem that there is a sufficient causal link to satisfy the requirements of the *SABS*. While the defendant's case law submitted on this point is correct and relevant, the cases where causation was denied can easily be distinguished on their facts from the present case, since it cannot be said that there were too many intervening causes here to deny a causal link between the accident and the injuries.

[860] With respect to the issue of the phrase "directly causes" in *SABS*, it should be noted that there might be an inconsistency in the plaintiff's submissions. Counsel for Ms. Monks indicated that "the word 'directly' is intended to be a descriptor of whether the *impairment* arises directly out of the 'use or operation' of an automobile, not whether the impairment flows directly from the accident" (emphasis added). However, later, counsel for Ms. Monks appears to add to his submissions a variation to this interpretation:

... the issue of "directly causing" is not as Ms. Orth would suggest that the impairment must be directly caused by the accident, rather it is *the accident giving rise to the impairment* [that] must be caused by the use or operation of an automobile.
(emphasis added)

[861] In my opinion these are two very different propositions. On the one hand, there is the proposition that the phrase "directly causes" in s. 2(1) of the *SABS* means that the use or operation of an automobile must directly cause the *impairment*; on the other hand, there is the proposition that the phrase means that the use or operation of an automobile must directly cause

the *accident giving rise to the impairment*. A careful reading of s. 2(1) and the case law on this issue cannot support the plaintiff's second interpretation – i.e., that it is the “accident giving rise to the impairment” which must be directly caused by the use or operation of an automobile. First, s. 2(1) clearly states that an “accident” is an incident in which the use or operation of an automobile directly causes an impairment; the plain wording of the section does not seem capable of supporting an interpretation such that an “accident” is defined as an incident in which the use or operation of an automobile directly causes an accident giving rise to an impairment. While this situation may result in many cases, this would nonetheless be a circular definition. Second, the case law submitted by the plaintiff also clearly establishes that it is the impairment, not the accident, which must be directly caused by the use or operation of an automobile (see *Greenhalgh, supra*).

[862] Nevertheless, in my opinion this distinction is not fatal to the plaintiff's arguments that s. 2(1) should not be read so as to exclude liability in situations such as the present case. Clearly the debate in the jurisprudence on the meaning of the phrase “directly causes” is directed towards situations where the role played by an automobile in the impairment or injury is so ancillary or banal that a finding of liability under SABS would defeat the purpose of the legislative scheme – i.e., where the claimant's injuries are caused by an event unrelated to the use or operation of a vehicle, or an event involving a vehicle only as the site of that event or as the method of transportation which brought the claimant into proximity with the event, then it cannot be said that the use or operation of an automobile directly caused the injuries. Such an interpretation of the phrase “directly causes” does not close the door on the possibility that there may be other contributing causes of the injury, as contemplated by the “material contribution” test for causation. Therefore, the defendant insurer's interpretation of the phrase “directly causes” does not prevent a finding of causation and liability in the present case, notwithstanding that the plaintiff's interpretation of the cases on this issue may be slightly inconsistent or over-reaching.

[863] Therefore, ING's arguments notwithstanding, I conclude that an impairment was “directly caused” by the use or operation of a motor vehicle, and that the accident made a significant or material contribution to the plaintiff's current condition and to the deterioration or

duration of her symptoms, in that it precipitated or reinforced the need for surgery. This conclusion does not negate or ignore that there were additional contributory causes to the plaintiff's condition, including pre-existing injuries and prior accidents.

ISSUE NO. 2 – PREVIOUS SETTLEMENT MONIES RECEIVED

Plaintiff's submissions

[864] Counsel for Ms. Monks responded to the defendant's argument that if Ms. Monks is entitled to accident benefits, then the defendant should not be ordered to pay any such benefits until the funds previously paid by Zurich Insurance to Ms. Monks in 2002 are fully exhausted.

[865] The plaintiff objected to this argument and submitted that there is no existing statutory provision or case law, which could support the defendant's position. The plaintiff cited and quoted the *Correia* case (page 188 of this decision) and indicated that the SABS is to be given a broad purposive interpretation; if the interpretation favoured by the defendant were intended by Parliament, then there would be express language in the SABS to support this interpretation. The plaintiff also cited the *Saliba* case (at para. 499, page 176 of this decision) where the arbitrator stated that the possibility existed for "fresh entitlement to statutory accident benefits" in relation to subsequent accidents, which occurred after the accidents for which benefits were sought. In relation to the two accidents, which formed the basis of the claim in *Saliba*, the arbitrator ruled that "each accident gives rise to entitlement to a fresh set of statutory accident benefits" (para. 71). Therefore, there is scope for separate awards arising from distinct accidents, even if the injuries are related, exacerbated or ongoing.

[866] The plaintiff argued that the settlement funds received in 2002 from Zurich Insurance were clearly and explicitly offered and accepted in relation to the 1995 accident only, and that no portion of the funds was to be paid or used in connection with the 1998 accident. Therefore, the plaintiff submitted that the present claim for benefits arising from the 1998 accident are separate

from the funds already received in connection with the 1995 accident, and should not be affected by the receipt of those funds.

[867] Furthermore, in response to the suggestion that Ms. Monks should use the amounts received through the Zurich settlement towards her increased medical care costs and rehabilitative needs, counsel for the plaintiff noted that a large portion of those amounts received (\$600,000) were intended to replace Ms. Monks' lost of past income as a result of the 1995 accident. Ms. Monks is therefore entitled to use these funds for her own needs and purposes, as she would have with any income, and she should not be forced to apply them towards medical and rehabilitative costs incurred as a result of a separate accident for which she is also entitled to separate accident benefits.

[868] Counsel for Ms. Monks further submitted that this does not mean that Ms. Monks is entitled to double recovery or a windfall; in fact Ms. Monks has made every attempt to account for all amounts receive to ensure there was no windfall or double recovery. It is simply a matter of receiving the appropriate funds relating to each accident for which Ms. Monks had been paying insurance premiums to ensure coverage in case of injury or loss.

[869] If the present claim for benefits arising from the 1998 accident are to be affected by the Zurich settlement funds, then the appropriate solution is not to prevent and prohibit the receipt of any benefits for the 1998 accident until the full amount of the 2002 settlement funds is exhausted, but rather to take into account the actual amounts Ms. Monks receives from Zurich on a monthly basis, given that Ms. Monks only receives the settlement funds through a structured monthly payment plan.

[870] Ultimately, the plaintiff submitted that the Zurich settlement funds should be accounted for according to a scheme she developed to reassure the Court that there has not been and would not be any windfall or double recovery accruing to Ms. Monks. The crux of the plan using the figures found in Exhibit 14 is that the monthly amounts currently paid to Ms. Monks from the 2002 settlement would be deducted from amount of benefits to be paid out to Ms. Monks from ING in connection with the 1998 accident, within each category of benefits. For example, ING

would be required to pay on-going medical and rehabilitation benefits minus the monthly amounts already received by Ms. Monks from the 2002 settlement relating to medical and rehabilitation needs. Likewise, ING would be required to pay the balance of Ms. Monks' home renovation and/or new home purchasing expenses after deducting the \$75,000 already received from the 2002 settlement funds for that purpose.

[871] Counsel for Ms. Monks explained the rationale for the plan and the plan in this manner.

[872] It is the contention of the plaintiff that if she meets the test of entitlement, which she does, then unless it is clearly stated to the contrary in the legislation or the regulations, she is entitled immediately to the full ambit of the coverage provided under the regime for all of her benefits. Obviously, she is not entitled to double recovery or a windfall and she does not seek one. But on the other hand, she paid her premiums for insurance coverage and she should receive the applicable benefits immediately and not when the Zurich funds are exhausted.

[873] Furthermore, it is to be remembered that Ms. Monks quite wisely placed a significant portion of the Zurich settlement into a structured settlement, which will pay her until she dies. If the argument put forth by Ms. Orth and ING were to be accepted that ING does not have to pay until the Zurich funds are exhausted, then ING would never have to pay at all because the Zurich benefits will not cease until Ms. Monks dies. Again, this result could clearly not have been the intention of the legislators. Otherwise, it would be imprudent to place settlement funds in a structured settlement and once again, if that were the intention of the legislators, one would have expected it to be clearly stated.

[874] It is the submission of the plaintiff that if the Zurich action had not been settled then it would have been incumbent on the two insurers (although they ironically would have ended up as being one and the same) to properly divide the various expenses but the plaintiff would have been entitled to the maximum under both policies. And why should she not? She paid premiums to both insurers. She had two separate accidents, which materially contributed to her condition. She has every right to receive the resulting benefits.

[875] The two insurers would have had to contribute equally to the monthly medical and rehabilitation expenses or home modifications. It is to be remembered that the 1995 accident occurred during the Bill 164 regime and so the accident benefit scheme was slightly different. But the logical result would have been that both insurers would have been liable to contribute to the monthly needs equally up to any monthly maximums and ultimately up to any total maximums under each regime.

[876] It might be tempting to find in these circumstances that ING's exposure should be determined as if the full Zurich benefits were available each month; i.e., ING would only be responsible for 50% of the monthly medical, rehabilitation and attendant care needs.

[877] However, this would still leave Ms. Monks with a shortfall each month. And for any significant expense, e.g. home modifications, vehicle modifications, assistive devices, etc. she would simply be unable to purchase them because she would not have the Zurich component of the cost. Effectively, ING would be able to avoid its obligation to its insured.

[878] Unless it can be demonstrated that the Zurich settlement was improvident, then the proper method to account for that settlement is to take into account the actual amounts Ms. Monks receives on a monthly basis.

[879] On the issue of improvident settlement, it is important to note that ING did not plead that the settlement was in fact improvident. And in order to establish that it was improvident, evidence would have had to be led by ING to demonstrate what a provident settlement was and that simply was not done. In the absence of that evidence, the Court is entitled to assume that it was provident and the only question then is the mechanism of how to take it into account. Ms. Monks is not seeking a windfall or double recovery and she has made every effort throughout the trial to account for what she has received and to ensure that there is no windfall or double recovery. But on the other side of that argument, neither is ING entitled to any windfall simply because there was a previous accident out of which there was an entitlement to accident benefits, which claim was settled.

[880] Furthermore, it can hardly lie in ING's mouth that the Zurich settlement was improvident for two reasons. The first is that ever since the Zurich settlement was reached, it has been the argument of ING starting with its pleadings in this case, that Ms. Monks has already received plenty of money. Secondly, it can also hardly argue the settlement was improvident given the relationship between ING and Zurich at the time of the settlement and the fact that Ed Nolan of ING had encouraged settlement of the Zurich claim because it seemed "indefensible" (Ex. 40, Tab 36).

[881] Ms. Monks is entitled to the full benefit of the coverage she purchased from ING. ING is in fact fortunate that it has the ability to reduce its exposure by virtue of the Zurich settlement. And it certainly, cannot now seek further benefit from that settlement given its role in bringing it about. ING sought a settlement of the Zurich action to keep its exposure down in that case. And it must be remembered that that settlement was reached under stressful conditions for Ms. Monks – Zurich had not paid a penny to Ms. Monks prior to the settlement and had continuously taken the position that there was no liability. There was an obvious power imbalance at the time of the settlement and her claim was compromised by virtue of the vagaries of litigation. But for ING to then turn around in this action and suggest that the settlement, which was to its advantage, was improvident would have been outrageous and clearly is outside the ambit of the nature of relationship intended in an insurance contract.

[882] It appeared to be suggested by Ms. Orth in the trial that Ms. Monks had plenty of money from the Zurich settlement and she should have been and should be able to provide for her needs better as a result of these dollars. It is to be remembered that \$600,000 of that settlement was intended to replace lost income. She has a right to earmark and utilize those funds in the manner any other person would and she should not be forced to use it to cover her medical, rehabilitative or attendant care needs unless absolutely necessary.

[883] Accordingly, it is the submission of the plaintiff that the Zurich settlement funds should be taken into account as follows.

[884] Ms. Monks received \$40,000.00 for past supplementary medical and rehabilitation benefits. Exhibit 52 was produced in response to questions from counsel for ING as to what the \$40,000 in the Zurich settlement, for past med/rehab expenses, was attributed. It is the position of the plaintiff that Ms. Monks actually did not need to account for this because much of it related to expenses incurred prior to the 1998 accident. It was not raised in the pleadings and the issue was raised for the first time at trial. But to ensure that the court was satisfied that there has been no “windfall” Ex. 52 was prepared by Ms. Monks during the trial. It is by no means complete. Ms. Monks pulled together as many expenses as she and Mr. Perrozzino could find during the trial. The costs in Exhibit 52 total \$49, 844 and the evidence of Ms. Monks is that there were more expenses. These expenses were either incurred prior to the 1998 accident or after but which were turned down by ING. Accordingly, no further accounting is required and there as clearly been no windfall – the \$40,000 does not even completely cover the past medical/rehabilitation expenses arising from the 1995 accident. Ms. Monks was entitled to receive those funds free and clear from any claim by ING.

[885] Ms. Monks also received \$150,000.00 for future medical and rehabilitation expenses. The evidence at trial was that Ms. Monks purchased a structured settlement with funding of \$800,000. Given that the \$150,000 was intended for future expenses and given that the \$800,000 was invested in a sound manner to provide for future expenses, the most reasonable approach to take is to take into account the monthly amount received by Ms. Monks through the structured settlement.

[886] This is all set out in Exhibit 14. Of the \$800,000 funding of the structured settlement, the \$150,000 initially produced \$769.25 per month. It is currently providing \$784.71 per month. In regard to medical and rehabilitation expenses being claimed by the plaintiff from May 2002 to the present, the monthly amounts received by her from the structure are being deducted. On a go forward basis, it is the submission of the plaintiff that ING should pay for her on-going medical and rehabilitation needs net of the collateral amounts she receives which are shown in the Sandra Guest schedules and also net of what she will receive each month from the \$150,000 portion of

the structure for medical benefits (there is a schedule setting out what that amount will be for the rest of the structure in Exhibit 14).

[887] Ms. Monks received \$75,000.00 for home renovations. Once it is determined that she will either renovate or purchase and renovate, these funds will be used for that purpose and ING should fund the rest.

[888] Ms. Monks received \$110,000.00 for future housekeeping expenses. This amount should also be considered in the context of the structured settlement. Initially it produced \$730.68 per month and it now produces \$745.37 per month. The testimony of Ms. Monks at trial was that the amount she receives from the Zurich settlement covers her housekeeping costs currently. However, to the extent that the costs at some stage in the future exceed the amount provided by the structure then ING should be required to pay them up to the maximums provided in the regulations (more of this will follow).

[889] Finally, Ms. Monks received \$300,000.00 for future attendant care needs. Again that should be taken into account in the context of the structured settlement. The initial amount it produced was \$1,538.50 and it currently produces \$1,569.42. The amounts received to date have been accounted for in the claim for attendant care to date. On a go forward basis, ING should be required to pay for attendant care needs up to the maximum monthly amount of \$6,000 net of the amount received each month from the structured settlement.

[890] In my opinion, there does not seem to be anything unreasonable or incorrect in the plaintiff's plan to account for the Zurich settlement funds received. The plan is reasonable in that it takes care to ensure that the plaintiff does not benefit from any windfall or double-recovery, while safeguarding Ms. Monks' right to adequate compensation under the statutory accident benefit scheme.

Defendant's Submission on Zurich Settlement

[891] First of all, the plaintiff's needs into the future are not \$6.2 million dollars, as the plaintiff would have the Court believe.

[892] The figure put before the Court by the plaintiff is based on an assumption that the plaintiff will not have benefits, and there was no evidence led before the Court to support any suggestion that the plaintiff will not have access to these benefits for her lifetime.

[893] The report of Sandy Guest-Poulin clearly states that the lifetime costs for the plaintiff with her in a wheelchair is \$2.2 million (Exhibit 8, Tab 1, page 79).

[894] As can be appreciated, this plaintiff's generated assessment is a Cadillac approach and will err on the side of favouring the plaintiff, in assessing the needs and the costs and, we submit, quite frankly, that her lifetime needs and costs will likely be less and would come below the amounts, which are estimated in that report.

[895] To date, the plaintiff has already received \$1.275 million (being the \$1.54 less the amount attributed to costs). We know that she has structured \$800,000 of that settlement amount, which will, if she lives to age 85, generate for her \$2.4 million, if she lives to age 93, it will exceed \$3 million (Exhibit 14, page 4). That amount alone exceeds the figure assessed by Ms. Guest-Poulin, which, as we suggest, is a Cadillac of assessments, but, in any event, ensures that her future care costs are clearly, already covered completely by the settlement with Zurich.

[896] In addition to the amount covering her lifetime expenses, the plaintiff also has what is left of the \$475,000 (being the balance of the settlement funds).

[897] If that's not enough, the plaintiff also has an annual income of approximately \$50,000 over and above these other monies. This is comprised of:

- a) income replacement benefits from ING in the annual amount of \$17,714.84;
- b) long term disability benefits in an amount in excess of \$30,000, which presumably is indexed on an annual basis.

[898] This clearly shows that the plaintiff has been fully indemnified for the expenses, which she will incur, as a result of her injury. Now, she asks to be doubly indemnified or be awarded what amounts to a windfall.

[899] The plaintiff and her counsel, both being very bright and careful business women, knew at the time of the settlement with Zurich that the plaintiff's future life expenses were taken care of by the settlement figure agreed upon, and knew that the plaintiff did not have to rely on receiving any further benefits from ING, in order to protect herself. In fact, the plaintiff was well aware that ING did not intend to pay benefits past the point of settlement with Zurich, as ING itself expected to be indemnified for the expenses that it had paid out to date, on her behalf. Again, the plaintiff was well aware that ING intended to discontinue benefits, once she resolved her differences with Zurich.

[900] Lets look at what would have happened had the Zurich lawsuit proceeded. At that time, the plaintiff had now sued Zurich for payment of all benefits, and had sued ING for payment of all benefits. Recognizing that we have two accidents, which may or may not have contributed to a continuing injury, the Court would have been left to make one of three decisions.

- a) decide that one or the other AB (accident benefit) insurer was responsible for payment of benefits;
- b) decide that each AB insurer pay its proportionate share, according to the contribution of each accident towards the injury; or finally,
- c) decide that both AB insurers had to pay all benefits.

[901] So, in a brilliant strategy, the plaintiff split her case by settling with Zurich and leaving the Court with what appeared to be only two choices – either make this AB insurer pay, or not. However, in doing so, this creates the optics that there are only two choices for the Court to make, however, we submit that there remain three choices, which are set out above.

[902] It is important to note that there is no case law within the Courts that have determined the issue of how accident benefits should be dealt with, where there have been more than one accident and more than one accident benefit insurer, and the injury is one where the subsequent accident aggravates a continuing injury.

A – ONE OR THE OTHER AB INSURER IS RESPONSIBLE FOR PAYMENT OF THE BENEFIT

[903] This has been discussed under the section of Causation. Clearly our position is that Zurich is the insurer, which should be found to be responsible for the plaintiff's expenses.

B – PROPORTIONATE SHARE ACCORDING TO RESPONSIBILITY FOR THE INJURY

[904] Our common law has long accepted that a party should only be responsible for that which it caused. In tort, it is a readily accepted principle, governed by the laws of negligence, however case has accepted that in the case of a contractual obligation that equity accepts that there can be an apportionment of liability (see *Wyeth v. Henry McWilliam et al*, [1986] O.J. No. 2655, page 7; *Tompkins Hardware Ltd. v. North Western Flying Services Ltd.*, [1982] O.J. No. 944, pages 7-8; *Pan-Oston Ltd. v. Bowes & Cokcs Insurance Brokers Ltd.*, [1998] O.J. No. 3294, page 23).

C – OBLIGATION FOR BOTH AB INSURERS TO PAY ALL BENEFITS

[905] Under this provision, the second payer would have the right to deduct collateral benefits, which would include the amount of benefits up to the maximum monthly entitlement arising out the first accident. Therefore, in our circumstances, ING should be allowed a deduction for the full amount of the monthly benefits, which would have been available under the Zurich policy, pursuant to Bill 164.

Defendant's summary

[906] ING submits that it would set a dangerous precedent to enable Ms. Monks to claim and receive accident benefits for future care costs from ING in addition to settlement funds already received for that purpose from the 2002 Zurich settlement; to permit this claim to go through would be tantamount to facilitating a windfall or double recovery for the Plaintiff.

[907] The funds already received by the Plaintiff through the 2002 Zurich settlement, and the additional funds expected to be generated through Ms. Monks' structured investment of those received funds, far surpass the actual lifetime future care costs which the Plaintiff could reasonably claim. ING argues that the settlement funds received from Zurich ensure that Ms. Monks future care costs will be generously met.

[908] Had the Plaintiff proceeded with her action against both Zurich and ING, the Court would have been asked to determine which insurer would have been responsible for which benefits; where there is aggravation of a pre-existing and continuing injury, the Court must apportion causation and liability in order to avoid the result of a double-indemnity or windfall. By reaping the benefit of the previous Zurich settlement funds, and by now asking for additional benefits from ING, the Plaintiff is essentially asking the Court to award a double indemnity to her.

[909] As well, ING argues that Ms. Monks has a substantial amount of funds leftover from the Zurich settlement, which is not tied up in structured investments, and that she enjoys current income replacement benefits from ING and long-term disability benefits. All these sources of income demonstrate that the Plaintiff has already been fully indemnified for her expenses and losses.

[910] The Defendant ING also argues that it is entitled to a deduction for any amounts that are already available to the Plaintiff under any benefit plan, through s. 60 of the *SABS*. Such other amounts need not actually be paid to an applicant, but must merely be "reasonably available" in order for the insurer to benefit from a deduction. ING submits that the Zurich settlement plan funds fall squarely within this category. ING relies on the *Orchover* case to support its position.

[911] *Orchover et al. v. Wright et al.* (1996), 28 O.R. (3d) 263 – The judge's analysis

[Tab 23 of the Defendant's Book of Authorities, vol. 1]

- FACTS AND ISSUES:

- o The plaintiff was injured in a motor vehicle accident with the defendant; she applied for and received weekly no-fault benefits from her own insurer before negotiating a cash-out settlement with the insurer whereby she received \$100,000 on account of future care costs and \$90,000 for future loss of income; she gave a release of any claims for benefits payable subsequent to the settlement.
 - o The defendant argued that he was entitled to deduct from the damages assessed against him the full present value for the no-fault benefits that the plaintiff was entitled to receive from the insurer.
 - o The plaintiff argued that the defendant should be entitled to credit only for what the insurer had already paid.
- OUTCOME:
 - o The defendant was entitled to deduct the full present value of the benefits that the plaintiff was entitled to receive from the insurer.
- RATIO:
 - o In cases of fault, the tortfeasor is entitled to benefit from the availability of other insurance benefits, not the injured plaintiff.
 - o If a plaintiff makes an improvident settlement with its own no-fault insurer, that fact should not prejudice the tortfeasor.
 - o A tortfeasor may be entitled to deduct from damages payable the full present value of insurance benefits available to the plaintiff irrespective of what might have been in the minds of the insurer and the plaintiff at the time of a settlement agreement.
- ANALYSIS:
 - o Although this case appears to support ING's position, it must be noted that it was in the context of a fault-based tort action, not a no-fault insurance benefits action.
 - o The Defendant does not offer any case law or authority to suggest why the same principles should apply in a no-fault accident benefit context.

Discussion

[912] In my opinion, the three cases submitted by the Defendant (Tabs 14,15 and 16 of the Book of Authorities, vol. 1) in support of its argument that the Court should engage in an apportioning of liability (or that it would have been required to do so had Ms. Monks pursued her claim against Zurich and ING) do not add much in the way of discussion on this issue, nor do they address the issue of how to account for the previous settlement funds received. The *Wyeth* (Tab 14), *Tompkins* (Tab 15) and *Pan-Oston* (Tab 16) cases are merely examples of where a Court will apportion liability, including the applicant's own contributory fault or negligence, and the extension of this principle from tort law to contract law.

[913] The Defendant does not really propose or argue a concrete plan for taking into account the Zurich settlement funds received by Ms. Monks in determining ING's liability. The Defendant merely asserts that Ms. Monks has already been compensated for costs and expenses related to her injuries.

[914] ING submitted that it should be liable to pay any benefits (if at all) only when amounts that are reasonably available from other sources (i.e., the Zurich settlement funds) have been exhausted; alternatively, ING submits that it is entitled to deduct the full amount of the settlement funds relating to medical benefits and attendant care benefits from whatever amount is owing to the Plaintiff.

Evidence heard at trial on Suzanne Monks' needs

Sandra Guest-Poulin

[915] Sandra Guest-Poulin testified that she has been a Canadian Certified Rehabilitation counsellor and a Registered Rehabilitation professional since 1990. She has also worked as a certified Life Care Planner since December 2001. She holds a Bachelor degree in Social Development Studies, and in 1990, she completed a major also in Social Development, both at the University of Waterloo. In 1991, she obtained a second Bachelor of Arts degree, Law Major, from Carleton University. She is qualified to work with spinal cord injured patients, as well as brain damaged and multiple injured patients. She was hired, as a Case Manager, by insurers and following her assessment of medical and rehabilitation needs of her patients, she devises and reports on a program to the financial source. She stated she has prepared 450 Life Care plans to date, averaging 35 to 40 such plans each year. She also stated 40% were prepared for tort cases and 60% for accident benefits cases.

[916] She accepted a referral by Ms. Monks' counsel on April 9, 2001 and she worked with Ms. Monks until the month of August 2001. She had previously met Ms. Monks in 1991, as Ms. Monks had hired her to work with her for a company where she was employed.

[917] Ms. Guest-Poulin's assessments of Ms. Monks' needs are found in Exhibit 8, Tabs 1 and 2. She met with Ms. Monks on May 3, 7 and 29, 2001. She stated that she became involved in 2001, to help Ms. Monks make the transition from a wheeled walker to a wheelchair. Ms. Guest-Poulin had prepared four scenarios to meet with Ms. Monks' needs, but she recalled that following Ms. Monks' receipt of the report CAT DAC, and the reports of Doctors Weinberg and Atack, it became obvious that Ms. Monks was quite disabled. Her reports, in 2001, make a list of Ms. Monks' disabilities. Ms. Guest-Poulin was asked to update Ms. Monks' Life Care plan and that report was filed at trial, as Exhibit 29. The report is dated May 1, 2003.

[918] As it will become important to know what Ms. Monks' needs have been since 2002 until the date of trial, and whether such needs were directly caused by the accident and were reasonable and necessary, I have reproduced part of Ms. Guest-Poulin's report.

Needs for 2001

[919] Part of Ms. Monks' needs, as estimated by Ms. Guest-Poulin, in 2001, were as follows:

Costing Considerations

Bladder Care Program – Ms. Monks self-catheterizes 4 to 5 times a day. For this procedure, following supplies are provided by the Community Care Access Centre: Apicare Banzalkonium Chloride 30 ml, Xylocaine 2% Jelly, 4-Sure 14" Fr Catheters or Med-RX 14Fx6" or Med-RX 14Fx16" Catheters, Urethral Cath Tray without Cath or, Meditron Cath Tray with Catheters, Non Sterile Gloves, Baby Wipes and Underpads.

Bowel Care Program – Ms. Monks must engage in digital evacuation of her bowels on a daily basis. For this procedure, she uses Non-sterile gloves, KY Jelly, Microlax enemas and, suppositories. Secondary to her bowel difficulties, Ms. Monks has a tendency to be bothered by hemorrhoids and requires creams and suppositories.

Pain Management Program – Ms. Monks is dependent on Fentanyl patches that are changed every 72 hours for pain management. Patch changes require – 3 Op-site adhesive bandages, Wipe Skin Prep Barrier, Adhesive Remover and hydrocortisone cream.

Peripheral Intravenous Central Catheter (PICC) Line Maintenance Program

– Ms. Monks has been on a Peripheral Intravenous Central Catheter for program since the fall of 2000. Nursing staff visits every 2nd day to monitor Ms. Monks' general health, the condition of the PICC line and order supplies. In addition, nursing staff is required to provide PICC line dressing changes every 2 days. Zofran medication is inserted into the central catheter 3-times a day.

PICC Line Administration requires the following supplies:

Blue Disposable Face Masks	Pad Alcohol Prep E-Z Ject Medium
Non-sterile grooves – medium	Dressing Gauze Sponge 2x2 12 ply Sterile
4x4 sterile gauze	Dressing Gauze Sponge 4x4 12 ply Sterile
Remover Wipe Mentor Adhesive	Dressing Op-Site 6x5 5cm IV 3000
Isopropyl Alcohol 70% - 450 ml	Syringe TE 10cc Luer Lok Sterile
Glove Syntehtetic Exam OMS Medium	Chlorhexidine Swab Sticks
CADD Admin Set 60" with Bag Spik	Cannula Interlink Threaded Lock
Tape Hapafix 10cmx10m Surgical	Cannula Interlink Vial Access
Interlink Syringe Cannula	IV bag/prepared Zofran 2 ml
Wipe Skin Prep Barrier	Battery Procell 9 Volt
Suture Steri-Strip ½"x4" Adhesive	Adapter Vial Universal
Mepore Dressing 15x9 cm	Adapter Interlink Injection Site
10ml Sodium Chloride 0.9%	Dextrose 15% 50ml
Universal Interlink	Terumo Needle 25Gx1"
PDI Antiseptic Iodine Swapsticks	Locking Liner Male Adapter Plug – short
Jelco Intermittent Injection Cap	Venisystem T-connector
Surgifix	Clinidine Povidone Iodine 10% USP Solution

[920] Ms. Monks does not use all of these items on a daily basis. Use of various items is dependent on nursing staff's preference. The Community Care Access Centre at special retail costs arranged with individual suppliers is presently funding these items. In order to determine the annual expense, Ontario Medical Supplies (OMS) provided the individual retail unit cost and each PICC Line Step materials usage was reviewed with Ms. Monks.

[921] In 2003, Ms. Guest-Poulin's report added the following changes:

Prescription Medication Regime

Changes to the Medication Schedule of the August 13, 2001 Life Care Plan would include,

- Dr. Pierscianowski has prescribed Flamazine, Celestoderm V2 and Fucidim H creams at specific sites with Duoderm CGH dressings, changed no more than three times a week for contact irritant dermatitis on her hands, neurodermatitis with folliculitis and possibly pyoderma gangrenosum on the breast and, avulsed nevus on the left anterior thigh.
- An Axis I diagnosis of Depression in partial remission was proposed by Dr. Gerin-Lajoie and Zoloft and Wellbutrin have been prescribed.

Medication prescriptions have been omitted, replaced and changed over the last one and a half years. The medication costs outlined in the appended table of costs were obtained from Shopper's Drug Mart.

Medical Supplies

Changes to the Medical Supplies Schedule of the August 13, 2001 Life Care Plan would include,

- Equipment and supplies that became required after the 1998 accident and the subsequent 1999 surgery are catheters for bladder dysfunction, enemas for bowel dysfunction, dietary supplements for gastrointestinal dysfunction and intravenous supplies for medication administration and symptom management.
- Supplies include supplies for port-a-cath management for the administration of Zofran to address her gastrointestinal symptoms (vomiting and nausea), a BIPAP machine for breathing difficulties, pressure stockings for lower extremity edema, dressings for breast and upper arm ulcers, dosette for prescription medication management and a cervical collar when ambulating or during car travel.
- As indicated, the port-a-cath was surgically inserted and the PICC line removed. While the use of a full arm cast bandage for PICC line protection is no longer applicable, Suzanne continues to use protective covering over her Fentanyl patches for showering and when she is exercising in the family swimming pool or at the summer home. Swimming and showering without protective covering causes her Fentanyl patches to fall off and the cast protector keeps the patches reasonably dry.
- Suzanne uses the Aspen and Miami J collars on a rotational basis when she is in a vehicle or using her scooter and walker. She interchanges her collars as they cause pressure points at different locations of the neck. She also perspires tremendously under the liners and perspiration contributes to her

problems with skin integrity. The solution, for Suzanne, is to have 2 or more collars and liners that she is able to interchange, which I believe is reasonable.

- Tegaderm or Op-site dressings are protection dressing for her Fentanyl Patches. Three are required every 2 to 3 days. Alcohol and adhesive remover is also required when changing the patch sites. Hydrocortisone cream is applied to the sores that develop under the patches.

Wheelchair Mobility / Accessories / Maintenance

Changes to the August 13, 2001 Life Care Plan would include omitting cane replacement costs and adjusting the future cost of walkers, scooters and the Bruno Lift and replacing those with earlier start-up dates (at the age of 50) for a power-wheelchair, as proposed by Dr. Vandewaetere.

- Cane replacement is now unnecessary, as Suzanne has become fully dependent on walker and powered mobility.
- Dr. Vandewaetere has advised that Suzanne will be power-wheelchair dependent within the next 2-5 years given the progressive deterioration of her condition physical condition.
- It is likely that Suzanne will continue to use a walker from bed to bathroom in her present home, long term. As it is impossible for her to carry her walker from one level of the home to another, without attendant care support, it is reasonable that she have two walkers, one on each level of the home, to increase her accessibility to walker equipment.
- The Elan Category II walker was no longer suitable due to the fact that Suzanne now requires a wider seat for safety reasons and to accommodate her increased weight. The cost of the new walker was \$695. A second walker has been purchased for second floor use in the home, as Suzanne is not able to carry her walker up between floors.

Transportation

Changes to the August 13, 2001 Life Care Plan would include adjusting the future cost of vehicle adaptations to exclude hand controls, as it is not expected that Suzanne will ever be an independent driver of a vehicle, given the combination of medications, somnolence, her inability to turn her neck to check traffic and her lack of strength and coordination in her arms.

- She will require a lift to accommodate her powered mobility equipment and lock-down mechanisms to reduce the need to transfer from mobility equipment to the passenger seat. An occupational therapy assessment is required to identify and recommend the modifications that will be required to support Suzanne, as a chauffeured passenger in the family vehicle.

Aids for Independent Function

- Suzanne now has difficulties with bed, toilet and tub transfers and a history of falls with mobility and stair climbing. New assistive aids recommended and some purchased over the last year and a half have included: a heavy duty, padded seat commode (OMS-\$289.31 or MS-\$315.70), two Ottobock hinge grab bars (\$332 plus installation), wireless door chime, Aquatic bath lift, a regular mattress (adjustable bed), a Lifeline and a second walker for second floor use. (Kaymar OT)
- Ms. McNabb reported the necessity for two rollator walkers, a portable commode (bedroom en-suite), a versa frame (main floor); grab bars, a transfer pole, an electric bed, long-handled reachers, scooter and car lift.

Health and Strength Maintenance

- Suzanne's physiotherapy program at the Rehab Centre involved use of a treadmill to walk 0.6 miles per hour for 5 minutes at a time. She was able to gradually increase her tolerance before discharge in early September 2001. Unfortunately, her walking tolerance deteriorated within six weeks of discharge and she was unable to maintain her discharge status. The treadmill purchased did not accommodate this deterioration. The treadmill that I had projected began at a speed of 0.1 mile per hour and would have been appropriate for resumption of a walking program, however, given the significant level of deterioration, it may be that any funding for a treadmill would not be appropriate at this time.
- Alternatively, Dr. Johnson recommends the use of a stationary bicycle and swimming exercises to prevent progressive deterioration on her knees. It is also recommended that Suzanne engage in an exercise program at some level to maintain her limited physical functional abilities. Rehabilitation literature supports that exercise prevents osteoporosis and further deterioration of the skeletal muscular structure.

Personal Services – Home / Community Support

- Suzanne's attendant care needs were increased from 60 to 81 hours a week in Ms. McNabb's September 2002 Assessment of Attendant Care Needs (Form 1). This report addressed Suzanne's needs related to daytime living activities such as personal care and home management.
- Suzanne should never be left alone, given her risk of falls. Additionally, she should never be left alone in the event of a fire or such event that she is required to leave the home. She requires maximal assist to get from bed to walker or powered chair to lift to outdoors, expediently. I have therefore considered a schedule to ensure 24-hour attendant care support (168 hours a week) within a program that combines CCAC funded home support and private housekeeping and attendant care with spousal support services.
- I consider that Mr. Perrozzino would be home from work from 6 in the evening to 6 in the morning, thereby providing 12 hours of family home support during the 5-day work week (60 hours) and 16 hours over weekend days (32 hours) totally 92 hours of family home support a week. I allow an expense of \$7 per hour as per the non-professional allowance under Bill 59 for family attendant care.
- I allow for four weeks (28 days/2,576 hours) of family respite a year to ensure that attendant care is available, while Mr. Perrozzino is away on business trips or attending to family and personal matters in the community. The cost of replacing family attendant care with private fee services would increase from \$7 an hour to \$17 an hour for this period of family designated time.
- With the family providing 92 hours a week (minus Respite time), Suzanne will require 76 hours a week of other attendant care services to ensure 24-hour attendant support. I have quantified the cost of a 24-hour attendant care support program, based on the following allocation of staff:
 - 5 hours a week CCAC funded home support worker
 - 9.5 hours a day, or 63.5 hours of personal support worker
 - 5 hours a week of housekeeping services
- I recognize that the current 6 hours a week of CCAC funded nursing services is not being provided on a set and reliable schedule. Additionally, nursing services are specific to the management and care of Suzanne's Port-a-Cath and once their task is complete, they leave the home to attend other patient care needs. Therefore, Registered Visiting Nurses are not part of the 24-hour Attendant Care program.

- In the event that Mr. Perrozzino was to predecease Suzanne, the cost of replacing his attendant care services would be \$17 an hour.

Architectural Renovations & Furnishing

I accept Mr. Puddicombe's April 8, 2003 updated Accessibility Costs Report, whereas the cost of renovations increased from \$74,200 (based on his report of May 29, 2001) to \$285,369, based on the following changes and/or medical recommendations:

- According to Dr. Vandewaetere, Suzanne is likely to require powered mobility within the next 2-5 years, as opposed to the original report's position of the use of a walker and the possibility of the eventual use of a manual wheelchair.
- According to Dr. Donald Johnson, Orthopaedic Surgeon, Suzanne "needs some form of exercise she can do at home. A swimming pool would be ideal; however, a stationary bicycle that she could exercise would be a reasonable compromise for the near future".
- Suzanne requires maximal assistance with ambulating up/down the stairs, including having a person lift each of her feet up/down onto the next step. Stair climbing is unsafe for Suzanne (risks of falls) and hazardous to her attendants (in the event of her falling or knocking the attendant down the stairs). A trial of a circular stair glide proved inadequate. The home will require an internal elevator system to access the top floor of the home.
- Recognition that the cost of construction (and renovations) has increased significantly since May 29, 2001.
- Suzanne spends up to 180 days per year at the family cottage, as a primary family activity that promotes her mental and physical health. A cottage assessment is needed to address safety at her family cottage. Although the cottage is not a primary residence, it is used year round and it would be foolish to protect Suzanne from safety related injuries in her home, but not offer the same protection at the cottage. The allocation of \$800 for an assessment of modifications is a nominal and reasonable consideration.
- The wording of Bill 59 – 15(2) states, "The rehabilitation benefit shall pay for reasonable and necessary measures undertaken by an insured person to reduce or eliminate the effects of any disability resulting from the impairment or to facilitate the insured person's reintegration into his or her family, the rest of

society and the labour market”. The Bill 59 accident benefits policy does not delineate primary versus secondary residences pertaining to an assessment of modifications and/or cost of modifications to more than one residence.

Summary Comments

Suzanne’s medical and disability needs have continued to change since authoring the Life Care Plan of August 13, 2001. In review of new medical issues and functional deterioration with Dr. Vandewaetere, the scenario of walker to wheelchair by the age of 65 is no longer applicable. Given the current rate of progressive deterioration, it is likely that Suzanne will be limited to using powered wheelchair mobility in the next few years.

In examining Suzanne’s current status and given changes to her personal care and attendant care needs, there are changes to and additional goods and services now required that were not addressed in August 13, 2001. The costs of these changes are reflected in the appended tables.

Scott Puddicombe’s evidence

[922] Scott Puddicombe testified during trial and addressed accessibility costs in his reports. He is a licensed Master Carpenter and I qualified him as an expert on lifting devices. He has testified previously in Ontario Courts on home adaptations and modifications for wheelchair accessibility. He has been involved in that type of work since 1991. Currently, he limits his work to design and consultancy work that includes working on disabled persons’ homes.

[923] He was asked to prepare an estimate on Ms. Monks’ home by Fiona Smith Bradley, who was hired by ING, as Ms. Monks’ occupational therapist. Ms. Bradley met with Mr. Puddicombe on October 31, 2000 to discuss immediate, intermediate and long term modifications that would facilitate Ms. Monks’ safety and independence in her home, specifically for stair mobility, home and kitchen accessibility.

[924] Mr. Puddicombe gave Ms. Bradley an estimate for a stair glide, providing travel from the main floor to the second floor of Ms. Monks home of \$13,750. The estimate for the front entry to the home totalled \$1,050, and for the renovations to the kitchen, \$7,500. The grand total

with GST came to \$23,668.40. Mr. Puddicombe stated that he never heard further from Ms. Bradley, once he delivered his estimate on November 22, 2000.

[925] Then, Mr. Puddicombe was asked by Sandy Guest-Poulin to provide an accessibility costs report to measure the cost of installing an elevator device for wheelchair use, as opposed to a walker use. He recalled that he did an investigative report and did not verify any of the information he obtained on the first visit. He did not take photographs of the interior of the house and he took no measurements of the different spaces in Ms. Monks' home. He did not check municipal by-laws and zoning requirements. He explained his report during the trial. The changes to the garage would have allowed Ms. Monks' access to the interior of the house by widening doors; he explained the change he proposed for the bedroom (upstairs master en suite) and its bathroom, main level powder room, the kitchen, the raising of the family room floor (it went down 7 inches with no room for a ramp), and a three step elevator with a weight capacity of 750 pounds. The bottom line was that, while such renovations for a walker would cost \$79,394 for a wheelchair, the modification cost amounted to \$146,911. This plan was submitted to Sandy Guest-Poulin and he never heard anything further.

[926] Mr. Puddicombe prepared another report dated April 8, 2003 to show what modifications would be needed to Ms. Monks' home to accommodate a wheelchair with power. The estimate included the cost of installing an indoor pool. The pool was recommended by Dr. Johnson. Dr. Johnson had been consulted by Ms. Monks, who was concerned with the state of her knees (the knees were buckling and could not support her). Mr. Puddicombe testified that a power chair requires a turning circle radius of more than five feet ... the standard for a wheelchair without power. I have reproduced Mr. Puddicombe's estimated costs, totalling \$362,206 and the itemized list gives a good picture of what was planned.

<u>ITEM</u>	<u>COSTS</u>
Entry and Exit	
Primary	\$ 19,500.00
Secondary	**
Master Bedroom	

Master Bathroom	\$ 27,000.00
Raised Section of Bedroom and Closets	**
Kitchen	\$ 40,000.00
Living space	\$ 12,000.00
Miscellaneous Items	
Flooring	\$ 10,000.00
Electrical	\$ 5,200.00
Travel between Levels	\$ 27,000.00
Pool	\$ 22,000.00
Alternate Accommodation (\$1,500.00/mo.)	\$ 15,000.00
Moving and Storage	\$ 4,000.00
Maintenance (Approx. \$1,200.00/yr)	not applied
Replacement	not applied
Proposed Addition	\$ 128,000.00
Includes: Secondary Means of Entry and Exit	**
Raised Section of Bedroom and Closets	**
Storage Space	**
Elevator Shaft	**
Eating Area of Kitchen	**
Anticipated consultation/management and Design fees (10% of total)	<u>\$ 30,970.00</u>
** Items that have been included in the proposed Addition	
Sub-total	\$ 340,670.00
Anticipated G.S.T.	<u>\$ 21,536.00</u>
Total Estimated Cost	<u>\$ 362,206.00</u>

The prices outlined in this report are estimated costs only.
Accurate pricing can be provided once approved plans have been completed.

[927] Mr. Puddicombe stated that the work for an elevator running along the outside wall, at the back of the house, was \$285,000 in 2003, and had increased to \$362,206 by December 2004, due to the scarcity of labourers in the construction trade in Ottawa. He stated that he also added \$15,000 for alternate accommodation for the Monks' family during the eight to ten months the renovations would be ongoing and \$4,000 for the moving and storage of furniture.

[928] Mr. Puddicombe was asked to comment on the criticism made of his plans by David Borthwick, an accessibility design consultant, hired by defendant. He pointed out that Mr. Borthwick could not compare his 2001 and 2003 reports, as they involved two different projects. The 2001 project was not sufficient to meet Ms. Monks' disability, but the 2003 report was sufficient. He also disputed Mr. Borthwick's estimate of 10 to 12 weeks to complete the job, as the backyard at the Monks' home cannot be accessed with heavy machinery.

[929] This Court heard a debate as to whether or not the purchase of a bungalow for the Monks' family would cost less and still help Ms. Monks. Even though there would be no change of elevation to go to a second floor, Mr. Puddicombe felt that the price would be \$200,000 for modifications to the bungalow and the installation of an indoor pool (small therapeutic tub like arrangement). The pool installation would add \$40,000 to \$60,000. Mr. Borthwick had estimated \$100,000 to \$150,000 to modify an existing bungalow. Mr. Puddicombe pointed out that Mr. Borthwick had not allowed for a pool and had not allowed for kitchen modifications. Dealing with a custom built home option, Mr. Puddicombe explained that, asking a builder to put up a custom built home would not translate into savings. The price of acquiring land and the number of intermediaries who collect a 30% fee for everything installed, would require the contractor to hire subcontractors and, thus, any savings would be eliminated.

[930] During cross-examination, Mr. Puddicombe agreed that Ms. Monks should look for a bungalow without a pool and without an elevator, as a more viable option. He was reminded by counsel for the plaintiff that the cost of moving, the 5% sale commission on the Monks' home, legal fees, land transfer tax, all have to be added to the total cost, if moving is considered. Mr. Puddicombe explained that in 2001, he would not have installed a stair glide in Ms. Monks' home, because Ms. Monks had problems with balance on flat surfaces at that time.

Fiona Smith Bradley's evidence

[931] Fiona Smith Bradley gave evidence. She is a registered member of both the Canadian Association of Occupational Therapists and the Ontario Society of Occupational Therapists. She graduated from Queen's University with a Bachelor of Science in occupational therapy. Since

1997, she has assessed over 600 clients in order to make recommendations to increase their functions in their homes or workplaces.

[932] She met with Ms. Monks on December 16, 1999 at her home, having been referred the file by ING. She assessed Ms. Monks' home making, home cleaning, meal preparation capabilities and she stated that she also assessed Ms. Monks' home office.

[933] Ms. Bradley stated that she had no concerns with Ms. Monks' mobility. On June 14, 2000, Ms. Monks had fallen down during a physiotherapy session at C.B.I. and Ms. Bradley explained that she had attributed the fall to Ms. Monks' gait training at C.B.I. She also confirmed that Ms. Monks was confined, in August and September 2000, to her bedroom on the second storey of her home, because of nausea, vomiting and extreme fatigue. She recalled that she had recommended to Ms. Monks to move to a bungalow to have her kitchen on the same floor as her bedroom. She had also recommended a stair glide for Ms. Monks' home, but the plan was abandoned because there was a curved staircase between the main floor and the second floor in Ms. Monks' home.

[934] Ms. Bradley testified that Ms. Monks was not prepared to move out of her home in the fall of 2000. She recalled that Ms. Monks felt she could not make decisions on home modifications. Ms. Monks said that her mother was waiting for the result of a functional status assessment from Elizabeth Bruyère (a long term care facility), and her decision would be geared to her mother's decision. Also, Ms. Monks had mentioned that her finances worried her and that she was going to attend an examination for discovery in a lawsuit involving another insurer. Ms. Bradley could not remember any complaints by Ms. Monks over any inaccuracies in her report.

[935] On October 23, 2000, Ms. Bradley recalled discussing a stair glide with Johanne Beehler, Case Manager at ING and with Scott Puddicombe. Mr. Monks had reported falling backwards on the stairs of her home. She also recalled the uncertainties that surrounded Ms. Monks in November and December 2000: Ms. Monks had applied for a CAT DAC assessment and Ms. Monks had important surgery coming up the following month, in January 2001.

[936] Following Ms. Monks January 2001 surgery, Ms. Bradley stated that Ms. Monks remained at a rehabilitation centre until April 5, 2001. At that time, she recalled Ms. Monks had said she would move to a 2,400 square foot home, but that she did not have the finances to move. Ms. Bradley did not meet with Ms. Monks during the period of May to September 2001. Ms. Bradley stated that she contacted Ms. Monks on one occasion, prior to going on maternity leave.

[937] During cross-examination, Ms. Bradley admitted that some of her reports to ING, concerning Ms. Monks, had not been given to Ms. Monks and the failure to provide copies to Ms. Monks, represents a breach of Ms. Bradley's code of ethics, as an occupational therapist. She also agreed that there had been no follow-up to provide a platform and steps to allow a Ms. Monks access to her pool, located in the backyard of the Monks' home. She further agreed that there was no follow-up on the safety in the home, following Ms. Monks' report of falling on the stairs on September 6, 2002. Nothing had been done by September 23, 2000, and Ms. Bradley admitted that Ms. Monks continue to fall in her home and fell backwards on the stairs of her home in October 2000.

[938] Ms Bradley also agreed that Ms. Monks had never refused the stair glide. Ms. Bradley corrected the impression that the installation of a bed on the main floor of the Monks' home was a desirable solution, as she agreed Ms. Monks could not access the bathroom and powder rooms with her walker on that level. She confirmed that she had recommended to Ms. Monks to come down the stairs in her home on her seat. Ms. Bradley stated that she made some recommendations that were not acted upon by ING and that it was not her job to press.

[939] More importantly, Ms. Bradley agreed that, following Ms. Monks release from the rehabilitation centre during April 2001, both the idea to renovate Ms. Monks' home or to move Ms. Monks to a bungalow had been placed on hold until ING received Ms. Monks' CAT DAC assessment. She also agreed that Ms. Monks had not been told of this decision.

David Borthwick's evidence

[940] He prepared an accessibility audit for ING. Mr. Borthwick obtained a licence to sell real estate in 1984, did a course in Construction Engineer Technology Management at Fanshawe

College, in London, Ontario, from 1985 to 1988, and he obtained a certificate of occupational, health and safety assessments from Ryerson Polytechnic, in Toronto, in 1994. He was qualified at trial as a home modification and accessibility design consultant and estimator.

[941] Mr. Borthwick never met Ms. Monks, nor did he visit her home. He stated that he was hired by ING after Christmas 2004 to review and criticize Scott Puddicombe's three reports on solutions for accessibility in Ms. Monks' home. He did not meet with Ms. Monks' assigned rehabilitation team.

[942] Mr. Borthwick came up with a price of \$200,000 to install an elevator at the rear of the Monks' home. He attempted to give evidence on a different location for the elevator at the rear of the Monks' home and I disallowed the receipt of that evidence, as it was outside of Mr. Borthwick's report submitted to the Court and found in Exhibit 30. Mr. Borthwick also testified that the renovations he proposed would take only eight weeks to complete and the Monks' family would not have to move out or send their furniture to storage.

[943] The option of moving Ms. Monks to a bungalow was explored. Mr. Borthwick referred to the report of real estate agent, Hamidi Rihi, found at Tab 4, Exhibit 30 to establish that a bungalow located in the Orleans area would cost \$350,000. Mr. Rihi also provided an opinion that the home of Ms. Monks would fetch \$290,000 to \$300,000. For renovations to an existing bungalow, Mr. Borthwick figured it would cost between \$100,000 to \$120,000. He also stated that he had no experience in the installation of a therapeutic pool.

[944] During cross-examination, Mr. Borthwick admitted to have managed only three projects in Ottawa. Mr. Borthwick also admitted that he had only testified in front of an arbitrator in the past. He agreed with plaintiff's counsel that a visit to the Monks' home would have been highly desirable to find out about the family dynamics and the use of their space. When told of the difference between the 2001 and 2003 Puddicombe reports, Mr. Borthwick stated that he no longer challenged Mr. Puddicombe's 2001 report. He was unsure, however, if the fact that he did not know that Mr. Puddicombe's walk through report of the Monks' home, had not been made with an expectation that the owner or the insurer would move on it, would affect his report.

Johanne Beehler's evidence

[945] Ms. Beehler has a grade 12 education and has progressed as an insurance adjuster to a senior field adjuster, in 1999. Her experience consists of on the job training (in 1988, she was handling first party losses in motor vehicle accidents) at different levels of insurance protection and seminars. She worked for Guardian Insurance, purchased by ING in 1998. She discussed her duty of good faith towards clients, her obligation to contact clients in a timely matter, the hiring of specialists for care giving and chasing forms to help with the determination of coverage.

[946] Ms. Beehler confirmed that the insurer is bound by the time limits imposed by the SABS, but the independent contractors working for the insurer are not bound by the time limits.

[947] Ms. Beehler testified that she first met Ms. Monks in January 1999. From January 1999 to March 1999, Ms. Monks had met with Dr. Vandewaetere and was waiting to meet with Dr. Lesiuk. Between March and September 1999, Ms. Monks kept Ms. Beehler informed of her health problems and Ms. Beehler learned that a July MRI showed Ms. Monks needed surgery. Ms. Beehler also testified that Ms. Monks had told her that she had needed surgery, prior to her 1998 motor vehicle accident. Ms. Beehler explained that commencing in August 1999, she gathered more medical information on Ms. Monks and that an attempt was made to have Dr. Lesiuk give an opinion on which of Ms. Monks' accident caused her to be catastrophically impaired.

[948] Mr. Gilles Rose, unit manager and Lori Hawn, corporate legal counsel, both employed by the defendant, received status reports from Ms. Beehler and gave Ms. Beehler directions as to how to handle Ms. Monks' case. She stated that Laurie Warren, an occupational therapist, kept her informed of Ms. Monks' needs after the first surgery in September 1999 to June 2000, and that she provided everything that was required to meet Ms. Monks' needs. In January 1999, Fiona Smith Bradley had been also assigned to meet with Ms. Monks and figure out Ms. Monks' transportation needs. By August of 2000, Ms. Beehler recalled that she had received reports that

Ms. Monks was falling, but that she did not think that the problem was related to Ms. Monks' 1998 motor vehicle accident.

[949] Ms. Beehler testified that Ms. Monks had refused attendant care, as she did not like strangers coming into her home. She stated that she had asked Ms. Monks if she was missing something and that nothing seemed to be missing in the fall of 2000. Ms. Beehler explained that if a person refuses attendant care, not much can be done about that.

[950] On December 1, 2000, Ms. Beehler stated that she was advised by Ms. Monks that there was a 95% chance that she would undergo a second surgery in January 2001, and that she would not make any decisions on renovations. Ms. Beehler recalled thinking that Ms. Monks' decision made sense, as Ms. Monks had a \$100,000 limit for home renovations and, since the proposed renovations would cost \$24,000, this last amount of benefit would be lost if the renovations did not prove useful. (Ms. Monks had already received \$75,000 from Zurich for home renovations. The explanation for what ING paid Ms. Monks for this item will be found in the plaintiff's submissions at page 286.)

[951] Discussing Ms. Monks' entitlement under the legislation, Ms. Beehler explained that on home modifications for rehabilitation benefits, Ms. Monks was limited to \$100,000 for a non-catastrophic injured person, pursuant to section 15, subsections 5(i) and 8. If Ms. Monks was declared to have a catastrophic impairment, her limit would increase to one million dollars. Until May 2001, ING was waiting for a report from Dr. Lesiuk and for the result of a CAT DAC investigation.

[952] The difference between the price of Ms. Monks' existing home and the purchase of a bungalow adequate to meet Ms. Monks' needs was determined to be \$150,000. Ms. Beehler stated that economically, it did not matter whether Ms. Monks' moved or renovated her existing home. She recalled that Ms. Monks, who was an experienced adjuster, would not tell ING what she wanted to do; Ms. Monks knew that all she had was a \$100,000 limit.

[953] When Zurich insurance paid Ms. Monks 1.5 million dollars, ING discontinued paying Ms. Monks her benefits, on May 31, 2002. Ms. Beehler's letter is reproduced in Exhibit 15. Even though ING never collected, ING paid out \$50,786.88 for home modifications, as Ms. Monks had received \$75,000 for that item in her settlement for the 1995 motor vehicle accident. Attendant care was stopped at 104 weeks and Ms. Monks was asked to repay \$33,064.37, and for housekeeping, \$1,811. Ms. Beehler also stated that, as Ms. Monks had recovered \$600,000 from Zurich for her Income Replacement Benefits, ING calculated it had overpaid Ms. Monks in the amount of \$17,730.44. ING then deducted 20% from Ms. Monks' Income Benefits, from 2002 until trial, when ING paid back to Ms. Monks the 20% deducted.

[954] During cross-examination, Ms. Beehler admitted that, Fiona Smith Bradley, the therapist she had assigned to look after Ms. Monks' needs, does not mention attendant care in her first two reports. At that time, Ms. Monks had fallen down the stairs of her home. Ms. Beehler further admitted that she had not made notes on the subject. Ms. Beehler also confirmed that she had not met with Ms. Monks, except once on December 1, 2000 and then following surgery on April 10, 2001, at the CBI cafeteria. Counsel for the plaintiff reviewed Fiona Bradley's reports with Ms. Beehler and Ms. Beehler admitted that Ms. Monks had gone through a substantial physical and psychological deterioration, by June 15, 2000. Ms. Beehler admitted she had no notes in her file to cover Ms. Monks' condition from April 2001 when Fiona Bradley stopped meeting with Ms. Monks, until April 2002. Ms. Beehler did not recall any change in Ms. Monks' mental capabilities, although she had noticed the physical deterioration.

[955] Ms. Beehler was asked about ING's internal reports involving her and ING's corporate counsel, Lori Hahn, and her replacement, John Berriault. On September 28, 2000, Mr. Stephen Gray, a claims representative from ING (Exhibit 40, Tab 10), described Ms. Monks as "extremely stoic and motivated" and as "a highly credible person". Ms. Beehler became argumentative and would not admit that ING had all the medicals on hand on Ms. Monks' condition to assess Ms. Monks' condition, and whether a causation link was established with the 1998 motor vehicle accident. John Berriault wrote to Ms. Beehler on October 23, 2000: "I believe experts will all state the impairment was caused by our loss and did start as of the date of

loss. All medicals in file are consistent with onset of disability being caused by the motor vehicle accident, despite insured's stoic attempts" (Exhibit 40, Tab 12). Ms. Beehler replied, when confronted with the above quote, that the word "will" indicates the future and that Ms. Monks had not met the catastrophic impairment test.

[956] After a long debate at trial, Ms. Beehler admitted that she placed her own interpretation on what Dr. Besemann had stated in his report, "I cannot reliably conclude that", but ignored what Dr. Lesiuk, Dr. Weinberg and Dr. Attack had said about the contribution of the 1998 motor vehicle accident.

[957] Ms. Beehler admitted that she was in breach of section 4 of the guidelines concerning CAT DAC assessment by communicating with the director of the assessment centre and not advising Ms. Monks.

[958] In connection with Ms. Monks' falling in her home, Ms. Beehler was asked if the one and a half months it took her to obtain an estimate on home modifications assured Ms. Monks "peace of mind", as defined in her contract. Ms. Beehler stated that she had not met with Ms. Monks until December 1, 2000 to give her an application for a CAT DAC assessment. Ms. Beehler had to agree that she had no notes on Ms. Monks' refusal to have home modifications until her first surgery and that she had not advised her superior of this in her December 18, 2000 report. Ms. Beehler was shown Exhibit 40, Tab 37, where Mr. Berriault had established ING had purchased Zurich on November 1, 2001 and ING paid out the Zurich settlement to Ms. Monks on April 1, 2002. Ms. Beehler replied that she did not know what arrangements were made, by her employer, at that time.

Entitlement to Benefits

Plaintiff's Submissions

Past Benefits

[959] The claim for past benefits is from May 2002 to the present. The foundation for these claims is contained in Exhibits 27 and 53 and Exhibit 8.

[960] Exhibits 27 and 53 contain the documentation of the actual expenses incurred on behalf of Ms. Monks since May 2002. Exhibit 8 contains the reports of Sandra Guest-Poulin. At Tabs 4 A, B & C she summarizes the expenses for 2002, 2003 and 2004. This was prepared in order to demonstrate to the Court and to ING what the actual needs and costs have been and what has been covered by collateral benefits (GSMIP, ODB, CCAC). Unfortunately, the final totals reached by Sandra Guest-Poulin and Ms. Monks do not completely accord with each other (but they are only off by about \$3,000 total for the whole period from May 2002 to January 2005).

[961] It is clear that it is extremely complicated to maintain the accounting for all of the expenses given the involvement of GSMIP, ODB and CCAC. Mr. Perrozzino and Ms. Monks both testified about the extraordinary task it is for him to simply manage the inventory of medications and medical needs, let alone the accounting. It was also their evidence that Mr. Perrozzino, who became responsible for the accounting following the denial of benefits by ING, became despondent about maintaining those accounts on top of everything else he had to do in terms of holding down his day job, taking care of Ms. Monks and taking care of his family. As a result, it is quite understandable that the accounting is not perfect.

[962] Ms. Guest-Poulin's reports were essentially prepared for illustration purposes to demonstrate in an orderly fashion what the various needs are, what they cost and what benefits are available from what sources to cover them. Exhibits 27 and 53 simply show the net amount Ms. Monks is out of pocket; Exhibit 8 shows how they got there.

[963] If anything, the claims being advanced in Exhibits 27 and 53 probably fall short of the true costs, which have been incurred by Ms. Monks. As a result, if there is a discrepancy, it works in favour of ING not against it. Furthermore, given that ING essentially washed its hands of Ms. Monks as of May 31, 2002, it can hardly be seen to complain if the accounting is less than perfect.

Medical/Rehabilitation Expenses to date

[964] Exhibit 27 tallies the total of the medical and rehabilitation expenses for May 2002 to the end of 2004 as \$68,815.87. This amount is net of all collateral benefits from GSMIP, ODB and CCAC.

[965] The Zurich contribution during that period from the \$150,000 portion of the structured settlement was \$24,195.44.

[966] The net amount after deducting the Zurich contribution is \$44,620.43.

[967] Exhibit 53 added a few more medical/rehabilitation expenses for that same period (May 2002 to the end of 2004). Those expenses added up to \$2,097.61. Therefore, the total net medical/rehabilitation expenses incurred by Ms. Monks during the period leading up to trial is \$46,718.04.

[968] In the letter from ING dated May 31, 2002 to Ms. Monks when they cut off her benefits, ING chose to pay the balance of the medical/rehabilitation benefits it felt it owed to the maximum of \$100,000 for a non-catastrophically impaired insured. It did so on the basis that if Ms. Monks wanted to do the renovations to her house she could do so. As a result, it paid Ms. Monks \$50,786.88.

[969] Even though ING had intended this money to go towards the home modifications, as the Court heard at trial, Ms. Monks did not proceed with those modifications. Firstly, the ING letter makes reference to the Puddicombe May 2001 report in which he gave a very basic estimate to modify the home of \$146,911. As Mr. Puddicombe testified and his subsequent report of April 2003 demonstrated, that amount was not nearly accurate for what is truly required to properly modify this home for Ms. Monks' needs. Secondly, given the position being taken by ING that either the 1998 accident did not materially contribute to her catastrophic condition and/or that even if it did, ING would not have to pay any benefits until her Zurich benefits are exhausted, Ms. Monks was not in a position to proceed with home modifications until her situation was

clarified. If either of ING's arguments were to be accepted, she faced the prospect of not having enough money to cover her on-going needs. If she had spent the funds she had on the modifications Mr. Puddicombe had recommended, she would be well short of funds required to take care of her on-going medical, rehabilitation and attendant care needs.

[970] However, because those dollars are equally applicable to any medical/rehabilitation expenses, and because Ms. Monks did receive the funds and because she did in fact use them for that purpose, the best method for the Court to take them into account would be to credit them to the accumulated medical/rehabilitation expenses incurred to date.

[971] This argument in fact benefits ING. If we were to take the position that the \$50,786.88 should simply be set aside for the eventual home modifications, then arguably ING would then be responsible for interest at 2% per month, compounded monthly for any overdue medical/rehabilitation expenses (as per s. 46 (2) of SABS-96).

[972] If this method of accounting for the medical/rehabilitation benefits to date is accepted by the Court, then there remains \$4,068.84 of the \$50,786.88 received from ING to be credited to any medical/rehabilitation benefits incurred after January 1, 2005.

Attendant Care expenses to date

[973] Again, these are summarized in Ex. 27 and 8. Tab 1H summarizes the time of John Perrozzino. As the evidence was heard at trial, it is not precise but again is probably less than the actual time he spent. It was based on 92 hours per week and was for the time when the hired caregivers were not present. It accounted for his time at work and also time with their sons. It does not take into account the time they were on holidays when his time taking care of her needs was greatly increased.

[974] In all of the circumstances, it is a fair estimate of his time. If the evidence of Ms. Monks that insurers paid family members \$10 an hour as a flat rate for the care provided by them, then it is a fair rate to be paid to Mr. Perrozzino given that the tasks he performs fall into all of the three categories in Form 1.

[975] The schedule at Tab 1-I reflects the actual money spent by Ms. Monks for attendant care between May 31, 2002 and January 2005. Again, it would appear that the amounts paid by Ms. Monks were more than fair given her needs during that time frame. And as can be seen from the spreadsheet at Tab 1-I and as Ms. Monks testified at trial, there are lengthy periods during which Ms. Monks has not had the attendant care she requires because she has not been able to afford it (this goes back to the problem that with the uncertainty of this litigation, Ms. Monks has not been able to spend the dollars she has from the Zurich settlement in the manner Ms. Orth suggests because she does not know what she will have at the end of the day until this litigation is resolved).

[976] The total of the two amounts is shown on the spreadsheet at the beginning of Tab 1 and is \$189,075.64. As suggested above, the Zurich contribution based on the \$300,000 portion of the structured settlement has generated \$48,094.62 toward attendant care costs. Therefore, the net amount owing is \$140,981.02.

[977] It is important to note that the monthly maximum payable by ING for attendant care under SABS-96 is \$6,000 pursuant to s. 16(5). However, given that the Zurich settlement contribution ranged from \$1,538.50 to \$1,569.42 during the period from May 2002 to January 2005, the actual monthly amount being claimed never exceeds \$7,538.50 to \$7,569.42 during that period. In other words, the amount being claimed from ING never reached \$6,000 in any one given month.

Visitor's Expenses

[978] Ms. Monks claims \$1,048.14 for visitor's expenses which were withheld by ING.

Housekeeping

[979] As indicated above, while housekeeping expenses have been incurred to date, no claim is being advanced at this time because the Zurich settlement funds from the structured settlement have completely covered that cost to date.

Interest

[980] As indicated above, s. 46(2) of SABS-96 provides:

(2) If payment of a benefit under this Regulation is overdue, the insurer shall pay interest on the overdue amount for each day the amount is overdue from the date the amount became overdue at the rate of 2 per cent per month compounded monthly

[981] As can be seen this provision is mandatory (“the insurer *shall* pay....”). Once the Court calculates the amounts owing to date, then we will assist in proving the appropriate interest calculation.

Current Benefits

[982] At the outset of the trial, the Court made the ruling that if entitlement to benefits was established by Ms. Monks in these proceedings, then an order should also go as to what that entitlement is at this time on an on-going basis. Of course, if the circumstances change following this trial, then the entitlement and/or quantum of the benefits will also change. At that time, the parties will turn to the process provided for in the regulations for such changes. But Ms. Monks after such a lengthy trial, which canvassed all of her conditions and needs, is entitled to an order now fixing what her current entitlements are.

[983] The following is the submission of the plaintiff for the on-going benefits:

Income Replacement Benefits

[984] At the outset of trial, it was agreed between the parties that ING should continue to pay \$340.96 to Ms. Monks for income replacement benefits and the Judgment should so provide. ING has brought Ms. Monks up to date in regard to these benefits but the order should reflect the on-going entitlement.

Medical/Rehabilitation

[985] It is the submission of the plaintiff that she is entitled on an on-going basis to the medical/rehabilitation benefits as set out in Ex. 8, Tab 4D of Sandra Guest-Poulin’s report.

[986] As indicated during the trial, Ms. Monks is not seeking an order from the Court that ING should pay the amounts, which are set out in that schedule. Rather, that schedule sets out her various needs for physical therapy, occupational therapy, nursing at the cottage, psychological counselling, parking and transportation to and from therapies and visits to health care providers, medications, bowel care supplies, bladder care supplies, Fentanyl administration, porth-a-cath administration, respiratory management, nutritional supplements, other needs, mobility equipment, vehicle and modifications and aids for independent living. It is the submission of the plaintiff that given the law covered in the section of these submissions under the heading of “*Directly Caused*”, all of the conditions described in Ms. Guest-Poulin’s reports and the on-going needs and expenses covered in the schedules flow from the accident and are the responsibility of ING to cover.

[987] It is submitted that an order requiring ING to pay all medical and rehabilitation expenses on an on-going basis for all of the needs summarized in the reports of Sandra Guest-Poulin and the accompanying schedules would suffice. Of course, ING is entitled to the benefit of any collateral benefits available to Ms. Monks to pay for these expenses and Ms. Monks must contribute an amount equal to the amount paid each month from the Zurich structured settlement from the \$150,000 portion of the structured settlement as provided in Exhibit 14.

Attendant Care

[988] In regard to attendant care, it was the evidence of Dr. Vandewaetere that Ms. Monks requires 24-hour attendant care. It was also the evidence of Sandra Guest-Poulin that she currently requires it and has required it since the beginning of 2003.

[989] Indeed, it was even the evidence of Fiona Smith-Bradley that Ms. Monks requires 24-hour attendant care. She however, felt that the real cause of the need was the narcolepsy-type condition. While it was not her place to say and she has no expertise in that regard, she attempted to suggest that this condition does not relate to the accident.

[990] For the reasons stated above in the section of these submissions entitled “*Directly Caused*”, it is the submission of the plaintiff that all of the conditions she currently has flow

directly from the accident, including the sleep disorders. Furthermore, it is not just the narcolepsy condition alone, which is cause for concern; it is a person with quadriparesis who also has narcolepsy who is so at risk.

[991] As a result, the only evidence before the court was that at this time Ms. Monks requires 24-hour attendant care and that she has since the early part of 2003.

[992] In order to establish what ING should pay, it is submitted that it should be ordered that a new Form 1 be prepared on the basis of 24-hour attendant care breaking down the hours into the three levels of care on the Form 1. Then the monthly amount should be established. Of course, the monthly amount generated by the Zurich settlement should be taken into account and ING would be ordered to pay the difference.

Expenses up to Judgment

[993] Of course, the claims being advanced to date at trial were only up to the end of 2004. There will of course have been expenses incurred since that time up to the time of the Court's decision. It is submitted that an order should go requiring ING to pay those expenses which have been incurred by Ms. Monks since the beginning of 2005 up to the date of judgment for all of the medical/rehabilitation needs summarized in the reports of Sandra Guest-Poulin and the accompanying schedules and for attendant care. ING should be able to deduct any collateral benefits and the amounts generated by the Zurich settlement toward those expenses. Interest pursuant to s. 46(2) of SABS-96 should be payable on those amounts as well.

Home Modifications

[994] In regard to home modifications, SABS-96, s. 15(5)(i) provides that the insurer shall pay all reasonable and necessary expenses for:

“home modifications and home devices, including communications aids, to accommodate the needs of the insured person, or the purchase of a new home if it is more reasonable to purchase a new home to accommodate the needs of the insured person than to renovate the insured person's existing home;

And section 15(8) provides:

“The amount of the rehabilitation benefit for the purchase of a new home shall not exceed the value of the renovations to the insured person’s existing home that would have been required to accommodate the needs of the insured person.”

[995] Ms. Monks has not yet determined what she proposes to do. It is clear from the evidence that her current house creates numerous difficulties for her and in fact, is dangerous for her. No one at trial disputed that an insured has the right to make the decision whether to renovate his/her existing home or to purchase and modify a new home.

[996] However, in order for her to be able to decide what to do, she needs to know what the cost to renovate her existing home is because that represents the maximum that can be spent either doing that or purchasing a new home and renovating it (as per s. 15(8)).

[997] It is submitted by the plaintiff that the only solid and credible evidence as to the cost to modify Ms. Monks’ home came from Scott Puddicombe. Mr. Borthwick provided a couple of reports and came and testified but he has never seen Ms. Monks’ house and never met her. He seemed to accept the estimate Mr. Puddicombe prepared in May 2001 as being a realistic figure to modify the home. But Mr. Puddicombe testified that that report was nothing more than a sketchy comparison of the costs to renovate on two scenarios – one with Ms. Monks using a walker and one with her using a wheelchair.

[998] On his own testimony, he felt that this May 2001 report was very unreliable. He had not done any measurements, he had not taken photos, he had not checked the infrastructure of the house to see how solutions could be implemented and he had not met with Ms. Monks to discuss how the house was used. He never intended it to be an accurate representation of the cost to properly modify the home. The report says this in its body and Mr. Puddicombe testified to this effect. On Mr. Puddicombe’s own evidence the May 2001 report was really of little value.

[999] It is not surprising that Mr. Borthwick was prepared to agree with that analysis because it is the lowest possible estimate and indeed he came up with one of his own which was similar in nature and cost. But frankly for all of the same reasons Mr. Puddicombe gave for why his May 2001 analysis was not of much value, Mr. Borthwick’s was equally of little value.

Furthermore, Mr. Borthwick's criticisms of Mr. Puddicombe's April 2003 report were more questions, which were in fact answered by Mr. Puddicombe during his testimony.

[1000] In the end, the best evidence before the court as to what is necessary to modify the Monks' residence and what it would cost was that of Scott Puddicombe. It is summarized in the report of April 2003, which was up-dated in December 2004. He did all of what is normally expected to establish the best method to modify the residence to meet her needs and to take into account how the family uses the house. It is also the best evidence of the cost and as of December 2004, it was \$362,206.00 (Ex. 9, Tab 3).

[1001] It was evident from his cross-examination that Mr. Borthwick was not in a position to challenge this evidence. He had not seen the house, he had not done any measurements, he did not know what the infrastructure of the house is and what complications it and the lot present for any modifications, he had not met Ms. Monks or the family members to establish how they use the house. A review of his report critiquing the Puddicombe reports discloses more questions on his part than anything else and Mr. Puddicombe answered them during the trial. In short, there was no evidence led by the defence of any substance to challenge the concepts and cost, recommended by Mr. Puddicombe, and it is submitted by the plaintiff that it should be accepted by the Court.

[1002] There was some concern expressed about the swimming pool contained in the house. It is to be remembered that it is not a swimming pool as a layperson might understand it. It is a smaller therapy pool, which provides resistance against which Ms. Monks could obtain exercise. Dr. Vandwaetere testified that this would be one of the best ways for Ms. Monks to improve her strength, endurance and weight. As long as she had the appropriate attendant care, it is submitted that this would be an appropriate addition to the home.

[1003] There was also a great deal of evidence about it being more ideal for Ms. Monks to be in a bungalow. Ms. Monks testified that she would be open to that solution as well. She just needs to know what the upper limit of what she can spend is before making the decision. There was clearly a significant difference between the evidence of Mr. Borthwick as opposed to

the evidence of Mr. Puddicombe as to how easy and how costly such a move would be, whether buying an existing bungalow, or having one built by an established home builder or a custom home builder. The evidence at trial was nothing more than speculation. Ms. Monks, herself though testified that she would be motivated to seek the most economic solution. The less she spends on home modifications, the more of the medical/rehabilitation limits of \$1,000,000 would be left for her on-going monthly needs.

[1004] Accordingly, the plaintiff submits that an order should go requiring ING to pay up to \$287,206 (\$362,206 less \$75,000 from the Zurich settlement) for home modifications either to her existing home or to go towards the cost of purchasing and modifying a new home.

Housekeeping and Home Maintenance Expenses

[1005] Section 22 of SABS-96 provides as follows:

“22. (1) The insurer shall pay for reasonable and necessary additional expenses incurred by or on behalf of an insured person as a result of an accident for housekeeping and home maintenance services if, as a result of the accident, the insured person sustains an impairment that results in a substantial inability to perform the housekeeping and home maintenance services that he or she normally performed before the accident.

(2) The amount payable under this section shall not exceed \$100 per week.”

[1006] While the housekeeping expenses are currently being met by the amounts generated by the Zurich settlement, they could fall short soon. As a result, an order should go requiring ING to pay on-going housekeeping expenses net of the amounts generated on a monthly basis by the Zurich settlement, up to a maximum amount of \$100 per week. Currently, ING would not have to pay because of the Zurich amounts but as soon as the costs exceed the funds generated by the structured settlement, Ms. Monks should not have to come back to this Court or turn to an arbitrator to obtain ING’s contribution. The need is well established and the only thing preventing ING from having to pay at this time is the fortuitous presence of the structured settlement amounts.

Case Manager

[1007] And finally, s. 17 of *SABS-96* provides that in the case of a person who sustains a catastrophic impairment, the insurer shall pay all reasonable and necessary expenses incurred by or on behalf of an insured person as a result of the accident for services provided by a qualified case manager. Ms. Monks testified that she would benefit from the assistance of a case manager and there was no evidence led to the contrary. Accordingly, an order should go requiring ING to provide and pay for a case manager.

Defendant's submissions: Entitlement to Benefits

[1008] In order to be entitled to Statutory Accident Benefits under Bill 59 and specifically medical/rehabilitation benefits and attendant care benefits, there are pre-requisites, which must be met in order to qualify for:

Entitlement

- The impairment must be as a result of an accident

s. 2 defines *accident* as “accident means an incident in which the use or operation of an automobile *directly* causes and impairment or *directly* causes damage to any prescription eyewear, dentures, hearing aid, prosthesis or other medical or dental device”. The previous definition under Bill 164, which covered the 1995 accident, said “*directly or indirectly*”. This change in Bill 59 reflects a significant narrowing of the scope of the coverage.

- The expense must be *incurred*; and
- The expense must be *reasonable* and *necessary*.

Quantification

- Medical/Rehabilitation Expenses – there are a number of provisions set out in s. 14, which will be referred to, as required:
- Attendant Care – s. 16(4) stipulates that the monthly amount payable by the attendant care benefit shall be determined in accordance with Form 1 – pursuant to s. 16(5)(b), the maximum amount payable to a person who sustained a catastrophic impairment, as a result of the accident, is \$6,000.

- In respect of both Medical/Rehabilitation and Attendant Care s. 60(2) applies, which relates to the deductibility of collateral benefits.
- Medical/Rehabilitation – if the person is catastrophic the maximum payment is \$1 million.
- Attendant Care – if the person is catastrophic, the maximum payment is \$1 million.

[1009] The plaintiff commenced an action against ING on May 3, 2002. It is noteworthy that, at the time the claim was issued, ING had not stopped paying benefits to the plaintiff and it is very telling that the claim was commenced within 4 days of settling the Zurich action.

[1010] Concerning the Zurich settlement, the plaintiff arbitrarily decided to designate a certain amount for each benefit. Notwithstanding that Bill 164 offered the plaintiff a larger pool of money to meet her needs (than what Bill 59 does), she chose to accept amounts, which do not accurately reflect what she was being advised were required to meet her future needs. ING should not be bound by the plaintiff's actions and submits that she improvidently settled her claim to the detriment of ING.

[1011] In terms of quantum, we take the position that s. 60(2) exonerates ING from paying any benefit, until such time as the amount that is reasonably available through other sources, i.e., the Zurich settlement, is exhausted. We note specifically that ING is entitled to a deduction of amounts that are reasonably available; the statute does not say that they have to be received. ING advances the following three scenarios:

(1) Under Bill 164, the plaintiff would have been entitled to amounts that were significantly higher than Bill 59 (i.e., Medical and Rehabilitation Benefits (\$1,000,000) and Attendant Care Benefits (no statutory maximum limit)). In our submission, those amounts were reasonably available to the plaintiff. Yet, the plaintiff, in her settlement with Zurich, chose to accept lower sums (i.e., Medical Benefits (\$190,000) and Attendant Care Benefits (\$300,000)). Given that the plaintiff has improvidently settled with Zurich, ING has been prejudiced by same. This fact should not be visited upon ING and, as such, no amounts are due and owing to the plaintiff; or

(2) Alternatively, if the Courts find that ING must accept the consequences of the plaintiff's actions in designating, from the Zurich settlement, the Medical Benefits and Attendant Care Benefits for only \$190,000 and \$300,000 respectively, it is our submission that ING is entitled to the full benefit of this amount and any accruing interest. As a result, ING has no obligation towards the plaintiff until such time as she has spent the value of the full benefit (i.e., \$190,000 and \$300,000);

(3) The amounts claimed by the plaintiff for the Medical and Rehabilitation expenses are inaccurate, both in fact and in law. The plaintiff has improperly interpreted the SABS and the amounts claimed are, in reality, significantly lower. With regards to the Attendant Care Benefits, the amounts claimed are not reasonable, given that the quantification provided was not done in accordance with Form 1, which is contrary to the SABS.

[1012] In our analysis below, ING's position is that the past expenses incurred are significantly lower than the amounts claimed by the plaintiff.

[1013] In the event that the Court is inclined to make an Order in favour of the plaintiff, we have prepared, for your ease of reference, a summary of ING's position regarding the amounts that are due and owing under the three scenarios described above (Addendum C).

Addendum C

ENTITLEMENT

Medical, Rehabilitation & Attendant Care Expenses ***Excluding home modifications**

<u>Description</u>	<u>Scenario 1</u> Available Amounts Under Bill 164	<u>Scenario 2</u> Full value of the Benefits (Zurich Settlement)	<u>Scenario 3</u> Quantification
Medical and Rehabilitation Expenses	\$ 10,936.55 \$ -1,000,000.00 (\$ 989,063.50) ¹	\$ 10,936.55 \$ -190,000.00 (\$ 179,063.45) ²	\$ 10,936.55 \$ -24,195.44 (\$ 13,258.89) ³
Attendant Care	Unlimited ⁴	\$ 59,713.36 \$ -300,000.00 (\$ 240,286.64) ⁵	\$ 59,713.36 \$ 48,094.02 \$ 11,619.34 ⁶

¹ The plaintiff had \$1,000,000 reasonably available to her, under Bill 164, yet chose to accept the sum of \$190,000 in the Zurich settlement. The actual past expenses of \$10,936.55 to be paid from this available resource, leaving a balance to be used in the amount of \$989,063.50 for future medical and rehabilitation expenses.

² ING is entitled to deduct the full value of the benefit, received by the plaintiff, in the Zurich settlement (\$190,000). The actual past expenses of \$10,936.55 to be paid from this available resource, leaving a balance to be used in the amount of \$179,063.45 for future medical and rehabilitation expenses.

³ ING is entitled to deduct the sums generated from the Zurich settlement (\$24,195.44). The actual past expenses of \$10,936.55 to be paid from this available resource, leaving a balance to be used in the amount of \$13,258.89 for future medical and rehabilitation expenses.

⁴ The plaintiff had an unlimited amount reasonably available to her under Bill 164, yet chose to accept the sum of \$300,000 in the Zurich settlement. As such, no monies are due and owing for past or future attendant care expenses, given that this unlimited resource was available.

⁵ ING is entitled to deduct the full value of the benefit, received by the plaintiff in the Zurich settlement (\$300,000). The actual past expenses of \$59,713.36 to be paid from this available resource, leaving a balance to be used in the amount of \$240,286.64 for future medical and rehabilitation expenses.

⁶ ING is entitled to deduct the sums generated from the Zurich settlement (\$48,094.02). The actual past expenses of \$59,713.36 to be paid from this available resource, leaving a balance to be paid in the amount of \$11,619.34. Future medical and rehabilitation expenses are to be submitted in accordance with Form 1.

Medical and Rehabilitation Expenses (not including Home Renovations)

[1014] ING submits that, pursuant to section 60(2) of the SABS, payment of a medical or rehabilitation expense is not required, if an amount is reasonably available to the plaintiff. The following is a summary of three scenarios advanced by ING:

(1) Under Bill 164, the maximum limit on supplementary medical benefits and rehabilitation benefits was \$1,000,000. Section 46(1) Ont. Reg. 776/93 – Statutory Accident Benefits Schedule – Accidents after December 31, 1993 and before November 1, 1996, reads as follows: “The total of all benefits paid under Parts VII and VIII in respect of an insurer person shall not exceed \$1,000,000 in respect of any one accident”. In Bill 164, Parts VII and VIII referred to Supplementary Medical Benefits and Rehabilitation Benefits, respectfully. That amount was reasonably available to the plaintiff, yet she chose to accept a sum of

\$190,000 from Zurich for Supplementary Medical Benefits. Given that the plaintiff has improvidently settled with Zurich, ING has been prejudiced. We submit that this should not be visited upon ING. As such, ING has no obligation to the plaintiff under this benefit, until such time as she has spent the entire \$1,000,000; or

(2) Alternatively, if the Court finds that ING must accept the consequences of the plaintiff's action in settling with Zurich and allocating \$190,000 to Supplementary Medical Benefits, it is our submission that ING is entitled to the full benefit of this amount (see the *Orchover* decision). No sums are due and owing to the plaintiff, until such time as these funds have been exhausted. That amount was reasonably available to the plaintiff;

(3) In the further alternative, the amount claimed by the plaintiff for medical and rehabilitation expenses is inaccurate, in that it does not comply with the statutory regulations, as outlined in the SABS. It is submitted that ING's interpretation and calculation of these expenses should be accepted.

[1015] The plaintiff argues that the sum of \$68,815.87 is owed under the past medical and rehabilitation expenses. We submit that the plaintiff has improperly calculated the amounts that may be due and owing. ING submits that the proper amount owed, if the 1998 accident materially contributed to the plaintiff's condition, is \$10,936.55. For the purposes of simplicity, in reviewing whether any amounts are due and owing under this heading, we will be using the same categories, provided by the plaintiff, in Exhibit 27, Tab 1.

(1) Drugs-supplies

[1016] ING has undertaken a careful review of all receipts provided in Exhibits 27 and 53, under this category. Surprisingly, there are duplications found in both Exhibits and we submit that, it may not be reliable. Notwithstanding, it is ING's position that many of these receipts are not, and should not, be ING's responsibility, in that it is not related whatsoever to the 1998 accident, as ING argued in the previous section dealing with directly related to the accident. This, of course, would be a moot point, if the Court finds that the 1998 accident did not materially contribute to the plaintiff's condition.

[1017] We have prepared a summary of the various receipts submitted by the plaintiff, providing the name of the prescription medication, the purpose, the dates and amounts (Addendum D). It is our submission that the medical evidence, (as detailed in the Directly related to the accident section), supports ING's position that the following prescription medications are not related to her cervical spondylosis myelopathy:

Alertec Modafinil, Apo Furosemide, Apo Prochlorazine, Apo Warfarin Sodium, Asacol, Bricanyl Turbuhaler, Diovan Valsartan, Duragesic Fentanyl, Flovent Fluticasone, Gen Doxazosin, Gen Serataline, Lipitor Atorvastatin, Matrixextomy, Micro K Potassium Chloride, Midamor Amiloride, Norvasc Amlodipine Besylate, Pantoloc Pantoprazole, Polytrim, Sodium Phosphate, Stemetil Prochlorperazine, Taro-Warfarin, Wellbutrin and Xenical Orlistat.

[1018] Addendum "D" provides the Court with the purpose of the above-noted prescriptions. It is our submission that these are not related to the 1998 accident, and as such, the amount claimed by the plaintiff should be reduced by \$3,928.49, which represents the sums associated with this medication, as outlined in Addendum D.

(2) Miscellaneous Expenses

[1019] The plaintiff provided a summary of the expenses incurred under this heading, totalling the sum of \$22,749 (Exhibit 27, Tab 1-B). The plaintiff also submitted additional miscellaneous expenses on the last day of trial (Exhibit 53) for a total amount of \$1,836.10.

[1020] It is our submission that most of the receipts submitted are not items that are covered in an accident benefits claim. During the trial, the plaintiff alluded to a bag full of receipts that she had not had a chance to review. The plaintiff, in the Zurich action, had only submitted expenses, which totalled no more than \$10,000 worth. Today, she is attempting to convince the Court that these are legitimate expenses that have been incurred as a result of the accident. In determining whether an expense should be paid, the test provides that the expense must be deemed to be reasonable and necessary. The insurer will indemnify an out-of-pocket expense that is reasonable and necessary in relation to a treatment. Can it be said that a pair of

pyjamas is reasonable and necessary? She would have purchased pyjamas had the accident not occurred. What about a business expense (i.e., claiming furniture for her business)? Surely, this does not fall within a payable expense that is in relation to her treatment. The plaintiff is simply trying to recreate it after the fact.

[1021] Rather than taking the Court through the minutia of each and every expense, we propose that a review of certain items will demonstrate that most of these expenses were either (a) paid out of the Zurich action settlement; (b) are not directly related to the 1998 accident or, (c) that they are not reasonable and necessary:

- Schwinn Windjammer (\$2,746.20) – Exhibit 27, Tab 1-B (receipt 34A) was paid by Zurich;
- Life Cycle Recumbant (\$3,401.65) – Exhibit 27, Tab 1-B (receipt 35A) was paid by Zurich;
- Revised Accessibility Cost Report (\$5,350.00) – Exhibit 27, Tab 1-B (receipt 43A) was paid by Zurich;
- Panasonic Intercom (\$815.31) – Exhibit 27, Tab 1-B (receipt 51C) was paid by Zurich;
- Air Purifier (\$574.97) – Exhibit 27, Tab 10B (receipt 52C) was paid by Zurich;
- Bi Pap (\$775.00) – Exhibit 27, Tab 1-B (receipt 25A) is not directly related to the 1998 accident;
- Bi Pap Filter (\$37.43) – Exhibit 27, Tab 1-B (receipt 26A) is not directly related to the 1998 accident;
- Drisdan/Claritan/Lactaid (\$57.45) – Exhibit 27, Tab 1-B (receipt 23A) is not reasonable and necessary;

- Listerine/Stress Tablets (\$26.20) – Exhibit 27, Tab 1-B (receipt 46A) is not reasonable and necessary;
- Drisdan (\$25.84) – Exhibit 27, Tab 1-B (receipt 12B) is not reasonable and necessary;
- Contact C (\$12.64) – Exhibit 27, Tab 1-B (receipt 13B) is not reasonable and necessary;
- Prunes Stewed (\$8.72) – Exhibit 27, Tab 1-B (receipt 31C) is not reasonable and necessary;
- Lysol (\$29.19) – Exhibit 27, Tab 1-B (receipt 36C) is not reasonable and necessary;
- Automatic Eyeglass Cleaner (\$82.79) – Exhibit 27, Tab 1- (receipt 53C) is not reasonable and necessary.

[1022] These are but a few examples that demonstrate the unreasonableness in the plaintiff's request to be compensated for these items. The above-noted expenses total approximately \$14,000, and we submit that most, if not all items listed, do not fall within the requirements of items that are covered under the SABS. As such, ING submits that the plaintiff has failed to demonstrate that these expenses are properly incurred in accordance with the SABS. Should the Court be inclined to order that the plaintiff should be reimbursed under this heading, it is our submission that it should be substantially discounted at 90%, leaving a sum of \$2,458.50 due and owing $((\$22,358.84 + \$1,836.10) \times 90\%)$.

(3) Transportation

[1023] The plaintiff testified that ING agreed to pay the sum of \$75 for transportation. This agreement was precipitated on the understanding that the plaintiff was previously using an outside provider, and a disagreement occurred. The agreement had provided that ING agreed to pay \$75 to the CCAC nurse, who had offered to drive the plaintiff to the various medical

appointments. However, the claim, now being advanced, is for her husband and/or her attendant to be paid to provide this service. This is not something which ING agreed to. This would amount to double indemnity, as a claim has been advanced for attendant care for both of these people, which includes the time and the services being provided for these trips. Further, any compensation for driving her to appointments should be paid on the basis of \$7 per hour, which is the amount mandated by the statute, according to the Form 1.

[1024] When the attendant, who was driving the plaintiff to the medical appointments, was in fact already being compensated under the attendant care benefits, the care provided and paid for, included driving the plaintiff. As such, it is our contention that the plaintiff's request for payment in the amount of \$9,525 (Exhibit 27, Tab 1-C, page 9-12) has been fully reimbursed and no monies are due and owing. It is our submission that she would be doubly indemnified, if she is compensated for this amount, however, if the Court is inclined to make an award, we would submit that, at best, she is entitled to payment of \$14 per trip, recognizing that it may take up to one hour to drive her each way.

[1025] In the event that the Court makes an award for reimbursement of transportation costs, ING submits that the amount due and owing is \$532.50, representing the parking costs (Exhibit 27, Tab 1-C, page 9).

(4) Lease of Vehicle

[1026] The plaintiff submits that the Court should order payment in the amount of \$22,576.74, which is the costs of leasing a vehicle, namely, a Ford Windstar from May 2002 until February 2005, which is calculated at the monthly rate of \$664.11.

[1027] Section 14(2) of the SABS provides that the medical benefit shall pay for all reasonable and necessary expenses incurred as a result of the accident. The evidence that was heard at trial, revealed that, prior to the accident, the plaintiff was driving a Ford Expedition, which she paid a lease payment in excess of that of the Windstar. The plaintiff wishes the Court to find that, had it not been for the accident, she would have used one of the other two vehicles

that was at home (other than the Ford Expedition). This is simply not true. The plaintiff did not lease the Ford Windstar, nor incur an expense, as a result of the accident. This expense existed prior to the accident.

[1028] The following is an excerpt of the evidence of Mr. Perrozzino during his cross-examination, which demonstrates that the plaintiff was not driving the two other vehicles:

Q. When we left off, we were talking about the different benefits that were being claimed and I had asked you about the 1998 Expedition?

A. Yes.

Q. What was the value?

A. \$40,000 - \$44,000.

Q. At the time, you also had 2 other vehicles?

A. Yes, a Jeep and a Chrysler Lebaron.

Q. Did she use these two vehicles?

A. She used the Expedition, not the other cars.

Q. How much were the lease payments for the Expedition?

A. Somewhere around \$600, it was a two-year lease.

Q. With respect to the Van, we know the amount?

A. This is a three-year lease. Following the Expedition, we got a 2000 Windstar. They all range in the \$600 region, a two-year lease.

Q. The current Windstar, is the amount that you have set out in your brief, a three-year lease?

A. Yes, 2002 and it ended.

Q. The 2000 Windstar, would it be fair to say that you would have got in December 1999 or 2000?

A. December of 1999.

Q. It's a four-year lease to bring it in 2004?

A. Had it for two years then got the 2002.

Q. Do you still have the Lebaron and Jeep?

A. I have the Lebaron and a Honda – got rid of the Jeep.

[1029] It is our submission that, in accordance with the SABS, ING is only obligated to pay the differential between the expenses that the plaintiff had prior to the accident and the increase in the expense, subsequent to the accident. As stated above in Mr. Perrozzino's testimony, the costs of the Expedition and of the Windstar were roughly the same. It is submitted that the plaintiff has not incurred a loss and is, therefore, not entitled to any payment, regarding the leasing of the Windstar.

(5) Unpaid expenses submitted prior to September 2002

[1030] Under this category, ING concedes that the Ergonomic Rehabilitation expense in the amount of \$1,795 (Exhibit 27, Tab E) is an expense that she is entitled to claim.

[1031] With respect to the out-of-pocket expense claimed, dated May 19, 2002 (Exhibit 51, Tab 2), in the amount of \$1,409.23, it is ING's submission that the expenses listed below are not proper:

- Transportation (\$75 X 12) for the same reasons outlined in transportation section above;
- Xenical prescription medication (\$98.73 X 2) is for obesity management, which is not related to the 1998 accident;

[1032] Accordingly, we submit that the amount of \$1,409.23 should be reduced by \$1,097.46.

[1033] With regards to the out-of-pocket expense claimed, dated June 24, 2002 (Exhibit 51, Tab 3), in the amount of \$794.32, it is ING's submission that the following expenses are not proper:

- Mouth infection products (\$69.51);
- Gaviscon (\$13.79); and
- Transportation (\$75 X 5).

[1034] Accordingly, we submit that the amount of \$794.32 should be reduced by \$458.30.

(6) Summary of ING's position regarding past medical and rehabilitation expenses

[1035] As detailed above, it is ING's submission that the proper quantification of the medical and rehabilitation expenses is as follows:

Description	Claimed by the Plaintiff	Less (as per ING's Submissions)	Total
Drug/Supplies	\$ 9,336.39	\$ 3,928.49	\$ 5,407.90
Misc. Expenses	\$24,585.10	\$22,126.60	\$ 2,458.59
Transportation	\$10,057.50	\$ 9,525.00	\$ 532.50
Lease Vehicle	\$22,579.76	\$22,579.76	\$ 0.00
Unpaid expenses	\$ 4,093.21	\$ 1,555.76	\$ 2,537.65
Total			\$10,936.55

[1036] In summary, it is ING's respectful submission that, with respect to past medical and rehabilitation expenses, the Court has three options:

- (1) Given that the plaintiff had a \$1,000,000 available to her under Bill 164, yet she chose to accept the sum of \$190,000 in the Zurich settlement, ING should not be prejudiced by her actions. Therefore, the \$1,000,000 was reasonably available to her and, as such, the past medical and rehabilitation expense, in the amount of \$10,936.55 (as noted above), is hereby satisfied; or

- (2) The plaintiff has earmarked \$190,000 from the Zurich settlement and regardless of the amount structured for future medical and rehabilitation benefits, ING should not be penalized and is entitled to the full value of the benefit. We submit that ING should have an order to go that it be entitled to deduct \$190,000 and that it is appropriate. Accordingly, it is our position that the past medical and rehabilitation expense, in the amount of \$10,936.55, is satisfied; or
- (3) As demonstrated above, the quantification for past medical and rehabilitation expenses is inaccurate. In ING's submission, the proper quantification amount for past medical and rehabilitation expenses is \$10,936.55. Given that the plaintiff has received the sum of \$24,195.44 (sums generated from the Zurich settlement), we submit that the past medical and rehabilitation expenses is satisfied, with a remaining credit of \$13,258.89, which can be applied towards future medical and rehabilitation expenses.

[1037] Finally, in the event that the Court finds that the 1998 accident materially contributed to the plaintiff's condition and that you make a declaration that she is entitled to certain benefits, it is our submission that all other claims, other than those raised in the pleadings, and/or the plaintiff's written submissions, should be subject to the normal process. In other words, once an expense has been incurred, it will be submitted to ING, at which time it will investigate and make a determination on whether it is reasonable and necessary.

Medical and Rehabilitation Expenses (future)

[1038] Following the same logic as above, it is our respectful submission that, with respect to future medical and rehabilitation expense benefits, the Court has three options:

- (1) The plaintiff had \$1,000,000 reasonably available to her under Bill 164, yet chose to accept the sum of \$190,000 in the Zurich settlement. ING should not be prejudiced by her actions and, accordingly, we submit that no order shall be granted for future attendant care benefits expenses, until such time as the plaintiff has spent \$989,063.50, representing the \$1,000,000 available, less payment in the amount of \$10,936.55 for past medical and rehabilitation expenses; or
- (2) The plaintiff is entitled to deduct the full value of the benefit received by the plaintiff in the Zurich settlement (i.e. \$190,000). As such, it is our

position that no amounts, with respect to future medical and rehabilitation expenses, are due and owing, until \$179,063.45 has been exhausted (i.e. full value of the benefit (\$190,000), less payment of the past medical and rehabilitation expenses (\$10,936.55)); or

- (3) ING has a remaining credit in the amount of \$13,258.89 (i.e., \$24,195.44 (sums generated from the Zurich settlement), less \$10,936.55 (past medical and rehabilitation expenses)). It is ING's position that it is not obligated to pay any future medical and rehabilitation expenses, until such time as the plaintiff has spent the sum of \$13,258.89.

Medical and Rehabilitation Expenses (Home Modifications)

[1039] It is ING's position that there is no evidence to substantiate the plaintiff's claim that \$362,206 is a reliable estimate for the plaintiff's home modifications. Rather, the evidence has demonstrated that an amount between \$180,000 and \$220,000 is most reasonable in the circumstances, and, we submit, should be accepted.

[1040] During the trial, the Court made a comment that, in your view, the May 29, 2001 report was not an accessibility report. It is our submission that, in carefully reviewing all reports, the December 9, 2004 report is not that much different than the May 29, 2001 report, in that it has no architectural drawings or specifications, no measurements whatsoever, has the same disclaimers, and they are all titled "Accessibility Costs Report" (Exhibit 9).

[1041] It is quite surprising that Mr. Cronyn, in his submissions, stated that the only solid and credible evidence, with respect to the costs of modifying the plaintiff's home, is that of Mr. Puddicombe. Mr. Cronyn may have forgotten that, during cross-examination, Mr. Puddicombe candidly admitted that the reports are to be deemed as proposals. He further admitted that the only way of getting a fixed cost was to have complete drawings, approved by the plaintiff, the Occupational Therapist, and the City. Although Mr. Cronyn wishes the Court to accept the last report, which, incidentally, is the most expensive, there are no architectural drawings or specifications that were developed to assist the Court in concluding that the pricing is accurate. By his own expert's admission, these cannot be relied upon to determine the fixed costs. One

must remember that the plaintiff has the onus of proving that these costs are accurate, which, we submit, she has failed to do.

[1042] It is our submission that, given the uncertainty of Mr. Puddicombe's reports and, more specifically, the last report rendered in December 2004, Mr. Borthwick's critique of Mr. Puddicombe's reports provide some useful insight. It is also worth noting that there is no notable difference between the second report (April 8, 2003) and the third report (December 9, 2004), other than the price, of course, being approximately \$77,000.

[1043] Mr. Borthwick is a very seasoned cost estimator (i.e. estimated thousands of jobs), who has completed between 1500 to 2000 accessibility reports. We submit that, based on his vast experience, Mr. Borthwick offered credible and dependable evidence on the cost estimate of the proposed home modifications. We submit that, despite not visiting the home, he was able, through the pictures and other documentation available, as well as his experience, to properly estimate the required renovations.

[1044] It is noteworthy that, in Mr. Puddicombe's cross-examination, he admitted that the amount of \$34,000 for the elevator (in the May 29, 2001 report) was a reasonable amount, which would come close to reality. However, this amount was considerably higher in the December 9, 2004 report, which now included an addition to the home. It is questionable as to why these costs have increased significantly, given that the plaintiff's needs did not change, other than Mr. Puddicombe's explanation that the new proposals dealt with a powered wheelchair. The evidence showed that, from a Building Code or federal accessibility guideline perspective, there was no difference in the construction of an elevator for a manual versus powered wheelchair. It is submitted that an addition to the home had to be contemplated in the May 29, 2001 report and, as such, included in the price provided by Mr. Puddicombe. There is no justification, we submit, in justifying an increase of approximately \$215,000 (i.e., May 29, 2001 report versus December 9, 2004 report.).

[1045] Although Mr. Borthwick agreed with Mr. Puddicombe's first report of May 29, 2001 (on the total cost estimate only), he offered a detailed explanation of every line item. The

conclusion reached was that, while some of Mr. Puddicombe's estimates were either high or low, he opined that for this size of a project, in his experience, with a minor variance of 5% to 10%, it would be in the vicinity of \$200,000.

[1046] The Court, ING submits, also has to consider whether the modifications to the kitchen, as proposed by Mr. Puddicombe, are necessary, in light of the evidence. The plaintiff testified that she no longer prepares any meals and relies on the family members or attendant care providers. An amount of \$40,000 was allocated to the kitchen in the December 9, 2004 report. This, in our submissions, would have to be discounted significantly to take into account that the plaintiff would simply require access to the kitchen and not be involved in the meal preparation, given her physical condition.

[1047] With respect to the proposed swimming pool, the plaintiff contends that she requires a swimming pool in her basement to assist her with exercises. In our submission, this is not a reasonable and necessary expense and there is no causal connection whatsoever.

[1048] In reviewing Dr. Donald Johnson's report, dated November 6, 2002 (Exhibit 1, Tab C-1, page 2), he states:

She wants to keep ambulating rather than go to a scooter, and I have suggested a couple of things. Perhaps a brace would help to stabilize her left knee, so she could keep walking without the knee giving way and having her fall. She needs some form of exercise she can do at home. A swimming pool would be ideal, however, a stationary bicycle that she could exercise would be a reasonable compromise for the near future.

[1049] This opinion does not, in our submission, amount to a recommendation that would meet the SABS test that it is a reasonable and necessary expense. The stationary bike was a reasonable compromise and she did obtain one.

[1050] The evidence advanced by ING, in this matter, is undisputed, in that Ms. Smith-Bradley, an experienced Occupational Therapist, expressed, not only concern regarding the plaintiff's narcolepsy and the possible dangers, but also opined that, in her expert opinion, would not fall within the statutory definition of "reasonable and necessary".

[1051] The plaintiff's history of knee problems dates back to 1993:

- The plaintiff tore the medial collateral ligaments of both knees playing slow pitch;
- Dr. Snook noted on January 12, 1993 that her right knee was sore. The plaintiff admitted during cross-examination that it might be residual from when she tore the ligaments playing slow pitch;
- In the February 1993 accident, the plaintiff hit her right knee on the console under the steering wheel;
- Dr. Snook noted on February 16, 1993 that Dr. Laflèche diagnosed the plaintiff with a right knee collateral strain;
- In August 1993, the plaintiff broke her left fibula, tibia and ankle. She was required to use crutches, which was hard on her knees, given her weight at the time;
- Dr. Snook noted on September 19, 1993 that the plaintiff weighed 204 pounds;
- The plaintiff continued to maintain a weight in excess of 200 pounds until January 1995, where she obtained a referral to see Dr. Dent for the Ensure supplement and fluids program;
- Dr. Vanderwaetere reports a knee injury, while in physiotherapy.

[1052] As the Court can see, the evidence clearly shows that the plaintiff's knee problem existed long before the 1998 accident. We submit that there is absolutely no causal connection at all to the 1998 accident, and the addition of a swimming pool should not be considered as an appropriate addition to the home.

[1053] Despite Mr. Cronyn's contention that the appropriate amount for home modifications is \$362,206, we submit, that the plaintiff's own belief, as to the costs of

renovations, is quite telling. The following excerpt of the plaintiff's cross-examination on the last day of trial, supports ING's position:

Q. Can you explain to us why, for safety reason, you felt you had to modify the cottage, but you took no steps with your home?

A. Well, the issue is the cottage was a reasonable easy amount. The home was totally out of our price range. I mean, you're talking about \$5,000 versus \$200,000.

[1054] In conclusion, we propose that the evidence supports ING's position that the cost of modifying the plaintiff's home is in the range of \$200,000. Now, applying the funds available from the Zurich settlement (\$75,000) and ING's previous contribution (\$50,786), it is submitted that the amount is \$74,214.

Attendant Care Benefits (past expenses)

[1055] ING submits that the plaintiff is not entitled to Attendant Care Benefits, as the need for same arises out of her unrelated medical conditions, including *inter alia* narcolepsy and sleep apnea and, therefore, is not a direct result of the motor vehicle accident.

[1056] ING submits that, pursuant to section 60(2) of the SABS, payment of an Attendant Care benefit expense is not required, if an amount is reasonably available to the plaintiff. The following is a summary of ING's theory:

- (1) Under Bill 164, the plaintiff would have been entitled to a maximum amount of \$10,000 per month for Attendant Care and there was no statutory maximum limit payable other than the foregoing monthly cap. That amount was reasonably available to the plaintiff, yet she chose to accept a sum of \$300,000 from Zurich for this head of benefit. Given that the plaintiff has improvidently settled with Zurich, ING has been prejudiced by same. That fact should not be visited on ING and, as such, it is our position that no amounts are due and owing to the plaintiff;
- (2) Alternatively, if the Court finds that ING must accept the consequences of the plaintiff's action in settling with Zurich and allocating \$300,000 to Attendant Care benefits, it is our submission that this amount was reasonably available to the plaintiff and ING is entitled to the full value of

this benefit. We submit that no sums are due and owing to the plaintiff until such time as these funds have been exhausted.

(3) In the further alternative, the amount claimed by the plaintiff is not reasonable, given that:

a) the assessment of needs provided was not done in accordance with Form 1, which is a statutory requirement;

b) the amounts are not calculated in accordance with the statute, as noted on Form 1, i.e., \$7, \$9, or \$15 dependant upon the level of services; and

c) the total monthly claim exceeds the statutory monthly amount mandated by statute, which is \$6,000. We submit that the only evidence that can be considered by the Court, are the assessments carried out by qualified occupational therapists. This analysis reveals that the past attendant care benefits expenses are substantially lower than those claimed by the plaintiff.

[1057] It is settled law that quantification of the Attendant Care benefits revolves around Form 1. This form outlines three levels of Attendant Care: (1) routine personal care; (2) basic supervisory functions; and (3) complex health/care and hygiene functions. Each level is compensated at the different rate of compensation, (1) \$7, (2) \$9, and (3) at \$15. This is mandated by statute. The Attendant Care assessments must be performed reliably and objectively, and reflect the claimant's needs at the time of the assessment. It is our submission that, in assessing the past Attendant Care benefits expenses that are allegedly due and owing, the Court must only consider the Form 1 that have been completed.

[1058] As stated above in paragraph 1 of this section, it is our submission that, in accordance with section 60(2) of the SABS, the Court must consider what was available to the plaintiff in May 2002. We know, at that time, under Bill 164, the plaintiff had "no maximum limits" available to her under the Attendant Care benefits. The fact that she chose to accept a significantly lower sum (i.e., \$300,000) puts the plaintiff in a prejudicial position. ING should have never been exposed to this benefit, and for that reason, it should not be penalized. It is

respectfully submitted that the plaintiff improvidently settled the Attendant Care benefits and that no sums are due and owing to the plaintiff, for past Attendant Care expenses.

[1059] In the alternative, should the Court conclude that ING must accept the Zurich settlement as is, we then submit that ING is entitled to deduct the full value of the benefit, which the evidence confirms is \$300,000 for future Attendant Care. According to the plaintiff, the Zurich settlement contribution ranges from \$1,538.50 to \$1,569.42. The fact that the plaintiff chose to structure it in this fashion, we submit, should not be visited upon ING. It is our submission that, in applying the *Orchover* decision and in accordance with section 60(2) of the SABS, ING is entitled to deduct the full value of the benefit, from any amounts that are allegedly due and owing for past Attendant Care expenses.

[1060] The following analysis will provide the Court with ING's position as to the proper amount of the past Attendant Care expenses that have been incurred to date.

[1061] The plaintiff argues that the sum of \$140,981.02 (\$189,075.64 (Exhibit 27, Tab 1-H and 1-I) less \$48,094.02 (the Zurich contribution, based on the \$300,000 portion of the structured settlement)) is due and owing. ING submits that, upon carefully reviewing the figures submitted, together with the applicability of the SABS, the plaintiff is grossly overestimating her entitlement.

[1062] In essence, the plaintiff is asking the Court to grant an order indirectly, which you cannot do directly. In granting the plaintiff's request, we submit that it would be circumventing the current legislation and not follow the proper process in assessing the value of attendant care benefits. Section 16(4) of the SABS provides that the amount payable for the Attendant Care benefits shall be determined in accordance with Form 1, and shall not exceed the sum of \$6,000 (section 16(5)(1)(ii) of the SABS). Section 16, in its entirety, is reproduced below:

16.(1) The insurer shall pay an insured person who sustains an impairment as a result of an accident an Attendant Care benefit. O. Reg. 403/96, s. 16(1).

(1.1) Despite subsection (1), no Attendant Care benefit is payable to an insured person whose impairment comes within the Grade I Whiplash Guideline or the

Grade II Whiplash Guideline, if the accident occurred after April 14, 2004. O. Reg. 458/03, s. 6(1).

(2) The Attendant Care benefit shall pay for all reasonable and necessary expenses incurred by or on behalf of the insured person as a result of the accident for,

(a) services provided by an aide or attendant; or

(b) services provided by a long-term care facility, including a nursing home, home for the aged, or chronic care hospital. O. Reg. 403/96, s. 16(2).

(3) Subsection (2) does not apply to expenses for which payment may be obtained under clause 14(2)(g), 15(5)(k), or 24(1)(c). O. Reg. 403/96, s. 16(3).

(4) The monthly amount payable by the Attendant Care benefit shall be determined in accordance with Form 1. O. Reg. 403/96, s. 16(4).

(5) The amount of the Attendant Care benefit payable in respect of an insured person shall not exceed the amount determined under the following rules:

1. If the accident occurred before October 1, 2003, the amount of the Attendant Care benefit payable in respect of the insured person shall not exceed,

i. \$3,000 per month, if the insured person did not sustain a catastrophic impairment, as a result of the accident, or

ii. \$6,000 per month, if the insured person sustained a catastrophic impairment as a result of the accident.

2. If the accident occurred on or after October 1, 2003, and the optional medical, rehabilitation and Attendant Care benefit referred to in section 27 has not been purchased and does not apply to the insured person, the amount of the Attendant Care benefit payable in respect of the insured person shall not exceed.

i. \$3,000 per month, if the insured person did not sustain catastrophic impairment as a result of the accident, or

ii. \$6,000 per month, if the insured person sustained a catastrophic impairment as a result of the accident.

3. If the accident occurred on or after October 1, 2003 and the optional medical, rehabilitation and Attendant Care benefit referred to in section 27 has been purchased and applies to the insured person, the amount of the Attendant Care benefit payable in respect of the insured person shall not exceed the monthly limit under subsection 27(5). O. Reg. 281/03, s. 5; O. Reg. 458/03, s. 6(2).

[1063] In reviewing Tabs 1-H and 1-I (Exhibit 27), we submit that it is clear the plaintiff is requesting payment over and above \$6,000 per month, which is contradictory to section 16(5)(1)(ii) of the SABS. For illustration purposes, the plaintiff is claiming the following for the months of June 2002 to November 2002:

Month	Expense (see Tab 1-H)	Expense (see Tab 1-I)	Total expense
June 2002	\$ 3,983.60	\$ 2,961.72	\$ 6,945.32
July 2002	\$ 3,983.60	\$ 2,841.72	\$ 6,825.32
August 2002	\$ 3,983.60	\$ 2,961.72	\$ 6,945.32
September 2002	\$ 3,983.60	\$ 3,221.52	\$ 7,205.12
October 2002	\$ 3,983.60	\$ 3,221.52	\$ 7,205.12
November 2002	\$ 3,983.60	\$ 3,269.04	\$ 7,252.64

[1064] Mr. Cronyn's argument is that, taking into consideration the contribution from the Zurich settlement, ING's portion never exceeded \$6,000. With all due respect to Mr. Cronyn, the plaintiff would, in reality, receive an amount, which is in excess of the allowable limit under the statute. It is not discretionary; it is mandatory. The Zurich settlement deduction has to be applied from the monthly maximum, no more. Although it may appear unfair that the plaintiff had to pay a higher rate for outside providers, ING cannot be penalized in this regard.

[1065] Further, the Form 1 is very specific in terms of addressing the needs of an insured. There was no evidence given as to the services provided, nor the amount of time spent providing specific services. This is needed in order to do an assessment and quantify it at the proper rates.

[1066] The plaintiff's claim cannot be accepted, as the hourly rates claimed are far in excess of the statutory mandated rates. The plaintiff has claimed rates as high as \$22 per hour for services, which she is entitled to claim \$7. However, as noted, we do not have the

information breakdown to determine how many hours were spent providing level 1 services, level 2 services or level 3 services.

[1067] Further, there has been no consideration item, which may influence the needs under the Form 1. For example, she now has a lifeline, which could serve to reduce the need for Attendant Care to some degree. Further, to the extent that the need for Attendant Care arises out of unrelated conditions, then the assessment should be reduced, as well.

[1068] It is ING's submission that the list provided by the plaintiff (Exhibit 27, Tabs 1-H and 1-I) cannot be used as a guide and are simply not reliable. The only reliable evidence, with respect to past Attendant Care benefits, is the Form 1 that has been completed on April 17, 2001 and September 17, 2002.

[1069] The first Form 1 was completed on April 17, 2001 (Exhibit 1, Tab B-9). At that time, the plaintiff's needs amounted to \$2,750.93 per month. This assessment was done shortly after the plaintiff's discharge from the hospital, following her second surgery. It is reasonable to assume that, at this point in time, the plaintiff's needs were most likely the greatest and that her condition would simply improve. After this assessment, the evidence is clear that the plaintiff never requested to be reassessed. We must not forget that the plaintiff, an experienced accident benefits adjuster, was well aware of her rights in this regard, and never once advised that the Form 1 did not properly reflect her requirements.

[1070] What is further telling, and again, is the strongest evidence available to assess the plaintiff's needs, pertaining to past Attendant Care benefits, is the second Form 1 that was completed by Ms. Susan McNab, an Occupational Therapist, on September 17, 2002 (Exhibit 19). The plaintiff, who had counsel at the time, and at her own choosing, is the one who had requested that Ms. McNab assess her level of functioning, personal safety and Attendant Care needs. We note that this assessment was done in September of 2002. In the plaintiff's testimony, she indicated that, at this point in time (September 2002), she was experiencing sleep attacks and had urinary track symptoms. There is no evidence to suggest that her needs, pertaining to Attendant Care in September 2002, are any different than those at the

commencement of trial. Ms. McNab concluded that the total assessed monthly Attendant Care benefit is \$3,872.25. Ironically, the plaintiff has claimed \$7,205.12 for September 2002 when her own Occupational Therapist assessed her needs at \$3,872.25.

[1071] In light of the evidence, which suggests that the Attendant Care is required due to her narcolepsy, it is ING's position that no amount is due, as this is not an accident related medical condition. If we accept that 50% of the reason she requires Attendant Care is due to the unrelated medical conditions, then the amount calculated should be reduced by that same percentage. As such, it is our submission that the proper amount for past Attendant Care benefits is \$119,426.72, reduced by half to \$59,713.36, which is calculated as follows:

Period	Description of Calculation	Total
June 2002-September 2002	In accordance with Form 1, completed by Ms. Smith-Bradley (see Tab B-9, Exhibit 1), the appropriate monthly amount is \$2,750.93 x 4 months	\$ 11,003.72
October 2002-January 2005	In accordance with Form 1, completed by Ms. McNab (see Exhibit 19), the appropriate monthly amount is \$3,872.25 x 28 months	\$ 108,423.00
Total		\$ 119,426.72 x 50%

Summary of ING's position regarding past Attendant Care expenses

[1072] In summary, it is ING's respectful submission that, with respect to past Attendant Care expenses, the Court has three options:

- (1) Given that the plaintiff had an unlimited exhaustive resource available to her under Bill 164, yet chose to accept the sum \$300,000 in the Zurich settlement, ING should not be prejudiced by her actions and, accordingly, we submit no order be granted for past Attendant Care benefits expenses;
or

- (2) The plaintiff has earmarked \$300,000 from the Zurich settlement, and regardless of the amount structured for future Attendant Care, ING should not be penalized, and is entitled to the full value of the benefit. It is our submission that an order entitling ING to deduct \$300,000, resulting in the past Attendant Care expenses in the amount of \$59,713.36 being paid, is appropriate; or
- (3) The quantification for Attendant Care must be made in accordance with Form 1. Two Attendant Care assessments were completed on April 17, 2001 and September 17, 2002, providing for monthly Attendant Care benefits in the amounts of \$2,750.93 and \$3,872.25, respectively. We submit that the amount of past Attendant Care expenses to date is \$59,713.36, less the Zurich contribution, in the amount of \$48,094.02, totaling \$11,619.34.

Attendant Care Benefits (future)

[1073] With regards to Attendant Care in the future, ING's position is that, in the event the Court grants a declaration for ongoing Attendant Care benefits, an order cannot be made that Attendant Care benefit is to be prepared, based on 24-hour Attendant Care. As stated above, the Occupational Therapist is the most qualified person to make that assessment, and the plaintiff's suggestion that the evidence supports such an order is without merit. Furthermore, the plaintiff's contention is not supported by law, and is contradictory to the statutory requirements. There is no jurisdiction for this Court to make such an order, and it should be left to the qualified professionals of assessing the plaintiff's needs. ING has the statutory right to complete their own assessment.

[1074] Following the same logic as above, it is our respectful submission that, with respect to future Attendant Care benefits, the Court has three options:

- (1) Given that the plaintiff had an unlimited exhaustive resource available to her under Bill 164, yet chose to accept the sum of \$300,000 in the Zurich settlement; ING should not be prejudiced by her actions. In light of the fact that the plaintiff would have had an unlimited amount of Attendant Care benefits available to her, ING would never have had the obligation to pay Attendant Care benefits. We submit that no order be granted for future Attendant Care benefits expenses, given that; or

- (2) The plaintiff is entitled to deduct the full value of the benefit received by the plaintiff in the Zurich settlement (i.e., \$300,000). As such, it is our position that no amounts, with respect to future Attendant Care benefits, are due and owing, until \$240,286.64 has been exhausted (i.e., full value of the benefit (\$300,000), less payment of the past Attendant Care expenses (\$59,713.36)); or
- (3) The quantification for future Attendant Care must be made in accordance with Form 1. No conditions can be imposed by the Court, as requested by the plaintiff (i.e., order that a new Form 1 be prepared on the basis of 24-hour Attendant Care). The assessment is best left to qualified professionals, who will assess the plaintiff objectively.

Housekeeping and Home Maintenance Expenses

[1075] In the plaintiff's written submissions, although Mr. Cronyn acknowledges that the housekeeping expenses are currently being met, he states that "they could fall short soon" and, as such, is requesting an order, requiring ING to pay on-going housekeeping expenses net of the amounts generated, on a monthly basis, by the Zurich settlement. We submit that there is no evidence to support this statement and, regardless, it is irrelevant.

[1076] Mr. Cronyn properly quoted section 22(1) and (2) of the SABS, and this text is also reproduced at the beginning of this section. The plaintiff is already receiving the maximum amount allotted under this benefit, and the benefit is limited by statute. The plaintiff cannot receive any amounts over and above it. In the Zurich settlement, the plaintiff allocated \$110,000 for housekeeping and home maintenance expenses. In our calculation, if the plaintiff had properly invested only \$67,800 in a structured settlement, this amount would have generated \$100 per week for the plaintiff's lifetime, thereby meeting all of her needs.

[1077] It is our submission that the law clearly provides that an order into the future is not within the Court's jurisdiction given that the expense has to have been incurred and it must be reasonable and necessary. Assuming that the Court grants the plaintiff her declaration, and if the amounts generated by the Zurich settlement are depleted, which we submit is an impossible scenario, the plaintiff will simply need to submit an expense under this benefit and ING will properly investigate the reasonableness and necessity of same. If it meets the SABS

requirements, payment will be made. We submit that this is the statutory and mandated process that needs to be followed, and that no order can be given in this regard.

Visitor Expenses

[1078] The plaintiff claims that she is entitled to an order in the amount of \$1,048.14 (see page 63 of the plaintiff's written submission). Once again, the plaintiff is clearly mistaken, and it is surprising that this relief is being requested. The evidence clearly demonstrated that this expense was in fact paid by ING by way of cheque no. 32388, dated April 27, 2001 in the amount of \$1,077.62, representing the visitor's expense in the amount of \$1,048.14, together with 2% interest in the amount of \$29.48 (see Exhibit 38). It is therefore submitted that this amount is not due and owing, and the plaintiff's request should be denied.

Income Replacement Benefits

[1079] For the sake of clarity, the parties agreed, on January 8, 2005, to settle the issue relating to the deduction of an overpayment of IRBs, as follows:

1. Since June 6, 2002, ING has deducted 20% of the IRB, on the basis of an overpayment to the plaintiff, arising out of the monies received as an IRB in the Zurich settlement. ING agrees to repay to the plaintiff those monies, which have been deducted, together with interest calculated in accordance with SABS, up to January 12, 2005, in the amount of \$12,887.82. ING acknowledges that it is not entitled to make a deduction in respect of its payment of an IRB, in respect of any monies received by the plaintiff in respect of the Zurich settlement in the past or in the future. ING acknowledges that the plaintiff's current entitlement is \$340.96 and reserves its rights, with respect to entitlement and calculation, and specifically reserves its right to investigation, deduction and/or the right to stop payment in accordance with the provisions of SABS.

2. The parties agree that this paragraph shall form part of the Judgment rendered in this matter, and agree to provide a full and final release, with respect to payment of past IRB benefits, up to January 12, 2005.

[1080] It is therefore submitted that the above-noted paragraph will need to be incorporated into the Judgment.

ISSUE NO. 3 – ENTITLEMENT TO BENEFITS

Discussion and Decision

Past benefits

[1081] The plaintiff's claim for past benefits is from May 2002 to the present, to cover the period of time in between the receipt of Zurich settlement funds and the present action against ING. The basis for the claim for past benefits is essentially that of compensation for expenses Ms. Monks has incurred during that period for which no benefit or settlement amount was offered or received.

[1082] The amount of past benefits being claimed for medical and rehabilitation expenses incurred from May 2002 until the present is \$68,815.87, which amount is net of all collateral benefits received from other sources (GSMPI, ODB, CCAC). After deducting the portion (\$24,195.44) of the Zurich settlement dedicated to medical and rehabilitation benefits, the net amount owing is \$44,620.43. This amount should then be increased to \$46,718.04 to account for additional expenses recently incurred.

[1083] The plaintiff noted that ING paid her \$50,786.88, when it terminated her benefits on May 31, 2002, to be used for home renovations. However, this amount was not used for home renovations given the uncertainty of the on-going litigation with ING concerning the 1998 accident, and was instead used for medical and rehabilitation expenses. Therefore, since ING has already paid \$50,768.88 for medical/rehabilitation needs, and only owes \$46,718.04 as past medical/rehabilitation benefits for the same time period, a credit of \$4,068.84 could be applied towards the medical/rehabilitation benefits ING will owe to Ms. Monks for the period beginning January 1, 2005.

[1084] In terms of attendant care expenses, the amount of attendant care expenses for the period between May 31, 2002 and January 2005 totals \$189,075.64, based on a \$10/hour fee, of which \$48,094.62 has already been paid through the Zurich settlement structure. Therefore, there is a balance owing of \$140,981.02 from ING to Ms. Monks for past benefits relating to attendant care expenses.

[1085] There is also an amount of \$1,048.14 owing to Ms. Monks for visitors' expenses incurred from May 2002 to the present (paid prior to trial).

[1086] As well, once the global amount has been determined, there will be a claim for interest on past benefits due, pursuant to s. 46(2) of the *SABS*; this was not discussed in any detail in the plaintiff's submissions.

Current on-going benefits

[1087] In terms of income-replacement benefits, the parties have agreed that ING should continue to pay \$340.96 per month to Ms. Monks for income replacement benefits.

[1088] In terms of medical/rehabilitation benefits, the plaintiff submitted that ING should be ordered to pay all medical and rehabilitation expenses on an on-going basis for all the medical-related needs summarized in Ex. 8, minus any collateral benefits available to Ms. Monks for medical/rehabilitation benefits, and minus any monthly amount received by Ms. Monks from the 2002 Zurich settlement designated for medical needs and expenses.

[1089] For attendant care benefits, the plaintiff submitted that Ms. Monks currently requires 24-hour attendant care, based on the sum total of all her medical problems, which stem from the accident. The plaintiff argued that a new "Form 1" should be prepared to account for the need for 24-hour care, and thus a new monthly amount would be generated. Then ING would be entitled to deduct the monthly amount received for attendant care arising from the 2002 Zurich settlement from the amount owing.

[1090] In terms of housekeeping and home maintenance expenses under s. 22 of the SABS, the plaintiff claimed that while the Zurich settlement funds designated for this type of expense currently meet Ms. Monks' needs, such needs could change in the future. The plaintiff therefore submitted that ING should be ordered to pay on-going benefits for housekeeping expenses minus the monthly amounts generated by the Zurich settlement funds, up to a maximum of \$100 per week. Currently, ING would not be required to pay anything, since the Zurich settlement funds cover Ms. Monks' present expenses; but Ms. Monks should not have to return to court if her expenses increase over time.

[1091] Finally, the plaintiff submitted that she is entitled to receive benefits under s. 17 of the SABS for expenses incurred for services provided by a qualified case manager. Ms. Monks has expressed a desire to obtain such benefits, given that she has a need for case manager services; such benefits should form part of the amounts owing by ING.

[1092] In addition to the amounts claimed for current on-going benefits up to the date of trial, there have been additional expenses incurred from the start of trial until the date of judgment; these additional expenses should also be accounted for in any order eventually made, with applicable interest minus any collateral benefits and monthly Zurich settlement monies received during that period.

Modifications to home and vehicle

[1093] The plaintiff submitted that ING is responsible for paying all reasonable and necessary expenses for home modifications and devices, or for the purchase of a new home if this purchase does not exceed the cost of renovations to the existing home, pursuant to s. 15 of the SABS. Ms. Monks has not yet decided whether it would be better to purchase a new home or renovate her existing home, since the full cost of renovating her home (which would be the maximum benefit allowable) has not yet been determined. The best estimate is a total renovation cost of \$362,206.00 provided by Scott Puddicombe in April 2003, updated to December 2004, found at Exhibit 9.

[1094] Counsel for the plaintiff asserted that Ms. Monks needs to know the upper limit of what she will be entitled to spend before making a decision as to home renovation vs. new home purchase (and what type of new home).

[1095] Using the estimated cost of modifications to her new home as an accurate estimate of the maximum benefit allowable, the plaintiff submitted that ING should be ordered to pay \$362,206.00 for home modification/new home purchase, but ING would be entitled to deduct \$75,000.00 from this amount to reflect the contribution towards home modifications already received through the 2002 Zurich settlement.

[1096] It should be noted that the *Saliba* case mentioned earlier (where the applicant had three undiagnosed medical conditions which became symptomatic following two car accidents, resulting in surgery which eventually rendered him a quadriplegic) dealt with the issue of benefits for home renovations in a subsequent proceeding.

[1097] In *Saliba v. Allstate Insurance Co. of Canada*, [2001] O.F.S.C.I.D. No. 31 (Ontario Financial Services Commission) [QL], an arbitrator from the Financial Services Commission awarded the applicant \$153,410.00 towards the cost of home renovations on the applicant's new home for his housing needs and those of his family members, including the cost of building an additional bedroom.

[1098] After moving out of a townhouse, which, according to the evidence, would be incapable of being modified to meet the applicant's disability needs, the applicant then purchased a resale home for approximately \$339,000.00. The applicant then claimed that the insurers were required to pay both the full cost of the new home, as well as the full cost of additional renovations which would need to be made to the new home – approximately \$146,000 – plus legal fees associated with the purchase and design fees associated with the renovations. In response, the insurers argued that they were only responsible for the cost of renovations to the former home (the townhouse).

[1099] The arbitrator found that the insurers' proposed amount was inadequate in that appropriate renovations to the townhouse were simply not possible; the preferable approach consistent with the legislation would be to award an amount which would respond to the applicant's actual current renovation needs and expenses. The arbitrator ruled that the townhouse could not be reasonably altered to meet the applicant's needs, and that the applicant acted reasonably in purchasing the new home. The applicant was therefore entitled to the full cost of all reasonable and necessary modifications to be made to the new home in order to accommodate the applicant's disability.

[1100] With respect to the applicant's claim that the insurer should provide additional benefits equal to the purchase price of the new home itself, the arbitrator rejected this claim given that an amount was already being awarded for home renovations: "I find no evidence that persuades me that the Insurers should fund the capital cost of the house itself, as a "good" under section 6(1)(f) of the Schedule, because I am not persuaded that Mr. Saliba requires a house "because of the accident." I agree with the Insurers' submission that everyone requires shelter, food and clothing" (para. 43). The arbitrator ruled that the impossibility of renovating the former home to make it suitable for the applicant's needs only meant that the applicant was entitled to the cost of *renovations* on the new home, not to the cost of *purchasing* the new home itself.

[1101] Mr. Justice Belch dealt partly with this issue: *d'Entremont v. Smallwood*, [1999] O.J. No. 4567 (Sup. Ct. Just.) [QL]. This case was not really about statutory benefits, but was rather an action brought by the victim of an automobile accident, who became paraplegic as a result of the accident, against the estate of another driver who died in the accident, and against the tavern where the defendant driver had become intoxicated prior to driving his car and causing the accident.

[1102] The case does partly touch on the issue of home modifications and the role of a statutory benefits insurer. Following the accident, the plaintiff's insurer advanced monies to the plaintiff's parents in order to modify their home, and to the plaintiff herself to modify her rental accommodation. The plaintiff later purchased a new home and approached the insurer for additional home modifications benefits; the parties could not agree on an amount, and mediation

on this issue was unsuccessful. The plaintiff then continued to look for an adequate replacement home, which either already contained the appropriate accommodations, or could be modified accordingly, and eventually settled on a design for the construction of a new (modified) home, which would cost approximately \$300,000. Justice Belch awarded this amount, but deducted the cost of an “average home” in the area – \$97,000 – to represent the plaintiff’s own contribution towards new housing had the accident never occurred, and also deducted amounts for sales taxes, as well as for certain modifications which the judge felt were not necessary. Overall, Justice Belch awarded a one-time housing allowance of \$190,348.00.

[1103] In addition, Justice Belch awarded \$53,992.00 in future care costs relating to projected home modifications. I also note that the insurer, in the *d’Entremont* case, paid for a modified van to meet the plaintiff’s disability needs when driving.

[1104] In its submissions, the defendant suggested that Ms. Monks should have spent her Zurich dollars (1.275 million, being 1.54 million less the amount attributed to costs) prior to making claims against the defendant ING. The defendant also argued that the Sandra Guest-Poulin estimate (Exhibit 8, Tab 1, page 79) amounted to a Cadillac approach generated by the plaintiff. I agree with counsel for the plaintiff that, during trial, he undertook not to use the Sandra Guest-Poulin assessment in his submissions on entitlement, but only in his submissions for aggravated damages. The Sandra Guest-Poulin assessment is also useful to the Court to consider the argument that Ms. Monks has received enough Zurich dollars, and that giving her more money will mean a windfall or double recovery for her. I find that the Sandra Guest-Poulin assessment clearly establishes that the Zurich dollars are, by far, inadequate to meet all of Ms. Monks’ needs. The defendant cannot make up its mind, because on the one hand it insists on a strict application of the regulations and that the Court not consider what Ms. Monks’ ultimate needs, while on the other hand, arguing that Ms. Monks has plenty of money to take care of herself without regard to the sources of those funds, in order to exonerate itself from its contractual and statutory obligations.

[1105] The defendant ignores in its submission that \$600,000 of the Zurich settlement was received for past income loss. The sum of \$600,000 was never intended to cover Ms.

Monks' medical, rehabilitation or attendant care needs. Similarly, monies received for income replacement benefit and long-term disability benefits were not intended to cover Ms. Monks' medical rehabilitation or attendant care needs. If that were the purpose, the plaintiff would have been foolish to litigate this matter.

[1106] The plaintiff in this case had two insurance contracts ... one from Zurich and one from ING. Ms. Monks will not obtain her full income losses from both the 1995 and the 1998 accidents, and that fact trumps the defendant's position that "Ms. Monks has been fully indemnified for the expenses that she will incur as a result of her injury". Both insurers have to look after medical, rehabilitation and attendant care needs, irrespective of the fact that Ms. Monks may or may not have money of her own.

[1107] Therefore, I conclude that the windfall argument of the defendant is not grounded in the legislation. Ms. Monks has an entitlement to an accident benefit scheme and she has not yet collected on it. Since Ms. Monks' funds were never intended to indemnify her for medical rehabilitation or attendant care needs, to the contrary, I find that, if the defendant were successful in its arguments on accident benefits, it is the defendant who would reap a windfall.

[1108] I do not understand how counsel for the defendant can submit that the plaintiff knew that ING did not intend paying benefits after the Zurich settlement, as ING itself expected to be indemnified for the expenses that it had paid out on her behalf. During trial, I did not receive any evidence on this point. Ms. Monks' evidence was contrary to this position. All the defendant accomplishes with this type of argument is to lengthen the trial and fatigue the trial judge.

[1109] The defendant, through its counsel, represented, on more than one occasion, that there was no jurisprudence in existence to guide this Court in a similar case. But the *Saliba* decision is a 1999 multi accident case and does provide me with guidance. In *Saliba*, the applicant's largely non-symptomatic pre-existing conditions were rendered symptomatic by the accidents, which resulted in tremendous pain and led to surgery; this surgery, brought on by the accidents, ultimately caused the plaintiff to suffer incomplete quadriplegia.

[1110] I find that, based on *Saliba v. Allstate Insurance of Canada, supra*, and *Z v. Dominion of Canada, supra*, there is no room for apportionment of the cost of paying benefits in accident benefit cases, as there is for the payment of damages in a tort case.

[1111] The defendant pleads that equity at common law exists in the contractual obligation field, for apportionment of liability. Then, the defendant argues that ING should be allowed a deduction for the full amount of the monthly benefits, within each category of benefits, which would have been available to Ms. Monks under the Zurich policy, pursuant to Bill 164. This, the defendant submits, would prevent Ms. Monks from receiving a windfall or double recovery. For example, ING would be required to pay ongoing medical and rehabilitation benefits, minus the monthly amounts already received by Ms. Monks from the 2002 settlement relating to medical and rehabilitation needs.

[1112] I agree with the plaintiff who submitted that, if Ms. Monks meets the test of entitlement (and I ruled that she has), then, unless stated to the contrary in the legislation or the regulations, she is immediately entitled to the full ambit of the coverage provided, under the regime, for all of her benefits. Ms. Monks is not entitled to a windfall, but on the other hand, she has paid for her insurance coverage with ING, and she should not have to wait until the funds under her Zurich settlement are exhausted. Ms. Monks has placed part of her Zurich settlement in a structured settlement that will ensure that, in certain benefit categories, she will receive funds until she dies. If I were to accede to the defendant's submissions on deduction of money received from the 1995 accident, the accident victims who would rarely place their accident settlements in structure settlements. If the legislators intended the result argued by the defendant, this should have been clearly stated.

[1113] In such a situation, ING would never have to pay benefits and would escape its contractual obligation. Had Ms. Monks not settled her 1995 accident claims, then both Zurich and ING would have had to contribute equally, to Ms. Monks' medical and rehabilitation expenses or home modifications. Even though the legislation is different for the 1995 and the 1998 accidents, both insurers would have been liable to contribute to Ms. Monks' monthly needs

equally, up to any monthly maximums, and ultimately up to any total maximums, under each regime.

[1114] I reject the defendant's argument that Ms. Monks made an improvident settlement when she settled her 1995 accident claims for 1.5 million dollars. The defendant did not plead that the settlement was improvident. The defendant did not adduce evidence at trial to prove that the settlement was improvident. So, this submission is also rejected, as it would mean that the whole Zurich action would be re-litigated. Counsel for the plaintiff indicates that the matter of amending pleadings to incorporate this argument was canvassed, prior to the trial, by the defendant, with the plaintiff, and was abandoned. The parties have already used two extra weeks of court time over and above their time estimate for this trial. To plead an improvident settlement would have meant an extra two weeks of trial. In any event, it is difficult to comprehend how the defendant can consider this argument, after submitting that the plaintiff is seeking a windfall with this action?

[1115] I also reject the defendant's submission that s. 60(2) applies in this case. There is no case law that supports the defendant's position that payment of an attendant care benefit expense is not required, if an amount is reasonably available to the plaintiff. I find that there is no improvident settlement for the 1995 accident benefits. I agree with the submissions of the plaintiff concerning the interaction of the Zurich settlement benefits with the benefits awarded to the plaintiff in this case. My decision incorporates all the plaintiff's submissions from page 296 to page 302.

[1116] It was suggested by counsel for ING during the trial that the SABS-96 provide benefits only for impairments, which are caused "directly" by an automobile accident. I agree with the plaintiff's submission that ING is required to provide benefits in relation to all of the impairments, which Ms. Monks has sustained, since the December 23, 1998 accident. I adopted the plaintiff's position in my analysis of *Greenhalgh v. ING Halifax Insurance Co.*, *supra*, and *Correia v. TTC Insurance Co.*, *supra*. The *Greenhalgh* decision favours the plaintiff, as the phrase "directly caused" was interpreted to mean that the restriction implied in the word "directly" refers to whether or not the impairment was caused directly by the use of operation of

an automobile notwithstanding that there may be additional or other causes of the impairment. Causation will not be established where there are too many intervening events or forces between the use /operation of an automobile and the claimant's injury; otherwise, the causation link should stand.

[1117] I agree with counsel for the plaintiff that in Ms. Monks' case, there are no extraordinary circumstances such as in the *Amos*, *Chisholm* or *Greenhalgh* decisions. In Ms. Monks' case, her injuries arose immediately following the impact of her vehicle with the bridge abutment. The use of the car was the direct cause of her injuries. There were no intervening acts. The dominant feature of the incident was the impact, while she was driving the car.

[1118] It is my decision that ING is responsible for all expenses of the two surgeries that followed Ms. Monks' accident and the numerous complications that resulted. In Sections 14, 15 and 16, the Ontario Regulation 403/96 Statutory Accident Benefits Schedule – accidents on or after November 1, 1996, the wording of the regulations reads: The insurer shall pay an insured person who sustains an impairment “as a result of” an accident, medical benefits, rehabilitation benefits, and attendant care benefits, accordingly. For the entitlement of these benefits, the impairment does not have to be a direct result of the accident, but simply “as a result of”.

[1119] This issue was dealt with squarely in *Correia v. TTC Insurance Co.*, *supra*, (page 201 of this decision). The arbitrator ruled “nothing in the wording of these sections limits benefits to those impairments directly caused by the accident. Moreover, in enacting Bill 59 amendments to the SABS, the legislature chose not to amend the provisions setting out the entitlement criteria for various types of benefits, all of which retain the “as a result of” test”. Counsel for the plaintiff reviewed the case law from pages 181 to 194 of this decision. I accept that the case of *Correia v. TTC Insurance Co.* is a complete answer to the defendant's argument that the expenses caused by the surgeries and the complications resulting from the surgeries should not be its responsibility. In the appeal at paragraph 41, the arbitrator stated:

New injury clearly may be related to an impairment sustained in the accident. The arbitrator (lower level of arbitration) gave examples of cases in which someone fell down the stairs due to dizziness related to, or a knee weakened from,

the effects of an accident and sustained new injuries. Those new injuries can be viewed as a direct consequence of the accident and as a result of it. Likewise, if someone undergoes surgery for accident related problems and, through no one's fault, suffers new impairment in the course of the operation, such consequences are directly traceable to an accident.

[1120] The same problem arose in *Saliba v. Allstate Insurance Co. of Canada, supra*, as arose in Ms. Monks' case. Mr. Saliba followed the advice of his treating practitioners and had an operation following his four accidents. He awoke with incomplete quadriplegia. The arbitrator stated in his decision:

Medicine remains an inexact science and it would appear that there is a great deal of room for differing opinions and judgment ...

Paragraph 66 of that decision continues:

I find no new intervening act, which would relieve Allstate and Progressive from responsibility for Mr. Saliba's statutory accident benefits. Although, Mr. Saliba's disability worsened following the surgery, this also does not relieve Allstate and Progressive from responsibility for Mr. Saliba's statutory accident benefits.

[1121] In Ms. Monks' case, she was told by her treating neurosurgeon that if she did not have surgery, she would go on to become a quadriplegic. She underwent surgery and, as a result, ended up catastrophically impaired. There is no intervening act in Ms. Monks' case.

[1122] I rely on the evidence of Dr. Lesiuk and Dr. Vanderwaetere, and I accept that all Ms. Monks' symptoms and the problems that she is currently suffering have their root in the original injury and the subsequent surgeries. The pain, weaknesses, numbness, all flow from the 1998 accident and the related surgeries. I find that the bowel and bladder dysfunction occurred immediately following the first surgery and derive from it. The cause of the nausea and vomiting has not been determined by the many doctors consulted in a hospital setting. I am satisfied that Ms. Monks' gastrointestinal problems have been appropriately studied and, as happens in medicine often enough, there are no explanations given for the ailment that afflicts the patient. Ms. Monks does not have to prove that her gastrointestinal problems relate to

neurological causes. As stated in the *Correia* decision, which I adopt fully, if a new impairment is caused by surgery, which was necessitated by the 1998 accident, then it flows from the accident. In this case, also included are Ms. Monks' condition of septicemia, edema, sleep apnea and narcolepsy, and I rule that they were all caused by the 1998 accident. Dr. Lesiuk and Dr. Vanderwaetere testified that Ms. Monks' weight problems flow from the accident. Ms. Monks could not remain active, following her surgeries and had difficulty keeping her food down. After the surgeries, she was nourished with milkshakes. She eventually was prescribed Ensure supplement and fluids, but the medical evidence I received at trial, confirmed that the weight problem could not be controlled by diet alone.

[1123] Ms. Monks' weight problem led her to a knee problem that will require surgery. I find that it too has its roots in the accident. The powerful medications Ms. Monks was required to take (reviewed in the evidence section) account for her sleep difficulties. Since the accident, Ms. Monks, who was an energetic person and a person defined by her work, has not been able to do anything, and I accept that she has difficulty reading. This has caused depression and psychological difficulties that in turn have had to be treated with medications. These problems are a result of the injuries and impairments she has sustained, due to the accident.

[1124] At the beginning of this trial, I gave a ruling on declaratory judgments. My ruling is attached as Appendix "A". The defendant brought a motion to obtain a ruling that the plaintiff's evidence on her past and future care needs was inadmissible. I decided that the type of relief in this action is the relief contemplated by s. 108(2) of the *Courts of Justice Act* – a declaration of the parties' rights with no coercive effect or remedial entitlement. I ruled that such was the relief granted in a declaration, despite the fact that such a declaration will likely result in the defendant paying certain amounts to the plaintiff.

[1125] I also declare that Ms. Monks' current needs are as set out in Exhibit 8, Tab 4-D, and because for the most part they are recurring and known, they fit within the definition of incurred. I also decided that the material that is being supplied to Ms. Monks, to meet such needs, is reasonable and necessary. The plaintiff should definitely not be obliged to come to court every other month for a determination of what is reasonable and necessary. If Ms. Monks'

needs do change and the material required to look after the needs also changes, then a court application must be made.

[1126] I declare that the defendant is obliged to meet Ms. Monks' needs on an ongoing basis, as, presently, they are recurring needs. The defendant has only to consult the spreadsheets found in Ms. Guest-Poulin's report to know what is to be provided, on a monthly basis.

[1127] Dealing with the expenses incurred by Ms. Monks to date and which expenses incurred were paid from the Zurich settlement, I declare that Ms. Monks has adequately accounted for such amounts received from Zurich and also from ING, from her husband's insurance at work, from community services, or from any other source. ING was not paying for a variety of expenses and Ms. Monks did not bother to fill a form No. 1. This is understandable, and it is also understandable that Mr. Perrozzino had little time left on his hands to do accounting, considering the evidence I heard concerning the time consumed in helping Ms. Monks.

[1128] I also rule that the amounts claimed by Mr. Perrozzino for transportation do not amount to double recovery. I find on the evidence of Ms. Monks, Mr. Perrozzino and Ms. Beehler that Mr. Perrozzino, despite the payments received, did not get paid for all his time and expenses.

[1129] I declare that it is the defendant's obligation to provide, and pay for, a leased vehicle to the plaintiff, equipped with a platform to hoist her wheelchair/scooter into the vehicle. When Ms. Monks was working, she had a leased vehicle for her business, because of her condition. When Ms. Monks ceased working, because of her condition, she no longer needed the leased vehicle. Because of her extraordinary needs for a wheelchair and then, a scooter, Ms. Monks was unable to use one of the other two vehicles, owned by the family. The need for the van arises solely out of the necessity to accommodate her physical needs.

[1130] I reject the submissions, made by the defendant, concerning unpaid expenses incurred by Ms. Monks, prior to September 2002, and to date, as they have no foundation in fact

or in the evidence received at trial. I declare that all such expenses be paid, in accordance with the plaintiff's proposal, reviewed earlier in this judgment.

[1131] I fail to comprehend why counsel for the defendant would want to waste the Court's time on the fact that the building code does not differentiate between a manual and an electric wheelchair. Once an expert testifies that the size and turning circle of an electric wheelchair is greater than for a manual wheelchair, then, in absence of clear evidence to the contrary, that is the information the Court will use in its decision. I agree that it is common sense.

[1132] I accept that Mr. Puddicombe's design is providing nothing more than what is reasonable to accommodate Ms. Monks' problems entering and leaving her home, and especially in helping her access the second floor of her home. Mr. Borthwick has not examined Ms. Monks' home, has not met with Ms. Monks, and he has not done any significant construction work in the Ottawa area.

[1133] I have to address the issue of whether the defendant is liable for more than the cost of making the Monks' present home accessible. In *Mortimer v. Cameron* (1992), 9 M.P.L.R., the court rejected the argument that the defendant should pay for the extraordinary purchase of a larger home, needed by the plaintiff, due to his accident related injuries. This approach was rejected because, i) it was based on an assumption with respect to the price of the house that the plaintiff would have purchased had he not been injured, ii) it assumed a certain level of contribution or lack of contribution by the plaintiff's partner, iii) it involved an element of betterment for which the court felt that the defendants should not be held responsible, notwithstanding the plaintiff's present needs. In this case, I find it debatable whether adding an elevator would mean a betterment to the plaintiff's residence. There will be a need to eliminate some existing rooms that could be used as a study, a sunroom, or a kitchen extension. Mr. Borthwick suggested \$200,000 would be required for an elevator and Mr. Puddicombe had an estimate of \$362,206. Mr. Puddicombe was adding, in his estimate, the price for a small type of pool in the basement, an allowance for alternate accommodation for the Monks family during renovations, the cost of furniture storage for 10 months, and an expensive \$40,000 kitchen

renovation. I would remove the cost of the pool, \$22,000, and the cost of kitchen renovations at \$40,000, and allow \$300,000, considering that that would be a fair, reasonable and necessary expense to accommodate Ms. Monks' needs. Ms. Monks, of course, will apply the \$75,000 received from Zurich for home renovations towards the \$300,000 needed for renovations. I feel that Ms. Monks will not be active in the kitchen and thus, that expense is not justifiable. The pool expense has been denied, as there are community facilities that will be accessible to Ms. Monks, now that she will have a van, adaptable to her needs, to access such facilities. I declare that the defendant is obliged to pay \$225,000 to Ms. Monks towards her home renovations. This award can be used to purchase and modify a bungalow should Ms. Monks elect to sell her home and purchase a bungalow.

[1134] On the specific matter of the lack of a Form No. 1 which Ms. Monks was supposed to have submitted, I rule that it is simply common sense that Ms. Monks did not have to submit such forms, once ING advised her that her accident benefits would be cut off as of May 31, 2002.

[1135] On attendant care, I declare that the average rate of \$10 per hour, submitted by Mr. Perrozzino, is more than fair for services rendered in all three categories, namely: (1) routine personal care, (2) basic supervisory functions, and (3) complex health/care and hygiene functions. As each rate is compensated at a different rate of compensation (1) \$7, (2) \$9, and (3) \$15, Mr. Perrozzino is giving the defendant a fair deal at \$10 per hour.

[1136] The plaintiff accepted \$300,000 in her Zurich settlement for attendant care benefit. Counsel for the defendant argues that this improvident settlement penalizes her client. She reminded me that in May 2002, the plaintiff, under Bill 164, had, available, attendant care, with no maximum limits. Therefore, she argued, ING should never have been exposed to a claim for attendant care and I should rule that ING owes nothing to Ms. Monks for such a benefit. The defendant's argument, based on section 16 of the SABS is reproduced at page 356 of this judgment. The defendant argues that the plaintiff's claim for \$140,981.02 for attendant care is grossly overestimated. Section 16(4) of the SABS sets a \$6,000 monthly limit and Ms. Monks' claim is over that limit.

[1137] I agree with counsel for the plaintiff that the maximum only applies to contributions to be made by ING. In the months shown on page 81, Exhibit 8, while the total amount of the attendant care costs exceeds the \$6,000, the Zurich contribution was \$1,538.50 per month. As a result, if the total attendant care cost reached \$6,945 to \$7,252 per month, if the Zurich contribution is subtracted, then what Ms. Monks is seeking is less than \$6,000 per month. I agree with counsel for Ms. Monks that nowhere, in the legislation, is it written that Ms. Monks cannot receive a sum that exceeds \$6,000 from all sources for attendant care.

[1138] I declare that Ms. Monks is entitled to receive from ING, 24-hour attendant care and that ING has to pay the cost remaining, after the contribution Ms. Monks receives from Zurich is factored in.

[1139] For housekeeping, I disagree that Ms. Monks is limited to \$100 per week. The monthly limit only applies to the defendant. I declare that the defendant must indemnify Ms. Monks for the housekeeping expenses to a maximum of \$100, once the expenses Ms. Monks is incurring exceeds the amounts she receives each month from the Zurich settlement.

[1140] There is no declaration given to the plaintiff for visitor's expenses, as the defendant proved, during trial, that they were paid.

Summary of Declarations

[1141] I declare/order that the plaintiff is entitled to the following benefits from the defendant:

1. \$46,718.04 for past benefits for medical and rehabilitation expenses, incurred from May 2002 until the present, as Ms. Monks used the sum of \$56,786.88, received from ING for home renovations, for medical and rehabilitation expenses, and since all ING owed was \$46,718.04, the difference of \$4,068.84 will be applied towards medical and rehabilitation benefits ING will owe to Ms. Monks commencing January 1, 2005.
2. ING will pay Ms. Monks \$140,901.02 for past benefits for attendant care for the period extending from May 31, 2002 to January 1, 2005. ING will provide Ms. Monks with attendant care on a 24-hour basis.

3. ING will pay interest on past benefits due, pursuant to s. 46(2) of the SABS.
4. ING will pay for Ms. Monks' current needs as ongoing needs, as described in Exhibit 8, Tab 4-D, as found in the spreadsheets of Sandra Guest-Poulin.
5. ING will pay for a leased van modified with a platform.
6. ING will pay \$225,000 to Ms. Monks for home renovations.
7. ING will pay housekeeping benefits already incurred, if any, and for the future, if the Zurich dollars are not sufficient to cover housekeeping benefits.
8. ING will provide the services of a case manager to Ms. Monks, pursuant to section 17 of the SABS.
9. ING will pay the amounts for current ongoing benefits up to the date of trial, and the additional expenses from the start of trial until the date of judgment; with applicable interest, minus any collateral benefits, and minus Zurich settlement dollars received during that period.
10. On consent of the parties, the following paragraph concerning payment of income, replacement benefit is added:

1. Since June 6, 2002, ING has deducted 20% of the IRB on the basis of an overpayment to the plaintiff arising out of the monies received as an IRB in the Zurich settlement. ING agrees to repay to the plaintiff those monies, which have been deducted, together with interest calculated in accordance with SABS up to January 12, 2005 in the amount of \$12,887.82. ING acknowledges that it is not entitled to make a deduction in respect of its payment of an IRB in respect of any monies received by the plaintiff in respect of the Zurich settlement in the past or in the future. ING acknowledges that the plaintiff's current entitlement is \$340.96 and reserves its rights with respect to entitlement and calculation and specifically reserves its right to investigation, deduction and/or the right to stop payment in accordance with the provisions of SABS.

2. The parties agree that this paragraph shall form part of the judgment rendered in this matter and agree to provide a full and final release with respect to payment of past IRB benefits up to January 12, 2005.

ISSUE NO. 4 – AGGRAVATED DAMAGES

Is Ms. Monks entitled to aggravated damages and if so, what quantum?

[1142] Due to the length of the parties' submissions on this item (30 pages each), I have selected the facts and the law that I find applicable to the issue of aggravated damages. I was advised by counsel for the plaintiff during trial that, prior to the defendant's termination of Ms. Monks' benefits on May 31, 2000, the claim was limited to the failure on the defendant's part to make proper accommodations for Ms. Monks within her home or within a replacement home.

[1143] The remainder of the plaintiff's claim for aggravated damages relates to the conduct of the defendant, which, the plaintiff argued, has been unreasonable and caused her actual damage.

[1144] The main complaints of the plaintiff can be summarized as follows:

1. The defendant's failure to recognize the extent of the plaintiff's injuries, which should have been obvious and its failure to provide for her needs;
2. The role played by the defendant in the Zurich action and the quick termination of the plaintiff's benefits following the Zurich settlement;
3. Its actions in asking that the benefits, paid to the plaintiff, be reimbursed by her, and the failure to advise the plaintiff, until the opening of trial, that such reimbursements would not be claimed back;
4. Its failure to accept the plaintiff's arguments on its obligation to give her the full Income Replacement Benefits, without deducting 20% from her monthly benefit;
5. Its conduct in the catastrophic impairment assessment of the plaintiff and its breach of the neutrality imposed by section 4 of the guidelines.

[1145] The plaintiff argued that the above actions of the defendant have caused her damages, for which she ought to be compensated.

[1146] The defendant submitted that I have to view the file as a whole and that, if I do, I will find that it has paid benefits to the plaintiff, over the past three and half years, and that it should therefore not be subjected to a claim for aggravated damages.

[1147] In particular, the defendant relied upon:

1. The plaintiff's satisfaction with the defendant's adjuster, Ms. Beehler, who looked after the plaintiff's needs in a timely manner.
2. The plaintiff was told by the defendant of its need to determine the impact of the 1998 accident on her benefits.
3. The fact that whatever needs Ms. Monks alleged were not met, these needs were the result of her own attitude, particularly concerning attendant care and housekeeping needs.
4. The fact that the termination of the plaintiff's benefits during the month of May 2002, was done in a balanced and reasonable manner.
5. The fact that the defendant did nothing to breach neutrality in the CAT DAC assessment.

The Law

[1148] The law is taken from the plaintiff's submissions, because I find that the jurisprudence, cited by the plaintiff, represents the current thinking by Canadian courts on aggravated damages. I have reproduced the relevant texts in what follows.

[1149] The most recent case involving aggravated and punitive damages is the B.C. case of *Fidler v. Sun Life Assurance Co. of Canada*, [2004] B.C.J. No. 982 (B.C.C.A.). It contains an authoritative review of the law as it has evolved to date.

[1150] The case involved a claim between an insured and her disability insurer. The insurer had terminated her benefits in 1997 and then, on the eve of trial in 2002. reinstated them. The trial was simply in regard to the claims for aggravated and punitive damages. The trial judge awarded \$20,000 for aggravated damages and denied the claim for punitive damages.

[1151] The B.C. Court of Appeal upheld the aggravated damages award but also awarded \$100,000 for punitive damages.

[1152] In regard to aggravated damages, Finch J. held:

¶ 38 The general principle in contract law is that damages will not be awarded for mental distress that results from the breach of a contract. However, there is an exception to this general principle for "peace of mind" contracts where the very purpose of the contract is to secure the peace of mind of the insured. The parties agree that the insurance contract at issue in this case is a "peace of mind" contract. It created an expectation of benefits and freedom from financial distress in the event Ms. Fidler experienced a long-term disability.

¶ 39 Accordingly, aggravated damages are available as additional compensation if the insured establishes that a breach of that contract caused her mental distress. There must be actual evidence of aggravation and mental distress. However, no independent, extra-contractual actionable wrong need be proven for such damages to be awarded. Nor is the nature of the insurer's conduct, other than the fact of breach, material since the purpose of such damages is compensatory and not punitive: see *Warrington*, supra [paragraphs] 13-22; *McIsaac*, supra [paragraphs] 5-12.

[1153] In *Warrington v. Great-West Life Assurance Co.*, 139 D.L.R. (4th) 18 (B.C.C.A.), the court was also faced with a circumstance where a disability insurer had denied its insured benefits. The Court provides an excellent review of the law of the contractual relationship between an insured and insurer. The Court first reviewed whether the relationship is a fiduciary one and found that in spite of the underlying requirement to act in the utmost of good faith it is not a contract imposing a fiduciary duty:

To my mind, the insurer's obligation to pay benefits under the policy upon receiving such proof is virtually indistinguishable from any other contractual duty undertaken by a contracting party and it is a long stretch indeed to characterize its role in this regard as a fiduciary one, even where the insured is "vulnerable" in the sense that he is in dire need of benefits. In this connection, I respectfully agree with the comments of the Ontario Court of Appeal in a recent case that was not cited to us, *Plaza Fiberglass Manufacturing Ltd. v. Cardinal Insurance Co.* (1994), [22 C.C.L.I. \(2d\) 161](#), [115 D.L.R. \(4th\) 37](#) (Ont. C.A.). There, Robins, J.A. said for the Court:

The fact that a contract is one of utmost good faith does not however mean that it gives rise to a general fiduciary relationship. The relationship between insured and insurer is not akin to the relationship between, say, guardian and ward, principal and agent, or trustee and beneficiary. In these latter instances, the inherent character of the relationship is such that the law has traditionally imported general fiduciary obligations. The insurer-insured relationship is contractual; the parties are parties to an arms-length agreement. The principle of uberrima fides does not affect the arms-length nature of the agreement, and, in my opinion, cannot be used to find a general fiduciary relationship. The insurance contract, as noted above, imposes certain specific obligations on its parties. These obligations, however, do not import general fiduciary duties to each and every insurance relationship. Before such fiduciary obligations can be imported there must be specific circumstances in the relationship that call for their imposition. [At 166.]

[1154] The Court also provided an excellent summary of development of the law as it relates to aggravated and punitive damages:

¶ 16 In Canada, it is problematic to define the relationship between damages for mental distress and so-called "aggravated damages", and to differentiate between those and punitive damages. This difficulty seems to be the result of the intertwining of punitive and aggravated damages in various judicial and academic pronouncements, even after the two were authoritatively severed -- in England by the House of Lords in *Rookes v. Barnard*, [1964] A.C. 1129 (H.L.) and in Canada by the Supreme Court in *Vorvis*, supra. Speaking for the majority in *Vorvis*, McIntyre, J. first of all equated damages for mental distress with aggravated damages: see the reference at p. 1092 to "damages for mental distress, properly characterized as aggravated damages". This pronouncement has sometimes been overlooked in subsequent cases: see, e. g., *Francis v. Canadian Imperial Bank of Commerce* (1994), [21 O.R. \(3d\) 75](#) at 88, [120 D.L.R. \(4th\) 393](#) (Ont. C.A.), and *Wallace v. United Grain Growers Ltd.*, supra, at 180.

¶ 17 Second, the Court in *Vorvis* made it clear that aggravated damages are purely compensatory. In McIntyre, J.'s analysis, they are awarded "to compensate for aggravated damage ... and may only be awarded for that purpose. Punitive damages, on the other hand, are punitive in nature and may only be employed in circumstances where the conduct giving the cause for a complaint is of such nature that it merits punishment." (Supra, at 1099.) Thus although aggravated damages "will frequently cover conduct which could also be the subject of punitive damages" (at 1099), the focus is on the effect of the injury on the plaintiff. Punitive damages, on the other hand, may "only be awarded in respect of

conduct which is of such nature as to be deserving of punishment because of its harsh, vindictive, reprehensible and malicious nature" (supra, at 1107-8) and the focus is therefore on the defendant's conduct, motives and even "attitude".

¶ 18 Third, *Vorvis* clarified that aggravated damages may be awarded in breach of contract cases. McIntyre, J. cited with apparent approval the English "peace of mind" cases, including [\[page29\]](#) *Jarvis v. Swans Tours Ltd.*, *Cox v. Philips Industries Ltd.* and *Heywood v. Wellers*, all supra, and a British Columbia case, *Cringle v. Northern Union Insurance Co.* (1981), [124 D.L.R. \(3d\) 22](#) (B.C.S.C.), to illustrate that "in some contracts the parties may well have contemplated at the time of the contract that a breach in certain circumstances would cause the plaintiff mental distress". His Lordship also noted that the Ontario Court of Appeal in *Brown v. Waterloo (City) Regional Board of Commissioners of Police* (1983), [150 D.L.R. \(3d\) 729](#) (Ont. C.A.), had implicitly accepted that the court has the power to award damages for mental distress in a contract case. (Supra, at 1102.) *Vorvis*, however, was found not to be such a case, since the damages concerned did not flow from an actionable wrong (the defendant's failure to give the plaintiff reasonable notice of termination of his employment) but from conduct that occurred prior to his dismissal.

The Court then went on to conclude:

¶ 19 In what circumstances, then, will aggravated damages be properly awarded where the claim is one in breach of contract? In contract, unlike tort, the purpose of damages is to compensate the wronged party for the loss of benefits that were within the reasonable contemplation of the parties when the contract was made: *Hadley v. Baxendale*, supra. Whereas mental suffering is often the foreseeable consequence of tortious conduct, its avoidance is not commonly a benefit contemplated by the contract. Still, the "peace of mind" decisions are proof that this is not always the case and that recovery for mental distress can be contained within reasonable limits. I read *Vorvis* to mean not that "damages for mental distress, properly characterized as aggravated damages" will be awarded only where there is an independent wrong in addition to the breach of contract (which is the result suggested by the Court in *Wallace v. United Grain Growers*, supra), but to leave it open to courts to develop guidelines as to the limited circumstances that will justify such awards. I for one am content to adopt the suggestion made in *Hayes v. Dodd*, supra, that damages of this kind should be recoverable "when the subject matter of the contract ... is to provide peace of mind or freedom from distress."

[1155] In both *Fidler* and *Warrington*, the B.C. Court of Appeal found that there is the potential for aggravated damages to be awarded in cases involving breaches of contracts which

are intended to provide “peace of mind” and that insurance contracts are such contracts. A review of a number of cases, decided before these, discloses that typically, an award of aggravated damages requires not only that some actual damage has been suffered by the plaintiff, but also a separate actionable wrong must have been committed by the insurer.

[1156] As seen in *Fidler*, Finch J. held that “There must be actual evidence of aggravation and mental distress. However, no independent, extra-contractual actionable wrong need be proven for such damages to be awarded. Nor is the nature of the insurer’s conduct, other than the fact of breach, material since the purpose of such damages is compensatory and not punitive.” He cites *Warrington* as the authority for that statement. And in *Warrington*, the following is the analysis that caused the court to arrive at that conclusion:

¶ 22 In summary, I conclude that a disability insurance policy is one of the few contracts in which damages for mental distress are recoverable when they are proven to result from the breach of contract. The trial judge here dealt with these damages in another way. Perhaps considering McIntyre, J.’s comment, at p. 1103 of *Vorvis*, that he was not saying aggravated damages could never be awarded in a case of wrongful dismissal “particularly where the acts complained of were also independently actionable,” the trial judge apparently felt constrained to find an independent “actionable wrong”, a breach of a duty of good faith apart from the breach of contract. My difficulty with this approach is that logically, Mr. Warrington’s mental distress did not result from Great-West’s motive or bad faith (or breach of a duty of good faith) or mental state or motives in refusing Mr. Warrington’s claim. It resulted from the delay itself. The effect on him would have been the same whether the insurer had been well-motivated, reasonable, unreasonable, or even malicious in that delay. Thus in my view it was erroneous to find that an independent tort had been proven that resulted in injury in the form of mental distress to Mr. Warrington.

¶ 23 Subject to the issue of quantum, then, I would set aside the trial judge’s award of damages for mental distress arising from an independent wrong known as “breach of the duty of good faith” and substitute an award of damages for mental distress, or aggravated damages, for breach of contract.

¶ 24 Given this conclusion, it is unnecessary to determine whether there is a duty (whether contractual or delictual) of good faith, the breach of which is an independent wrong, owed by insurers to their insureds with respect to the payment of benefits. Nor need we embark on an examination of the esoteric points ably argued by Mr. Roberts, as to whether proof of “bad faith” is [\[page32\]](#) necessary to show a breach of such duty, whether proof of recklessness or

dishonesty on the part of a defendant is required, or whether any of these was proven in this instance.

[1157] In Ontario, the leading case on aggravated damages was decided in 1984 in the case of *Thompson et al. v. Zurich Insurance Co.*, 45 O.R. (2d) 744 (Ont. H.C.) The *dicta* in that case was quoted with approval in *Warrington*:

I separately consider the prayer for aggravated damages. In a learned essay by counsel for the plaintiffs (Cherniak and Morse, *Aggravated Punitive and Exemplary Damages*, Torts in the 80's, p. 151) appears the following passage:

While in some cases, some of the elements which justify an award of aggravated compensatory damage will also justify or even make larger an award of punitive or exemplary damages, each head should stand alone and be separately assessed, although the court must be careful not to duplicate the damages. The term "aggravation of damage" (or damages) should properly refer to either element of these damages. This ought to avoid confusing the aggravated damage award in any way with the exemplary or punitive damage award.

It was for a long time the rule that no damages could be received in contract for injury to the feelings. The rigidity of the rule has yielded, however, to the spirit of the times. The parties' contemplation and the scope of the contract has now become the basic criterion:

The reason for the general rule is that contracts normally concern commercial matters and that mental suffering on breach is not in the contemplation of the parties as part of the business risk of the transaction. If however the contract is not primarily a commercial one, in the sense that it affects not the plaintiff's business interests but his personal, social and family interests, the door should not be closed to awarding damages for mental suffering if the court thinks

that in the particular circumstances the parties to the contract had such damages in their contemplation.

McGregor on Damages, 14th ed. (1980), para. 70A; *Jarvis v. Swans Tours Ltd.*, [1973] Q.B. 233; *Jackson v. Horizon Holidays Ltd.*, [1975] 1 W.L.R. 1468.

No one rule as to damages can be adopted to fit every case. As the circumstances differ, so must the rule.

Few contracts could affect one's personal interests more than a contract for medical and rehabilitation benefits. In my judgment, both insured and insurer

would contemplate that a failure of the insurer to pay medical and rehabilitation accounts in a timely fashion will foreseeably occasion mental distress and emotional upset. The predominant, if not the sole object of the contract was to provide ease of mind to the insured that his medical accounts would be taken care of by timely payments during the period of rehabilitation.

The “peace of mind” contract

[1158] During trial, Ms. Beehler, the defendant’s adjuster in this case, agreed that Ms. Monks’ insurance policy is a “peace of mind” insurance policy. Ms. Beehler also identified the text that follows, as the defendant’s peace of mind policy, as promoted on the defendant’s website:

Our Priority: Your Peace of Mind

When it comes to insurance, your main concern is probably how your insurer will handle your claim.

At **ING Insurance** we are committed to delivering prompt, fair, professional and courteous claims service to our customers.

Thanks to our network of claims offices, you can expect to receive quality service, from a local claims representative.

Our main concern: your satisfaction

At ING Insurance, we take nothing for granted. That’s why we constantly monitor the quality of the service we provide. We do this by:

- Customer surveys
- Broker surveys
- An internal quality review process

To find out more, talk to a broker today.

[1159] I therefore find that the plaintiff’s policy falls into the category of “peace of mind” contracts referred to in the jurisprudence just cited. I find that all that is needed following the decisions in *Warrington and Fidler, supra*, is a simple breach of contract, which causes aggravation or mental distress and that there is no need to prove a separate actionable wrong. I

agree with counsel for the plaintiff that to award aggravated damages, I require proof of the damage sustained by the plaintiff. In awarding punitive damages, the Court does not attempt to compensate a plaintiff, but to punish and deter. The article by Earl Cherniak and Jerome Morse, referred to in the *Thompson* case, *supra*, stated the following:

While in some cases, some of the elements, which justify an award of aggravated compensatory damage, will also justify or even make larger an award of punitive or exemplary damages, each head should stand alone and be separately assessed, although the Court must be careful not to duplicate the damages.

[1160] In this case, punitive damages have not been pleaded and as a consequence, I need not worry whether the damages awarded as compensation have also been reflected in a punitive damage award.

The Plaintiff's Position

[1161] On the first matter in the plaintiff's claim for compensatory damages, the failure to appreciate the extent of injuries and meet her resulting needs, the plaintiff points to evidence Ms. Monks gave at trial that, despite her having fallen down the stairs repeatedly from June 2000 to November 2000, while descending from the second floor to the main floor of her home, even though nothing was done to correct the situation. Ms. Fiona Bradley Smith had made a report to the defendant about this serious condition, as early as June 2000. Scott Puddicombe, the construction consultant, did not arrive at the plaintiff's home to assess the situation until the end of October 2000. Ms. Monks had sustained, among many falls, a rather serious fall in her home, at the beginning of October 2000.

[1162] The estimate, given by Scott Puddicombe of \$23,688, would have provided modifications to the entries of the plaintiff's home and a stair glide to permit the plaintiff's access to her bedroom on the second floor. Fiona Smith Bradley, the therapist, did not get the renovations done and Ms. Beehler did not ensure that the work was done. Ms. Beehler was unable to find, in her notes, any reference to the plaintiff having said that she did not want the

work to be done. Counsel for the plaintiff submitted that, if the plaintiff's comments were not recorded by Ms. Beehler, the inference is that the plaintiff never made that comment.

[1163] The next complaint is about the defendant's delay in giving the plaintiff the accident benefits she had contracted for. The plaintiff relied on the internal notes and letters of the employees of the defendant, contained in Exhibit 40, filed at trial.

[1164] Ms. Beehler understood, from the beginning, the consequences the 1998 accident had for the plaintiff. Her note to file made on September 21, 1999, states:

Insured visited her family doctor, Dr. Anik Vandewaetere on January 15/99. On impact, insured's head hit the steering wheel, glasses were thrown off, seat belt caused severe bruising to breasts, right breast was leaking blood. Doctor recommended a mamogram if not completely healed in two weeks time. Insured is experiencing numbness in righthand side. Right arm weakness is much more evident. Prior to motor vehicle accident, she had decrease of 20%. Now feels she's at 40% - she has difficulty buttoning things and anything that requires strength and repetitiveness - ie, mashing and peeling potatoes, had falls asleep when using mouse on computer. She's had difficulty falling asleep - wakes up and arms feel like she's been in boxing match - arms are heavy, has muscular soreness, gets pins and needles down top of upper arm, then goes down the back of lower arm. She has standard whiplash muscular pain in back, but she is mainly concerned with deterioration of right arm. She also gets migraines in morning and when sitting at desk for long periods of time.

[1165] Less than one year later, Ms. Beehler gave her superior Gilles Rose the following evaluation of the plaintiff's condition:

We are currently 52 weeks post-surgery and prognosis for recovery is not looking good at all. Her future at present is very unstable and I think there is strong possibility that her impairment will be classified Catastrophic under section (f) of the definition: "evaluation of permanent impairment results in 55% or more impairment of the whole person". Once we are in receipt of the IME confirming claimant has reached her maximum recovery, I recommend we proceed with CAT DAC.

[1166] John Berriault wrote to Ms. Beehler on September 28, 2000 and had this to say on Ms. Monks' situation:

... She attempted a return to work November 14/99, within 8 weeks of undergoing a spinal fusion, only to quit in March 2000 after continually suffering from nausea, vomiting, headaches, fatigue and dizziness. Moreover, given her prior salary she states she has difficulty living on \$400 per week; clearly there is a significant shortfall. In short, there is no questioning either her character, motivation or the severity of her condition.

File Development:

After the September 1999 surgery and the return to work, it was felt there would be little future exposure. Lori Hawn wrote "... I'll remove from the [regional analyst] referral list given the apparent success of the surgery...". That apparently changed in March 2000, when she stopped working due to increasing symptomology.

Pre-existing Condition and File Issues:

She had several prior motor vehicle accident's, all clearly legitimate and contributing to a very serious medical condition. The file's complexities largely stem from issues of causation, catastrophic status, entitlement to prior AB, and prior AB deductibility. I will address them each in turn:

2. Causation:

I will not attempt a detailed medical chronology – Joanne has done a thorough job. I will merely state three key facts and my conclusion:

1. Even prior to our motor vehicle accident there was a diagnosis of herniation and at least one recommendation of surgery (Dr. Fehlings, April 98). It is unclear if all doctors agree that she would have continued to deteriorate regardless of our motor vehicle accident occurring.
2. The insured, who is highly credible, and several attending doctors attribute a "significant" aggravation of pre-existing problems to our motor vehicle accident. This is corroborated by the breast bruising which verifies a substantial impact.
3. Lori Hawn wrote Aug. 23/99: "... The SABS define accident as "*an incident in which the use or operation of an automobile directly causes an impairment.*"

I took a look at some FSCO arb. Decisions for "causation" and found that the arbitrators will rule in favour of benefits to a claimant if they can show the motor vehicle accident was a significant and contributing factor, or a material factor, to the claimant's ongoing disability. One case stated "it is sufficient if the contribution by the accident was more than minimal and made a material contribution".

“In our case, we have a claimant who knew surgery would be required eventually, but after this motor vehicle accident it is now immediately necessary. Causation is a difficult hurdle to overcome at the best of times...” I agree.

Conclusion:

Causation and Entitlement hinge entirely upon the medical experts’ opinion as to whether our motor vehicle accident caused an impairment leading to a substantial inability to perform her job. You must ensure all experts address the exact wording of the test(s) of benefits. For IRB, that test is specified in section 4. The definition of impairment in section one reads “...a loss or abnormality of a psychological, physiological or anatomical structure or function...”.

In my opinion there can be no doubt this insured will meet that test. You must ensure the medical experts agree with me.

[1167] On October 23, 2000, John Berriault wrote to Johanne Beehler and stated:

- I appreciate insured did not require housekeeping from week one of loss (in large part because she was stoic and did attempt to return to functioning).
- Agree wording states entitlement ceases 104 weeks after onset of disability.
- I believe experts will all state the impairment was caused by our loss, and did start as of the date of loss. All medicals in file are consistent with onset of disability being caused by the motor vehicle accident, despite insured’s stoic attempts.

Therefore, in my opinion, our position would be the impairment and disability began at date of loss and the benefit entitlement ceases as of 104 weeks thereafter, regardless of other factors.

Joanne I am not aware of any FSCO decisions to the contrary. If the insured’s lawyer points out any that are contrary to our position, we can look at the matter further.

[1168] Joanne Beehler’s next communication to Gilles Rose, another person holding a decision making position in the defendant’s employment, took place on December 13, 2000:

Determination of Catastrophic Impairment:

I supplied Sue with the appropriate application form, following her request. I expect that it will be returned duly completed very shortly. She tells me that Dr. Lesiuk’s diagnosis of her present condition is “incomplete quad”; in which case, there is no doubt that her impairment will be deemed catastrophic per part (a) of

the definition; she will not need to reach maximum recovery, as she would have if she had needed to rely on art (f) – impairment resulting in 55% or more impairment of the whole person.

Causation:

So it appears clear that Sue's impairment will be deemed catastrophic; all that will remain is the assessment of causation. We had addressed this before and we thought this accident would certainly be considered a contributing factor as doctor's reports have indicated that the impact caused a "significant" aggravation of pre-existing problems. Upon receipt of Sue's application, we will proceed with CAT DAC, and have them address the issue of causation.

[1169] On December 14, 2000, Ms. Beehler has a note made during a telephone conversation with her superior, John Berriault:

Don't agree to any home renovations or changes without discussing same with John ... short or long-term.

[1170] Then, on December 18, 2000, John Berriault wrote the following to Ms. Beehler:

Causation:

Recent Supreme Court of Canada case law held a tortfeasor that caused as little as 5% of a plaintiff's condition 100% responsible for damages. I fully expect the same principle will be applied to accident benefit carriers, particularly in view of first party obligations and utmost good faith. In view of the above fact situation and the dramatic functional impact our loss had upon the insured, I see little in the way of a causation defence. She was able to be gainfully employed up to our loss. Thereafter she was stoic and struggled right up until she required back surgery. The insured therefore will be entitled to accident benefits under our policy. Moreover, we have never disputed same.

So it appears clear that Sue's impairment will be deemed catastrophic; all that will remain is the assessment of causation. We had addressed this before and we thought this accident would certainly be considered a contributing factor as doctor's reports have indicated that the impact caused a "significant" aggravation of pre-existing problems. Upon receipt of Sue's application, we will proceed with CAT DAC, and have them address the issue of causation.

[1171] Counsel for the plaintiff relied on the above notes to argue that, by June 2000, Ms. Beehler had all the information she needed to understand the plaintiff's history to that point. Then, on May 29, 2001, the defendant received the opinion of Dr. Lesiuk, the treating neurosurgeon. The defendant knew at that point in time that, because of the nature of the plaintiff's complaints, and the MRI findings after the 1998 accident, Dr. Lesiuk would attach enough significance to the 1998 motor vehicle accident to pin liability on the defendant.

[1172] In May 2002, the defendant received the report of the CAT DAC assessment from Dr. Besemann, who stated that he could "not reliably conclude that this patient is catastrophically impaired on the basis of the December 23, 1998 accident".

[1173] Counsel for the plaintiff recalled that the report of Dr. Hansebout was received on February 4, 2003 and Dr. Stewart's report was received on June 11, 2002. He argued that the defendant should have noticed that some of the evidence relied upon by its two experts did not correspond to the weight of the evidence. Such a critical look was a must, because the notes, cited above, show that, throughout the year 2000, the defendant's decision makers thought that they could not win the point, namely, attribute all the blame of the plaintiff's conditions to the 1995 motor vehicle accident *sequelae*.

[1174] Counsel also argued that Ms. Beehler knew the legal test to be applied to the medical evidence. During trial, when Ms. Beehler was asked to review the May 29, 2001 report, made by Dr. Lesiuk, which ascribed a significant role to the 1998 motor vehicle accident, she stated that, as she was not a medical expert, she was unable to draw a conclusion. Counsel for the plaintiff recalled that Ms. Beehler's September 26, 2000 evaluation, sent to Gilles Rose on the plaintiff's medical condition and quoted in the previous pages, belies that assertion. Ms. Beehler had no difficulty reaching the conclusion, at that time, that Ms. Monks was catastrophically impaired. During cross-examination, when Ms. Beehler was reminded that Dr. Besemann had written in his report that he could not "reliably conclude" that the catastrophic impairment was caused by the 1998 motor vehicle accident, she looked surprised. She also stated that somewhere else in Dr. Besemann's report, it was written that Ms. Monks was not

catastrophically impaired by the 1998 motor vehicle accident. She was challenged to find that statement in Dr. Besemann's report, but could not find those words, because they are not there. Ms. Beehler then suggested that the words were implied at page 16, second last paragraph.

[1175] Counsel for the plaintiff pointed out that Dr. Lesiuk's report is clear and concise, compared to having to imply words at page 16 of Dr. Besemann's report. Although she could interpret Dr. Besemann's unclear report, Ms. Beehler claimed to be unable to understand Dr. Lesiuk's report, one that required no interpretation on her part.

[1176] The plaintiff argued that, based on her history of injuries, a history that was in the defendant's hands on May 31, 2002, the defendant was patently unreasonable in terminating her benefits. She also argued that the defendant, by taking the position that all Zurich settlement monies had to be exhausted before it had to pay her, was also patently unreasonable. It clearly deprived Ms. Monks of "peace of mind" when the defendant knew that the facts and the applicable law were clearly favouring Ms. Monks' claims.

[1177] She submitted that had she known that the defendant would have terminated her accident benefits, she might not have settled her claim against Zurich Insurance.

[1178] Counsel for the plaintiff claimed that, not only did Ms. Monks not enjoy "peace of mind", but suffered the contrary ... significant stress. She had to live with the additional stress of the threat of an adverse result, because the defendant so arduously pursued this case and the stress was worse for Ms. Monks, who had the experience of a nurse, rehabilitation consultant, and insurance adjuster.

[1179] The key to aggravated damages is that there must be evidence of damage. The key to punitive damages is that it is not intended to compensate but rather to punish and deter. As was indicated in the quote taken from the article by Earl Cherniak and Jerome Morse which was referred to in the *Thompson* case, "While in some cases, some of the elements which justify an award of aggravated compensatory damage will also justify or even make larger an award of

punitive or exemplary damages, each head should stand alone and be separately assessed, although the court must be careful not to duplicate the damages.”

[1180] In this case, it is the submission of the plaintiff that the conduct of ING could well justify an award of punitive damages. However, because such a claim has not been advanced, there is no need of concern on the part of the Court to keep the two heads of damage separate. As a review of the evidence demonstrates every action on the part of ING, which might justify an award of punitive damage has also caused aggravated damage to Ms. Monks, and so the award can properly reflect this without concern for whether it has also been reflected in a punitive damage award.

Facts relied upon by the plaintiff

[1181] As indicated above, the claim for aggravated damages is based upon the following actions by ING:

- the failure to provide any home modifications prior to May 31, 2002.

[1182] In this matter, the evidence was clear from the reports of Fiona Smith-Bradley that, as early as June 2000, Ms. Monks was suffering falls within her house. Certainly, by September 2000, it was evident that she was falling often on the stairs in the house. This was noted by Smith-Bradley in her September report (Ex. 1, Tab B5) which was copied to Joanne Beehler. In spite of agreement that something should be done about this, Scott Puddicombe did not arrive at the house to assess the situation until October 31, 2000, and it was clear that this step was expedited when Ms. Monks reported a serious fall in her home, in early October.

[1183] On November 22, 2000, (Exhibit I, Tab B7), Mr. Puddicombe provided an estimate for some minor work, which would have provided safety, and some mobility, within her own home. This consisted of a stair glide, minor modifications to the front door and kitchen, which would have totaled \$23, 668. None of this work was ever done.

[1184] Ms. Smith-Bradley agreed that in her role as occupational therapist, it was her responsibility to see that these things were done or at least to find some solution and she candidly admitted that she did not. However, Ms. Smith-Bradley was hired by the defendant, and it is the defendant who must bear any responsibility for her failures – she was ING’s agent. In any event, it is the submission of the plaintiff that it was incumbent on Ms. Beehler’s responsibility, in her role as the adjuster for Ms. Monks, to ensure that this work was done, and it was not.

[1185] Ms. Beehler stated that the reason for not doing so was that Ms. Monks told her that she did not want her to go ahead with renovations to her home, or to move out to another home. However, while there are notes of various conversations with Ms. Monks throughout this period, there is no one specific conversation in which this statement by Ms. Monks was actually recorded by Ms. Beehler. Ms. Beehler agreed that it would have been an important communication from Ms. Monks, and that, in the normal course, she should have recorded such a conversation, but she did not. The plaintiff submitted that it was not recorded because it was not said.

[1186] However, what is clearly recorded in Ms. Beehler’s notes is a telephone conversation with John Berriault (Ex. 40, Tab 15) on December 14, 2000 right after an e-mail exchange between them in regard to setting up the CAT DAC – “DON’T agree to any home renovations/changes without discussing it John – short or long term.”

[1187] It is the submission of the plaintiff that the only reasonable conclusion which can be drawn from all of this, is that the real reason why no modifications were done was the concern as to whether the case would be considered catastrophic or not, and the defendant did not want to be on the hook for any extensive modifications, given that concern.

[1188] Ms. Beehler and Ms. Smith-Bradley both insisted that the best solution for Ms. Monks was to relocate to a bungalow. But the most that they did was find a couple of listings in April 2001 and then there was no follow-up.

[1189] Mr. Puddicombe was brought in again in May 2001, and he did the analysis referred to earlier in this decision. As he testified, it was a very loose estimate of the difference between modifications for a walker as compared to a manual wheelchair. It was not intended to be a proposal or a quote and something much more formal would have been required. Nothing appears to have been done by Ms. Beehler or Ms. Smith-Bradley, after he provided his report.

[1190] Both ms. Beehler and Ms. Smith-Bradley stated that they were held up by Ms. Monks not making a decision. But in the meantime, none of the minor modifications, suggested in the fall of 2000, were ever done and Ms. Monks continued to live in a home which was dangerous in terms of mobility between floors.

[1191] ING was aware of the danger this posed and yet, did nothing. Ms. Monks did suffer a number of falls from the fall of 2000 to May 2002, when the benefits were terminated.

[1192] It is submitted by the plaintiff that ING's actions were unreasonable and the "peace of mind" intended to be provided by her insurer on this issue was breached and it caused damage. Therefore, aggravated damages are warranted because of:

- the failure on its part to recognize the obvious - that the December 23, 1998 accident did materially contribute to Ms. Monks' catastrophic condition and to properly provide for her on an on-going basis.

[1193] It is submitted that ING knew, almost immediately, that it would have liability to pay benefits to Ms. Monks, arising out of the 1998 accident. There are e-mail exchanges in which it is clear that Lori Hawn, John Berriault, Gilles Rose and Joanne Beehler all understood the legal test, which would be applied. They all also understood the factual background. (See Ex. 40, Tabs 1-6, 9, 10, 12, 14, 17).

[1194] By June of 2000, Ms. Beehler had all of the necessary medical information to know the entire history up to that point. By May 29, 2001, ING had the opinion of Dr. Lesiuk the treating neurosurgeon. His opinion was anticipated in earlier e-mails and it was understood, even beforehand, because of the nature of Ms. Monks' complaints, and the MRI findings after

the 1998 accident, that his opinion would attach enough significance to the 1998 accident to make the defendant liable.

[1195] Prior to the letter of termination in May 2002, the only other opinion the defendant had was the CAT DAC in which Dr. Besemann stated that he could “not reliably conclude that this patient is catastrophically impaired on the basis of the December 23, 1998 accident”.

[1196] The plaintiff relied on the sum total of the evidence reviewed earlier in her submissions and argued it is clear and obvious that the evidence in favour of Ms. Monks’s case on causation is overwhelming, and any objective observer would have seen this. It is the submission of the plaintiff that the defendant was patently unreasonable in terminating her benefits on May 31, 2002.

[1197] In addition, the plaintiff submitted that the arguments advanced by the defendant that the impairments Ms. Monks has were not ‘directly’ caused by the accident and that the defendant does not have to pay any benefits until the Zurich benefits are exhausted are also patently unreasonable. The defendant clearly has deprived Ms. Monks of “peace of mind” and such an argument should not have been raised against their insured when the law and facts on both accounts are clear.

[1198] According to counsel for the plaintiff, there simply was not at any time, and never has been, any sustainable evidence or law to support the decision on the part of the defendant to terminate benefits and their action has caused significant damage to Ms. Monks.

[1199] The plaintiff also claimed that she sustained an immediate and significant source of damage derived from the fact that, had she known that the defendant was going to terminate her benefits, she might have settled the Zurich claim. She might have taken a completely different course, had she known that the defendant proposed to do this.

[1200] The plaintiff reiterated her argument that, because she lost the opportunity to ask for more money from Zurich, she has not had “peace of mind”, since May 31, 2002; she has had

the contrary – significant stress. The stakes in this action are very significant. If the defendant were to succeed, all Ms. Monks could look forward to is the receipt of the weekly IRB. If she succeeds, she has the benefit of a further \$1.87 to \$2 million of benefits available to her when one considers the medical/rehabilitation, attendant care and housekeeping and maintenance obligations which would ensue. The outcome will, it goes without saying, determine how she lives out the rest of her days.

[1201] Counsel pointed out that living with that uncertainty, dreading an adverse result in a case assiduously pursued by the defendant ... this has caused great stress. She said, during her testimony, when asked what she would do if she lost, “I just don’t know”.

[1202] Ms. Monks pointed out that Dr. Vandewaetere testified about her serious health problems she suffered during the months of November and December 2004, and Dr. Vandewaetere attributed them to the up-coming trial. That, said the plaintiff, was clear manifestation of the stress and damage caused by the defendant’s decision and actions in this case.

[1203] Counsel for the plaintiff argued that the cruelest irony, in this case, is that Ms. Monks is at once, among other things, a nurse, a rehabilitation consultant and an insurance adjuster. As a result, as a nurse, she has had a better than average comprehension of what has happened to her body and mind. At the same time, in her role as a rehabilitation consultant she has had full knowledge of what she has needed by way of therapies, medications, assistive devices and attendant care throughout and what she will need in the future. And finally, as an insurance adjuster, she has known what she should have been receiving from her insurer to assist in fulfilling all of these needs.

The Defendant’s Position

[1204] The defendant argued that the plaintiff’s claim for aggravated damages should fail because:

1. Ms. Monks did not prove a separately actionable course of conduct.

2. The defendant showed utmost faith in addressing Ms. Monks' needs for home renovations and in cancelling her accident benefits.

3. Ms. Monks did not show that the defendant's action amounted to flagrant, calculated, outrageous and malicious conduct.

[1205] Counsel for the defendant argued that I should not follow the decision in *Fidler v. Sun Life Insurance, supra*, as the case is under appeal to a higher court. She maintained that the defendant paid benefits to Ms. Monks for three and a half years and, therefore, her client should not be penalized with an award for compensatory damages. She also claimed that even though it was Johanne Beehler's first complicated file, Ms. Beehler was knowledgeable. Counsel for the defendant further submitted that Ms. Monks had expressed satisfaction with the services of Ms. Beehler.

[1206] According to the defendant, Ms. Monks abandoned her claim as set out in her statement of claim paragraph 15(c), (d), (e), (f), and (g). The defendant submitted that it was attentive to Ms. Monks' needs by offering her housekeeping help from January 1999 to March 1999; Ms. Monks was granted a two-month extension to file her disability certificate. Ms. Monks also received assurances, on August 31, 1999, that the defendant would pay on a 'without prejudice' basis, her benefits.

[1207] The defendant further argued that it provided Ms. Monks with attendant care. The only periods of time Ms. Monks did not have attendant care was when she refused the care. Laurie Warren, the occupational therapist, recommended in September 1999 that Ms. Monks be given attendant care, as John Perrozzino was having problems with all the demands made on his time and energy through looking after Ms. Monks' needs. The defendant also reimbursed the transportation expenses and the attending nurse's expenses Ms. Monks incurred while at her cottage in the summer.

[1208] Counsel for the defendant pointed out that the defendant's file on accident benefits remained open until April 2002, some three and a half years following the 1998 accident.

[1209] The defendant claimed that attendant care was refused by Ms. Monks when she was falling down the stairs in September and October 2000. Reference is made by the defendant to Ms. Monks' refusal to allow the defendant instal a bed for Ms. Monks in the kitchen of her home. In December 2000, Ms. Monks, knowing she was facing a second surgery in January 2001, stopped all renovation plans that would have enhanced her accessibility to her home.

[1210] Following Ms. Monks release from the rehabilitation facilities, on April 11, 2001, the defendant argued that a form 1 was presented to Ms. Monks to cover transportation arrangements and a ramp to access her home among other items. Prior to Ms. Monks' release, Ms. Beehler's notes for April 10, 2001 (Exhibit 37, Tab 25) read as follows:

Will need to discuss moving for renovations – will advise on cost of ramp – should look at a temporary one for now – gave her application for expenses.

[1211] The defendant blamed Ms. Monks for the lack of decision on the issue of home renovations or of moving to a bungalow elsewhere. The defendant also claimed that Ms. Monks was aware that monies were available for renovations. Moreover, Ms. Beehler stated that Ms. Monks had the capacity to make the decisions. Counsel for the defendant argued again that Ms. Monks did not want to make a decision for surgery, until her Zurich action settled. Her client, she submitted, was entitled to have its opinions substantiated by medical evidence. She also submitted that Dr. Lesiuk's opinion did not translate into an irrefutable endorsement of her client's responsibility to Ms. Monks.

[1212] The defendant reminded me that the result of the catastrophic impairment assessment coupled with Ms. Monks' own assessment that she required surgery, prior to her 1998 motor vehicle accident, meant that her client acted with prudence and with an absence of malice. As further proof that the defendant did not cause delay, but rather that Ms. Monks did, the defendant recalled that Ms. Monks did not contest her CAT DAC assessment, because she wanted that assessment as evidence in her 1995 motor vehicle accident case against Zurich Insurance.

Decision

[1213] The plaintiff has met her onus of proving that her “peace of mind” contract has been breached.

[1214] However, I do not accept the plaintiff’s argument that the defendant failed to recognize the extent of the plaintiff’s injuries, which should have been obvious. Mr. Justice Rosenberg decided, in *Lurtz v. Duchesne et al* (February 4, 2005) Docket C40027 (Ont. C.A.), a medical malpractice case, that the trial judge had:

“erred in taking into account his view of the apparent weakness of the defence case. The defence had the opinions of a number of reputable experts that supported their case on liability and damages. The trial judge fell into error identified in *Foulis v. Robinson* (1978), 21 O.R. (2d) 769 (C.A.) at page 776, of “second guessing, after the event, how a trial should be conducted”.”

[1215] The above comment by Rosenberg J.A. was in relation to a premium granted in an award for costs. I agree with the principle that a defendant has an absolute right to mount as vigorous a defence as it sees fit, and if its conduct is found to be patently unreasonable, it can be dealt with in assessing costs. I believe Justice Rosenberg’s principle applies to this issue of compensatory damages and the defendant was entitled to rely on its medical experts.

[1216] I find that the defendant did not look after Ms. Monks’ needs properly from August to December 2000. During those months, Ms. Monks had difficulty in getting in and out of her home, and moving up and down to her second floor bedroom.

[1217] On June 15, 2000, Fiona Smith Bradley reported to Joanne Beehler, as follows:

Ms. Monks continues to experience intermittent, unpredictable nausea and vomiting on a daily basis. She reports that she frequently awakens during the night to vomit, and experiences additional nausea in the morning and throughout the day. On June 14, 2000, Ms. Monks had to cancel her appointments with both the physiotherapist and the occupational therapist due to dizzy spells that she had been experiencing since the previous week. She reported that the dizzy spells had resulted in falls both at physiotherapy and at home.

Pool Accessibility

Ms. Monks has a swimming pool in her backyard. The pool is an above ground pool that has been sunken into the ground with approximately 2' remaining above ground. Ms. Monks is presently unable to access the pool independently. The installation of a staircase (\$319 + installation) would be required for safe transfers into and out of the pool. Because the pool does not have a flush deck surrounding it, an external set of steps and a platform would also have to be built to which the pool stairs could be secured.

[1218] On September 22, 2000, Ms. Bradley Smith reports to Ms. Beehler:

Cognitive/Psychological Status

Ms. Monks continues to report feelings of depression regarding the significant changes in her physical and functional status following her motor vehicle accident. She is taking Wellbutrin and Zoloft, anti-depressant medications. She commented to this therapist that if she did not have a family to consider she might have committed suicide by now. She has stated that she is not suicidal at this time.

Ms. Monks is experiencing difficulty with her short-term memory and concentration. This may be caused by several factors including medication, fatigue and pain. She is also experiencing difficulties with word finding, which is frustrating her during her social interactions. She is reportedly only able to concentrate for 15 minutes at a time while reading.

Ms. Monks stated that she experiences sudden dizziness while ascending or descending stairs, which causes her to fall. There is a banister for her to grasp onto for support on one side of the stairs. There is no banister on the wall; however, given the width of the staircase she would be unable to grasp both banisters simultaneously if there was one installed on the wall. This therapist has suggested to Ms. Monks that she consider moving to a one-level dwelling where she would not be required to climb stairs on a regular basis. Ms. Monks and her family are not prepared at this point in time to make a decision regarding moving. Another consideration would be to install a stair glide in Ms. Monks' home between the main level and the second level. The minimum cost for a custom-built stair glide on a circular staircase is approximately \$10,000.

3. Living arrangement

Ms. Monks has been experiencing falls on the stairs in her home. She is also unable to manage the housekeeping in her present living arrangement. This therapist has discussed with Ms. Monks a few options, including modifying her

own home by installing railings or a ramp at the garage steps, a third step at her front door, and a stair glide between the main level and the second level of her home. Alternately, this therapist has recommended that Ms. Monks consider selling her home and moving to a single-level dwelling that would be easier for her to mobilize in. Homemaking would also be more manageable in a smaller dwelling. Ms. Monks is not prepared to make any of these decisions at this time due to financial uncertainty and due to uncertainty with her mother's future living requirements.

Plan

The contents of this report have been discussed with Joanne Beehler. This therapist will continue to follow up with Ms. Monks in order to facilitate her functioning of her pre-accident normal life activities. Ms. Monks is nearing her maximum medical recovery as per Dr. Lesiuk's suggestion that maximum medical recovery would be reached by December 2000. The focus of the occupational therapy intervention is therefore on long-term compensatory strategies to maximize functioning rather than short-term solutions anticipating a pre-accident level of functioning.

[1219] Ms. Smith further reported on November 3, 2000:

On October 23, 2000 Ms. Monks reported that she had fallen backwards down her stairs at home approximately 2 weeks prior to this visit. Since the fall she has had a constant headache, increased overall pain, and her balance during ambulation is decreased. X-rays taken following the fall reportedly indicated that no fracture occurred; however, Ms. Monks reported that her general practitioner feels it is possible that bruising occurred at the neck and back. Ms. Monks reported that she has fallen on her stairs at home at least 8 times post-surgery. Her falls occur following a sudden, unpredictable loss of balance.

[1220] Following Ms. Monks' first surgery, Ms. Smith reported to Ms. Beehler, the following:

Ms. Monks requires supervision for safety when ascending and descending the stairs to the second level of her home. During this assessment, she was observed while descending the stairs, holding on to the railing for support. She completed this activity safely, however this therapist has ongoing concerns regarding Ms. Monks' safety using stairs on a daily basis, due to her fluctuations in symptoms, including fatigue and nausea, and her reportedly deteriorating functional status. Ms. Monks has stated that she is attempting to limit her trips on the stairs during

the day. She is also aware of the need to sit on a step to rest if she becomes dizzy, and to finish ascending or descending the stairs on her buttocks.

[1221] The above evidence convinces me that the defendant did not look after Ms. Monks' needs. When she was asked, during cross-examination, why it took so long to remedy the reported problems, Ms. Smith Bradley answered: "It is not my job to push". The excuses given to me at trial by the defendant's representatives are not acceptable. Ms. Monks told me at trial that "financially, we could not afford bungalows; they were too expensive... (to buy new) and older models would have cost too much in renovations. It was a financial consideration".

[1222] Ms. Beehler did not feel that Ms. Monks had cognitive impairments. Her occupational therapist repeats in her reports, quoted earlier, that Ms. Monks suffers dizzy spell, nausea, vomiting and she reports feeling of depression over significant changes in her physical and functional status. Ms. Bradley Smith in one of her reports speaks of two prescribed drugs that Ms. Monks takes to deal with her depressive state. Ms. Monks had to be x-rayed on October 23, 2000, as a result of having fallen backwards on her stairs. Ms. Fiona Bradley Smith finished her report of November 23, 2000 by stating that Ms. Monks could descend the stairs on her buttocks.

[1223] The lack of action, coupled with the absence in the notes of Ms. Beehler that Ms. Monks did not want any renovations made, leads me to find neglect on the part of the defendant, neglect that caused Ms. Monks damage. I also accept that Ms. Monks' cognitive powers were diminished in the autumn of 2000. Ms. Monks was in a depressive state. The depression coupled with an inability to concentrate due to a constant state of nausea, vomiting and headaches should have caused a "hands on" approach on Ms. Beehler's part. Ms. Monks was only visited by Ms. Beehler in December 2000. At that time, Ms. Monks had just been told that she would face a very invasive surgery through her throat to enlarge her spinal canal. I cannot fault Ms. Monks for lack of timeliness in fixing access to and from the second floor of her home. Mr. Puddicombe, who had visited Ms. Monks' home on October 31, 2000, with a view to calculate the cost of renovations, never reported back to Ms. Monks prior to her January 2001 surgery.

[1224] Then, there was a recommendation made by the therapist to ease the loading of Ms. Monks' walker, in the van. Ms. Monks never heard anything more from Ms. Bradley Smith.

[1225] Commenting on the therapist's suggestion of installing a bed in the kitchen of her house, Ms. Monks testified that a narrow powder room with a toilet was all there was adjacent to the kitchen. She could not have catheterized there, four to five times each day. Thus, the recommendation was totally impractical.

[1226] Contrary to the argument of the defendant, I cannot find fault with the plaintiff's rejection of an impractical solution.

[1227] Ms. Monks never did get a stair glide despite the occupational therapist's recommendation. The defendant also failed to provide a second walker for Ms. Monks' use on the second floor of her home. When Ms. Monks' mother died, in December 2002, Ms. Monks testified that she used her mother's walker on the second level of her home. Ms. Monks stated that she felt everything that was needed in home renovations, to insure her safe mobility inside and outside her home, was placed on hold. She testified that she now understands that the defendant was awaiting the result of her litigation with Zurich Insurance, before committing itself to any expenses to meet her needs. In the matter of the pool accessibility, Ms. Monks testified that the fence was removed, a platform installed, but no steps were supplied and, therefore, she could not use the pool. Ms. Beehler testified she had assumed the stairs had been installed.

[1228] Ms. Beehler appeared to be a responsible adjuster, but some of her statements leave me perplexed. She had gathered experience working her way up the ladder in the insurance industry, and claimed that she had a good working relationship with Ms. Monks. However, I feel that she temporised and thus, she neglected Ms. Monks. Rather than writing long and elaborate reports, there was a need for action on her part and Ms. Beehler did not discharge her responsibility of ensuring Ms. Monks' needs were met. I find that Ms. Beehler seemed to have been more interested in saving her employer some money than giving her client "peace of mind", as guaranteed by her contract. She claimed to have a good working

relationship with Ms. Monks, but for the most part, she avoided visiting Ms. Monks as much as she could and, consequently, did not find out if Ms. Monks' needs were being met.

[1229] Ms. Beehler insisted, during her evidence at trial, that Ms. Monks had no cognitive difficulties and should be held accountable for her lack of decision on making home renovations or moving out. This, in the face of the reports she kept receiving from Fiona Bradley Smith; in addition, Ms. Smith told me during her testimony: "It was not my job to push". I find that the defendant caused physical and emotional damage to Ms. Monks in the delay taken to fix the access problems to her home, on the main level, to fix her access to the second level of her home, as well as access to her outside swimming pool. Because of Ms. Monks' weight, attendants could not help much, in helping her leave her second floor bedroom. So, Ms. Monks was prevented from sitting outside in the summer of 2001, because of her fear of falling downstairs.

[1230] For all that was said by the defendant concerning Ms. Monks' inappropriate settlement of the Zurich Insurance action, I am aware that the defendant had asked and received an intervenor status in that action. As a result, I cannot entertain any complaint that the defendant was short changed by the plaintiff's settlement. The defendant had purchased Zurich Insurance and the settlement money came to Ms. Monks by way of a cheque from the defendant in this action. The defendant then cancelled all accident benefits to the plaintiff, following the Zurich Insurance settlement, and the quick discontinuance of benefits was very stressful to Ms. Monks, as she had been the major earner in her household. Her husband, Mr. Perrozzino, was working so hard at his regular employment and in helping Ms. Monks that she feared he would fall sick and she and her family would quickly run out of money. I find that this was a reasonable and realistic concern for Ms. Monks.

[1231] The defendant then took the reasonable step of asking Ms. Monks to reimburse the monies it had advanced to her, stating that she had to exhaust her settlement money, before claiming against ING Insurance. Exhibit 15 is a letter to Ms. Monks, dated May 31, 2002, claiming \$33,064.37 for attendant care benefits, \$1,811 in housekeeping benefits, and \$17,730.44 for income replacement benefits that the defendant commenced to collect

immediately from Ms. Monks by deducting 20% of the income replacement benefit from each subsequent monthly payment of the benefit. I find that this action by the defendant clearly increased the stress level of the plaintiff and did her damage. She testified that, because of her structured settlement, she incurred debts in order to pay the mounting bills of her medical and rehabilitation needs. The toll the defendant's financial decision took on Mr. Monks' physical and mental health cannot be measured, but it is undoubtedly high.

[1232] The defendant did not advise the plaintiff until the opening of the trial that it would not seek repayment of those funds. The defendant only advised Ms. Monks at trial that it would cease the 20% deduction of the monthly income replacement benefit. Throughout the actions of the defendant, Ms. Monks had no "peace of mind" and was left to ponder her future on a daily basis. Her mental and physical suffering was enough without having to worry that she would run out of money, if she happened to live a normal life span. Who knows what his report might have concluded, had Dr. Besemann applied the correct test or causation; this trial might conceivably have been averted, as the evidence I heard was that the Besemann report changed the defendant's attitude on compensation to Ms. Monks.

[1233] The breaches of section 4 of the Commissioner's Guidelines are serious. The defendant first interfered to have the designated assessment centre ascribe responsibility for the 1998 motor vehicle accident to Ms. Monks' pre-existing condition. The plaintiff and her counsel should have been told that the assessment doctor was asked to determine which accident had contributed to her condition. They were not told and Dr. Besemann, who was seeing his very first case of incomplete quadriplegia, received the mandate to assess responsibility, again for the first time. His assessment of responsibility was ambiguous compared to that of Dr. Lesiuk, and he did damage to Ms. Monks' case, by not following Guideline number 4, and maintaining his neutrality. His statement at trial should have been "my report is there and I stand by it". Instead, he attempted to shore up his report after having read the opinions of other doctors.

[1234] I agree that all I am required to do is compensate the plaintiff for the delays she incurred in having her needs met in a "peace of mind" contract. I am not analysing good faith, utmost faith or bad faith in an aggravated damage claim. The notes to file, I referred to earlier in

this section, show that the defendant and its employees knew what they had to do, namely, settle Ms. Monks' claims. Is it possible that the defendant gambled on weak internal advice and decided to cut benefits to Ms. Monks? The notes are full of references to obligations to pay until the CAT DAC assessment came in, on Dr. Besemann's report, equivocal at best, and containing factual omissions. The defendant then sought the opinion of Doctors Stewart and Hansebout, and they too were unaware of many relevant facts that came out at trial, facts that the experts, by their own admissions, stated could have changed their opinions on causation. Obtaining more and more medical opinions was a course of action the defendant had a right to take. However, if such a course of action caused the plaintiff damage, then the defendant has to compensate for the delay. Ms. Beehler's December 14, 2000 note resonates with assurance: "Don't agree to any home renovations on changes without discussing same with John ... short-term or long-term". Meanwhile, Ms. Monks was falling all over the place.

[1235] The defendant played for time, while awaiting Dr. Besemann's catastrophic impairment assessment. Once the defendant obtained Dr. Besemann's report, it seems possible that it was not satisfied with Dr. Besemann's equivocal conclusion. Then, the defendant obliged Ms. Monks to visit Dr. Stewart, in Toronto, for a defence medical that was completed in a mere one hour to one and a half hours. Ms. Monks was suffering from edema in her legs and in her lungs, but she attended, nevertheless. The trip to Toronto took place in 2002. In 2003, the defendant ordered Dr. Hansebout's elaborate written report that was based on a number of erroneous facts, taken from previous reports made, in part, for the Zuric action. This exemplifies to me that the defendant had embarked, at all cost, on a gamble that it could get evidence to escape liability.

[1236] I am far from satisfied that Ms. Beehler acted with zeal in the determination of whether Ms. Monks' home should be renovated or a bungalow purchased. She had a clear obligation to meet with Ms. Monks to help her decide by giving her better examples of the financial considerations involved. The pricing of two bungalows, in my opinion, was not a sufficient investigation to help Ms. Monks decide what best suited her needs.

[1237] Did the defendant compromise Ms. Monks' overall position by not telling her that as soon as it paid the Zurich settlement for the 1995 accident, her benefits would be cut off? Whatever settlement Ms. Monks made would have a great impact on her for the rest of her life, and cutting her out of accident benefits meant that she had to live with uncertainty and the threat of losing this action. The same inability to concentrate and the same uncertainty that Ms. Monks would have enough money to pay her medical and rehabilitation expenses, also affected Ms. Monks' decision on attendant care and housekeeping needs. I disagree with counsel for the defendant that the discontinuance of Ms. Monks' benefits during May 2002 was done in a balanced and reasonable manner.

[1238] The defendant was not 'up front' with Ms. Monks in the period prior to the CAT DAC assessment until the cancellation of her benefits. Arguably, the defendant was waiting from December 2000 for some opportune, which would enable it to cut Ms. Monks' benefits, and money was the only consideration. While it is true that they paid money to Ms. Monks, it was the bare minimum; witness the assessment of the cost for a stair glider and then no news of the defendant's proposed course of action. The same scenario was repeated for Ms. Monks' access to her pool. Even in April 2001, Ms. Beehler was still looking at ramps "a temporary one for now". Her insured had had, by that time, two surgeries.

[1239] I also disagree with counsel for the defendant that Dr. Lesiuk's opinion on causation did not translate into an irrefutable endorsement of her client's responsibility to Ms. Monks. A simple reading of Ms. Beehler's notes to Mr. Berriault, *supra*, and Mr. Berriault's answers, including notes on the state of the law, belies this.

[1240] Ms. Monks' attempts to have Zurich Insurance state its position concerning her future need for surgery was perfectly normal. Ms. Monks knew about limitation periods, as did her friend, Ms. Warren, also an adjuster. Ms. Monks had been placed in the 60% category of persons suffering from a spinal canal reduction, who remained stable without other misfortune, but Ms. Monks had imagination and experience, and it was her right to investigate what might lie down the road for her, in accident benefits, if she had to resort to an operation.

[1241] I am not concerned with whether or not the defendant acted with or without malice, for the reasons set out earlier on the state of the law. I am not concerned either with the fact that Ms. Monks did not contest her CAT DAC assessment. There is no merit in the defendant's attempt to have me second guess why the plaintiff, represented by a lawyer, decided not to appeal the catastrophic impairment assessment.

[1242] I award the plaintiff \$50,000 in aggravated damages. I would have awarded \$100,000 in aggravated damages to the plaintiff, had she not been responsible for some of the delays incurred in renovating her home or in choosing to move to another home. Ms. Monks was concerned with her mother's future and wanted to wait on Elizabeth Bruyère's assessment of her mother's needs, before she would consider her own needs. At one point in time, Ms. Monks lost the help of the persons giving her attendant care and did not replace them, because she did not want strangers in her home. However, the defendant's behaviour caused the delays in home renovations, following the catastrophic impairment assessment, and following its May 2002 letter to Ms. Monks advising all benefits were cut off. These have meant that for the past three years, Ms. Monks was a prisoner in the second storey of her home. When she had to leave her bedroom to come downstairs, Ms. Monks had to come down on her buttocks, as suggested by the defendant.

[1243] Finally, if the parties cannot resolve the issue of costs, written submissions may be made in accordance with the Practice Direction. The plaintiff's submissions of no more than ten pages in length shall be filed within 21 days of the release of these Reasons, and the defendant's response of no more than ten pages shall be delivered within ten days thereafter. The parties are asked to add the totals for all fees required, when making their presentations.

The Honourable Mr. Justice Paul F. Lalonde

Released: June 15, 2005

COURT FILE NO.: 02-CV-20559
DATE: 2005/06/15

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

SUZANNE MONKS

Plaintiff

- and -

ING INSURANCE COMPANY OF CANADA

Defendant

REASONS FOR JUDGMENT

The Honourable Mr. Justice Paul F. Lalonde

Released: June 15, 2005